

Medical Surgical Nursing - Focus on Clinical Judgement (3rd by Honan)

Chapter 1 The Nurse's Role in Adult Health Nursing

MULTIPLE CHOICE

1. The nurse ensures that a client's bedspace is neat and clean with the call light within easy reach. The nurse is focusing on which nursing theorist who realized the importance of the environment for care?

- | | |
|----|----------------------|
| 1. | Florence Nightingale |
| 2. | Sister Callista Roy |
| 3. | Dorothea Orem |
| 4. | Martha Rogers |

ANS: 1

Florence Nightingale's theory focused on the environment for care. Sister Callista Roy's model is based in systems theory and an individual's ability to adapt. Dorothea Orem's model is the self-care deficit theory. Martha Rogers' model is the science of unitary human beings.

PTS:1 DIF:Apply

REF:Emergence of Contemporary Nursing in the United States

2. The nurse is instructing a client on self-administration of insulin so that the client will not need a health care provider to do this activity. The nurse is implementing which of the following aspects of Virginia Henderson's theory of nursing?

- | | |
|----|--|
| 1. | A caring relationship |
| 2. | Helping the client achieve independence from the nurse's assistance as quickly as possible |
| 3. | Integration of objective and subjective data |
| 4. | Application of critical thinking |

ANS: 2

Virginia Henderson's theory of nursing is to help people achieve health or a peaceful death so that they can be independent from the nurse's assistance as quickly as possible. A caring relationship, integration of objective and subjective data, and application of critical thinking are included in the American Nurses Association's essential features of professional nursing.

PTS:1 DIF:Analyze

REF:Emergence of Contemporary Nursing in the United States

3. A client tells the nurse that he has an HMO for his health insurance. The nurse understands that the purpose of this type of health plan is to:

-
1. ensure payment is made to Medicare for services rendered.
 2. maximize the utilization of health care resources.
 3. efficiently manage costs while providing quality care.
 4. focus on the illness when providing care.
-

ANS: 3

TESTBANKSELLER.COM

Health maintenance organizations (HMOs) were created to efficiently manage health care costs while providing quality care. An HMO is a type of managed care plan with the goal of providing wellness care and not focusing on the illness during the provision of care. HMOs do not ensure payment is made to Medicare for services rendered. HMOs also do not maximize the utilization of health care resources but rather use financial incentives to decrease care costs.

PTS: 1 DIF: Understand REF: Cost of Care

4. A client tells the nurse that he does not have a primary care physician but rather makes an appointment with a doctor who specializes in the area in which he is experiencing a problem. The nurse realizes this client is at risk for which of the following?

-
1. Fragmented care
 2. Overpayment of services
 3. Inability to sustain health
 4. Finding an appropriate general practitioner
-

ANS: 1

In the 1980s, the close and trusting relationship between an individual and the individual's physician waned and was replaced by acquaintances with specialists based upon particular health care problems. These episodes of care cause fragmentation of care. The client who utilizes specialists is not at risk for overpayment of services, the inability to sustain health, or finding an appropriate general practitioner.

PTS: 1 DIF: Analyze REF: Providers of Care

5. The nurse is attending a masters degree program in efforts to be educationally prepared to serve as a hospital leader. The nurse realizes that this educational preparation will:

-
1. hinder the nurses ability to work with physicians.
 2. be viewed as not supporting the profession of nursing by other nurses.
 3. ensure the nurse is biased towards clinicians interests.
 4. prepare the nurse to serve as strong clinical support with the ability to integrate business and caring.
-

TESTBANKSELLER.COM

ANS: 4

The nurse is attending an educational program to serve as a hospital leader. This education will prepare the nurse to serve as strong clinical support with the ability to integrate business and caring. This education will not hinder the nurses ability to work with physicians. This education will not be viewed as unsupportive to the profession of nursing. The education will ensure that the nurse is not biased towards clinicians interests.

PTS: 1 DIF: Analyze REF: Clinical Systems Leadership

6. A client tells the nurse that all hospitals care about is doing the minimum for a client regardless of the outcome. Which of the following should the nurse respond to this client?

-
1. It does feel like that sometimes.
 2. Health insurance companies have caused this problem.
 3. The doctors will get paid regardless of the clients outcomes.
-

-
4. There are quality programs in place to make sure clients receive the best quality of care regardless of the cost.

ANS: 4

In response to concerns about safety and quality of care voiced by clients and providers, total quality management and continuous quality improvement programs were initiated. These programs ensure society that cost management is not compromising safety or quality. This is what the nurse should respond to the client. The other choices do not address the clients concerns nor do they explain quality management programs.

PTS: 1 DIF: Apply REF: Quality Measure Shift

7.The nurse is providing care at a time that is the most beneficial to the client. The nurse is implementing which of the following Joint Commission Dimensions of Quality Performance?

-
- | | |
|-------|--------------|
| 1. | Safety |
| <hr/> | |
| 2. | Timeliness |
| <hr/> | |
| 3. | Efficiency |
| <hr/> | |
| 4. | Availability |

ANS: 2

The dimension of timeliness means the degree in which interventions are provided at the most beneficial time to the client. Safety means the degree in which the risk of an intervention and risk to the environment are reduced for both client and health care provider. Efficiency means the degree in which care has the desired effect with a minimum of effort, waste, or expense. Availability means the degree in which appropriate interventions are available to meet the clients needs.

PTS:1DIF:Analyze

REF:Box 1-1 Joint Commission Dimensions of Quality Performance

8.The nurse is providing care while adhering to safety as a Joint Commission Dimension of Quality Performance. Which of the following did the nurse provide to the client?

-
1. Using a needleless device when providing intravenous medications

-
2. Keeping the siderails of the bed in the down position after providing a pain medication to a client
 3. Having the client sit in a wheelchair with the wheels in the unlocked position
 4. Placing cloth towels over a spill in the room of an ambulatory client
-

ANS: 1

The dimension of safety means the degree in which the risk of an intervention and risk to the environment are reduced for both client and health care provider. The nurse who uses a needleless device when providing intravenous medications is adhering to this dimension. Keeping the siderails in the down position is not a safe practice. Having a client sit in a wheelchair with the wheels unlocked is not a safe practice. Placing cloth towels over a spill in the room of an ambulatory client is not a safe practice.

PTS:1DIF:Analyze

REF:Box 1-1 Joint Commission Dimensions of Quality Performance

9.The nurse is planning and providing care while adhering to the American Nurses Association definition of professional nursing. Which of the following does the nurse include when implementing client care?

TESTBANKSELLER.COM

-
1. Follows the NANDA nursing diagnoses process
 2. Integrates objective and subjective data
 3. Respects cultural diversity of peers
 4. Acknowledges the experience and training of physicians
-

ANS: 2

The American Nurses Association acknowledges six essential features of professional nursing. These include: 1) a caring relationship, 2) attention to the full range of human health and illness experiences, 3) integrates objective and subjective data, 4) applies scientific knowledge and critical thinking, 5) advances nursing knowledge through scholarly inquiry, and 6) promotes social justice. The nurse integrating objective and subjective data is implementing one of the six essential features of professional nursing. The other choices are not essential features of professional nursing.

PTS:1DIF:Analyze

REF: Emergence of Contemporary Nursing in the United States

10. The nurse has shifted her practice from an illness focus to a health focus. Which of the following has this nurse implemented?

1. Standardized care plans
2. Critical pathways
3. Instructing a client on relaxation techniques to aid with sleep
4. Holding around-the-clock medication when a client is asleep

ANS: 3

The use of client education as a strategy to attain and maintain the potential for health is an example of the shift of care from an illness focus to a health focus. The nurse instructing a client on relaxation techniques to aid with sleep is implementing a health focus of care. The other choices do not support the shift from an illness focus to a health focus.

PTS: 1 DIF: Analyze REF: Leadership

TESTBANKSELLER.COM

11. A client is admitted with a highly communicable disease. The nurses do not want to participate in the care of this client. Which of the following should be done to ensure the client receives the highest quality of care?

1. Adhere to strict standard precautions.
2. Plan to have the client transferred to another health care organization.
3. Ask the physician if the client can be cared for in the home.
4. Suspend the nurses without pay who refuse to care for the client.

ANS: 1

When providing care in a highly global environment, the risks of communicable diseases increases. In the event that a client is admitted with a highly communicable disease and the nurses are fearing for their own health and safety, the only safe approach is to ensure all staff adhere to strict standard precautions. The other choices do not ensure that the client will receive the highest quality of care. The nurses must learn emotional intelligence and resolve issues under fire.

PTS:1DIF:AnalyzeREF:Globalization

12.The nurse has been an employee of an organization for 2 years and is considering a job change. Which of the following does this nurses plan suggest to any future employers?

1. The nurse moves to other jobs too frequently.
2. The nurse is inflexible.
3. The nurse is searching for a more challenging environment with career opportunities.
4. The nurse is willing to sacrifice home and personal life for a job.

ANS: 3

At one point in time, job changes every 2 or 3 years was considered a red flag for employers. This does not hold true today. The nurse who changes jobs every 2 or 3 years is interested in career advancement and success. Creativity is valued and opportunities are desired. Moving to another job in 2 to 3 years does not mean the nurse is inflexible. The new generation of nurses does not want to sacrifice home and personal life for a job.

PTS: 1 DIF: Analyze REF: Care Delivery Models

13.The nurse is experiencing pain and fatigue in both arms when using the computer to document client care. Which of the following can the nurse do to reduce these symptoms?

1. Refuse to use the computer and document using a pen and paper.
2. Stand up when using the computer.
3. Adjust the keyboard and chair to reduce the pressure on the wrists and arms.
4. Ask another nurse to input the information for client care activities.

ANS: 3

Ergonomic hazards are increasing with health care providers and nurses in particular. Many of these hazards are because of the implementation of computers for documentation. The nurse should adjust the keyboard and chair to reduce the pressure on the wrists and arms when documenting with the computer. The nurse cannot refuse to use the computer. Standing up may not reduce the nurses symptoms. The nurse cannot legally ask another nurse to document client care.

PTS:1DIF:ApplyREF:Ergonomic Hazards

MULTIPLE RESPONSE

1.The nurse is planning care for a client and reviewing appropriate educational materials to use for discharge instructions. Which domains of nursing is this nurse implementing? (Select all that apply.)

1.	Nursing process
2.	Clinical practice
3.	Education
4.	Literature
5.	Administration
6.	Research

TESTBANKSELLER.COM

ANS: 2, 3

The four domains of nursing are: 1) clinical practice, 2) education, 3) administration, and 4) research. When the nurse plans care for a client, the domain being implemented is clinical practice. When reviewing appropriate educational materials to use for discharge instructions, the domain being implemented is education. The nurse is not utilizing the domains of research or administration. Nursing process and literature are not domains of nursing.

PTS:1DIF:Apply

REF:Emergence of Contemporary Nursing in the United States

2.The nurse suspects that another health care colleague may be chemically dependent when which of the following is assessed? (Select all that apply.)

1.	Prolonged work breaks
2.	Clinical care omissions
3.	Mood stability

-
- | | |
|----|-------------------------------|
| 4. | Extraordinary accomplishments |
|----|-------------------------------|
-
- | | |
|----|-------------------------|
| 5. | Heavy use of fragrances |
|----|-------------------------|
-
- | | |
|----|-----------------------------------|
| 6. | Inability to recall recent events |
|----|-----------------------------------|
-

ANS: 1, 2, 4, 5, 6

Clues of possible chemical dependency include tardiness, late sick calls, frequent or prolonged work breaks, inability to recall recent events, heavy use of fragrances, clinical care omissions or errors, patient complaints or requests for a change in care provider, mood instability, and extraordinary accomplishments. Mood stability is not a characteristic of a colleague who is experiencing chemical dependency.

PTS:1DIF:Apply

REF:Box 1-6 Clues to the Possibility of Chemical Dependence

3.The nurse is a member of a health care team that includes a physician and other health care providers. These providers work together to ensure the client is relieved of suffering, has diseases cured, and experiences enhanced health and performance. Which of the following are the levels of care represented by this team of health care providers? (Select all that apply.)

-
- | | |
|----|--------------|
| 1. | Sustain life |
|----|--------------|
-
- | | |
|----|-----------------|
| 2. | Maintain health |
|----|-----------------|
-
- | | |
|----|---------------|
| 3. | Regain health |
|----|---------------|
-
- | | |
|----|-----------------|
| 4. | Minimize injury |
|----|-----------------|
-
- | | |
|----|---------------|
| 5. | Maximize cost |
|----|---------------|
-
- | | |
|----|------------------------|
| 6. | Attain enhanced health |
|----|------------------------|
-

ANS: 1, 2, 3, 6

The medical teams mission is to relieve suffering and cure disease. This involved the three levels of care: 1) sustain life, 2) regain health, and 3) maintain health. Once the shift toward health care occurred, the fourth level of attaining enhanced health was added. Minimize injury and maximize cost is not a level of care.

PTS:1 DIF:Analyze REF:Providers of Care

4. A client tells the nurse that she is disappointed that her employer is offering a health maintenance organization for a health care benefit. Which of the following can the nurse use as responses to the client as advantages of this type of health plan? (Select all that apply.)

-
1. Since there is a nursing shortage, clients need to stay out of the hospital.

 2. This type of plan provides wellness care at a minimal cost to keep people healthy.

 3. This type of plan helps clients avoid illnesses with high costs.

 4. An HMO standardizes diagnostic and treatment decisions across the nation.

 5. This type of plan ensures coordinated services from wellness to death.

 6. This type of plan costs as much as the traditional plans, but the insurance companies get the extra money from premiums.

ANS: 2, 3, 4, 5

TESTBANKSELLER.COM

There are several missions and visions of managed care. The first is to provide wellness care at a minimal cost to keep people healthy and avoid providing illness care at a higher cost. Another mission is to standardize diagnostic and treatment decisions across the nation. Managed care emphasizes the delivery of coordinated services across the care spectrum from wellness to death and uses financial incentives to decrease length of stay and achieve cost efficiency. Managed care was not implemented to address the nursing shortage. This type of plan does not cost as much as a traditional health plan nor do the insurance companies receive the extra money from premiums.

PTS: 1 DIF: Apply REF: Cost of Care

5. The nurse has incorporated several criteria that are essential for being a member of a profession. Which of the following has this nurse done? (Select all that apply.)

-
1. Has passed the licensure examination

 2. Works regularly scheduled shifts

 3. Completed a bachelors degree in nursing

4. Limits absences from work

5. Joined the American Nurses Association

6. Reads evidenced-based information to incorporate into planning client care

ANS: 1, 3, 5, 6

There are seven essential criteria for a profession. The nurse has incorporated four of these criteria by passing the licensure examination, the nurse has implemented a code of ethics; by completing a bachelors degree in nursing, the nurse has been educated in an institution of higher education; by joining the American Nurses Association and reading evidenced-based information, the nurse is affiliated with a professional association that promotes and ensures quality practice. Working regularly scheduled shifts and limiting absences from work are not essential criteria for a profession.

Chapter 2 Chronic Illness and End-of-Life Care

MULTIPLE CHOICE

1.The nurse believes that a client is eligible as a participant for The National Hospice Reimbursement Act of 1986. This act mandated that:

-
1. clients with terminal illnesses are reimbursed.
 2. a physician must order hospice to be reimbursed.
 3. to receive reimbursement that client must be eligible for Medicare.
 4. to receive benefits, the physician must certify that the client has a limited life expectancy of 6 months or less.
-

ANS: 4

The Medicare hospice benefit is a reimbursement benefit for those with a prognosis of 6 months or less to live (certified by a physician). The act does not mandate reimbursement to clients with terminal illnesses, physicians do not have to order hospice for reimbursement, nor does a client have to be eligible for Medicare for hospice eligibility.

PTS: 1 DIF: Analyze REF: History and Overview of Hospice Care

2.After a Native American client has died, the family begins the practice of purifying the body. The nurse realizes that the deceased client may stay with the family for what period of time?

-
1. 12 hours
 2. 24 hours
-

3.	36 hours
4.	48 hours

ANS: 3

Native Americans believe that the soul departs from the body 36 hours after death. The family may want the body to remain at the place of death for this period. The other choices are incorrect lengths of time according to Native American culture.

PTS:1DIF:Analyze

REF: Table 7-1 Cultural Considerations Related to Dying

3.A client is receiving care for symptoms; however, the treatment will not alter the course of the disease. This client is receiving which type of care?

1.	Hospital-based
2.	Managed
3.	Palliative
4.	Therapeutic

ANS: 3

Palliative care, or comfort care, is directed at providing relief to a terminally ill client through symptom and pain relief. The goal is not curative. Care for symptoms that will not alter the course of the disease does not need to be provided in the hospital. Managed care is guided through the direction of a primary care physician. Therapeutic is a type of care that focuses on a specific treatment for a health problem.

PTS: 1 DIF: Analyze REF: Overview of Palliative Care

4.A client diagnosed with a terminal illness is receiving an opioid/acetaminophen combination for pain control. The nurse realizes this client is being managed at which step of the World Health Organization approach to pain management?

1.	Step 1
2.	Step 2

3. Step 3

4. Step 4

ANS: 2

The World Health Organization approach to pain management involves three steps. Step 1: Clients are treated with around-the-clock doses of nonopioids. Step 2: The use of opioid/acetaminophen combinations are used to treat mild to moderate pain. Step 3: Strong opioids are used. There is no Step 4 in the World Health Organizations approach to pain management.

PTS:1DIF:Analyze

REF: Figure 7-2 Conceptual Model of Ladder Approach to Pain Management

5.A dying client is surrounded by family and friends at home. The hospice nurse talks with the spouse of the dying client to ensure that everything the family needs during this time is being done. The nurse is providing support to:

1. the client.

2. the bereaved.

3. ensure compliance with the hospice rules and regulations.

4. determine if the spouse understands that the client is dying.

ANS: 2

Supporting the familys rituals and cultural practices gives structure to support the bereaved through this painful process when people are vulnerable and feel off balance. The nurse is not providing support to the client. The nurse is not providing support to ensure compliance with the hospice rules and regulations. The nurse is also not providing support to determine if the spouse understands that the client is dying.

PTS: 1 DIF: Analyze REF: Role of the Hospice and Palliative Care Nurse

6.A client of the Hispanic culture is nearing death and the family requests that the client be prepared for discharge. The nurse realizes that the reason the family and client want to return home is because:

-
1. individuals within this culture do not trust hospital caregivers.

 2. the family wants to have a spiritual healer care for the client.

 3. it is bad luck to die in the hospital.

 4. the spirit may get lost if the client dies in the hospital, and it will not be able to find its way home.

ANS: 4

Within the Hispanic culture, the client and family may not want to die in the hospital because the spirit may get lost and will not be able to find its way home. The reason the family and client want to return home is not because of a distrust of hospital caregivers. The family may want to have a spiritual healer conduct a ceremony for the client, but this does not need to be done in the home. Members of the Hispanic culture do not believe that it is bad luck to die in the hospital.

PTS:1DIF:Analyze

REF: Table 7-1 Cultural Considerations Related to Dying

7. During the period of time when a client diagnosed with a terminal illness became comatose, a health care proxy made decisions about the client's care. When the client regained consciousness a few days later, the nurse consulted whom regarding the client's ongoing care decisions?

-
- | | |
|----|------------|
| 1. | The client |
|----|------------|
-
- | | |
|----|-----------------------|
| 2. | The health care proxy |
|----|-----------------------|
-
- | | |
|----|---------------------|
| 3. | The client's family |
|----|---------------------|
-
- | | |
|----|------------------------|
| 4. | The client's physician |
|----|------------------------|

ANS: 1

A health care proxy is in effect whenever the client is unable to communicate and ceases to be in effect as soon as the client regains decision-making capacity. The nurse should consult with the client regarding the client's ongoing care decisions. The nurse should not consult with the health care proxy, the family, or the physician.

PTS:1DIF:Apply

REF: Ethics in Practice: Legal and Ethical Considerations Related to Dying

8.The nurse is concerned that the spouse of a terminally ill client is experiencing Anticipatory Grieving when which of the following is assessed?

1. Confidence in the ability to care for the ill client at home
2. Expressing anger about the clients pending death and crying throughout the day
3. Large social support system
4. Knowledge of equipment function

ANS: 2

Anticipatory grieving is the intellectual and emotional responses and behaviors by which individuals work through the process of modifying self-concept based on the perception of potential loss. Anger and crying about the clients pending death are signs of Anticipatory Grieving. The other assessment findings are evidence that the spouse is accepting the caregiver role.

PTS:1DIF:AnalyzeREF:Nursing Diagnoses

9.The nurse administers additional intravenous medication to a hospice client with uncontrollable pain. After receiving the additional medication, the client demonstrates apneic periods and bradycardia. Which of the following does this nurses actions suggest?

1. Euthanasia
2. Assisted suicide
3. Double effect
4. Malpractice

ANS: 3

The principle of double effect means that increasing the dose of medication to achieve pain control, even if death is hastened, is ethically justified. Euthanasia is the administration of medication to purposefully cause anothers death. Assisted suicide is the practice of providing medication to a client with the intent that the client use the medication to voluntarily commit suicide. Malpractice is conducting some aspect of care that causes a client harm.

PTS:1DIF:AnalyzeREF:Managing Pain

10. A client with a terminal illness was ingesting morphine sulfate 10 mg by mouth every 6 hours for pain. To ensure that the client receives the same degree of pain control when delivering the same medication through the intravenous route, which of the following should the nurse do?

1. Provide morphine sulfate 10 mg intravenous every 6 hours.
2. Provide morphine sulfate 20 mg intravenous every 4 hours.
3. Provide a different medication since morphine sulfate cannot be given through the intravenous route.
4. Consult a dose equivalent table to determine the dose of morphine sulfate the client will need through the intravenous route.

ANS: 4

Dose equivalent tables should be used by the nurse when analgesics or the routes of administration are changed. The nurse should not provide the same dosage of the medication through the intravenous route since this may be too much. Morphine sulfate can be administered through the intravenous route.

PTS:1DIF:ApplyREF:Managing Pain

11. A terminally ill client is experiencing nausea. Which of the following interventions can be used to help the client at this time?

1. Administer diphenhydramine (Benadryl) as prescribed.
2. Provide three regular meals.
3. Limit mouth care.
4. Restrict iced fluids.

ANS: 1

Diphenhydramine (Benadryl) acts on the vomiting center in the medulla. This is the intervention that would be the most helpful to the client at this time. The client should be provided with small, frequent meals. Mouth care should be provided when necessary. Iced fluids are helpful for dry mouth.

PTS:1DIF:Apply

REF:Managing Loss of Appetite, Constipation, Nausea, and Vomiting

12.A terminally ill client is more alert and talkative, and she is requesting specific foods to eat. The nurse should caution the family regarding the clients behavior because this could indicate:

1. total remission of the disease process.
2. final surprising rally before retreating.
3. the client is cured of the terminal illness.
4. the client was misdiagnosed.

ANS: 2

Nurses should prepare the family of a terminally ill client for an occasional final surprising rally in which the client becomes temporarily more alert and responsive before retreating. The period of alertness does not indicate total remission of the disease process, the clients being cured of the terminal illness, or the clients being misdiagnosed.

PTS: 1 DIF: Apply REF: Providing Care in the Active Phase of Dying

TESTBANKSELLER.COM

13.The nurse is concerned that a hospice client is approaching death when which of the following is assessed?

1. Respiratory rate 16 and regular
2. Blood pressure 110/60 mmHg
3. Restlessness, irritability, and anxiety
4. Periods of wakefulness are greater than periods of sleep

ANS: 3

Symptoms of hypoxia include restlessness, irritability, and anxiety. Respirations of 16 and regular is a normal respiratory rate. Blood pressure of 110/60 mmHg is within normal limits. Periods of wakefulness being greater than periods of sleep is also a normal physiological finding.

PTS: 1 DIF: Analyze REF: Table 7-2 Physiology of Dying

MULTIPLE RESPONSE

1.The nurse is discussing end-of-life wishes with a client and his family. Since the client is not sure of what type of care he wants, the nurse provides the document Five Wishes because this document provides which of the following types of information? (Select all that apply.)

- | | |
|----|---|
| 1. | What the client wants his loved ones to know |
| 2. | The level of comfort that the client wants |
| 3. | Comments and ideas for health care providers |
| 4. | The person designated by the client to make health care decisions |
| 5. | The kinds of medical treatment that the client wants or does not want |
| 6. | The way in which the client wants to be treated |

ANS: 1, 2, 4, 5, 6

The Five Wishes document helps clients express themselves if they are seriously ill and unable to communicate their wishes for themselves. It looks at all of a clients needs: medical, personal, emotional, and spiritual. Comments and ideas for health care providers is not a part of the Five Wishes document.

PTS: 1 DIF: Apply REF: Role of the Hospice and Palliative Care Nurse

2.The nurse is making a home visit to a client receiving hospice care. Which of the following symptoms will the nurse assess in the client during the visit? (Select all that apply.)

- | | |
|----|--------------------|
| 1. | Aggression |
| 2. | Anxiety |
| 3. | Confusion |
| 4. | Depression |
| 5. | Increased appetite |

6. Urinary continence

ANS: 2, 3, 4

Common symptoms of the client receiving hospice care include pain, dyspnea, nausea, vomiting, constipation, loss of appetite, urinary urgency and incontinence, insomnia, confusion, delirium, anxiety, and depression. Aggression, increased appetite, and urinary continence are not symptoms typically assessed in a client receiving hospice care.

PTS:1DIF:Apply

REF:Assessment of the Patient Receiving Hospice and Palliative Care

3.The nurse, assessing pain in a client receiving hospice care, uses the ABCDE model to guide pain management. Which of the following is a part of this pain management approach? (Select all that apply.).

-
1. Ask about the pain regularly.
-
2. Believe the patient and family in their reports of pain.
-
3. Confront the patient if you believe pain control was not achieved.
-
4. Deliver interventions only when requested.
-
5. Enable the patient to control her course of pain management to the greatest extent possible.
-
6. Utilize complementary alternative medicine approaches first.

ANS: 1, 2, 5

The ABCDE model is a guide to pain management. For A, the nurse should regularly ask about pain. For B, the nurse should believe the patient and family in their reports of pain and what relieves it. For C, the nurse should choose pain control options that are appropriate for the patient. The nurse should not confront the patient about pain control since this is not therapeutic. For D, interventions should be delivered in a timely, logical, and coordinated manner and not only when requested. For E, patients and families should be empowered. Complementary alternative medicine approaches should not be used first.

PTS: 1 DIF: Apply REF: Box 7-2 ABCDE Guide to Pain Assessment

4.The nurse is providing a terminally ill client with morphine for pain control. In addition to this medication, which of the following can be provided to enhance analgesic effect? (Select all that apply.)

1.	Antihypertensive
2.	Antidepressant
3.	Antibiotic
4.	Antiemetic
5.	Anticonvulsant
6.	Corticosteroid

ANS: 2, 5, 6

Adjuvant medications can enhance analgesic effect and include antidepressants, anticonvulsants, and corticosteroids. Antihypertensives, antibiotics, and antiemetics are not considered adjuvant medications for pain control.

PTS:1DIF:ApplyREF:Managing Pain

5.A client with a terminal illness refuses pain medication. The nurse realizes that the client may decline pain medication for which of the following reasons? (Select all that apply.)

1.	Fear that the pain means the disease is worse
2.	Insufficient health plan benefits to pay for the medication
3.	Cultural background prevents the use of pain medication
4.	Fear of becoming addicted to pain medication
5.	Fear of side effects
6.	Concern about being labeled as a bad client

ANS: 1, 4, 5, 6

Client barriers to sufficient pain management include fear that the disease is worse, fear of becoming addicted to pain medication, fear of side effects, and concern about being labeled as a bad client. Insufficient health plan benefits to pay for the medication and cultural background preventing the use of pain medication are not identified client barriers to sufficient pain management.

Chapter 3 Fluid and Electrolyte and Acid–Base Imbalances

MULTIPLE CHOICE

1.The nurse is concerned that a client can become dehydrated when which of the following is assessed?

- | | |
|----|------------------------------------|
| 1. | History of arthritis |
| 2. | Appendicitis diagnosis 3 years ago |
| 3. | Age 30 |
| 4. | Obese female |

TESTBANKSELLER.COM

ANS: 4

An adult female has 50% of body weight that is fluid. Adipose cells contain less fluid than other cells. Females have more fat cells than males. Overweight people have less body fluid than thin people. A history of arthritis and appendicitis does not predispose the client to dehydration.

PTS:1DIF:AnalyzeREF:Fluid Balance

2.A client has lost a significant amount of blood. The nurse realizes that the fluid compartment most effected with the blood loss will be:

- | | |
|----|----------------|
| 1. | intracellular. |
| 2. | interstitial. |
| 3. | intravascular. |

4. transcellular.

ANS: 3

Intravascular fluid is the fluid in the bloodstream. Intracellular fluid is the fluid inside each cell. Interstitial fluid is the fluid between cells. Transcellular fluid is the fluid outside all of the other fluid compartments, and it includes cerebrospinal fluid, joint fluid, and fluid within the gastrointestinal tract.

PTS:1DIF:AnalyzeREF:Fluid Balance

3.A client is diagnosed with chronic renal failure. Which of the following electrolytes should the nurse monitor for this client?

1. Hydrogen

2. Phosphorus

3. Calcium

4. Vitamin D

ANS: 1

The kidneys contribute to the regulation of electrolyte levels. Two electrolytes regulated by the kidneys are hydrogen and bicarbonate. The kidneys do not directly influence a clients phosphorus level. The kidneys affect calcium by activation of vitamin D; however, the kidneys do not regulate calcium levels. Vitamin D is not an electrolyte.

PTS: 1 DIF: Analyze REF: Control of Fluid and Electrolyte Balance

4.A client had a 2 kg weight loss in one day. The nurse realizes this change in weight is due to:

1. fluid loss.

2. poor appetite.

3. medications.

4. bed rest.

ANS: 1

A weight loss of more than 0.5 kg over 24 hours generally is the result of fluid loss and not of body mass. The client would not lose 2 kgs of body weight because of poor appetite, medications, or bed rest.

PTS:1DIF:Analyze

REF:Fluid Imbalances: Assessment with Clinical Manifestations

5.A client has a serum sodium level of 129 mEq/L. The nurse should prepare to administer which of the following intravenous solutions?

-
- | | |
|----|---------------------------------|
| 1. | Dextrose 5% and Lactated Ringer |
|----|---------------------------------|
-
- | | |
|----|-------------------------------------|
| 2. | Dextrose 5% and 0.45% Normal Saline |
|----|-------------------------------------|
-
- | | |
|----|--------------------|
| 3. | 0.9% Normal Saline |
|----|--------------------|
-
- | | |
|----|------------------------------------|
| 4. | Dextrose 5% and 0.9% Normal Saline |
|----|------------------------------------|

ANS: 3

TESTBANKSELLER.COM

Normal saline (0.9%) is commonly provided to restore extracellular fluid volume and increase sodium levels. Dextrose 5% and Lactated Ringers, Dextrose 5% and 0.45% Normal Saline, and Dextrose 5% and 0.9% Normal Saline are hypertonic solutions, and they will move water from the cells into the bloodstream.

PTS:1DIF:Apply

REF: Table 12-3 Isotonic IV Solutions; Table 12-4 Hypertonic IV Solutions

6.A client is diagnosed with fluid volume excess. Which of the following will the nurse most likely assess in this client?

-
- | | |
|----|------------------|
| 1. | Poor skin turgor |
|----|------------------|
-
- | | |
|----|-------------------------|
| 2. | Jugular vein distention |
|----|-------------------------|
-
- | | |
|----|-----------|
| 3. | Dry mouth |
|----|-----------|
-
- | | |
|----|----------------------|
| 4. | Increased heart rate |
|----|----------------------|

ANS: 2

Excess fluid in the intravascular space causes an elevation in blood pressure, and increased jugular venous pressure may be visible in distended neck veins. Poor skin turgor, dry mouth, and increased heart rate are findings consistent with fluid volume deficit.

PTS:1DIF:Apply

REF:Fluid Volume Excess: Assessment with Clinical Manifestations

7.A client is demonstrating dizziness and lightheadedness upon standing. The nurse is concerned the client is experiencing postural hypotension when which of the following is assessed?

1. Lying BP 120/70 mmHg, P 70; standing BP 116/78 mmHg, P 78
2. Lying BP 116/64 mmHg, P 62; standing BP 94/58 mmHg, P 78
3. Lying BP 130/80 mmHg, P 84; standing BP 118/72 mmHg, P 90
4. Lying BP 126/74 mmHg, P 74; standing BP 108/62 mmHg, P 84

ANS: 2

TESTBANKSELLER.COM

A decrease in systolic blood pressure of more than 20 mmHg when going from lying to standing, along with an increase in heart rate of 10 beats per minute or a decrease in diastolic blood pressure of more than 10 mmHg, along with a 10 beats per minute increase in heart rate, is considered postural hypotension. The other vital sign measurements do not support the criteria for postural hypotension.

PTS:1DIF:Analyze

REF:Fluid Imbalances: Assessment with Clinical Manifestations

8.The nurse assesses a client to have mild pitting edema of the lower extremities. The nurse would document this finding as being:

1. 0+.
2. 1+.
3. 2+.
4. 3+.

ANS: 2

Mild pitting edema is documented as being +1. No pitting edema would be documented as 0+. Moderate pitting edema would be documented as 2+. Moderately severe pitting edema would be documented as 3+.

PTS: 1 DIF: Apply REF: Figure 12-4 Pitting Edema Grading Scale

9. An elderly client is demonstrating new signs of confusion. Which of the following should the nurse consider when caring for this client?

- | | |
|----|--|
| 1. | Assess for signs of elevated sodium level. |
| 2. | Restrict fluids. |
| 3. | Administer prescribed diuretic medication. |
| 4. | Monitor daily weights. |

ANS: 1

Elderly clients who develop a new onset of confusion should have their serum sodium levels checked for an elevated serum sodium level. Restricting fluids, administering diuretics, and monitoring daily weights are all interventions appropriate for a client with a low-serum sodium level.

PTS: 1 DIF: Apply

REF: Excess Sodium Ion: Assessment with Clinical Manifestations

10. A client diagnosed with hypokalemia should have which of the following electrolytes also assessed?

- | | |
|----|-------------|
| 1. | Sodium |
| 2. | Calcium |
| 3. | Bicarbonate |
| 4. | Magnesium |

ANS: 4

Clients with hypokalemia often have concurrent hypomagnesemia. Hypokalemia is resistant to treatment unless the hypomagnesemia is corrected. Sodium, calcium, and bicarbonate changes are not associated with hypokalemia.

PTS:1DIF:Apply

REF: Deficient Potassium Ion: Planning and Implementation

11.A client is diagnosed with hypophosphatemia. The nurse realizes that this electrolyte imbalance is most likely associated with:

- | | |
|----|---------------------------|
| 1. | diabetes mellitus. |
| 2. | congestive heart failure. |
| 3. | arthritis. |
| 4. | chronic alcoholism. |

ANS: 4

A diet deficient in phosphorous may cause hypophosphatemia and reduced absorption of phosphorous occurs with chronic alcoholism. Hypophosphatemia is not associated with diabetes mellitus, congestive heart failure, or arthritis.

PTS: 1 DIF: Analyze REF: Deficient Phosphorus Ion: Etiology

12.A client diagnosed with chronic renal failure is experiencing muscle weakness, paresthesias, and depression. Which of the following do these assessment findings suggest to the nurse?

- | | |
|----|-----------------|
| 1. | Hyperkalemia |
| 2. | Hyponatremia |
| 3. | Hypocalcemia |
| 4. | Hypermagnesemia |

ANS: 4

Signs and symptoms of hypermagnesemia are similar to those seen with hypercalcemia and include paresthesias, muscle weakness, anorexia, nausea, diminished bowel sounds, and

constipation. Confusion, depression, lethargy, and coma can also occur. Muscle weakness, paresthesias, and depression are not seen in hyperkalemia, hyponatremia, or hypocalcemia.

PTS:1DIF:Analyze

REF:Excess Magnesium Ion: Assessment with Clinical Manifestations

13.A client begins rapid breathing and demonstrates anxiety after learning of a diagnosis of breast cancer. After a short while, the client complains of tingling lips and fingers. Which of the following should the nurse do to assist this client?

1. Provide oxygen.
2. Coach the client in the use of an incentive spirometer.
3. Help the client slow the respiratory rate or breathe into a paper bag.
4. Administer intravenous fluids.

ANS: 3

With the clients rapid respirations, too much carbon dioxide is being excreted. This leads to alkalosis. Symptoms of respiratory alkalosis include tingling of the lips and fingers. If the client is unable to control the respiratory rate, the nurse may have the client breathe into a paper bag, which forces the rebreathing of carbon dioxide. Providing oxygen, using an incentive spirometer, and intravenous fluids is not going to help correct the clients rapid respiratory rate and respiratory alkalosis.

PTS:1DIF:ApplyREF:Respiratory Alkalosis

MULTIPLE RESPONSE

1.A client is diagnosed with hyponatremia. Which of the following assessment findings would cause the nurse to become concerned? (Select all that apply.)

1. Confusion
2. Poor appetite
3. Restlessness
4. Lethargy

5.	Seizures
----	----------

6.	Coma
----	------

ANS: 1, 3, 4, 5, 6

The change in osmolality that occurs with hyponatremia causes fluid to shift into the intracellular space. Signs and symptoms associated with an expanded intracellular compartment include confusion, restlessness, lethargy, seizures, and coma. Poor appetite is not an assessment finding of hyponatremia.

PTS:1 DIF:Analyze

REF: Efficient Sodium Ion: Assessment with Clinical Manifestations

2. After reviewing a client's most recent electrocardiogram, the nurse suspects the client is experiencing hyperkalemia. Which of the following did the nurse assess on the client's rhythm strip? (Select all that apply.)

1.	Tall peaked T-waves
----	---------------------

2.	Short QRS complex
----	-------------------

3.	Dysrhythmias
----	--------------

4.	Wide QRS complex
----	------------------

5.	Bradycardia
----	-------------

6.	Tachycardia
----	-------------

ANS: 1, 3

Tall peaked T-waves and dysrhythmias are seen on the electrocardiogram of a client experiencing hyperkalemia. The other choices are not seen with hyperkalemia.

PTS: 1 DIF: Analyze REF: Excess Potassium Ion: Diagnostic Tests

3. A client has a serum potassium level of 2.9 mEq/L. Which of the following should be done to assist this client? (Select all that apply.)

-
- | | |
|----|--|
| 1. | Implement continuous cardiac monitoring. |
|----|--|
-
- | | |
|----|-----------------------------------|
| 2. | Check for an elevated ST segment. |
|----|-----------------------------------|
-
- | | |
|----|---|
| 3. | Assess muscle strength, tone, and reflexes. |
|----|---|
-
- | | |
|----|-------------------------|
| 4. | Monitor digoxin levels. |
|----|-------------------------|
-
- | | |
|----|-------------------------------|
| 5. | Monitor for seizure activity. |
|----|-------------------------------|

ANS: 1, 3, 4

Interventions for a patient with hypokalemia are continuous cardiac monitoring; assessing for flattening T-waves; monitoring for digoxin toxicity, which may cause dysrhythmias; and assessing muscle strength, tone, and reflexes. Seizure activity is a sign of a sodium imbalance.

PTS:1DIF:Apply

REF: Deficient Potassium Ion: Assessment with Clinical Manifestations

4. Which of the following assessment techniques can the nurse use to determine if a client is experiencing hypocalcemia? (Select all that apply.)

-
- | | |
|----|------------|
| 1. | Allen test |
|----|------------|
-
- | | |
|----|----------------|
| 2. | Chvosteks sign |
|----|----------------|
-
- | | |
|----|---------------------------|
| 3. | Percussion of the abdomen |
|----|---------------------------|
-
- | | |
|----|---------------------------|
| 4. | Auscultation of the lungs |
|----|---------------------------|
-
- | | |
|----|-----------------|
| 5. | Trousseaus sign |
|----|-----------------|
-
- | | |
|----|-----------------------|
| 6. | Palpation of the neck |
|----|-----------------------|

ANS: 2, 5

Trousseaus sign is assessed by inflating a blood pressure cuff for up to 4 minutes and assessing for hand spasms as a sign of hypocalcemia. Chvosteks sign is done by tapping on the facial nerve

and assessing for a spasm of the facial muscle on the same side as evidence of hypocalcemia. The Allens test, percussion of the abdomen, auscultation of the lungs, and palpation of the neck are not performed specifically for hypocalcemia.

PTS:1DIF:Apply

REF: Efficient Calcium Ion: Assessment with Clinical Manifestations

5. A client is diagnosed with a serum calcium level of 11.2 mEq/L. Which of the following interventions would be appropriate for this client? (Select all that apply.)

- | | |
|----|--|
| 1. | Administer diuretics as prescribed. |
| 2. | Restrict fluids. |
| 3. | Administer intravenous fluids as prescribed. |
| 4. | Continuous cardiac monitoring. |
| 5. | Administer intravenous sodium as prescribed. |
| 6. | Change to a low fat diet. |

TESTBANKSELLER.COM

ANS: 1, 3, 4, 5

Management of hypercalcemia is focused on removing calcium, which is accomplished by administering diuretics, administering intravenous fluids, and administering intravenous sodium. Continuous cardiac monitoring is needed for clients at risk for developing dysrhythmias. Restricting fluids and changing to a low-fat diet are not used to treat hypercalcemia.

PTS: 1 DIF: Apply REF: Excess Calcium Ion: Planning and Implementation

6. Which of the following components of the arterial blood gas will the nurse focus when on determining a client's acid-base status? (Select all that apply.)

- | | |
|----|------------------|
| 1. | pH |
| 2. | PO ₂ |
| 3. | PCO ₂ |

4.	HCO ₃
5.	O ₂ Sat
6.	Hgb

ANS: 1, 3, 4

Interpretation of the clients acid-base status involves the evaluation of three components of the arterial blood gas: pH, PCO₂ and HCO₃-. PO₂ and O₂ Sat are not used to evaluate the clients acid-base status. Hgb level is not a component of the arterial blood gas.

Chapter 4 Perioperative Nursing

MULTIPLE CHOICE

1.The nurse is identifying diagnoses appropriate for a client scheduled for a surgical procedure. Which of the following is a diagnosis commonly used for preoperative client?

1.	Anxiety TESTBANKSELLER.COM
2.	Sleep deprivation
3.	Excess fluid volume
4.	Disturbed body image

ANS: 1

The preoperative experience may be one of the most tension-producing periods of hospitalization. The nursing diagnosis anxiety is commonly used for preoperative clients. The other diagnoses are not commonly used as preoperative diagnoses.

PTS:1DIF:Apply

REF:Nursing Diagnoses Used During Preoperative Assessment

2.The preoperative nurse cares for the client until the client progresses into the intraoperative phase of care which begins when the client:

-
1. signs the surgical consent form.
 2. arrives at the surgical suite doors.
 3. is transferred to the postanesthesia care unit.
 4. accepts that surgery is pending.
-

ANS: 2

The preoperative period ends and the intraoperative period begins when the patient and family are at the door to the surgical suites. Intraoperative care does not begin when the client signs the surgical consent form, is transferred to the postanesthesia care unit, or accepts that surgery is pending.

PTS: 1 DIF: Analyze REF: Introduction

3.The nurse is ensuring that a client is able to make knowledgeable decisions regarding an upcoming surgery and can provide informed consent. What is the responsibility of the nurse regarding informed consent?

-
- TESTBANKSELLER.COM
1. Explain the surgical options
 2. Explain the operative risks
 3. Describe the operative procedure to be done
 4. Witness a patients signature
-

ANS: 4

The nurse may concurrently sign that he has witnessed a patients signature. It is the physicians responsibility to explain the other answer choices.

PTS: 1 DIF: Apply REF: Decision Strategies and Informed Consent

4.A client being prepared for surgery has a pulse oximeter placed on one digit of his hand. The nurse is applying this device to monitor the clients:

-
1. oxygen level.
-

2. heart rate.

3. blood pressure.

4. urine output.

ANS: 1

Pulse oximeters are used to precisely identify the clients peripheral tissue oxygenation. Pulse oximeters are not to measure heart rate, blood pressure, or urine output.

PTS: 1 DIF: Analyze REF: Trends

5.A client is scheduled for surgery in 2 weeks. Which of the following should the nurse instruct the client regarding healthy lifestyle behaviors?

1. Eat nutritious meals.

2. If obese, cut calories before the surgery.

3. If sedentary, exercise more before the surgery.

4. Stop all prescribed medications.

ANS: 1

The client should be encouraged to adopt healthy dietary, rest, and exercise habits before the surgery. A client who has not followed healthy lifestyle habits should not suddenly make these changes before a surgical procedure. The nurse should encourage the client to eat nutritious meals. A client who is obese should not be encouraged to cut calories before the surgery. The client who is sedentary should not be encouraged to suddenly exercise before the surgery. The client should not be instructed to stop prescribed medications unless a physician has prescribed this action.

PTS: 1 DIF: Apply REF: Time Frames and Tasks

6.The nurse wants to reduce the stress level for a preoperative client. Which of the following communication techniques can the nurse use to achieve this result?

1. Allow the client to be alone before the surgery.

-
2. Observe and ask the client if there is anything that can be done to help reduce her anxiety.
-
3. Refer to the client by her first name.
-
4. Make tasteful jokes or comments to help the client laugh.

ANS: 2

Strategies to reduce preoperative stress include observing and asking the client if there is anything that can be done to help reduce her anxiety. Leaving the client alone before the surgery will not help reduce stress. Referring to the client by her first name might be considered unprofessional and should not be done. Making jokes is also not a professional behavior and should not be done by the nurse.

PTS: 1 DIF: Apply REF: Nurse/Patient Communication

7. Which of the following can the nurse do to help an elderly client scheduled for a surgical procedure?

-
1. Work at a slower pace.
-
- TESTBANKSELLER.COM
2. Speed up the pace so the client has time to rest.
-
3. Talk to family members and leave the client alone.
-
4. Send them to the surgical holding area in advance.

ANS: 1

When caring for elderly clients, pace is important. Nurses should slow the pace. The nurse should not ignore the client. The nurse should also not send the client to the surgical holding area in advance since this could prove to be uncomfortable for the elderly client.

PTS: 1 DIF: Apply REF: Age-Related Issues

8. The nurse is concerned that a client scheduled for surgery will be at risk for hypothermia. Which of the following did the nurse assess in this client to determine the risk?

-
1. Client is a vegetarian.
-
2. Client exercises 5 days a week for 30 minutes.

3. Client has a history of congestive heart failure.

4. Clint is 48 years old.

ANS: 3

Clients at risk for hypothermia include the very young, the very old, those with a history of heart disease, those with a bleeding tendency, having complex surgery, and having surgery on a large body area that will be exposed. Being a vegetarian or exercising does not predispose a client to developing hypothermia during surgery.

PTS:1 DIF:Analyze REF:Environmental Safety

9.The nurse is concerned that a client may have an undocumented allergy to latex when which of the following is assessed?

1. Recent episode of appendicitis

2. Recovered from bronchitis 3 months ago

3. Allergy to specific foods

4. Does not like to wear wool clothing

ANS: 3

Risk factors for latex allergy include a history of allergies, for example, food allergies or contact dermatitis (eczema). Appendicitis and bronchitis do not increase the clients risk of a latex allergy. The clients not wearing wool clothing does not increase the clients risk of a latex allergy.

PTS: 1 DIF: Analyze REF: Personal Patient Safety

10.The nurse is providing a medication to reduce the preoperative clients anxiety. Which of the following medications is the nurse most likely providing to the client?

1. Hydrogen ion antagonist

2. Anticholinergic

3. Calcium channel blocker

4. Opioid

ANS: 4

Opioids provide analgesia, decrease anxiety, and provide sedation. Calcium channel blockers treat specific heart problems. Hydrogen ion antagonists are used to reduce gastric secretions. Anticholinergics are used to reduce oral and respiratory tract secretions.

11.A nurse is filling the role of circulator during a surgical procedure. Which of the following will this nurse do to provide care to the client during the case?

-
1. Maintain the sterile field.
-
2. Assist the surgeon.
-
3. Serve as the client advocate.
-
4. Assist with the administration of anesthesia.

ANS: 3

TESTBANKSELLER.COM

The circulating nurse serves as the client advocate while the client is least able to care for himself. Maintaining the sterile field is a responsibility of the scrub nurse. Assisting the surgeon is an activity of the registered nurse first assistant. Assisting with the administration of anesthesia is an activity of the nurse anesthetist.

PTS: 1 DIF: Apply REF: Circulator/Circulating Nurse

12.An elderly client is scheduled for a surgical procedure. The nurse realizes that the outcome of the clients operation will depend upon the clients:

-
1. age.
-
2. severity of illnesses.
-
3. nutritional status.
-
4. activity status.

ANS: 2

Severity of illness is a much better predictor of outcome of surgery when compared to age. Nutritional status and activity status would be characteristics that are associated with severity of illness.

PTS: 1 DIF: Analyze REF: Geriatric Considerations

13. During a surgical procedure, the client's body temperature spikes to a dangerous level. Which of the following will be done to help this client?

1. Reduce the flow of the anesthetic agent.
2. Provide 50% oxygen.
3. Stop the surgery for cardiac dysrhythmias.
4. Administer a Dantrolene infusion.

ANS: 4

Malignant hyperthermia is a medical emergency. The anesthetic agent should be stopped immediately and the client should be hyperventilated with 100% oxygen. The surgery should be stopped if it is an elective case. Dantrolene should be provided.

PTS: 1 DIF: Apply REF: Malignant Hyperthermia

14. A perioperative nurse is identified as being the scrub nurse for a surgical procedure. Which of the following are this nurse's responsibilities during the surgery? (Select all that apply.)

1. Don surgical attire and personal protective equipment.
2. Maintain the sterile field.
3. Pass instruments and supplies to the surgeon.
4. Prepare medication.
5. Remove used instruments.
6. Organize the sterile field for use.

ANS: 2, 3, 4

Responsibilities of the scrub nurse during a surgical procedure include maintaining the sterile field, passing instruments and supplies to the surgeon, and preparing medication. Donning surgical attire and organizing the sterile field are responsibilities done before the surgery begins. Removing used instruments are done after the surgery has concluded.

PTS: 1 DIF: Apply REF: Box 21-3 Duties of the Scrub Nurse

15.The perioperative nurse is identifying nursing diagnoses appropriate for a client currently having surgery. Which of the following would be appropriate for the client at this time?

1.	Risk for infection
2.	Risk for impaired skin integrity
3.	Risk for injury
4.	Risk for inadequate nutrition
5.	Risk for hypothermia
6.	Risk for fluid volume overload

ANS: 1, 2, 3, 5

Nursing diagnoses for the perioperative client include risk for infection, risk for impaired skin integrity, risk of injury, and risk of hypothermia. Risk for inadequate nutrition and risk for fluid volume overload would be more appropriate during the postoperative period of client care.

PTS: 1 DIF: Analyze REF: NANDA and the Nursing Process

16.Which of the strategies can a perioperative nurse use to make a child feel less anxious prior to a surgical procedure? (Select all that apply.)

1.	Take the client on a tour of the operating room.
2.	Allow the client to bring a toy or stuffed animal.
3.	Allow the parents to stay with the child as much as possible.

-
4. Have the chaplain say a prayer with the child.
 5. Use age-appropriate explanations.
 6. Respond to questions in a straightforward manner.
-

ANS: 1, 2, 3, 5, 6

Strategies to help a preoperative pediatric client feel less anxious prior to a surgical procedure include taking the client on a tour of the operating room, allowing the client to bring a toy or stuffed animal, allowing the parents to stay with the client as much as possible, using age-appropriate explanations, and responding to questions in a straightforward manner. Having a chaplain say a prayer with the child is good, but it may not be age appropriate.

PTS: 1 DIF: Apply REF: Pediatric Considerations

17. The circulating nurse is performing a time out prior to the beginning of a surgical procedure. Which of the following will be assessed during this time out? (Select all that apply.)

-
1. Correct client
 2. Correct procedure
 3. Correct site and side
 4. Correct surgeon
 5. Correct day
 6. Correct time
-

ANS: 1, 2, 3, 4

A correctly performed time out includes verifying the right client; the correct procedure; the correct site and side; the correct surgeon; the correct position; the correct equipment, instruments, and implants if necessary. The correct day and time are not parts of the surgical time out.

PTS: 1 DIF: Apply REF: Time Out

18.The nurse determines that a client is experiencing a risk associated with the use of anesthesia for a surgical procedure. Which of the following are considered risks of anesthesia? (Select all that apply.)

1.	Nausea and vomiting
2.	Sore throat
3.	Seizure
4.	Postoperative myocardial infarction
5.	Surgical wound infection
6.	Hypothermia

ANS: 1, 2, 3, 4, 6

Risks of anesthesia include adverse reaction to the anesthetic, nausea and vomiting, sore throat, seizure, myocardial infarction, hypothermia, malignant hyperthermia, numbness or loss of function of a body part, and disseminated intravascular coagulation. Surgical wound infection is not a risk associated with anesthesia.

19.The nurse, caring for a postoperative client, will assess vital signs:

1.	every 15 minutes for the first hour.
2.	every 20 minutes for the first hour.
3.	every 30 minutes for the first hour.
4.	not important at this point.

ANS: 1

Vital signs are performed every 15 minutes for the first hour and may be done more often if the client is less stable. Vital sign assessment is extremely important and should be done more frequently than every 20 or 30 minutes.

PTS: 1 DIF: Apply REF: Postoperative Physiological Stabilization

20. The nurse, caring for a postoperative client, will apply supplemental oxygen because:

1. the client needs it.
2. of anesthetic gasses in the lungs.
3. it helps control blood pressure.
4. it helps with wound healing.

ANS: 2

Postoperative clients require supplemental oxygen because they may still be retaining anesthetic gasses in the lungs. The client will not be able to state that they need oxygen. Oxygen will not control blood pressure nor will it help with wound healing.

PTS: 1 DIF: Analyze REF: Postoperative Physiological Stabilization

21. A client recovering from anesthesia in the care unit has an artificial airway. The nurse knows the purpose of an artificial airway is to:

1. keep the mouth open. TESTBANKSELLER.COM
2. keep the tongue from blocking the airway.
3. keep the client from vomiting.
4. allow the client to talk.

ANS: 2

The artificial airway ensures that the tongue does not block the upper airway. An artificial airway may or may not keep the mouth open. An artificial airway will not prevent the client from vomiting and is not used to facilitate client communication.

PTS: 1 DIF: Analyze REF: Postoperative Physiological Stabilization

22. The nurse, caring for a client recovering from surgery, is monitoring the urine output and will notify the surgeon if the output falls below:

1. 10 mL/hr.

2.	20 mL/hr.
----	-----------

3.	30 mL/hr.
----	-----------

4.	50 mL/hr.
----	-----------

ANS: 3

With proper renal function, the kidneys will produce a minimum of 30 mL of urine per hour. A urine output of 10 or 20 mL/hr should be reported to the physician. A urine output of 50 mL/hr does not need to be reported.

PTS: 1 DIF: Apply REF: Postoperative Physiological Stabilization

23.The nurse assesses an area of drainage on the dressing of a postanesthesia care clients surgical wound. Which of the following should the nurse do?

1.	Call the surgeon right away.
----	------------------------------

2.	Cover the dressing with a new dressing.
----	---

TESTBANKSELLER.COM

3.	Circle the area and mark it with the date and time.
----	---

4.	Pass it off to the next shift.
----	--------------------------------

ANS: 3

If any drainage is showing on the dressing, the nurse is to circle the area and mark it with the date and time. The surgeon does not need to be phoned unless excessive bleeding or hematoma formation has occurred. The dressing does not need to be covered with a new dressing. The nurse should not pass this finding off to the next shift.

PTS:1DIF:ApplyREF:Wound Stabilization

24.The nurse coaches a postoperative client to utilize a breathing device that prevents the complication of atelectasis. This device would be a(n):

1.	IPPB.
----	-------

2.	blow bottles.
----	---------------

3. incentive spirometer.

4. postural drainage.

ANS: 3

An incentive spirometer assists the patient with deep breathing exercises that can help prevent atelectasis. A client would not use an intermittent positive pressure breathing device without the presence of a nurse and/or respiratory therapist. Blow bottles are not a medical device used to prevent atelectasis. Postural drainage is a technique used to drain secretions from the lung lobes.

PTS: 1 DIF: Apply REF: Nursing Care Beyond Transfer

25. Which of the following nursing interventions would be appropriate after a wound evisceration?

1. Place the client in high-Fowlers position.

2. Give the client fluids to prevent shock.

3. Push the organs back inside and tape up the wound.

4. Apply a sterile saline-soaked dressing and cover.

ANS: 4

The nurse is to cover the wound with a sterile saline-soaked dressing and maintain it until the client is taken to surgery. High-Fowlers position will not help with wound evisceration. Providing fluids would be contraindicated since the client will be returning to surgery. The nurse should not manipulate the exposed organs.

PTS: 1 DIF: Apply REF: Anticipating Complications

26. The nurse should instruct the postoperative client that antiembolic stockings are used to:

1. keep the legs warm.

2. serve as a nonslip slipper.

3. promote venous return.

-
4. make it easier to ambulate after surgery.

ANS: 3

Surgery may result in swelling that could impede blood return. Antiembolic stockings will aid in blood return and reduce lower extremity edema postoperatively. These stockings are not used to keep the legs warm, serve as a nonslip slipper, nor make it easier to ambulate after surgery.

PTS: 1 DIF: Apply REF: Recovery Milestones Beyond the Day of Surgery

27.The nurse is planning to teach a postoperative client about discharge medication. Which of these nursing interventions would best assist the client in learning?

-
1. Withhold any pain medication so that the client can concentrate better.
 2. Schedule the teaching after physical therapy so the client will be relaxed.
 3. Place the client in a comfortable position and have the patient use the bathroom.
 4. Plan the teaching at night right before bed so that the client can sleep on the new information given.
-
- TESTBANKSELLER.COM

ANS: 3

Placing the client in a comfortable position and having him use the bathroom will allow him to concentrate on the learning to take place. The client will not be able to concentrate on the instructions if he is in pain. The client may be tired after physical therapy and would not want to engage in instruction at this time. Waiting until night to conduct instruction is also not a good time considering the client may be fatigued from activities throughout the day and needs to rest.

PTS: 1 DIF: Apply REF: Box 22-4 Discharge Teaching Tips

28.The nurse is instructing a family member on how to change a clients postoperative wound dressing at home. Which of the following should be included in these instructions?

-
1. Wear gloves to remove the old dressing.
 2. Wear sterile gloves to apply the new dressing.
 3. Clean hands prior to applying the new dressing.
-

-
4. Reposition the new dressing after application.

ANS: 3

If the client is to change the dressing at home, there is no need to wear gloves when the old dressing is removed. Clean hands are sufficient to apply the new dressing. Sterile gloves are not needed to apply the new dressing. Once the new dressing has been placed over the wound, it should be left alone and not repositioned.

PTS: 1 DIF: Apply REF: Patient and Family Teaching

29. Which of the following should the nurse do when caring for an elderly postoperative client?

-
1. Allow rest periods between activities.
 2. Address the client by the first name.
 3. Assess for confusion if the client takes a long time to complete a task.
 4. Avoid eye contact.
-

TESTBANKSELLER.COM

ANS: 1

Caring for an elderly postoperative client, the nurse should allow rest periods between activities, avoid using the client's first name, not mistake slow activity for confusion, and maintain eye contact and full attention.

PTS: 1 DIF: Apply

REF: Respecting Our Differences: Postoperative Considerations for the Older Adult

30. The nurse is instructing a postoperative client regarding signs of complications. Which of the following should be included in these instructions?

-
1. Notify the physician with a body temperature greater than 99°F.
 2. Expect the pain level to increase.
 3. Report a change in drainage or increase in bleeding.
 4. Dizziness and fainting is an expected side effect of anesthesia.
-

ANS: 3

Signs and symptoms of postoperative complications include fever, usually greater than 100 or 101F; sudden change in pain; change in drainage or bleeding; dizziness and fainting. The client should not be instructed to notify the physician with a body temperature of 99F. Pain level should not increase once discharged. Dizziness and fainting should be reported immediately.

PTS: 1 DIF: Apply REF: Patient and Family Teaching

31. When a client is brought from the surgical suite to the postanesthesia care unit, the nurse will conduct a rapid head-to-toe visual assessment. Which of the following statuses will be assessed during the initial assessment? (Select all that apply.)

1.	Surgical site
2.	Vital signs
3.	Respiratory stability
4.	Circulatory stability
5.	Range of motion of lower extremities
6.	Bowel sounds

ANS: 1, 2, 3, 4

When a client is admitted to the postanesthesia care unit, the initial head-to-toe assessment includes surgical site, vital signs, respiratory stability, and circulatory stability. Range of motion of the lower extremities and bowel sounds are not a part of the initial head-to-toe assessment.

PTS: 1 DIF: Apply REF: Postoperative Physiological Stabilization

32. The postanesthesia care unit nurse is caring for clients with different types of wound drains. Which are the most common types of drains? (Select all that apply.)

1.	Plantar drain
2.	Penrose drain
3.	Davol

-
- | | |
|----|---------|
| 4. | Hemovac |
|----|---------|
-
- | | |
|----|------------------|
| 5. | Ostomy appliance |
|----|------------------|
-
- | | |
|----|------------------------------|
| 6. | Chest tube collection device |
|----|------------------------------|
-

ANS: 2, 3, 4

The most common types of wound drains include the Penrose, Davol, and Hemovac. An ostomy appliance is not a postoperative wound drain. A chest tube collection device is not a postoperative wound drain.

Chapter 5 Cancer Care

MULTIPLE CHOICE

1.The nurse realizes that for a cell to become cancer, it needs to progress through four stages. Which of the following is *not* a stage of this process?

- TESTBANKSELLER.COM
-
- | | |
|----|------------|
| 1. | Initiation |
|----|------------|
-
- | | |
|----|------------|
| 2. | Metastasis |
|----|------------|
-
- | | |
|----|-------------|
| 3. | Progression |
|----|-------------|
-
- | | |
|----|-------------|
| 4. | Stimulation |
|----|-------------|
-

ANS: 4

The four stages of oncogenesis or carcinogenesis are: 1) initiation, 2) promotion, 3) progression, and 4) metastasis. Stimulation is not a stage of carcinogenesis.

PTS:1DIF:AnalyzeREF:Carcinogenesis

2.A clients most recent prostate-specific antigen level has decreased since starting treatment for prostate cancer. The nurse realizes this level would indicate that the client:

-
- | | |
|----|----------------------------|
| 1. | no longer has the disease. |
|----|----------------------------|
-

-
2. has an increase in the severity of the disease process.
 3. is responding to treatment.
 4. should be retested.
-

ANS: 3

A decrease in a tumor marker is important in the assessment of cancer, monitoring tumor response during treatment strategies, and diagnosis of recurrence of disease. A decrease in the prostate-specific antigen level once treatment has begun for prostate cancer would indicate that the client is responding to treatment. A drop in the level does not mean that the client no longer has the disease, that the disease is progressing, or that the client needs to be retested.

PTS:1DIF:AnalyzeREF:Laboratory Tests

3.A client's tumor was staged using the TNM system. The tumor was staged as T4,N1,Mx. The nurse realizes that this staging means:

-
1. tumor in situ, minimal node involvement, no presence of metastasis.
 2. large tumor, no node involvement, presence of metastasis.
 3. medium tumor, multiple nodes involvement, no presence of metastasis.
 4. large tumor, single node involvement, unable to assess metastasis.
-

ANS: 4

The larger the number in the TNM staging system, the increasing involvement or larger size of the tumor, node, and metastasis. T4 reflects the size of the tumor. N1 describes the regional node involvement. Mx signals the inability to assess the presence or absence of distant metastasis.

PTS:1DIF:AnalyzeREF:Staging and Grading

4.Which of the following statements made by a client after receiving instruction regarding internal radiation would indicate that teaching has been successful?

-
1. My children can come visit me after school.
 2. Individuals will need to keep at least 3 feet away when possible.
-

-
3. I will be sharing a room near the nursing station.
 4. The hospital staff will limit the amount of time in my room.
-

ANS: 4

General guidelines include assigning the patient to a private room; postradiation precaution signage; limiting the amount of time in the room; observing a distance of at least 6 feet from the source when possible; and prohibiting pregnant staff, family, visitors, and children from interacting or visiting with the patient. The other choices would indicate the need for additional instruction and are incorrect.

PTS:1DIF:AnalyzeREF:Internal Radiation

5.A client, prescribed to begin chemotherapy, asks the nurse How does chemotherapy work? Which of the following should the nurse respond to this client?

-
1. It prevents the process of cell growth and replication.
 2. It kills only cancer cells.
 3. It treats the exposed area only with high-energy rays.
 4. Agents are implanted in an area to inhibit cancer growth.
-

ANS: 1

Chemotherapy is the use of drugs that prevent, kill, or block the growth and spread of cancer cells. Some noncancerous cells can be damaged during chemotherapy. External radiation treats an exposed area with high-energy rays. Internal radiation uses implanted agents.

PTS:1DIF:ApplyREF:Chemotherapy

6.A client is prescribed interferon as part of treatment for cancer. Which of the following should the nurse instruct the client regarding this medication?

-
1. Flu-like symptoms should be reported to the physician.
 2. General fatigue while receiving this medication is common.
 3. Seek emergency care with a high fever.
-

-
4. Side effects are short term and will resolve in a few days.

ANS: 2

Side effects vary by the type of biological agent, including a flu-like illness, high fever, headache, and general fatigue. These are expected effects and do not need to be reported to the physician. Side effects of these medications are long term and can vary in intensity during the course of treatment.

PTS:1DIF:ApplyREF:Biological Therapy

7.A client recovering from bone marrow transplantation is experiencing vomiting, fatigue, and skin reactions. Which of the following should the nurse do to help this client?

-
1. Prepare to administer platelets as prescribed.
-
2. Prepare to administer red blood cells as prescribed.
-
3. Limit fluids.
-
4. Explain that the client is experiencing expected short-term side effects.

ANS: 4

Clients who undergo bone marrow transplantation may experience short-term side effects, including nausea, vomiting, fatigue, loss of appetite, mouth sores, hair loss, and skin reactions. These side effects are not treated with platelets or red blood cells. Limiting fluids can make the side effects worse.

PTS: 1 DIF: Apply REF: Blood and Bone Marrow Transplantation

8.A client receiving chemotherapy for cancer has a hemoglobin level of 9.7 g/dL. Which of the following should the nurse anticipate as treatment for this client?

-
1. Place client in reverse isolation.
-
2. Administer antibiotics as prescribed.
-
3. Administer epoetin alfa as prescribed.
-
4. Administer filgrastim as prescribed.

ANS: 3

Treatment for moderate anemia in the client receiving chemotherapy for cancer would include the administration of epoetin alfa as prescribed. This medication elevates hemoglobin levels and improves the quality of life for clients. The other choices would be appropriate for the client diagnosed with neutropenia and not anemia.

PTS: 1 DIF: Apply REF: Anemia

9.A client receiving chemotherapy has a platelet count of 85,000. Which of the following should the nurse do to assist this client?

-
1. Assess for bruising and frank bleeding.
 2. Provide a razor for shaving.
 3. Remind the client to floss before brushing the teeth each day.
 4. Provide NSAIDs as prescribed.
-

ANS: 1

TESTBANKSELLER.COM

A platelet count of less than 100,000 indicates thrombocytopenia, and the client should be assessed for bruising and frank bleeding. The client should avoid the use of a razor, avoid flossing, and NSAIDs should not be provided since they promote bleeding.

PTS:1DIF:ApplyREF:Thrombocytopenia

10.A client receiving chemotherapy tells the nurse that he is concerned that he may be developing Alzheimers disease since he is having a new onset of memory loss. Which of the following should the nurse do to help this client?

-
1. Discuss the clients memory issues with the physician.
 2. Suggest the client use a journal to aid with short-term chemo fog problems.
 3. Assess for signs of pending stroke.
 4. Notify the physician and plan for transferring the client to an intensive care area.
-

ANS: 2

Twenty to 50% of cancer clients receiving chemotherapy describe cognitive changes such as being in a fog. To aid this client, the nurse should suggest the client keep a log or journal to document activities in order to identify when the fog is more acute. Chemo fog can last up to 2 years after treatment, but it is not permanent. The clients memory issues do not need to be discussed with a physician. The client is not experiencing a stroke. The client does not need to be transferred to an intensive care area.

PTS:1DIF:ApplyREF:Cognitive Disorders

11.A client is experiencing nausea and vomiting 1 day after chemotherapy has begun for cancer treatment. The nurse realizes this clients nausea and vomiting would be considered:

- | | |
|----|---------------|
| 1. | anticipatory. |
| 2. | acute. |
| 3. | delayed. |
| 4. | chronic. |

ANS: 3

TESTBANKSELLER.COM

Delayed nausea and vomiting occurs more than 24 hours after chemotherapy. Anticipatory nausea and vomiting occur before, during, or after chemotherapy, and they appear earlier than expected. Acute nausea and vomiting occur within 24 hours after starting chemotherapy. Chronic nausea and vomiting affect people with advanced cancer and is not well understood.

PTS: 1 DIF: Analyze REF: GI System

12.The nurse is planning interventions to address the potential problem of mucositis for a client receiving chemotherapy. Which of the following assessment findings caused the nurse to identify the client as being at risk for this side effect?

- | | |
|----|--------------------------------|
| 1. | Client prescribed chemotherapy |
| 2. | Client age 50 |
| 3. | Client lives alone |
| 4. | Client is fatigued |

ANS: 1

High risks for developing mucositis include age younger than 20, hematologic or head and neck cancer, preexisting oral disease, and chemotherapy and radiation. Age greater than 50, living arrangements, and level of fatigue do not increase a client's risk of developing mucositis.

PTS: 1 DIF: Analyze REF: Mucositis

13. Even though a client has completed a course of chemotherapy and has been found to be cancer free at this time, she continues to experience fatigue. Which of the following should the nurse instruct this client?

1. Fatigue is the first warning sign of cancer and should be reported to the physician.
2. Fatigue indicates a poor diet.
3. Fatigue is caused by poor fluid intake.
4. Fatigue can persist after treatment ends, but it will eventually improve.

ANS: 4

Fatigue is the most common symptom associated with cancer and cancer treatment. Fatigue is more often a result of the treatment than the cancer itself. The client should be informed that fatigue may persist after cancer therapy is completed, but it will eventually improve.

PTS: 1 DIF: Apply REF: Fatigue

MULTIPLE RESPONSE

1. A client is diagnosed with cancer. The nurse realizes that which of the following are characteristics of this type of cell? (Select all that apply.)

1. Aneuploid
2. Cohesive
3. Migratory
4. Poorly differentiated
5. Specific morphology

-
6. Abnormal chromosomes

ANS: 1, 3, 4, 6

Characteristics of malignant cells include uncontrolled cell division; large, variably shaped nuclei; anaplasia; poor differentiation; noncohesion; migration; lack of contact inhibition; aneuploidy; and abnormal chromosomes. Specific morphology and cohesiveness are characteristics of either benign or normal cells.

PTS:1DIF:AnalyzeREF:Malignant Cells

2.A nurse is teaching at a health fair about the early warning signs of cancer. Which of the following would the nurse include as early warning signs? (Select all that apply.)

-
1. A sore that does not heal
-
2. Change in bladder or bowel habits
-
3. Family history
-
4. Unusual discharge
-
5. Obvious change in nevus
-
6. Nagging cough

ANS: 1, 2, 4, 5, 6

Early warning signs can be easily remember using the acronym CAUTION: C, change in bladder or bowel habits; A, a sore that does not heal; U, unusual bleeding or discharge; T, presence of a lump or thickening; I, indigestion; O, obvious change in a wart or mole; and N, a nagging cough or hoarseness.

PTS: 1 DIF: Apply REF: Box 15-1 Warning Signs of Cancer

3.A client is experiencing nausea and vomiting related to chemotherapy. Which of the following strategies can the nurse use to improve nutrition in this client? (Select all that apply.)

-
1. Adding peppermint to foods
-
2. Administering ondansetron

- | | |
|----|---------------------------------|
| 3. | Drinking adequate fluids |
| 4. | Drinking hot beverages |
| 5. | Eating food at room temperature |
| 6. | Sipping ice water |

ANS: 1, 2, 3, 5

Strategies to improve nutrition in the client experiencing nausea and vomiting from chemotherapy include using herbs such as peppermint, administering prescribed anti-emetics, ensuring an adequate intake of fluids, and ingesting foods at room temperature. Foods and fluids of extreme temperatures such as hot beverages and ice water should be avoided by the patient with nausea and vomiting.

PTS: 1 DIF: Apply REF: Chemotherapy: Side Effects

4.A client asks the nurse what he can do to prevent the onset of cancer. The nurse realizes that which of the following contribute to the development of cancer? (Select all that apply.)

- | | |
|----|--------------------------------|
| 1. | TESTBANKSELLER.COM
Heredity |
| 2. | Environment |
| 3. | Lifestyle |
| 4. | Stress |
| 5. | Age |
| 6. | Blood pressure |

ANS: 1, 2, 3, 5

The factors known to contribute to the development of cancer include heredity, environment, and lifestyle. Aging has a direct effect on one's risk of developing cancer. The longer one lives, the greater the risk for developing cancer. Stress and blood pressure are not factors known to contribute to the development of cancer.

PTS: 1 DIF: Analyze REF: Etiology

5.The nurse is planning to instruct a client on strategies to lessen the impact of lifestyle on the development of cancer. Which of the following should the nurse include in these instructions? (Select all that apply.)

-
- | | |
|----|------------------------|
| 1. | Follow a low-fat diet. |
|----|------------------------|
-
- | | |
|----|-------------------------------|
| 2. | Avoid prescribed medications. |
|----|-------------------------------|
-
- | | |
|----|---------------------|
| 3. | Exercise regularly. |
|----|---------------------|
-
- | | |
|----|---------------------|
| 4. | Limit sun exposure. |
|----|---------------------|
-
- | | |
|----|-------------------------------------|
| 5. | Sleep less than 7 hours each night. |
|----|-------------------------------------|
-
- | | |
|----|---|
| 6. | Do not smoke or use any tobacco products. |
|----|---|

ANS: 1, 3, 4, 6

TESTBANKSELLER.COM

Strategies to lessen the impact of lifestyle on the development of cancer include following a low-fat diet, exercising regularly, limiting sun exposure, and avoiding all use of tobacco products. Prescribed medications will not lessen the impact of lifestyle on the development of cancer. Sleeping less than 7 hours each night will not lessen the impact of lifestyle on the development of cancer.

PTS: 1 DIF: Apply REF: Lifestyle

6.A client is prescribed a selective estrogen receptor modulator as treatment for ovarian cancer. Which of the following should the nurse instruct the client regarding side effects of this medication? (Select all that apply.)

-
- | | |
|----|-------------|
| 1. | Hot flashes |
|----|-------------|
-
- | | |
|----|-------------|
| 2. | Blood clots |
|----|-------------|
-
- | | |
|----|------------------------|
| 3. | Drop in blood pressure |
|----|------------------------|
-
- | | |
|----|---------------|
| 4. | Reduce libido |
|----|---------------|

5. Increased risk of developing other cancer

6. Weight gain

ANS: 1, 2, 4, 5

Side effects of selective estrogen modulator medications include hot flashes, blood clots, loss of interest in sex, and a higher risk of other cancers. Drop in blood pressure and weight gain are not side effects associated with this classification of medication.

Chapter 6 Pain Management

MULTIPLE CHOICE

1.A client tells the nurse that she rarely experiences pain, but when she does, she seeks medical attention. The nurse realizes this client understands that pain is important because it:

1. is a protective system.

2. includes the automatic withdrawal reflex.

3. creates sensitivity to pain.

4. helps with healing.

ANS: 1

Pain is a protective system that includes protection from unsafe behaviors by use of reflexes, memory, and avoidance. Even though the automatic withdrawal reflex is a part of the pain response, it does not explain why pain is important. Pain does not create sensitivity to pain. Pain does not help with healing.

PTS: 1 DIF: Analyze REF: Definitions and Implications of Pain

2.A client complains that the bed sheets touching his skin are extremely painful. The nurse realizes this client is experiencing:

1. allodynia.

-
- | | |
|----|-------------|
| 2. | modulation. |
|----|-------------|
-
- | | |
|----|--------------|
| 3. | kinesthesia. |
|----|--------------|
-
- | | |
|----|-----------------|
| 4. | proprioception. |
|----|-----------------|

ANS: 1

Allodynia or hyperalgesia is a state where a slight or nonpainful stimulus is interpreted as very painful. Kinesthesia is the awareness of movement. Proprioception is the awareness of body position. Modulation is an influencing factor in the perception of pain.

PTS: 1 DIF: Analyze REF: Peripheral Nervous System

3.A client is complaining of severe abdomen pain. The nurse realizes this client is experiencing which type of pain?

-
- | | |
|----|-----------|
| 1. | Neuralgia |
|----|-----------|
-
- | | |
|----|------------------------------------|
| 2. | Pathological
TESTBANKSELLER.COM |
|----|------------------------------------|
-
- | | |
|----|---------|
| 3. | Somatic |
|----|---------|
-
- | | |
|----|----------|
| 4. | Visceral |
|----|----------|

ANS: 4

Visceral pain is pain arising from the body organs or gastrointestinal tract. Somatic pain is pain that originates from the bone, joints, muscles, skin, or connective pain. Neuralgia and pathological pain are both types of pain that result from injury to a nerve or malfunction of the neuronal transmission process or due to impaired regulation.

PTS:1DIF:AnalyzeREF:Types of Pain

4.A client, diagnosed with acute appendicitis, is experiencing abdominal pain. The best way for the nurse to describe this clients pain would be:

-
- | | |
|----|----------|
| 1. | chronic. |
|----|----------|
-
- | | |
|----|--------------|
| 2. | neuropathic. |
|----|--------------|

3. referred.

4. acute.

ANS: 4

Acute pain onset is sudden and of short duration. Chronic pain is a sudden or slow onset of mild to severe pain that lasts longer than 6 months. Referred pain is the result of the transfer of visceral pain sensations to a body surface at a distance from the actual origin. Neuropathic pain is paroxysmal pain that occurs along the branches of a nerve.

PTS:1DIF:ApplyREF:Types of Pain

5.A client is observed holding a pillow over the abdominal region with both knees flexed in a side-lying position. Vital signs assessment reveals an elevated blood pressure and heart rate. Which of the following should the nurse say to this client?

1. Can I get you anything?

2. Would you like something for pain?

3. You look comfortable.

4. Your blood pressure is up.

ANS: 2

Sympathetic responses to pain include elevated blood pressure and heart rate. And since the client is hugging a pillow over the abdominal region with both knees flexed in a side-lying position, the best thing for the nurse to say to this client is Would you like something for pain? The other responses are incorrect because they do not acknowledge that the client is experiencing pain.

PTS: 1 DIF: Apply REF: Assessing the Clinical Manifestations of Pain

6.A client experiencing chronic pain asks the nurse why she is not prescribed Demerol like she received when she had a total knee replacement. Which of the following should the nurse respond to this client?

1. You dont need something that strong.

-
2. That medication does not exist anymore.
 3. That medication does not last very long.
 4. It can cause you have high blood pressure.
-

ANS: 3

Meperidine is no longer a major drug for acute or chronic pain due to its short analgesic duration of 2 to 3 hours and the potential for accumulative toxic effects of its metabolite, normeperidine. The best response for the nurse to make to the client would be that medication does not last very long. The other responses are inaccurate.

PTS:1DIF:ApplyREF:Opioid Analgesics

7.A client is informed that a tricyclic antidepressant medication is going to help control his chronic pain. The nurse would expect the physician to prescribe:

-
1. Amitriptyline.
 2. Baclofen.
 3. Gabapentin.
 4. Diazepam.
-

ANS: 1

Amitriptyline is an antidepressant. Gabapentin is an anticonvulsant. Baclofen is a muscle relaxant. Diazepam is a benzodiazepine.

PTS: 1 DIF: Analyze REF: Adjuvant Medications

8.A client receiving around-the-clock medication for terminal cancer experiences additional pain when performing activities of daily living. The nurse realizes this client is experiencing:

-
1. breakthrough pain.
 2. intractable pain.
-

3. psychosomatic pain.

4. acute pain.

ANS: 1

Breakthrough pain is commonly seen in the advanced stages of cancer. It is spontaneous, unpredictable, and can be initiated by certain activities such as during activities of daily living. Intractable pain is resistant to some or all forms of therapy. Psychosomatic pain is that which has a psychological origin. The client is diagnosed with terminal cancer. Acute pain has a sudden onset and resolves within 6 months.

PTS:1 DIF:Analyze REF:Breakthrough Pain

9.A client recovering from surgery tells the nurse that she is nauseated and is experiencing an increase in pain. Which of the following does this clients symptoms suggest to the nurse?

1. The client is becoming dependent upon the pain medication.

2. The clients pain threshold is lower when experiencing nausea.

3. The client is experiencing withdrawal symptoms from pain medication.

4. The client is experiencing referred pain.

ANS: 2

Pain threshold is influenced by nausea, fatigue, and lack of sleep. The client experiencing an increase in pain during nausea is demonstrating an alteration in the pain threshold. The client is not becoming dependent upon the pain medication. The client is not experiencing withdrawal symptoms. The client is also not experiencing referred pain.

PTS: 1 DIF: Analyze REF: Pain Threshold and Pain Tolerance

10.A client with a history of malingering pain tells the nurse that he needs a prescription for pain medication. Which of the following should the nurse do first to assist this client?

1. Ask the physician for a pain medication prescription for the client.

2. Remind the client that he does not have pain but just wants the medication.

-
3. Thoroughly assess the client for pain.
 4. Suggest the client seek counseling for his pain medication-seeking behavior.
-

ANS: 3

Pain of a psychological origin is when an individual seeks treatment for pain when no actual pain exists. This is also referred to as malingering or pretending pain. The nurse should not assume that the pain does not exist but rather should conduct a thorough pain assessment to rule out an actual physiological problem. The nurse should not immediately ask the physician for pain medication. The nurse should not remind the client that he does not have pain but just wants the medication. The nurse should also not suggest the client seek counseling for pain medication-seeking behavior.

PTS: 1 DIF: Apply REF: Box 16-1 Pain Descriptions

11.The nurse is implementing the five Cs of pain management for a client. Which of the following is included in this intervention?

-
1. Caring for the client in a holistic manner
 2. Creating a calm environment
 3. Comparing the degree of pain reported with previous episodes
 4. Continuously assessing the clients pain
-

ANS: 4

The five Cs of pain management include comprehensive assessment, consistent use of assessment tools, continuous reassessment, customize the plan of care, and collaborate with other health care providers to plan pain management. The other choices are not included in the five Cs of pain management.

PTS: 1 DIF: Apply REF: Planning and Implementation

12.A client, diagnosed with arthritis, should be instructed to avoid the use of NSAIDs because of which of the following prescribed medications?

-
1. Penicillin
-

2.	Coumadin
3.	Digoxin
4.	Diazide

ANS: 2

Persons at greatest risk for adverse reactions to NSAIDs include those who are prescribed warfarin (Coumadin) since the NSAID can increase the effects of the Coumadin and promote bleeding.

PTS: 1 DIF: Apply REF: Box 16-2 Groups of NSAID Drugs

MULTIPLE RESPONSE

1. Prior to hospitalization, a client had been ingesting high doses of oxycodone. The nurse suspects the client is experiencing symptoms of withdrawal when which of the following are assessed? (Select all that apply.)

1.	Muscle twitching and spasms
2.	Restlessness
3.	Increased heart rate
4.	Drop in blood pressure
5.	Increase in blood pressure
6.	Irritability

ANS: 1, 2, 3, 5, 6

Withdrawal symptoms include myoclonus or muscle twitching and spasms, restlessness, irritability, increased heart rate, and increased blood pressure. A decrease in blood pressure is not a symptom of narcotic medication withdrawal.

PTS: 1 DIF: Analyze

REF: Potential and Actual Side Effects of Opioid Analgesics

2.The nurse would be concerned that a client is at risk for developing chronic pain when which of the following health problems are diagnosed? (Select all that apply.)

- | | |
|----|---------------------------|
| 1. | Osteoarthritis |
| 2. | Osteoporosis |
| 3. | Heart disease |
| 4. | Diabetes mellitus |
| 5. | Chronic pulmonary disease |
| 6. | Anemia |

ANS: 1, 2, 5

Common health problems associated with chronic pain include osteoarthritis, osteoporosis, and chronic pulmonary disease. Heart disease, diabetes mellitus, and anemia are not associated with chronic pain.

PTS:1DIF:AnalyzeREF:Chronic Pain

3.An 84-year-old client is experiencing severe arthritis pain. The nurse realizes that which of the following pain management approaches would be the most beneficial for this client? (Select all that apply.)

- | | |
|----|---|
| 1. | Avoid NSAIDs. |
| 2. | Utilize morphine or morphine-like medication. |
| 3. | Provide medication through the oral route. |
| 4. | Utilize diazepam. |
| 5. | Suggest Darvocet. |
| 6. | Provide medication through the intramuscular route. |

ANS: 1, 2, 3

When providing pain medication to a geriatric client, pain management approaches include the utilization of morphine or morphine-like medication to control pain and provide medication using the oral route. NSAIDs should also be avoided because of the risk of gastrointestinal bleeding. Diazepam should be avoided because of a long half-life. Darvocet should be avoided because of toxic effects with renal insufficiency. Medication should not be provided using the intramuscular route because of muscle wasting and loss of fatty tissue in the elderly client.

PTS: 1 DIF: Apply REF: Geriatric Considerations

4.A client with severe pain from spinal stenosis is prescribed Methadone. The nurse realizes that the advantages of this medication are what? (Select all that apply.)

-
1. Decrease in the need for antidepressant adjuvant medication
-
2. Less frequent dosing schedule
-
3. Long half-life
-
4. Inexpensive
-
5. Can be used for intermittent pain
-
6. Does not cause respiratory depression

ANS: 1, 2, 4

The advantages of methadone include that it decreases the need for antidepressant adjuvant medication because it increases the release of serotonin and norepinephrine, dosing is every 12 hours, and it is inexpensive. Disadvantages of this medication include: it has a long half-life; it cannot be used for intermittent pain management; and it does cause respiratory depression.

PTS:1DIF:AnalyzeREF:Intractable Pain

5.The nurse is using the PAINAID Scale to assess a clients level of pain. Which of the following are assessed with this pain scale? (Select all that apply.)

-
1. Breathing rate
-
2. Assign a number to the degree of pain

-
- | | |
|----|------------------------|
| 3. | Negative vocalizations |
|----|------------------------|
-
- | | |
|----|--|
| 4. | Assign a facial expression to the degree of pain |
|----|--|
-
- | | |
|----|-------------------|
| 5. | Facial expression |
|----|-------------------|
-
- | | |
|----|---------------|
| 6. | Body language |
|----|---------------|
-

ANS: 1, 3, 5, 6

The PAINAID scale assesses breathing, negative vocalizations, facial expression, body language, and comfort. The Numerical Rating Scale assigns a number to the degree of pain. The Wong-Baker FACES Scale assigns a facial expression to the degree of pain.

PTS: 1 DIF: Apply REF: Skills 360: Pain Assessment Tools

6.A client diagnosed with severe arthritis tells the nurse that she always has some degree of pain. Which of the following could explain this clients poor pain management? (Select all that apply.)

-
- | | |
|----|---------------------------------------|
| 1. | Client does not appear to be in pain. |
|----|---------------------------------------|
-
- | | |
|----|------------------------------|
| 2. | Client does not report pain. |
|----|------------------------------|
-
- | | |
|----|---------------------------------------|
| 3. | Client cannot afford pain medication. |
|----|---------------------------------------|
-
- | | |
|----|--|
| 4. | Client is fearful of becoming addicted to pain medication. |
|----|--|
-
- | | |
|----|---|
| 5. | Client believes pain medication means the condition is worse. |
|----|---|
-
- | | |
|----|-----------------------------------|
| 6. | Client has a high pain tolerance. |
|----|-----------------------------------|
-

ANS: 1, 2, 4, 5

Barriers to pain assessment and management include that the client is not demonstrating overt signs of pain, and therefore she does not need pain medication; the client does not report pain, so therefore she does not need pain medication; the client is fearful of becoming addicted to pain medication; and the client believes pain medication means the condition is worse. The fact that the client is unable to afford pain medication and is having a high pain tolerance are not identified barriers to pain assessment and management.

PTS: 1 DIF: Analyze REF: Barrier to Pain Assessment and Pain Management

7.The nurse determines that a client is experiencing chronic pain when which of the following is assessed? (Select all that apply.)

1.	Suffering
2.	Fatigue
3.	Sleeplessness
4.	Apathy
5.	Sadness
6.	Anger

ANS: 1, 3, 5

The descriptor triad for chronic pain is suffering, sleeplessness, and sadness. Fatigue, apathy, and anger do not describe chronic pain.

Chapter 7 Nursing Assessment: Respiratory Function

MULTIPLE CHOICE

1.A client is experiencing the ventilation-perfusion mismatch termed *shunting*. The nurse realizes that the client most likely is *not* experiencing which of the following disorders?

1.	Hemothorax
2.	Intrapulmonary fistulas
3.	Pneumothorax
4.	Pulmonary embolus

ANS: 4

Shunting is the portion of the cardiac output that does not exchange with alveolar air. Examples of shunting include hemothorax, pneumothorax, and intrapulmonary fistulas. Pulmonary embolus is the other type of ventilation-perfusion mismatch called dead space.

PTS: 1 DIF: Analyze REF: Ventilation-Perfusion Dysfunction

2.A client has a slight shift to the left on the oxygen-hemoglobin dissociation curve. Which of the following assessment findings will support this curve configuration?

- | | |
|----|--|
| 1. | Arterial pH less than 7.35 |
| 2. | Increased levels of 2,3-diphosphoglycerate |
| 3. | Hyperthermia |
| 4. | Hypothermia |

ANS: 4

Factors that cause increased affinity of oxygen for hemoglobin will shift the oxyhemoglobin dissociation curve to the left. These factors include alkalemia and hypothermia. Arterial pH less than 7.35, increased levels of 2,3-diphosphoglycerate, and hyperthermia indicate a shift to the right, not the left.

PTS: 1 DIF: Analyze REF: Oxygen-Hemoglobin Dissociation Curve

3.A client, experiencing an acid-base imbalance, demonstrates signs of full compensation within 3 days. The nurse realizes that the full compensation was accomplished by which of the following systems?

- | | |
|----|----------------------|
| 1. | Extracellular buffer |
| 2. | Intracellular buffer |
| 3. | Pulmonary |
| 4. | Renal |

ANS: 4

The extracellular and intracellular buffer systems act immediately, the pulmonary system acts within 2 to 3 hours, and the renal system responds within 2 to 3 days.

PTS: 1 DIF: Analyze REF: Compensatory Mechanisms

4.A client with a nasogastric tube connected to low continuous suction has the following arterial blood gas (ABG) results: pH 7.49, PaO₂ 91, PaCO₂ 42, and HCO₃ 31. Interpreting these result, the nurse concludes that the client is in:

1. metabolic acidosis.
2. metabolic alkalosis.
3. respiratory acidosis.
4. respiratory alkalosis.

ANS: 2

Because the pH is greater than 7.45, this is not an acidosis. The PaCO₂ is within normal limits. The HCO₃ is elevated. An elevated pH and HCO₃ indicates metabolic alkalosis.

PTS: 1 DIF: Analyze REF: Arterial Blood Gas Analysis

5.A client is demonstrating signs of respiratory alkalosis. The nurse realizes that this alteration is *least likely* caused by which of the following?

1. Diarrhea
2. Fever
3. Pain
4. Severe anemia

ANS: 1

Diarrhea is a cause of metabolic acidosis. Causes of the respiratory alkalosis are hypoxia, increased minute ventilation, hyperventilation, pregnancy, fever, pain, and severe anemia.

PTS: 1 DIF: Analyze REF: Table 30-1 Causes of Acid-Base Imbalances

6.A client has a productive cough that produces green sputum with a musty odor. The nurse realizes that the client may be experiencing:

1. emphysema.

2. pneumococcal pneumonia.

3. *Pseudomonas* infection.

4. pulmonary edema.

ANS: 3

A client with a *Pseudomonas* infection can have a cough that produces green sputum with a musty odor. The sputum from emphysema is gray-white and mucoid. The sputum from pneumococcal pneumonia and pulmonary edema are rust colored and pink, frothy, respectively.

PTS: 1 DIF: Analyze REF: Table 30-2 Sputum in Pulmonary Conditions

7.A client is experiencing a gradual increase of pleuritic pain. In which of the following pulmonary conditions would the nurse expect to see this type of pain?

1. Pneumococcal pneumonia

2. Pneumothorax

TESTBANKSELLER.COM

3. Pulmonary embolism

4. Tuberculosis

ANS: 4

A more gradual onset of pleuritic pain is seen in tuberculosis and malignancy. Acute pleuritic pain is associated with pneumococcal pneumonia, pneumothorax, and pulmonary embolism.

PTS: 1 DIF: Analyze REF: Assessment: History Taking

8.The nurse is assessing a client diagnosed with emphysema. Which of the following will most likely be assessed during the clients physical examination?

1. Barrel chest

2. Pectus carinatum

3. Pectus excavatum

4. Scoliosis

ANS: 1

Barrel chest is often seen in chronic emphysema as a result of long-term air trapping. Pectus carinatum is an abnormal protuberance of the sternum, and pectus excavatum is an abnormal depression of the sternum.

PTS:1DIF:ApplyREF:Skeletal Deformities

9.The nurse is assessing an adult patient experiencing hypoxia. Which of the following findings would be considered a late sign of hypoxia?

1. Confusion

2. Cyanosis

3. Drowsiness

4. Headache

TESTBANKSELLER.COM

ANS: 2

Cyanosis is a late sign of hypoxia. Confusion, drowsiness, and headache are early signs.

PTS: 1 DIF: Analyze REF: Signs of Respiratory Distress

10.A client is diagnosed with a large pneumothorax. The percussion note the nurse would expect to find is:

1. dullness.

2. flatness.

3. resonant.

4. tympany.

ANS: 4

Air-filled areas have a percussion note of tympany. A resonant note can be elicited by percussing a patient with normal lungs. Flatness is heard over bone and dullness is heard over the organs.

PTS:1DIF:Apply

REF: Table 30-3 Percussion Notes and Associated Conditions

11.A client is demonstrating a crescendo-decrescendo pattern of breathing with periods of apnea. The nurse would document this breathing pattern as being:

1.	Cheyne-Stokes.
2.	apnea.
3.	bradypnea.
4.	Kussmaul.

ANS: 1

Cheyne-Stokes breathing is a pattern of crescendo-decrescendo breathing. Apnea is the absence of breathing. Bradypnea is a breathing rate of less than 12 respirations per minute. Kussmaul breathing is rapid and deep and often associated with diabetic ketoacidosis.

PTS: 1 DIF: Apply REF: Figure 30-14 Rhythms of Breathing

12.The nurse, assessing a clients breath sounds, has the stethoscope placed over the second intercostal space next to the sternum. The sound the nurse is most likely going to hear would be:

1.	vesicular.
2.	bronchovesicular.
3.	bronchial.
4.	absent.

ANS: 2

Bronchovesicular breath sounds are loud and harsh and are most likely heard over the trachea. Vesicular breath sounds can be heard anywhere over the lung fields. Bronchial sounds are only normally heard over the trachea. They are loud and harsh in quality, high-pitched, and sound hollow. Absent breath sounds can also be heard throughout the lung fields.

PTS: 1 DIF: Analyze REF: Table 30-4 Normal Breath Sounds

13.A client is determined to be a candidate for a low-flow oxygen delivery system. Which of the following will the nurse most likely assess in this client?

1. Active bleeding
2. Change in level of consciousness
3. Cardiac arrhythmias
4. Respiratory rate of 16, unlabored breathing

ANS: 4

Low-flow oxygen systems are used for clients who are clinically stable and have a normal ventilatory pattern such as the client with a respiratory rate of 16 and unlabored breathing. A high-flow oxygen system would be indicated for a client who is not clinically stable such as bleeding, change in level of consciousness, or who is experiencing cardiac arrhythmias.

PTS: 1 DIF: Apply REF: Oxygen Delivery Systems

TESTBANKSELLER.COM

MULTIPLE RESPONSE

1.The nurse is assessing a client for decreased fremitus. Which of the following conditions are associated with decreased fremitus? (Select all that apply.)

1. Atelectasis
2. Emphysema
3. Pneumonia
4. Pneumothorax
5. Pulmonary fibrosis
6. Pulmonary infarction

ANS: 1, 3, 5, 6

Atelectasis, pneumonia, pulmonary fibrosis, and pulmonary infarction cause decreased fremitus. Pneumothorax and emphysema would cause increased fremitus.

PTS: 1 DIF: Analyze REF: Fremitus

2.The nurse is documenting that a client has adventitious breath sounds. Which of the following would be considered this type of sound? (Select all that apply.)

1.	Rales
2.	Vesicular
3.	Rhonchi
4.	Wheeze
5.	Bronchovesicular
6.	Pleural friction rub

TESTBANKSELLER.COM

ANS: 1, 3, 4, 6

Adventitious breath sounds include rales, rhonchi, wheezes, and pleural friction rubs. Vesicular and bronchovesicular are considered normal breath sounds.

PTS: 1 DIF: Analyze REF: Table 30-5 Adventitious Breath Sounds

3.The nurse is assessing the thorax of an elderly client. Which of the following would be considered normal age-related changes in this clients respiratory system? (Select all that apply.)

1.	Hyperresonance
2.	Pain with inspiration
3.	Vital capacity reduced
4.	Hemoptysis
5.	Productive cough

6. Wheezes

ANS: 1, 3

Normal age-related changes seen in the elderly include hyperresonance with palpation and a reduction in the vital capacity. Pain with inspiration, hemoptysis, productive cough, and wheezes are not normal age-related changes of the respiratory system.

PTS: 1 DIF: Analyze REF: Age-Related Changes in the Respiratory System

4.A client is scheduled for a ventilation-perfusion scan. The nurse realizes that this diagnostic test is used to diagnose which of the following? (Select all that apply.)

1. Pulmonary emboli

2. Congestive heart failure

3. Bronchitis

4. Asthma

TESTBANKSELLER.COM

5. Pneumonia

6. COPD

ANS: 1, 3, 4, 5, 6

The purpose of the ventilation-perfusion scan is to diagnose and locate pulmonary emboli. It is also helpful in diagnosing bronchitis, asthma, pneumonia, COPD, and cancer. This scan is not used to diagnose congestive heart failure.

PTS: 1 DIF: Analyze REF: Ventilation-Perfusion Scan

5.A client is prescribed a bedside diagnostic test to assess pulmonary status. The nurse will prepare to administer which of the following to the client? (Select all that apply.)

1. Capnography

2. Thoracentesis

-
- | | |
|----|----------|
| 3. | Oximetry |
|----|----------|
-
- | | |
|----|--------------|
| 4. | Bronchoscopy |
|----|--------------|
-
- | | |
|----|-----------------|
| 5. | Polysomnography |
|----|-----------------|
-
- | | |
|----|-------------|
| 6. | Lung biopsy |
|----|-------------|
-

ANS: 1, 3, 5

The three diagnostic tests that can be administered at the bedside include capnography, which measures exhaled carbon dioxide; oximetry, which measures oxygenation; and polysomnography, which measures breathing while asleep. Thoracentesis, bronchoscopy, and lung biopsy are all invasive procedures and cannot be administered at the bedside.

Chapter 8 Nursing Management: Patients with Upper Respiratory Tract Disorders

MULTIPLE CHOICE

1. A child is diagnosed with severe allergic rhinitis. Which of the following manifestations would the nurse most likely assess in this client?

-
- | | |
|----|-----------------------|
| 1. | Edematous neck glands |
|----|-----------------------|
-
- | | |
|----|-----------------|
| 2. | Reduced hearing |
|----|-----------------|
-
- | | |
|----|----------|
| 3. | Pruritis |
|----|----------|
-
- | | |
|----|---|
| 4. | Frequent wiping of the nose with the palm of the hand |
|----|---|
-

ANS: 4

Frequent wiping of the nose with the palm of the hand is one symptom seen in the client diagnosed with severe allergic rhinitis. Edematous neck glands, reduced hearing, and pruritis are not manifestations of severe allergic rhinitis.

PTS:1 DIF:Analyze

REF:Allergic Rhinitis: Assessment with Clinical Manifestations

2.A client tells the nurse that she experiences a stuffy nose, nasal pain, and postnasal drip every time she works in her companys office. Which of the following types of allergic rhinitis is this client most likely experiencing?

1.	Infectious
2.	Perennial
3.	Occupational
4.	Seasonal

ANS: 3

Occupational allergic rhinitis occurs from airborne substances in the workplace. Seasonal allergic rhinitis occurs during a specific time of the year. Perennial allergic rhinitis occurs in response to exposure to environmental allergens that can occur throughout the year. Infectious rhinitis is a nonallergic type of rhinitis.

PTS: 1 DIF: Analyze REF: Table 31-1 Types of Allergic Rhinitis

3.A client asks the nurse if there is an antihistamine that does not cause drowsiness. Which of the following medications would this client most likely prefer to treat allergic rhinitis?

1.	Diphenhydramine
2.	Chlorpheniramine maleate
3.	Clemastine
4.	Fexofenadine

ANS: 4

Fexofenadine (Allegra) is a second-generation antihistamine, and second-generation antihistamines exhibit less sedation than first-generation medications such as diphenhydramine, chlorpheniramine maleate, and clemastine.

PTS:1DIF:Apply

REF: Table 31-3 Medications Used in Treatment of Rhinitis

4. A client diagnosed with hypertension is experiencing allergic rhinitis. The nurse realizes that the medication that would *not* be indicated for this client would be:

1. loratadine.
2. montelukast.
3. pseudoephedrine.
4. zafirlukast.

ANS: 3

Pseudoephedrine can be contraindicated for the patient with hypertension. Loratadine, montelukast, and zafirlukast should be used cautiously for patients with hepatic impairment.

PTS: 1 DIF: Analyze

REF: Table 31-3 Medications Used in Treatment of Rhinitis

5. A 16-year-old client is being prescribed a medication to treat acute sinusitis. The nurse realizes that this client should *not* be prescribed:

1. amoxicillin.
2. cefuroxime.
3. ciprofloxacin.
4. erythromycin.

ANS: 3

Quinolones such as ciprofloxacin (Cipro) and levofloxacin (Levaquin) are contraindicated in children younger than 17 years of age.

PTS: 1 DIF: Analyze REF: Acute Sinusitis: Pharmacology

6. The nurse is caring for a client diagnosed with acute sinusitis. Which of the following symptoms is the client most likely experiencing?

1. Anosmia

-
- | | |
|----|-------|
| 2. | Fever |
|----|-------|
-
- | | |
|----|-----------|
| 3. | Halitosis |
|----|-----------|
-
- | | |
|----|----------------|
| 4. | Metallic taste |
|----|----------------|
-

ANS: 1

Clients often complain of unilateral face pain, purulent nasal discharge, pain during mastication, anosmia (absence of smell), and headache. Less common symptoms include fever, nasal congestion, halitosis, toothache, metallic taste, and cough.

PTS:1DIF:Apply

REF:Acute Sinusitis: Assessment with Clinical Manifestations

7.The nurse is planning care for the client diagnosed with viral rhinitis. Which of the following would be the best goal of care for this client?

-
- | | |
|----|--|
| 1. | Prevent secondary bacterial infection. |
|----|--|
-
- | | |
|----|---------------------------------|
| 2. | Prevent rhinitis medicamentosa. |
|----|---------------------------------|
-
- | | |
|----|---------------------------------|
| 3. | Refrain from use of analgesics. |
|----|---------------------------------|
-
- | | |
|----|---|
| 4. | Encourage complete participation in activities. |
|----|---|
-

ANS: 1

Treatment of acute rhinitis, or the common cold, is aimed at decreasing the impact of the symptoms and preventing secondary bacterial infection. Rhinitis medicamentosa occurs from misuse of nasal decongestants. Acetaminophen or a nonsteroidal anti-inflammatory agent is useful for fever, aches, and pain. Rest is encouraged.

PTS: 1 DIF: Apply REF: Viral Rhinitis: Planning and Implementation

8.The nurse is instructing the mother of a client recovering from a tonsillectomy. Which of the following should the nurse instruct the mother to report?

-
- | | |
|----|-----------------------|
| 1. | Difficulty swallowing |
|----|-----------------------|
-

-
- | | |
|----|--------------------|
| 2. | Difficulty talking |
|----|--------------------|
-
- | | |
|----|----------------------|
| 3. | Excessive swallowing |
|----|----------------------|
-
- | | |
|----|------|
| 4. | Pain |
|----|------|

ANS: 3

Excessive swallowing is a sign of bleeding and should be reported. Pain and difficulty talking and swallowing are expected.

PTS:1DIF:Apply

REF:Tonsillitis and Adenoiditis: Planning and Implementation

9.Which of the following should the nurse instruct a client recovering from a tonsillectomy?

-
- | | |
|----|--------------------------------|
| 1. | Drink milk to promote healing. |
|----|--------------------------------|
-
- | | |
|----|-------------------------|
| 2. | Gargle with salt water. |
|----|-------------------------|
-
- | | |
|----|--------------------------|
| 3. | Maintain good hydration. |
|----|--------------------------|
-
- | | |
|----|-----------------------|
| 4. | Use a straw to drink. |
|----|-----------------------|

ANS: 3

Drinking milk does not promote healing and may encourage production of mucus. Gargling and drinking with a straw may disrupt the clot at the operative site and cause bleeding. Maintaining good hydration and eating soft foods are encouraged.

PTS:1DIF:Apply

REF:Tonsillitis and Adenoiditis: Planning and Implementation

10.A client is experiencing epistaxis. Which of the following interventions would the nurse complete?

-
- | | |
|----|------------------|
| 1. | Call the doctor. |
|----|------------------|
-
- | | |
|----|--------------------------------|
| 2. | Check laboratory test results. |
|----|--------------------------------|

-
3. Obtain an emesis basin.
 4. Show the patient how to pinch the nose.
-

ANS: 4

The initial intervention for a client with epistaxis is to show the client how to lean forward and pinch the nose against the nasal septum for about 5 to 10 minutes continuously. The other interventions are not necessary at this time.

PTS: 1 DIF: Apply REF: Epistaxis: Planning and Implementation

11.A client has been diagnosed with stage IV cancer of the larynx. The nurse realizes that which of the following surgeries is recommended for this type of cancer?

-
1. Hemilaryngectomy
 2. Partial laryngectomy
 3. Supraglottic laryngectomy
 4. Total laryngectomy
-

TESTBANKSELLER.COM

ANS: 4

In clients diagnosed with invasive or infiltrating tumors such as those of stage III or stage IV, the entire larynx is removed. The other surgeries only remove portions of the larynx and would be appropriate for lesser stages of the disease.

PTS: 1 DIF: Analyze REF: Laryngeal Obstruction: Surgery

12.A client is recovering from a total laryngectomy with the placement of a tracheostomy. The nurse should include which of the following instructions to this client?

-
1. Clean the tracheostomy tube with soap and water daily.
 2. Limit protein in the diet.
 3. Restrict fluids.
-

-
4. The nasogastric tube will be in for 2 weeks.

ANS: 4

Clients recovering from a laryngectomy are unable to take nutrition orally for about 10 to 14 days. During this time the client will receive nutrition via intravenous fluids, enteral feedings through a nasogastric tube, or parenteral nutrition. Protein and fluids are not limited. The tracheostomy tube is not cleaned with soap and water.

PTS: 1 DIF: Apply REF: Laryngeal Obstruction: Nutrition

13.A client diagnosed with viral rhinitis tells the nurse that she has been using a decongestant nasal spray for several weeks and the symptoms are getting worse. Which of the following does the nurse suspect is occurring with this client?

-
1. Developing pneumonia
-
2. Subacute rhinitis
-
3. Rhinitis medicamentosa
-
4. Chronic otitis media

TESTBANKSELLER.COM

ANS: 3

Rhinitis medicamentosa can occur with overuse of decongestant nasal sprays, and it leads to rebound nasal congestion that is often worse than the original nasal congestion. The use of nasal sprays does not cause pneumonia, subacute rhinitis, or chronic otitis media.

PTS: 1 DIF: Analyze REF: Viral Rhinitis: Planning and Implementation

MULTIPLE RESPONSE

1.The nurse is teaching a client how to use a nasal spray. Which of the following should be included in these instructions? (Select all that apply.)

-
1. Blow the nose before instilling the spray.
-
2. Tilt the head back and angle the tip of the bottle to the side of the nostril.
-
3. Use a finger to occlude the nostril that is not receiving the spray.

-
4. Inhale gently and evenly while discharging the spray into the nostril.
 5. If a second spray is recommended, immediately repeat the procedure.
 6. Blow the nose after administration of the spray.
-

ANS: 1, 3, 4

For the steps to be correct, the head should be slightly forward, the second spray should be given 15 to 20 seconds after the spray, and the client should not blow the nose after the administration of the spray. The client should be instructed to blow the nose before instilling the spray, to use a finger to occlude the nostril that is not receiving the spray, and to gently inhale while the spray is being delivered into the nostril.

PTS: 1 DIF: Apply REF: Patient Playbook: Installation of Nasal Spray

2.A client has been diagnosed with allergic rhinitis. Which of the following should the nurse instruct the client regarding strategies to avoid this disorder? (Select all that apply.)

-
1. Remove home carpeting
 2. Reduce the use of an air conditioner
 3. Remove pets from the home
 4. Open windows in the spring and summer
 5. Use feather pillows
 6. Wash bed linens in cold water
-

ANS: 1, 3

Strategies to reduce the symptoms of allergic rhinitis include removing home carpeting and removing pets from the home. The client should be instructed to use an air conditioner, keep windows closed during allergy season, avoid feather pillows, and wash bed linens in hot water.

PTS: 1 DIF: Apply REF: Nursing Strategy: Allergy Avoidance Measures

3.A client is demonstrating signs of chronic sinusitis. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

1.	Facial pain
2.	Fever
3.	Headache
4.	Toothache
5.	Fatigue
6.	Swollen neck glands

ANS: 1, 3, 4, 5

Manifestations of chronic sinusitis include facial pain, headache, toothache, and fatigue. Fever and swollen neck glands would indicate the disorder has spread beyond the sinuses.

PTS:1DIF:Apply

REF:Chronic Sinusitis: Assessment with Clinical Manifestations

4. With which of the following can the nurse instruct a client who is experiencing pain from a sore throat? (Select all that apply.)

1.	Gargle with warm salt water.
2.	Eat salty foods.
3.	Suck on hard candy.
4.	Drink fluids.
5.	Avoid citrus fruits.
6.	Suck on popsicles.

ANS: 1, 3, 4, 6

Interventions to reduce the pain from a sore throat include gargling with warm salt water, sucking on throat lozenges or hard candy, sucking on flavored frozen desserts or popsicles, using a humidifier in the bedroom, and drinking fluids. The client should not be instructed to eat salty foods or avoid citrus fruits.

PTS: 1 DIF: Apply REF: Patient Playbook: Easing Sore Throat Pain

5.A client is demonstrating signs of peritonsillar abscess. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

1.	Bradypnea
2.	Drop in blood pressure
3.	Hot potato voice
4.	Trismus
5.	Dysphagia
6.	Sore throat

ANS: 3, 4, 5, 6

Assessment findings consistent with peritonsillar abscess include: hot potato voice; trismus, or difficulty fully opening the mouth; dysphagia, or painful swallowing; and sore throat. Bradypnea and drop in blood pressure are not assessment findings consistent with peritonsillar abscess.

PTS:1DIF:Apply

Chapter 9 Nursing Management: Patients with Chest and Lower Respiratory Tract

Disorders

MULTIPLE CHOICE

1.The nurse is reviewing clients for risk factors in the development of pneumonia. Which of the following clients would be at the highest risk for developing this disorder?

1.	A 48-year-old client experiencing menopause
----	---

-
2. An 18-year-old client with abdominal pain
 3. A 23-year-old client diagnosed with sickle-cell anemia and a cough
 4. A 3-year-old client with fever
-

ANS: 3

High-risk groups for acquiring pneumonia are people with diabetes, infants 6- to 23-months old, and those with a chronic illness such as sickle-cell anemia. Menopause and abdominal pain are not symptoms associated with pneumonia. Fever in a 3-year-old client could be caused by many disorders and not necessarily pneumonia.

PTS:1DIF:Analyze

REF: Box 32-1 High-Risk Indicators for Acquiring Pneumonia

2.A client diagnosed with chronic obstructive pulmonary disease is experiencing pneumonia. The nurse applies oxygen at 2 L/min via nasal cannula. When the nurse leaves the room, a family member increases the oxygen to 5 L. Which complication may occur?

-
1. AnginaTESTBANKSELLER.COM
 2. Apnea
 3. Metabolic acidosis
 4. Respiratory alkalosis
-

ANS: 2

The COPD clients drive to breathe is hypoxia. Increasing the oxygen removes this drive and leads to apnea. Angina occurs because of decreased oxygen to the myocardial tissues. Neither respiratory alkalosis nor metabolic acidosis would occur with the increased oxygen level.

PTS: 1 DIF: Analyze REF: Safety First: Oxygen Therapy

3.The nurse has a positive PPD during the last testing cycle for tuberculosis. Which of the following is indicated for this nurse?

-
1. Nothing
-

-
2. Chest x-rays every 2 months
 3. Pharmacological treatment
 4. Admission for inpatient treatment
-

ANS: 3

Latent tuberculosis infection occurs when a person exposed to the mycobacterium has a positive PPD test. This person is without an active clinical picture and has a 10% chance of developing TB if preventive pharmacological treatment is not initiated. The nurse needs pharmacological treatment. Doing nothing could result in active disease. The nurse does not need chest x-rays every 2 months or admission for inpatient treatment.

PTS: 1 DIF: Apply REF: Tuberculosis: Pathophysiology

4.A client undergoes a purified protein derivative (PPD) test. The test should be read:

-
1. immediately after the test.
 2. 24 to 48 hours after the test.
 3. 48 to 72 hours after the test.
 4. anytime after 72 hours.
-

ANS: 3

A small amount of tuberculin is injected directly under the skin at the site and is read 48 to 72 hours after the test. The test should not be read immediately afterwards or within 24 to 48 hours. If the test is read after 72 hours, the test may need to be repeated.

PTS: 1 DIF: Apply REF: Tuberculosis: Diagnostic Tests

5.The nurse is instructing a client on ways to reduce the transmission of tuberculosis. Which of the following should be included in these instructions?

-
1. The disease is transmitted by inhaling droplets exhaled by an infected person.
 2. The disease is transmitted by not fully cooking foods.
-

3. The disease is transmitted by not washing hands.

4. The disease is transmitted by sexual contact.

ANS: 1

Tuberculosis is transmitted by inhaling the bacillus present in the air. The bacillus is present in the air after an infected person has coughed, sneezed, or expectorated. Tuberculosis is not transmitted through poorly cooked foods, poor handwashing, or sexual contact.

PTS: 1 DIF: Apply REF: Tuberculosis: Patient and Family Teaching

6. A client receiving oral medications for the treatment of tuberculosis develops hepatitis. Which of the following medications would be indicated for the client at this time?

1. Ethambutol

2. Isoniazid

3. Rifampin
TESTBANKSELLER.COM

4. Streptomycin

ANS: 4

Streptomycin is a medication that can be used until the cause of hepatitis is identified or the liver tissue heals. It is also given for those who have a first-line drug intolerance. First-line drugs are isoniazid (INH), rifampin (RIF), ethambutol (EMB), and pyrazinamide (PZA).

PTS: 1 DIF: Apply REF: Tuberculosis: Pharmacology

7. The spouse of a client diagnosed with tuberculosis is to begin isoniazid prophylactic therapy. Which of the following should the nurse instruct the spouse regarding length of time to take this medication? The medication should be taken for:

1. 10 to 24 days.

2. 1 to 3 months.

3. 4 to 7 months.

-
4. 6 to 12 months.

ANS: 4

Isoniazid therapy lasts 6 to 12 months. Taking the medication less than 6 months can be ineffective. The spouse should not be instructed to take the medication for 10 to 24 hours, 1 to 3 months, or 4 to 7 months.

PTS:1DIF:Apply

REF: Table 32-4 Treatment Regimens and Nursing Considerations for Tuberculosis

8.A client diagnosed with a lung abscess is being prescribed antibiotic therapy. Which of the following medications would be indicated if this client has a history of penicillin allergy?

-
1. Metronidazole

-
2. Clindamycin

-
3. Ampicillin

-
4. TESTBANKSELLER.COM
Steroid

ANS: 2

Clients allergic to penicillin are often given clindamycin since this medication is not part of the penicillin family. Metronidazole and ampicillin should not be administered to this client. Steroid is not an antibiotic.

PTS: 1 DIF: Analyze REF: Lung Abscess: Pharmacology

9.A client diagnosed with a hemothorax has had a chest tube inserted and attached to a portable water-seal drainage system. Which of the following interventions would be *inappropriate* for this client?

-
1. Clamp the tubing when ambulating.
-
2. Date and mark the amount of drainage in the collection chamber every shift.
-
3. Monitor the suction chamber for continuous bubbling.
-
4. Watch the water-seal chamber for fluctuation.
-

ANS: 1

The chest tube should not be clamped or raised above the chest when ambulating. All other options are appropriate.

PTS: 1 DIF: Apply REF: Pneumothorax: Planning and Implementation

10.A clients chest tube has been accidentally dislodged while the client was being transferred from the bed to a stretcher. Which of the following should the nurse do to help this client?

1. Cover the site with occlusive petroleum jelly gauze and tape to four sides.
2. Cover the site with occlusive petroleum jelly gauze and tape to three sides.
3. Cover the site with occlusive petroleum jelly gauze and tape to two sides.
4. Cover the site with occlusive petroleum jelly gauze and tape to one side.

ANS: 2

In the case of accidental dislodging of the chest tube, the site should be covered with occlusive petroleum jelly gauze and taped on three sides to prevent the development of a tension pneumothorax. If the gauze is taped on all four sides, the client can develop a tension pneumothorax. Taping the gauze on one or two sides will not be effective to support this client and should not be done.

PTS: 1 DIF: Apply REF: Red Flag: Ensuring Chest Tube Connections

11.A client is diagnosed with fractured ribs. Which of the following should the nurse instruct this client?

1. Engage in routine activities of daily living after taking pain medication.
2. Splint the rib cage when deep breathing and coughing.
3. Restrict fluids.
4. Stay on bed rest until the ribs heal.

ANS: 2

Nursing care for a client recovering from fractured ribs include splinting the rib cage when deep breathing and coughing. The client should be encouraged to avoid dangerous activities when

taking pain medication. Fluids should not be restricted. Bed rest would not be necessary for fractured ribs.

PTS:1DIF:Apply

REF: Fractured Rib: Planning and Implementation; Patient and Family Teaching

12.A client is prescribed a diuretic for treatment of pulmonary hypertension. Which of the following should the nurse instruct the client regarding this medication?

-
1. This medication expands the blood vessels.

 2. This medication causes smooth muscle relaxation to reduce pulmonary engorgement.

 3. This medication reduces the amount of water in the body.

 4. This medication keeps the blood from clotting.

ANS: 3

Diuretics in the treatment of pulmonary hypertension are used to reduce the amount of water in the body. Vasodilators expand the blood vessels. Sildenafil causes smooth muscle relaxation to reduce pulmonary engorgement. Anticoagulants keep the blood from clotting.

PTS: 1 DIF: Apply REF: Pulmonary Arterial Hypertension: Pharmacology

13.The nurse is assessing a client experiencing manifestations of cor pulmonale. Which of the following will the nurse most likely assess in this client?

-
1. Low blood pressure

 2. Low heart rate

 3. Hoarseness

 4. Lumbar pain

ANS: 3

Manifestations of cor pulmonale include hoarseness, chest pain, distended neck veins, liver enlargement, peripheral edema, abnormal heart sounds. Low blood pressure, low heart rate, and lumbar pain are not manifestations of cor pulmonale.

PTS:1DIF:Apply

REF:Cor Pulmonale: Assessment with Clinical Manifestations

MULTIPLE RESPONSE

1.The nurse is caring for a client diagnosed with pneumonia. Which of the following signs and symptoms would the nurse most likely assess in this client? (Select all that apply.)

1. Abdominal pain

2. Anorexia

3. Cough

4. Dyspnea

5. Fever

6. Frequent wiping of the nose

TESTBANKSELLER.COM

ANS: 1, 2, 3, 4, 5

Specific symptoms suggestive of pneumonia include fever, chills or rigor, sweats, new cough (with or without sputum), pleuritic chest pain, and dyspnea. Nonspecific symptoms include malaise, fatigue, abdominal pain, headaches, anorexia, and worsening of an underlying illness. Frequent wiping of the nose is a sign of allergic rhinitis.

PTS:1DIF:Apply

REF: Pneumonia: Assessment with Clinical Manifestations

2.The nurse is planning to administer the pneumococcus vaccination to a client. Which of the following would indicate that a client is a candidate for this vaccination? (Select all that apply.)

1. Age 70

2. Age 55

3. Diagnosis of heart failure

-
- | | |
|----|--|
| 4. | Recovering from knee replacement surgery |
|----|--|
-
- | | |
|----|---------------------|
| 5. | Diagnosis of asthma |
|----|---------------------|
-
- | | |
|----|---------------------------------|
| 6. | Recovering from an appendectomy |
|----|---------------------------------|
-

ANS: 1, 3, 5

Criteria for the pneumococcus vaccination include high-risk groups such as people over age 65, diagnosed with chronic heart disease, and diagnosed with asthma. Age 55, recovering from knee replacement surgery; and recovering from an appendectomy are not criteria for the pneumococcus vaccination.

PTS: 1 DIF: Analyze REF: Pneumonia: Pharmacology

3.The nurse is planning care for a client diagnosed with bronchiolectasis. Which of the following would be goals for this clients care? (Select all that apply.)

-
- | | |
|----|----------------------|
| 1. | Treat the infection. |
|----|----------------------|
-
- | | |
|----|------------------------|
| 2. | Reduce the heart rate. |
|----|------------------------|
-
- | | |
|----|--------------------------|
| 3. | Minimize further damage. |
|----|--------------------------|
-
- | | |
|----|-----------------------|
| 4. | Improve urine output. |
|----|-----------------------|
-
- | | |
|----|--------------------|
| 5. | Promote breathing. |
|----|--------------------|
-
- | | |
|----|--------------------|
| 6. | Remove secretions. |
|----|--------------------|
-

ANS: 1, 3, 5, 6

Treatment goals for the client diagnosed with bronchiolectasis include treat the infection, minimize further damage, promote effective airway breathing, and remove secretions. Treatment goals do not include reducing heart rate and improving urine output.

PTS: 1 DIF: Apply REF: Bronchiolectasis: Planning and Implementation

4.The nurse, planning care for a client diagnosed with a pneumothorax, identifies which types of pneumothorax? (Select all that apply.)

1.	Spontaneous
2.	Radical
3.	Traumatic
4.	Incomplete
5.	Iatrogenic
6.	Tension

ANS: 1, 3, 5, 6

The four types of pneumothorax are spontaneous, traumatic, iatrogenic, and tension. Radical and incomplete are not types of pneumothorax.

PTS: 1 DIF: Analyze REF: Pneumothorax: Etiology

5. Which of these instructions are for a client diagnosed with a pneumothorax? (Select all that apply.)

1.	Remove air from the pleural space.
2.	Correct acid-base imbalances.
3.	Treat infection.
4.	Minimize damage.
5.	Reexpand the lung.
6.	Improve fluid balance.

ANS: 1, 2, 4, 5

Treatment goals for pneumothorax include removing the air and fluid from the pleural space, correcting acid-base imbalance, minimizing further damage, and reexpanding the lung. Treating infection and improving fluid balance are not treatment goals for a pneumothorax.

Chapter 10 Nursing Management of Patients Diagnosed with COPD and Asthma

MULTIPLE CHOICE

1. A client states, I don't know why I should quit smoking. It can't improve anything. The nurse responds by informing the client about the decrease in lung cancer rates over time after a person quits smoking. Which of the following is correct?

-
1. The lung cancer rate corresponds to that of nonsmokers 1 year after quitting smoking.
 2. The lung cancer rate corresponds to that of nonsmokers 2 years after quitting smoking.
 3. The lung cancer rate corresponds to that of nonsmokers 5 years after quitting smoking.
 4. The lung cancer rate corresponds to that of nonsmokers 10 years after quitting smoking.

ANS: 4

Ten years after quitting smoking, the client's lung cancer rate will correspond to a nonsmoker's rate. After 1 year of no smoking, the risk of coronary heart disease decreases to half that of a smoker. After 2 years of no smoking, the risk of coronary heart disease equals that of a nonsmoker. After 5 years of no smoking, the lung cancer rate drops by half.

PTS:1 DIF: Apply

REF: Table 33-4 Changes in Physiological Function of Patients after Smoking Cessation

2. A client has been smoking for the last 40 years and has a history of emphysema. Which of the following findings would the nurse *not* expect to find?

-
1. Decreased forced vital capacity (FVC)
 2. Increased anterior-posterior chest diameter
 3. Increased forced expiratory volume (FEV₁)

-
4. Pursed lip breathing

ANS: 3

The FEV₁ does not increase; it decreases. The FVC does decrease, and the client can exhibit increased anterior-posterior chest diameter and pursed lip breathing.

PTS:1DIF:Apply

REF: Chronic Obstructive Pulmonary Disease: Assessment with Clinical Manifestations

3.A client is being treated for exacerbation of chronic obstructive pulmonary disease. Which of the following nursing interventions will the nurse expect to be completed?

-
1. Initiate oxygen at 1 L/min via nasal cannula.
-
2. Limit fluids.
-
3. Place on respiratory isolation.
-
4. Schedule all activities at one time.
-

ANS: 1

Oxygen for a client diagnosed with COPD should be low flow so as not to diminish the clients drive to breath. Fluids are encouraged, and activities should be interspersed with rest periods so the client will not become overtired. Isolation is not necessary at this time.

PTS:1DIF:Apply

REF:Chronic Obstructive Pulmonary Disease: Oxygen Therapy

4.A client has been diagnosed with chronic obstructive pulmonary disease. Which of the following nursing diagnoses would be the most important at this time?

-
1. Activity intolerance
-
2. Anxiety
-
3. Impaired gas exchange
-

-
4. Nutrition, imbalance

ANS: 3

Airway and breathing are always a top priority for a client. Once gas exchange is ensured for the client, the other diagnoses of activity intolerance and nutrition imbalance can be addressed. Anxiety would be addressed last for this client.

PTS:1DIF:Apply

REF:Chronic Obstructive Pulmonary Disease: Nursing Diagnoses

5.The nurse is caring for a client who has completed pulmonary function testing. Which of the following indicates the amount of air inhaled or exhaled with each breath during normal breathing?

-
1. Expiratory reserve volume
-
2. Minute volume
-
3. Tidal volume
-
4. Vital capacity

ANS: 3

Tidal volume is the amount of air inhaled or exhaled with each breath during normal breathing. The expiratory reserve volume is the maximum amount of air exhaled forcefully after a normal exhalation. Minute volume is the amount of air breathed per minute. Vital capacity is the maximum amount of air exhaled after maximum inspiration.

PTS:1DIF:Analyze

REF: Table 33-3 Pulmonary Function Spirometry Measures

6.The nurse is caring for a client diagnosed with chronic obstructive pulmonary disease. Which of the following interventions require extra care by the nurse?

-
1. Administering pain medications
-
2. Applying a cardiac monitor

-
3. Encouraging fluids
 4. Teaching the client diaphragmatic breathing
-

ANS: 1

Administering pain medications (narcotics) requires extra care by the nurse because these medications can depress respiratory status and worsen hypercapnia. Increasing fluids helps thin the client's secretions and is encouraged. Applying a cardiac monitor and monitoring the rhythm is part of a normal assessment. Teaching diaphragmatic breathing does not require extra care.

PTS:1DIF:Apply

REF: Chronic Obstructive Pulmonary Disease: Pharmacology

7. A client diagnosed with chronic obstructive pulmonary disease has the complication of cor pulmonale. Which of the following instructions will be included in the client's discharge teaching?

-
1. Adjust oxygen higher depending on activity level.
 2. Increase sodium in the diet.
 3. Maintain bed rest.
 4. Weigh self daily, and call the physician with a weight gain of 2 pounds
-

ANS: 4

A weight gain of greater than 2 pounds would indicate fluid retention and need to be reported to the physician. Oxygen would not be increased past the prescribed level because this may eliminate the client's drive to breathe. Increasing sodium will encourage fluid retention. Moderate activity is desired to maintain a level of cardiovascular health.

PTS:1DIF:Apply

REF: Chronic Obstructive Pulmonary Disease: Patient and Family Teaching

8. A client is being admitted with the diagnosis of asthma. To facilitate breathing, in what position would the nurse place the client?

-
1. Lateral
-

2.	Prone
----	-------

3.	High-Fowlers
----	--------------

4.	Supine
----	--------

ANS: 3

Only the high-Fowlers position facilitates breathing. The other positions could make breathing more difficult.

PTS: 1 DIF: Apply REF: Asthma: Assessment and Clinical Manifestations

9.A client diagnosed with asthma is having an acute episode at home. Which of the following medications should the client be instructed *not* to use during this episode?

1.	Albuterol
----	-----------

2.	Proventil
----	-----------

3.	Serevent
----	----------

4.	Ventolin
----	----------

ANS: 3

Serevent is a long-acting agent and is not to be used as rescue medication during acute episodes. Albuterol (also sold under the brand names Proventil and Ventolin) is a short-acting agent used as a rescue medication.

PTS: 1 DIF: Apply REF: Asthma: Pharmacology

10.A client diagnosed with asthma is receiving instructions about the use of albuterol. The client should be aware that albuterol may cause:

1.	bradycardia.
----	--------------

2.	drowsiness.
----	-------------

3.	nasal congestion.
----	-------------------

-
4. nervousness.

ANS: 4

Albuterol causes nervousness, tachycardia, insomnia, dizziness, tremors, hypertension, headache, and irritation to the nasal and throat passages. Albuterol does not cause bradycardia, drowsiness, or nasal congestion.

PTS: 1 DIF: Apply REF: Asthma: Complications

11.The nurse is assessing a client diagnosed with asthma. The clients breath sounds initially had wheezing but are diminishing until no audible sounds are heard. This has occurred because:

-
1. swelling has increased, and it has blocked airways.
-
2. the attack has passed.
-
3. the client used an inhaler.
-
4. no mucus is present.

TESTBANKSELLER.COM

ANS: 1

This client needs to be evaluated immediately and receive prompt treatment to reduce the airway obstruction and reverse inflammation. Lack of audible breath sounds does not mean that the attack has passed, the client has used an inhaler, or there is no mucus present.

PTS: 1 DIF: Analyze REF: Asthma: Complications

12.The parents of a child diagnosed with cystic fibrosis ask the nurse how their child developed the disease. Which of the following should the nurse explain to these parents?

-
1. Cystic fibrosis is a disease that has an extra chromosome.
-
2. Cystic fibrosis is an X-linked disorder.
-
3. Cystic fibrosis is passed on by a defective gene from both parents.
-
4. Cystic fibrosis is passed on by one defective gene from one parent.

ANS: 3

Cystic fibrosis is an inherited, autosomal recessive disease that is passed on by a defective gene from both parents and not one parent. This disease does not occur because of an extra chromosome. This disease is not an X-linked disorder. Cystic fibrosis is a chronic, progressive, and frequently fatal disease of the body's exocrine mucus-producing glands that primarily affects the respiratory, digestive, intestinal systems, and the pancreas.

PTS:1 DIF:Apply REF:Cystic Fibrosis

13. The nurse is to collect a stool specimen from a client diagnosed with cystic fibrosis. The nurse would expect to see:

1. black, tarry stool.
2. bulky, foul-smelling stool.
3. clay-colored stool.
4. green stool.

ANS: 2

Bulky, foul-smelling stool is characteristic of clients diagnosed with cystic fibrosis as a result of malabsorption. Black, tarry stool can be observed in a client with upper gastrointestinal bleeding. Clay-colored stool can indicate bile obstruction. Green stool may indicate gastrointestinal infection.

PTS: 1 DIF: Analyze REF: Box 33-3 Nutrition in CF

14. A client is experiencing a sudden onset of headache, nausea, cough, fever, myalgia, and fatigue. The nurse suspects this client is experiencing:

1. seasonal influenza.
2. chronic obstructive pulmonary disease.
3. asthma.
4. cystic fibrosis.

ANS: 1

Seasonal influenza has a sudden onset with a headache, nausea, cough, chills, fever, rhinitis, myalgia, and extreme fatigue. These symptoms are not seen in chronic obstructive pulmonary disease, asthma, or cystic fibrosis.

PTS:1 DIF:Analyze REF:Seasonal Influenza

MULTIPLE RESPONSE

1.The nurse is caring for a client diagnosed with cystic fibrosis. Which of the following medications does the nurse realize are commonly used to help treat this disorder? (Select all that apply.)

1.	N-acetylcysteine (Mucomyst)
2.	Acetaminophen (Tylenol)
3.	Dornase alfa (Pulmozyme)
4.	Furosemide (Lasix)
5.	Ibuprofen (Motrin)
6.	Digitalis (Digoxin)

ANS: 1, 3, 5

Medications commonly used to treat cystic fibrosis include N-acetylcysteine (Mucomyst), Dornase alfa (Pulmozyme), and Ibuprofen (Motrin). Acetaminophen, furosemide, and digitalis are not routinely prescribed in the treatment of cystic fibrosis.

PTS: 1 DIF: Analyze REF: Cystic Fibrosis: Pharmacology

2.The nurse suspects a client is experiencing chronic obstructive pulmonary disease when which of the following is assessed? (Select all that apply.)

1.	Peripheral edema
2.	Jugular vein distention
3.	High blood pressure

4. Dyspnea on exertion

5. Sputum production

6. Cough

ANS: 4, 5, 6

Chronic obstructive pulmonary disease is characterized by a history of three primary symptoms: 1) cough, 2) sputum production, and 3) dyspnea on exertion. Peripheral edema, jugular vein distention, and high blood pressure are not symptoms of chronic obstructive pulmonary disease.

PTS:1DIF:Analyze

REF: Chronic Obstructive Pulmonary Disease: Assessment with Clinical Manifestations

3.A client is diagnosed with stage I mild, chronic obstructive pulmonary disease. Which of the following assessment findings will support this diagnosis? (Select all that apply.)

1. Chronic cough

2. Sputum production

TESTBANKSELLER.COM

3. Forced expiratory volume in 1 second of greater than 80%

4. Mild airflow limitations

5. Extreme dyspnea on exertion

6. Right-sided heart failure

ANS: 3, 4

In stage I mild chronic obstructive pulmonary disease, the client will demonstrate mild airflow limitations and have a forced expiratory volume in 1 second of greater than 80%. Chronic cough and sputum production are signs of stage 0 of the disease. Extreme dyspnea on exertion and right-sided heart failure are indications of stage III severe chronic obstructive pulmonary disease.

PTS:1DIF:Analyze

REF:Table 33-2 Classification of Patients with COPD by Severity

4. A client diagnosed with chronic obstructive pulmonary disease is scheduled for diagnostic tests. Which of the following are used to aid in the diagnosis of this disorder? (Select all that apply.)

- | | |
|----|-------------------------------------|
| 1. | Pulmonary function spirometry tests |
| 2. | Chest x-ray |
| 3. | Electrocardiogram |
| 4. | Medication levels |
| 5. | Sputum samples |
| 6. | Electrolyte levels |

ANS: 1, 2, 3, 4

Tests used to aid in the diagnosis of chronic obstructive pulmonary disease include pulmonary function spirometry tests, chest x-ray, electrocardiogram, and medication levels. Sputum samples are not useful and are not recommended in the diagnosis of chronic obstructive pulmonary disease. Electrolyte levels are not indicated.

Chapter 11 Nursing Assessment: Cardiovascular and Circulatory Function

MULTIPLE CHOICE

1. A client, receiving a transfusion of packed red blood cells, asks the nurse why it is needed since she knows the body makes new blood. Which of the following should the nurse respond to this client?

- | | |
|----|---|
| 1. | It will take 30 days for your body to make the new blood cells. |
| 2. | Your body will make the new blood cells in about 60 days. |

-
3. It takes at least 3 months for your body to make enough blood cells to replace what you have lost.
 4. Red blood cells last about 120 days. Your body needs to have a constant supply to replace them.
-

ANS: 4

Red blood cells have a life span of about 120 days. Abnormal red blood cells have a shorter life span and are lysed and extracted from the circulation. The nurse should explain that since red blood cells live 120 days, the body needs a constant supply. The transfusion will provide the cells until the body makes new ones.

PTS: 1 DIF: Apply REF: Erythrocytes

2.The nurse is reviewing a clients white blood cell count. Which of the following would not be affected by the presence of an infection?

-
1. Basophils
 2. Eosinophils
 3. Lymphocytes
 4. Neutrophils
-

ANS: 3

Basophils, eosinophils, and neutrophils are considered granulocytes which are present in increased amounts in the presence of an infection. Lymphocytes are common in the lymphatic system and work with acquired immunity.

PTS:1DIF:AnalyzeREF:White Blood Cells

3.A client is diagnosed with a parasitic infection. The nurse realizes that which of the following white blood cell counts will be elevated?

-
1. Basophils
 2. Eosinophils
 3. Lymphocytes
-

4. Neutrophils

ANS: 2

Eosinophils are classically associated with infections from parasites. Eosinophils are also more involved in the reactions to allergies. Basophils are central to the inflammatory process and neutrophils respond to bacterial infections. Lymphocytes are cells that work in acquired immunity.

PTS:1DIF:AnalyzeREF:White Blood Cells

4.A client is diagnosed with a low red blood cell count. Which of the following should the nurse assess in this client?

-
1. Urine output
-
2. Bowel sounds
-
3. Respirations
-
4. Consciousness

ANS: 3

The function of the erythrocyte or red blood cell is to transport oxygen from the lungs to the cells of the body. With a low red blood cell count, the nurse should assess the clients respiratory status. The urine output, bowel sounds, and consciousness may or may not be affected by the decrease in red blood cells.

PTS: 1 DIF: Apply REF: Erythrocytes

5.A client is prescribed a vitamin B-12 injection every month. Which of the following should the nurse explain to the client as the purpose of this medication?

-
1. It is needed to make new red blood cells.
-
2. It makes the red blood cells more flexible.
-
3. It makes the red blood cells hold more oxygen.
-
4. It makes the red blood cells hold their shape.

ANS: 1

Vitamin B-12 is necessary to make erythrocytes and help make thymine, which is a precursor to the red blood cell. The erythrocyte cytoskeleton makes the red blood cell more flexible.

Cholesterol helps the red blood cells hold their shape. There is nothing physiologically that will make a red blood cell hold more oxygen.

PTS: 1 DIF: Apply REF: Erythrocytes

6. During the assessment of a client, the nurse identifies pinpoint hemorrhages under the skin that are dark red and less than 2 mm in size. How should the nurse document this finding?

1.	Petechiae
2.	Ecchymosis
3.	Hematoma
4.	Purpura

ANS: 1

TESTBANKSELLER.COM

Petechiae are pinpoint hemorrhages under the skin which are round, dark red and less than 2 mm in diameter. Ecchymosis is superficial bleeding under the mucous membrane or skin. Hematoma is a deeper palpable bleeding under the skin. Purpura is abnormal bleeding under the skin as seen from blood leaking from capillaries after minor trauma.

PTS: 1 DIF: Apply

REF: Table 23-3 Physical Assessment for Hematological Problems

7. The nurse is auscultating the area to the right of the sternum at the second intercostal space. The nurse is listening to which of the following valves?

1.	Aortic
2.	Mitral
3.	Tricuspid
4.	Pulmonic

ANS: 1

The aortic area is the second intercostal space to the right of the sternum. The pulmonic area is the second intercostal space on the left side of the sternum. The tricuspid area is the fifth intercostal space on the left side of the sternum. The mitral area is the fifth intercostal space at the midclavicular line on the left side of the pericardium.

PTS: 1 DIF: Apply REF: Assessment

8.A client is diagnosed with a normal ejection fraction. The nurse realizes that the clients ejection fraction is most likely between:

- | | |
|----|-------------|
| 1. | 10% to 20%. |
| 2. | 30% to 40%. |
| 3. | 60% to 70%. |
| 4. | 80% to 90%. |

ANS: 3

Ejection fraction is the percentage of blood that is emptied from the ventricle during systole. An ejection fraction of 60% to 70% is considered normal. Lower ejection fraction findings indicate damage to the ventricle. Ejection fractions are not usually as high as 80% to 90%.

PTS:1DIF:AnalyzeREF:Cardiac Cycle

9.A client is experiencing chest pain that occurs in the third costochondral joint. The onset was sudden; it radiated to the shoulders; and it becomes worse when taking a deep breath or twisting the torso. The nurse suspects that this client is experiencing:

- | | |
|----|----------------------------------|
| 1. | aortic dissection. |
| 2. | pulmonary embolus. |
| 3. | pneumothorax. |
| 4. | musculoskeletal-costochondritis. |

ANS: 4

The pain of costochondritis includes locations at the 3rd, 4th, or 5th costochondral joint; a sudden or gradual onset; and radiation to the shoulders. Costochondritis is aggravated by deep

inspirations or twisting. The pain of aortic dissection is sudden and tearing, and it radiates to the shoulders, neck, back, and abdomen. The pain of a pulmonary embolus includes a sudden, sharp pleuritic pain that varies with respiration. The pain of a pneumothorax is a sudden onset of tearing or pleuritic pain that is worsened by breathing.

PTS: 1 DIF: Analyze REF: Table 23-7 Differentiating Chest Pain

10. The nurse applies pressure to the fingernail of a client and watches for the color to return after releasing the pressure. The nurse is assessing this client's:

- | | |
|----|-------------------|
| 1. | skin changes. |
| 2. | capillary refill. |
| 3. | skin turgor. |
| 4. | peripheral edema. |

ANS: 2

Capillary refill is assessed by applying pressure to the fingernail and then quickly releasing the pressure and watching for the return of color. Skin changes, skin turgor, and peripheral edema are all assessments of the extremities; however, do not use the application of pressure to a fingernail to assess.

PTS: 1 DIF: Apply REF: Assessment

11. A client is having a diagnostic test that will evaluate the heart's structure and function using an ultrasound. The client is most likely having a(n):

- | | |
|----|-------------------------------|
| 1. | exercise electrocardiography. |
| 2. | electrocardiogram. |
| 3. | echocardiogram. |
| 4. | chest x-ray. |

ANS: 3

An echocardiogram is the evaluation of the heart's structure and function with images and recordings using ultrasound. Exercise electrocardiography is the use of exercise while assessing

a client's 12-lead electrocardiogram. A 12-lead electrocardiogram is a standardized recording of the electrical activity of the heart. A chest x-ray provides information on the size of the heart and pulmonary circulation, lung disease, and abnormalities of the aorta.

PTS:1DIF:AnalyzeREF:Diagnostic Tests

12. A client is scheduled for a cardiac catheterization. The nurse realizes that the indications for this diagnostic test would be:

-
- | | |
|-------|-----------------------------------|
| 1. | hypertension. |
| <hr/> | |
| 2. | peripheral edema. |
| <hr/> | |
| 3. | cerebral vascular accident. |
| <hr/> | |
| 4. | diagnose coronary artery disease. |

ANS: 4

Clinical implications for cardiac catheterization are to diagnose coronary artery disease and assess for atherosclerotic lesions. Hypertension, peripheral edema, and cerebral vascular accident are not indications for a cardiac catheterization.

PTS:1DIF:AnalyzeREF:Diagnostic Tests

13. Which of the following should the nurse instruct a client who is scheduled for a stress test?

-
- | | |
|-------|---|
| 1. | Eat nothing before the test. |
| <hr/> | |
| 2. | Expect to feel chest pain. |
| <hr/> | |
| 3. | The test will take between 20 and 50 minutes. |
| <hr/> | |
| 4. | You will be videotaped performing the test. |

ANS: 3

The nurse should instruct the client that the test will take between 20 to 50 minutes to complete. The client may be permitted to eat a light meal before the test. Eating nothing before the test is not standard. Chest pain during the test should be reported. The client will not be videotaped performing the test. The client will be monitored during the test.

PTS:1DIF:Apply

REF: Patient Playbook: Preparing the Patient for a Stress Test

MULTIPLE RESPONSE

1.The nurse is administering an anticoagulant to a client. Which of the following medications are anticoagulants? (Select all that apply.)

1.	Acetylsalicylic acid
2.	Clopidogrel
3.	Dalteparin
4.	Reteplase
5.	Tirofiban
6.	Warfarin

ANS: 3, 6

TESTBANKSELLER.COM

Warfarin (Coumadin) and dalteparin (Fragmin) are anticoagulants. Acetylsalicylic acid (aspirin), clopidogrel (Plavix), and tirofiban (Ticlid) are antiplatelet agents. Reteplase (Retavase) is a thrombolytic.

PTS: 1 DIF: Analyze REF: Table 23-2 Medications That Alter Coagulation

2.During the assessment of a clients thoracic region, the nurse determines that the client has a chest abnormality. Which of the following are common chest abnormalities? (Select all that apply.)

1.	Barrel chest
2.	Jugular vein distention
3.	Pectus excavatum
4.	Retracting rib cage

5.	Pectus carinatum
----	------------------

6.	Displaced trachea
----	-------------------

ANS: 1, 3, 5

Three common physical abnormalities of the chest are barrel chest; pectus excavatum, or funnel chest; and pectus carinatum, or pigeon chest. Jugular vein distention would be noted on the neck and not the chest. Retracting rib cage would occur with rib fractures and is considered a flail chest. Displaced trachea would be a medical emergency since this indicates a hemo- or pneumothorax.

PTS: 1 DIF: Analyze REF: Table 23-8 Physical Abnormalities of the Chest

3.The nurse is assessing a client for a heart murmur. Which of the following should be included in this assessment? (Select all that apply.)

1.	Blood pressure
----	----------------

2.	Heart rate
----	------------

3.	TESTBANKSELLER.COM Location
----	--------------------------------

4.	Radiation
----	-----------

5.	Timing
----	--------

6.	Intensity
----	-----------

ANS: 3, 4, 5, 6

When assessing heart murmurs, the following are evaluated: location, radiation, timing, and intensity. Blood pressure and heart rate are not assessed during the assessment of a heart murmur.

PTS: 1 DIF: Apply REF: Box 23-5 Assessment of Heart Murmur

4.The nurse is evaluating a client who is experiencing chest pain. Which of the following should be included in this assessment? (Select all that apply.)

1.	Grade the pain on a scale of 1 to 10.
----	---------------------------------------

-
2. Precipitating factors.

 3. Assess for sleep disturbance.

 4. Determine what was used to eliminate the pain.

 5. Assess for sweating.

 6. Determine if the pain is related to an injury or surgery.

ANS: 1, 2, 4, 5, 6

When evaluating a client's chest pain, the following should be assessed: pain intensity with a pain scale; duration; characteristics to include precipitating factors, onset, pattern of disappearance; recurrence and time of day; what is used to treat the pain; other symptoms such as shortness of breath or sweating; and if the pain is related to an injury or surgery. Assessment for sleep disturbance is not a part of the chest pain assessment.

PTS: 1 DIF: Apply REF: Box 23-1 Chest Pain Evaluation

5. The nurse is assessing a client's precordium. Which of the following should be included in this assessment? (Select all that apply.)

-
1. Palpate the precordium.

 2. Palpate temperature and pulses of extremities.

 3. Assess blood pressure.

 4. Percuss the precordium.

 5. Auscultate heart sounds.

 6. Identify the location and timing of heart sounds.

ANS: 1, 2, 4, 5, 6

When assessing a client's precordium, the following should be included: palpation of the precordium, palpation of extremities for temperature and pulses, percussion of the precordium,

auscultation of heart sounds, and location and timing of heart sounds. Blood pressure is not a part of the precordium assessment.

Chapter 12 Nursing Management: Patients with Hypertension

MULTIPLE CHOICE

1. Which of the following should the nurse instruct a client who is newly diagnosed with hypertension?

-
1. It is a lifelong process.

 2. It can be managed easily.

 3. It is a short-term problem.

 4. It happens only in the very poor and treatment is expensive.

ANS: 1

TESTBANKSELLER.COM

Treatment of hypertension is a lifelong process. It requires lifestyle modification and occurs in all racial and economical groups. Hypertension can either be easy or difficult to manage.

PTS: 1 DIF: Apply REF: Introduction

2. A client is diagnosed with isolated systolic hypertension. The nurse realizes that this diagnosis means the client is experiencing a systolic pressure:

-
1. greater than 140 mmHg and a diastolic pressure greater than 90 mmHg.

 2. greater than 90 mmHg and a diastolic pressure greater than 60 mmHg.

 3. greater than 140 mmHg and a diastolic pressure lower than 90 mmHg.

 4. lower than 140 mmHg and a diastolic pressure greater than 90 mmHg.

ANS: 3

The likelihood of developing isolated systolic hypertension is greater with age and is confirmed with a systolic pressure greater than 140 mmHg while the diastolic pressure remains less than 90 mmHg.

PTS: 1 DIF: Analyze REF: Hypertension: Nonmodifiable Risk Factors

3.The nurse is instructing a client on the impact of cigarette smoking and the development of hypertension. Which of the following would *not* be appropriate for the nurse to include in these instructions?

1. Tobacco damages the lining of the artery walls.
2. Tobacco temporarily constricts blood vessels, increasing pulse and blood pressure.
3. Tobacco thins the blood and makes the person at risk for bleeding.
4. Carbon monoxide in tobacco smoke replaces the oxygen in the blood, forcing the heart to work harder to supply oxygen.

ANS: 3

Tobacco and smoking have been shown to increase heart rate and blood pressure because of vasoconstriction and the accumulation of plaque on the artery walls. Because of the replacement of oxygen with carbon monoxide from tobacco smoke, the heart has to work harder to supply oxygen to the organs. There is no evidence that smoking thins the blood and causes bleeding.

PTS: 1 DIF: Apply REF: Hypertension: Modifiable Risk Factors

4.The nurse is assessing a clients pulse pressure. His blood pressure reading is 130/82 mmHg. Which of the following is the correct pulse pressure?

- | | |
|----|-----|
| 1. | 40 |
| 2. | 48 |
| 3. | 130 |
| 4. | 82 |

ANS: 2

The pulse pressure is the difference between the systolic and diastolic pressure: $130 - 82 = 48$. The other choices represent miscalculations or not understanding the correct way to calculate pulse pressure.

PTS: 1 DIF: Apply REF: Hypertension: Pathophysiology

5. A client is surprised to learn that she has high blood pressure. Which of the following should the nurse assess in this client? The presence or occurrence of:

- | | |
|----|-----------|
| 1. | nausea. |
| 2. | pain. |
| 3. | headache. |
| 4. | fear. |

ANS: 3

With very elevated blood pressure, headache is the most commonly reported symptom. Although pain and nausea may be reported, they are not the most common. Fear is not commonly associated with hypertension though it may occur with an onset of pain or nausea.

PTS: 1 DIF: Apply

REF: Hypertension: Assessment with Clinical Manifestations

6. A client's blood pressure has been measured at 130/86 mmHg on two separate occasions. The nurse realizes this client's blood pressure reading would be categorized as being:

- | | |
|----|-----------------------|
| 1. | normal. |
| 2. | prehypertension. |
| 3. | stage 1 hypertension. |
| 4. | stage 2 hypertension. |

ANS: 2

Prehypertension is a new designation used to identify individuals at high risk for the development of hypertension. Systolic blood pressure of 120 to 139 and diastolic blood pressure of 80 to 90 are values for prehypertension. A normal blood pressure is less than or equal to 120

mmHg systolic and less than or equal to 80 mmHg diastolic. Stage 1 hypertension is a systolic blood pressure between 140 to 159 and a diastolic pressure between 90 to 99. Stage 2 hypertension is a systolic reading greater than or equal to 160 and a diastolic pressure of greater than or equal to 100 mmHg.

PTS:1DIF:Analyze

REF:Table 28-6 JNC VII Classification of Blood Pressure in Adults

7.The nurse uses a blood pressure cuff that is too small for the circumference of the clients arm. How will this size of blood pressure cuff affect the clients blood pressure measurement?

1.	Falsely low
2.	Falsely high
3.	Not clearly heard
4.	More time consuming

ANS: 2

TESTBANKSELLER.COM

The blood pressure cuff must be the appropriate size to get an accurate reading. A cuff that is too small could result in a falsely high reading. A blood pressure cuff that is too large could result in a falsely low reading. The cuff size may not affect the nurses ability to hear the blood pressure sounds. An incorrect blood pressure cuff size will not be more time consuming to use.

PTS:1DIF:Analyze

REF:Table 28-2 Factors Causing False Blood Pressure Readings

8.A client diagnosed with hypertension should be instructed by the nurse to avoid which of the following foods?

1.	Cold cuts
2.	Bananas
3.	Milk
4.	Oatmeal

ANS: 1

Cold cuts are processed meats that are usually high in sodium and may cause water retention and an increase in blood pressure. The rest of the foods really have no effect on blood pressure.

PTS:1DIF:Apply

REF: Hypertension: Planning and Implementation: Evidence-Based Care

9.A client is instructed to reduce his intake of daily sodium intake so that the total amount is what his body needs. The nurse should instruct the client to reduce sodium intake to:

- | | |
|----|----------------|
| 1. | 500 mg a day. |
| 2. | 1000 mg a day. |
| 3. | 2500 mg a day. |
| 4. | 4500 mg a day. |

ANS: 1

A human body needs about 500 mg of sodium each day. The average intake of sodium for individuals in the United States is between 4000 to 6000 mg a day.

PTS:1DIF:Apply

REF: Hypertension: Planning and Implementation: Evidence-Based Care

10.A client asks the nurse why she should be concerned about the amount of sodium in ice cream. Which of the following should the nurse respond to this client?

- | | |
|----|---|
| 1. | Sodium is used to enhance the flavor. |
| 2. | Sodium is used to emulsify the ice cream. |
| 3. | Sodium is used to prevent mold. |
| 4. | Sodium is used as a preservative. |

ANS: 2

Sodium is used in ice cream as an emulsifier. Sodium in canned or processed foods is used to enhance flavor. Sodium is used to prevent mold in cheese, breads, and cakes. Sodium is used as a preservative in cured meats and sausages.

PTS: 1 DIF: Apply REF: Table 28-6 Sodium-Based Food Additives

11. Which of the following should the nurse instruct a client who desires to reduce his blood pressure through increasing physical activity?

1. Regular exercise can lower the blood pressure by 5 to 10 mmHg.
2. Regular exercise must be done 7 days a week.
3. Regular exercise has to be done for at least 2 hours each day.
4. Regular exercise is the participation in aerobic activities.

ANS: 1

Regular exercise can lower blood pressure by 5 to 10 mmHg. Regular exercise should be done 5 days a week for 60 minutes or 20 minutes of vigorous exercise at least 3 times a week to be effective. Regular exercise includes aerobic activity, flexibility, and strengthening exercises.

PTS: 1 DIF: Apply

REF: Hypertension: Planning and Implementation: Evidence-Based Care

12. A client is prescribed Spironolactone (Aldactone) for blood pressure control. Which of the following should the nurse assess in this client as a potential side effect?

1. Hypokalemia
2. Hyperkalemia
3. Hyponatremia
4. Hypernatremia

ANS: 2

Spironolactone (Aldactone) is a potassium-sparing diuretic. Side effects include hyperkalemia. Hypokalemia and hyponatremia are side effects of the thiazide diuretics. Hypernatremia is not a known side effect of any antihypertensive medication.

PTS: 1 DIF: Apply

REF: Table 28-9 Pharmacologic Management of Hypertension

13. A client is prescribed an ACE inhibitor for management of hypertension. Which of the following side effects should the nurse instruct the client as being expected with this medication?

1.	Tachycardia
2.	Constipation
3.	Bizarre dreams
4.	Persistent dry cough

ANS: 4

One side effect of ACE inhibitors that is expected with this medication is a persistent dry cough. Tachycardia, constipation, and bizarre dreams are not side effects associated with ACE inhibitors.

PTS:1 DIF:Apply

REF: Table 28-9 Pharmacologic Management of Hypertension

MULTIPLE RESPONSE TESTBANKSELLER.COM

1. The nurse is considering the risk factors for a client's development of primary hypertension. Which of the following would be considered nonmodifiable risk factors for the client? (Select all that apply.)

1.	Age
2.	Stress
3.	Gender
4.	Ethnicity
5.	Regular exercise
6.	Limits fat and salt in diet

ANS: 1, 3, 4

Nonmodifiable risk factors are those thing we cannot change or control, such as age, gender, and ethnicity. Stress, exercise, and diet are considered modifiable risk factors or those the client can control.

PTS: 1 DIF: Analyze REF: Hypertension: Risk Factors

2. Which of the following should the nurse tell a client when instructing on ways to reduce the risk factors for hypertension? (Select all that apply.)

1.	Smoking
2.	Diet
3.	Exercise
4.	Family history
5.	Race
6.	Stress

ANS: 1, 2, 3, 6

Modifiable risk factors can be changed or modified to help control hypertension. Smoking, diet, stress, and exercise can be changed to affect blood pressure. Persons with more risk factors have a greater chance of having hypertension during their lives. Family history and race cannot be modified.

PTS: 1 DIF: Apply REF: Hypertension: Risk Factors

3. Which of the following assessment questions would be appropriate for the nurse to use when assessing a client for hypertension? (Select all that apply.)

1.	Do you consume alcohol products? How much? How long?
2.	Do you use nicotine products? How much? How long?
3.	Do you experience nosebleeds?
4.	Do you get hungry at night?

5. Do you experience cold sweats?

6. Do you experience headaches?

ANS: 1, 2, 3, 6

The nurse will often ask the client questions about risks of hypertension. Asking about alcohol and nicotine product use will tell you about increased risk factors. Nosebleeds and headaches are often associated with hypertension. Although cold sweats and hunger are symptoms a patient may report, they are not indicative of hypertension.

PTS: 1 DIF: Apply REF: Box 28-2 Hypertension Assessment

4. The blood pressure measurement for a client is very different from the one that was assessed a few hours previously. The nurse should suspect that the blood pressure measurement is false when which of the following is assessed in the client?

1. Client needs to void.

2. Client smoked a cigarette 10 minutes prior to the measurement.

3. The examination room is very warm.

4. Doors are slamming and children are crying in the environment.

5. Client just had lunch.

6. Client slept for 8 hours the previous night.

ANS: 1, 2, 3, 4

Factors that cause false blood pressure readings include anxiety, full urinary bladder, excessively warm room, recent tobacco use, and loud or repetitive noises. Eating a meal or having 8 hours of sleep are not known to cause a false blood pressure reading.

PTS: 1 DIF: Analyze

REF: Table 28-2 Factors Causing False Blood Pressure Readings

5. A client is planning to use nicotine gum to aid with cigarette cessation. Which of the following should the nurse instruct the client as adverse effects of using nicotine gum? (Select all that apply.)

-
- | | |
|----|------------------------------|
| 1. | Rapid heart rate may result. |
|----|------------------------------|
-
- | | |
|----|---------------------------|
| 2. | Mild headaches can occur. |
|----|---------------------------|
-
- | | |
|----|---------------------------------------|
| 3. | A sore mouth and throat are possible. |
|----|---------------------------------------|
-
- | | |
|----|-----------------------------|
| 4. | Abnormal dreams are common. |
|----|-----------------------------|
-
- | | |
|----|-----------------------|
| 5. | Pruritis is possible. |
|----|-----------------------|
-
- | | |
|----|-------------------|
| 6. | Nausea can occur. |
|----|-------------------|
-

ANS: 1, 2, 3, 6

Adverse effects associated with the use of nicotine chewing gum include tachycardia, mild headache, sore mouth and throat, and nausea. Abnormal dreams and pruritis are adverse effects of nicotine patches, nicotine nasal spray, and nicotine inhalers.

TESTBANKSELLER.COM

Chapter 13 Nursing Management: Patients with Coronary Vascular Disorders

MULTIPLE CHOICE

1. A client is learning about cholesterol. The nurse explains that the good cholesterol transports plasma cholesterol away from plaques and to the liver for metabolism. This type of cholesterol is called:

-
- | | |
|----|---------------------------|
| 1. | high-density lipoprotein. |
|----|---------------------------|
-
- | | |
|----|--------------------------|
| 2. | low-density lipoprotein. |
|----|--------------------------|
-
- | | |
|----|--------------------------------|
| 3. | very-high-density lipoprotein. |
|----|--------------------------------|
-
- | | |
|----|-------------------------------|
| 4. | very-low-density lipoprotein. |
|----|-------------------------------|
-

ANS: 1

High-density lipoprotein transports plasma cholesterol away from atherosclerotic plaques and to the liver for metabolism and excretion. Low-density lipoproteins, or bad cholesterol, are the main component of the atherosclerotic plaque. Very-low-density lipoproteins are considered more atherogenic and are more common in men and people with diabetes.

PTS:1DIF:ApplyREF:Hyperlipidemia

2.A client has a blood pressure of 124/78 mmHg and a triglyceride level of 160 mg/dL. Based on these results, the nurse knows that the client has:

1. an optimal blood pressure and triglyceride level.
2. a prehypertensive blood pressure and an optimal triglyceride level.
3. a prehypertensive blood pressure and a borderline high triglyceride level.
4. stage I hypertension and a high triglyceride level.

ANS: 3

Prehypertensive blood pressure ranges systolically from 120 to 139 mmHg or diastolically from 80 to 90 mmHg. Stage I hypertension is systolic blood pressure (SBP) of 140 to 159 mmHg or a diastolic blood pressure (DBP) of 90 to 99 mmHg. Optimal triglyceride levels are less than 150 mg/dL. Triglyceride levels from 150 to 199 mg/dL are considered borderline high. Triglyceride levels at 200 to 499 mg/dL are considered high.

PTS:1DIF:Analyze

REF: Table 24-1 Classification of Lipid Levels; Table 24-2 Blood Pressure Classification

3.The nurse measures a clients blood pressure to be 158/92 mmHg. The nurse recognizes that this blood pressure is classified as:

1. normal.
2. prehypertension.
3. stage I hypertension.
4. stage II hypertension.

ANS: 3

Normal blood pressure is SBP less than 120 mmHg and DBP less than 80 mmHg. A prehypertensive state is SBP of 120 to 139 mmHg or DBP of 80 to 90 mmHg. Stage I hypertension is SBP of 140 to 159 mmHg or DBP of 90 to 99 mmHg. Stage II hypertension is a SBP of 160 mmHg or higher or a DBP of 100 mmHg or higher.

PTS: 1 DIF: Analyze REF: Table 24-2 Blood Pressure Classification

4.A client is complaining of chest pain that occurs during exercise. This pain is relieved when the client rests. The nurse realizes that this client is experiencing which type of angina?

- | | |
|----|----------------------------|
| 1. | Prinzmetals variant angina |
| 2. | Silent angina |
| 3. | Stable angina |
| 4. | Unstable angina |

ANS: 3

Stable angina is precipitated by factors that increase oxygen demand or reduce oxygen supply. Chest pain occurs predictably with the same onset, duration, and intensity and is relieved when the precipitating factor is removed or with nitroglycerin. Unstable angina is typified by an increase in frequency, duration, and intensity of symptoms at lower levels of activity and even at rest. Prinzmetals variant angina is a coronary artery spasm. Silent angina can occur with no pain at all and is common in diabetic patients.

PTS:1DIF:AnalyzeREF:Types of Angina

5.A client diagnosed with stable angina is undergoing a 12-lead electrocardiogram. Which of the following results is not expected?

- | | |
|----|-----------------------|
| 1. | ST segment depression |
| 2. | ST segment elevation |
| 3. | T-wave flattening |
| 4. | T-wave inversion |

ANS: 2

During an episode of angina, T-wave flattening or inversions and ST segment depression may be seen on the electrocardiogram due to subendocardial ischemia. ST segment elevation is seen with impending or acute myocardial infarction.

PTS: 1 DIF: Analyze REF: Diagnostic Tests: Electrocardiogram

6.A client is scheduled for a cardiac angiogram. Which of the following should the nurse instruct the client about this diagnostic test?

-
1. It is noninvasive.
 2. Contrast dye is injected.
 3. Clients can move about after the procedure.
 4. General anesthesia is used.
-

ANS: 2

A cardiac angiogram is a procedure that visualizes the structures of the heart and vessels. This is an invasive procedure; however, it does not need general anesthesia. The client is awake during the procedure. A contrast dye is injected, and the client may feel a warm sensation. The client must maintain bed rest with the leg straight for up to 4 to 6 hours after the catheter is removed.

PTS: 1 DIF: Apply REF: Diagnostic Tests: Coronary Angiography

7.When planning the care of a client diagnosed with stable angina, which of the following would be considered a goal of treatment?

-
1. Decrease in ischemia and episodes of angina
 2. Prevent myocardial infection
 3. Reduction of risk factors
 4. Reduction of stress by education
-

ANS: 1

The primary goal for the treatment of stable angina is to improve the quality of life by decreasing episodes of angina and ischemia. The second goal is to increase the quantity of life by preventing

progression to myocardial infarction and death. Reduction of risk factors and education are both parts of a treatment plan.

PTS: 1 DIF: Apply REF: Planning and Implementation: Goals

8.A client is prescribed a beta-blocker for treatment of coronary artery disease. Which of the following is the client most likely going to be prescribed?

- | | |
|----|-------------------------|
| 1. | Amlodipine |
| 2. | Atenolol |
| 3. | Diltiazem hydrochloride |
| 4. | Nicardipine |

ANS: 2

Amlodipine, diltiazem hydrochloride, and nicardipine are all calcium channel blockers. Atenolol is a beta-blocker.

PTS:1DIF:Analyze

TESTBANKSELLER.COM

REF:Table 24-4 Common Medications for the Treatment of CAD

9.A client tells the nurse that using nitroglycerin tablets causes a tingling sensation and a headache. The nurse knows that this is:

- | | |
|----|-----------------------|
| 1. | an emergency. |
| 2. | an allergic reaction. |
| 3. | evidence of toxicity. |
| 4. | expected. |

ANS: 4

Nitroglycerin tablets will cause a tingling sensation and can cause feelings of the heart pounding, as well as flushing and headache. These symptoms are not an emergency, an allergic reaction, or evidence of toxicity. These symptoms are expected with nitroglycerin tablets.

PTS:1DIF:AnalyzeREFPharmacology

10. A nurse is considering contraindications to fibrinolytic therapy. Which of the following patients is an appropriate candidate for fibrinolytic therapy?

1. A patient with a peptic ulcer disease
2. A patient with a history of hemorrhagic stroke
3. A patient with a history of a motor vehicle accident 1 year ago
4. A patient with inflammatory bowel disease

ANS: 3

Contraindications to fibrinolytic therapy include active internal bleeding, active inflammatory bowel disease, active peptic ulcer disease, active pericarditis, defective homeostasis, gastrointestinal/genitourinary bleeding for less than 6 months, history of hemorrhagic stroke, known bleeding disorders, neurologic procedure within the past 2 months, recent surgery or trauma within 2 months, pregnancy, suspected aortic dissection, and uncontrolled hypertension.

PTS: 1 DIF: Analyze REF: Acute Coronary Syndrome: Pharmacology

11. A client is participating in cardiac rehabilitation and is currently engaging in supervised exercise, counseling, and education. The nurse realizes this client is in which phase of cardiac rehabilitation?

1. Phase I
2. Phase II
3. Phase III
4. Phase IV

ANS: 3

Phase I of cardiac rehabilitation begins in the hospital. Phase II of cardiac rehabilitation is the transitional phase and centers around recovery at home with increasing activity. Phase II of cardiac rehabilitation occurs in an outpatient rehabilitation facility, and it focuses on supervised exercise, counseling, and education. Phase IV of cardiac rehabilitation is the maintenance phase and focuses on long-term changes.

PTS: 1 DIF: Analyze REF: Patient and Family Teaching

12.A client tells the nurse that he ingests an NSAID when the angina pain gets really bad, and it eliminates the pain. The nurse suspects the client is experiencing:

1. musculoskeletal pain.
2. aortic dissection.
3. mitral valve prolapse.
4. pericarditis.

ANS: 1

Musculoskeletal pain is relieved with NSAIDs. The pain of aortic dissection and pericarditis would not be relieved with NSAIDs. Mitral valve prolapse may or may not have associated chest discomfort.

PTS:1DIF:Analyze

REF: Diagnostic Tests: Differential Diagnosis for Angina

13.A client is prescribed nicotinic acid as part of treatment for coronary artery disease. Which of the following should the nurse instruct the client regarding this medication?

1. Ingest an aspirin 30 minutes before taking the medication and after eating.
2. Expect a gritty taste.
3. Anticipate constipation.
4. Expect fatigue with this medication.

ANS: 1

Instructions to the client prescribed nicotinic acid include ingesting an aspirin 30 minutes to 1 hour before the medication and after food. A gritty taste is not associated with this medication. Constipation is not an expected gastrointestinal side effect of this medication. This medication does not cause fatigue.

PTS:1DIF:Apply

REF:Table 24-4 Common Medications for the Treatment of CAD

Chapter 14 Nursing Management: Patients with Complications from Heart Disease**MULTIPLE CHOICE**

1. The nurse is assessing the pain of a client experiencing angina. Which of the following should be included in this assessment? (Select all that apply.)

1.	Precipitating event
2.	Quality
3.	Radiation
4.	Severity
5.	Timing
6.	Medication

ANS: 1, 2, 3, 4, 5

The memory aid PQRST can be used to assess a client experiencing symptoms of angina, and it includes precipitating event, quality, radiation, severity, and timing. Medication is not a part of this assessment.

PTS:1DIF:ApplyREF:Table 24-3 PQRST

2. A client is at risk for coronary artery disease. Which of the following should the nurse instruct as modifiable risk factors for this health condition? (Select all that apply.)

1.	Alcohol consumption
2.	Diabetes mellitus
3.	Family history
4.	Gender

5. Low daily fruit intake

6. Psychosocial index

ANS: 1, 2, 5, 6

Nonmodifiable risk factors are age, gender, and family history. Modifiable risk factors include hyperlipidemia, hypertension, tobacco abuse, diabetes mellitus, abdominal obesity, lack of physical activity, low daily fruit and vegetable intake, alcohol consumption, and psychosocial index.

PTS:1DIF:Apply

REF:Etiology: Nonmodifiable Risk Factors; Modifiable Risk Factors

3.A client is diagnosed with angina after describing the type of pain she experiences. Which of the following are characteristics of anginal pain? (Select all that apply.)

1. Pressure

2. Heavy

TESTBANKSELLER.COM

3. Squeezing

4. Stabbing

5. Sharp

6. Demonstrates a clenched fist over the sternum

ANS: 1, 2, 3, 6

Angina pain is typically described as pressure, heavy, squeezing, and it is demonstrated by placing a clenched fist over the sternum. This hand posture is referred to as Levines sign which is the universal sign for angina. Angina pain is not stabbing or sharp.

PTS: 1 DIF: Analyze REF: Assessment with Clinical Manifestations

4.A client is experiencing a sudden onset of chest pain. Which of the following will the nurse do to manage this chest pain?

-
1. Administer intravenous morphine as prescribed.

 2. Provide oxygen.

 3. Insert an indwelling urinary catheter.

 4. Position the client on the left side.

 5. Administer nitroglycerin as prescribed.

 6. Administer aspirin as prescribed.

ANS: 1, 2, 5, 6

The emergency management of chest pain follows the memory aid MONA; that is, morphine, oxygen, nitroglycerin, and aspirin. An indwelling urinary catheter and positioning the client on the left side are not interventions for the emergency management of chest pain.

PTS: 1 DIF: Apply REF: Red Flag: Emergency Management of Chest Pain

5. Which of the following will the nurse instruct a client being discharged to home after experiencing an acute myocardial infarction? (Select all that apply.)

-
1. Understand cardiac condition

 2. How to manage chest pain

 3. Activity level

 4. Medications

 5. Risk factors

 6. Immunizations

ANS: 1, 2, 3, 4, 5

Discharge instructions for a client being discharged after experiencing an acute myocardial infarction include understanding cardiac condition, chest pain management, activity, medications, risk factors, diet, and signs and symptoms to report to the physician. Immunizations are not a part of discharge instructions after an acute myocardial infarction.

6.The nurse suspects a client is experiencing left-sided heart failure when which of the following is assessed? (Select all that apply.)

- | | |
|----|-------------------------------|
| 1. | Decreased basilar lung sounds |
| 2. | Distended neck veins |
| 3. | Extra heart sounds |
| 4. | Lung crackles |
| 5. | Tachycardia |
| 6. | Weight gain |

TESTBANKSELLER.COM

ANS: 1, 3, 4, 5

Signs of left-sided heart failure are dysrhythmic heart rate, tachycardia, heart murmurs, extra heart sounds, lung crackles, and decreased basilar lung sounds. Distended neck veins and weight gain are symptoms of right-sided heart failure.

PTS: 1 DIF: Analyze REF: Box 25-2 Heart Failure Clinical Manifestations

7.A client diagnosed with heart failure is prescribed furosemide (Lasix). Which of the following should this client be monitored for because of this medication? (Select all that apply.)

- | | |
|----|-------------------------------|
| 1. | Dehydration |
| 2. | Rebound fluid volume overload |
| 3. | Hyponatremia |
| 4. | Hypokalemia |

5. Hyponatremia

6. Hyperkalemia

ANS: 1, 3, 4

Any client prescribed diuretics should be monitored for dehydration, hyponatremia, and hypokalemia. Rebound fluid volume overload is not possible with diuretic therapy. Hyponatremia and hyperkalemia are also not possible with diuretic therapy.

PTS: 1 DIF: Apply REF: Heart Failure: Pharmacology

8.The nurse is reviewing the medications prescribed for a client diagnosed with dilated cardiomyopathy. Which of the following medications are commonly prescribed for this disease process? (Select all that apply.)

1. ACE Inhibitor

2. Beta-blocker

3. Diuretic

4. Anticoagulant

5. Antiarrhythmic

6. Antibiotic

ANS: 1, 2, 3, 4, 5

Pharmacological management of dilated cardiomyopathy includes ACE inhibitor to prevent further dilation of the heart, beta-blocker to reduce the strain that heart failure produces on the heart muscle, diuretics to decrease the amount of circulating fluid, anticoagulants to decrease blood clots, and antiarrhythmics to maintain the normal electrical stimulation of the heart. Antibiotics are not routinely prescribed for a client diagnosed with dilated cardiomyopathy.

PTS: 1 DIF: Analyze REF: Dilated Cardiomyopathy: Pharmacology

9.Which of the following should the nurse instruct a client diagnosed with hypertrophic cardiomyopathy? (Select all that apply.)

-
- | | |
|----|-----------------------------------|
| 1. | Follow recommended activity level |
|----|-----------------------------------|
-
- | | |
|----|-------------------|
| 2. | Avoid all alcohol |
|----|-------------------|
-
- | | |
|----|------------------------------|
| 3. | Take hot tub baths routinely |
|----|------------------------------|
-
- | | |
|----|--------------------|
| 4. | Avoid overexertion |
|----|--------------------|
-
- | | |
|----|-------------------|
| 5. | Avoid dehydration |
|----|-------------------|
-
- | | |
|----|--|
| 6. | Unexplained breathlessness is a common symptom |
|----|--|
-

ANS: 1, 4, 5

The nurse should instruct the client diagnosed with hypertrophic cardiomyopathy to follow the recommended activity level, avoid overexertion, and avoid dehydration. The client should be instructed to use alcohol in moderation, to avoid hot tub baths or showers, and to report unexplained breathlessness to a health care provider.

PTS:1DIF:Apply

TESTBANKSELLER.COM

REF:Hypertrophic Cardiomyopathy: Planning and Implementation

10.The nurse determines that a client diagnosed with pericarditis is demonstrating the classic signs of the Beck triad. What are the signs of the Beck triad? (Select all that apply.)

-
- | | |
|----|-------|
| 1. | Fever |
|----|-------|
-
- | | |
|----|---------|
| 2. | Dyspnea |
|----|---------|
-
- | | |
|----|----------------------|
| 3. | Muffled heart sounds |
|----|----------------------|
-
- | | |
|----|--------------------------------|
| 4. | Elevated jugular vein pressure |
|----|--------------------------------|
-
- | | |
|----|-------------|
| 5. | Hypotension |
|----|-------------|
-
- | | |
|----|----------------|
| 6. | Abdominal pain |
|----|----------------|
-

ANS: 3, 4, 5

The symptoms of Beck triad include muffled heart sounds, elevated jugular vein pressure, and hypotension. Fever, dyspnea, and abdominal pain are not considered findings within the Beck triad.

TESTBANKSELLER.COM

Chapter 15 Nursing Management: Patients with Structural, Inflammatory and Infectious Cardiac Disorders

MULTIPLE CHOICE

1. The nurse suspects a client's heart is failing when which of the following heart sounds is assessed?

1.	S ₁
2.	S ₂
3.	S ₃
4.	S ₄

ANS: 3

An auscultated S₃ is a sign that increased blood volume remains in the ventricle with each beat and that the heart is beginning to fail. S₁ and S₂ sounds are the first and second sounds heard when auscultating the heart. An S₄ sound may indicate increased resistance to ventricular filling.

PTS: 1 DIF: Analyze REF: Assessment with Clinical Manifestations

2.A client is diagnosed with heart failure. Which of the following diagnostic tests is useful to determine the degree of the failure?

- | | |
|----|---------------------------------|
| 1. | Brain natriuretic peptide level |
| 2. | Blood cultures |
| 3. | Sedimentation rate |
| 4. | Arterial blood gas |

ANS: 1

Brain natriuretic peptide is a hormone found in the left ventricle; it is used to help diagnose and grade the severity of heart failure. Blood cultures are used to diagnose carditis. Sedimentation rate is used to diagnose pericarditis. Arterial blood gasses are not used to determine the degree of heart failure.

PTS: 1 DIF: Analyze REF: Heart Failure: Diagnostic Tests

3.A nurse is instructing a client regarding medications and substances contraindicated for the client with heart failure. Which of the following would *not* be contraindicated?

- | | |
|----|--------------|
| 1. | Alcohol |
| 2. | Furosemide |
| 3. | Metformin |
| 4. | Pioglitazone |

ANS: 2

Loop diuretics (e.g., furosemide) are part of the recommended medications for heart failure. Alcohol, metformin, and pioglitazone (a thiazolidinedione) are contraindicated.

PTS:1DIF:Apply

REF: Table 25-1 Recommended and Contraindicated Medications in Heart Failure

4.The nurse is determining nursing diagnoses appropriate for a client demonstrating productive cough with pink frothy sputum, shortness of breath, and crackles. Which of the following nursing diagnoses is of the most importance?

- | | |
|----|---|
| 1. | Activity intolerance |
| 2. | Anxiety |
| 3. | Impaired gas exchange |
| 4. | Risk for ineffective respiratory function |

ANS: 3

The first priority is to maintain adequate oxygenation. The next diagnoses in priority would be risk for ineffective respiratory function. Activity intolerance would be the third diagnosis. Anxiety would be the last diagnosis in order of priority.

PTS: 1 DIF: Apply REF: Heart Failure: Nursing Diagnoses

TESTBANKSELLER.COM

5.In planning the care for a client diagnosed with heart failure, which of the following would be an appropriate goal?

- | | |
|----|----------------------------------|
| 1. | Reduce myocardial contractility. |
| 2. | Increase cardiac workload. |
| 3. | Decrease ejection fraction. |
| 4. | Increase activity levels. |

ANS: 4

An increase in activity levels would be an appropriate goal for the client diagnosed with heart failure. The other options would be a decrease in ability, function, or management of the heart failure patient.

PTS: 1 DIF: Analyze REF: Heart Failure: Collaborative Management

6.The nurse is instructing a client diagnosed with mild heart failure on dietary modifications. Which of the following client statements indicates that the instruction has been effective?

-
- | | |
|----|---------------------------|
| 1. | I will avoid green beans. |
|----|---------------------------|
-
- | | |
|----|----------------------------|
| 2. | I will avoid orange juice. |
|----|----------------------------|
-
- | | |
|----|-------------------------|
| 3. | I will avoid soy sauce. |
|----|-------------------------|
-
- | | |
|----|---------------------------|
| 4. | I will avoid apple sauce. |
|----|---------------------------|

ANS: 3

Soy sauce is a high-sodium food choice; all the other choices are low sodium. Treatment for mild symptoms of heart failure includes dietary restriction of salt.

PTS: 1 DIF: Analyze REF: Heart Failure: Planning and Implementation

7.A client is undergoing diagnostic testing for infective endocarditis. Which of the following laboratory tests would be most useful in diagnosis?

-
- | | |
|----|-----------------------|
| 1. | Basic metabolic panel |
|----|-----------------------|
-
- | | |
|----|----------------|
| 2. | Blood cultures |
|----|----------------|
-
- | | |
|----|--------------------|
| 3. | Reticulocyte count |
|----|--------------------|
-
- | | |
|----|------------------|
| 4. | Prothrombin time |
|----|------------------|

ANS: 2

Blood cultures identify the causative organisms. A basic metabolic panel gives the current status of the clients acid/base balance and electrolytes. The reticulocyte count determines bone marrow function and evaluates erythropoietic activity. The prothrombin time is useful in monitoring anticoagulant therapy.

PTS: 1 DIF: Analyze REF: Infective Endocarditis: Diagnostic Tests

8.Which of the following would the nurse most likely assess in a client diagnosed with right-sided heart failure?

-
- | | |
|----|----------------------|
| 1. | Distended neck veins |
|----|----------------------|

-
- | | |
|----|----------|
| 2. | Oliguria |
|----|----------|
-
- | | |
|----|---------------------------------------|
| 3. | Cough with frothy blood-tinged sputum |
|----|---------------------------------------|
-
- | | |
|----|---------|
| 4. | Syncope |
|----|---------|
-

ANS: 1

An assessment finding in a client diagnosed with right-sided heart failure is distended neck veins. Oliguria, cough with frothy blood-tinged sputum, and syncope are all clinical manifestations of left-sided heart failure.

PTS: 1 DIF: Apply REF: Box 25-2 Heart Failure Clinical Manifestations

9. Which of the following diagnostic tests is useful to diagnose mitral valve prolapse?

-
- | | |
|----|-------------------|
| 1. | Electrocardiogram |
|----|-------------------|
-
- | | |
|----|----------------|
| 2. | Echocardiogram |
|----|----------------|
-
- | | |
|----|---------------------|
| 3. | Cardiac angiography |
|----|---------------------|
-
- | | |
|----|----------------------------------|
| 4. | Transesophageal echocardiography |
|----|----------------------------------|
-

ANS: 4

Transesophageal echocardiography is useful in the assessment of cardiac murmurs, stenosis, and regurgitation of all four cardiac valves. An electrocardiogram, echocardiogram, and cardiac angiography may or may not be useful when diagnosing mitral valve prolapse.

PTS: 1 DIF: Analyze

REF: Mitral Valve Prolapse: Assessment with Clinical Manifestations

10. A client diagnosed with mitral valve prolapse is experiencing palpitations. Which of the following should the nurse instruct this client?

-
- | | |
|----|---------------|
| 1. | Avoid tobacco |
|----|---------------|
-
- | | |
|----|------------------------------|
| 2. | Ingest alcohol in moderation |
|----|------------------------------|
-

3. Avoid weight loss

4. Limit caffeine intake

ANS: 1

Clients with palpitations associated with mitral valve prolapse should be instructed to avoid caffeine, alcohol, and tobacco. Weight loss should be encouraged in overweight clients.

PTS:1DIF:Apply

REF: Mitral Valve Prolapse: Planning and Implementation

11.A client tells the nurse that she had rheumatic heart disease as a child. For which of the following valvular disorders should this client be assessed?

1. Mitral valve prolapse

2. Mitral stenosis

3. Aortic regurgitation

4. Aortic stenosis

ANS: 2

Mitral stenosis is most commonly caused by rheumatic heart disease. Rheumatic heart disease has not been linked to mitral valve prolapse, aortic regurgitation, or aortic stenosis.

PTS:1DIF:AnalyzeREF:Mitral Stenosis

12.A client, recovering from surgery to replace a calcified aortic valve with a mechanical valve, should be instructed that which of the following medications will be needed long term?

1. ACE inhibitor

2. Beta-blocker

3. Antibiotic

-
4. Anticoagulant

ANS: 4

The mechanical valve requires long-term anticoagulation therapy to prevent the risk of thromboembolism. ACE inhibitors, beta-blockers, and antibiotics are not indicated as long-term therapy for this surgery.

PTS:1DIF:ApplyREF:Valvular Surgery

13.A client is scheduled for annuloplasty surgery to the aortic valve. Which of the following will most likely occur during this clients procedure?

-
1. A catheter will be inserted through the femoral vein.
-
2. A heart bypass machine will be used.
-
3. Local anesthesia will be provided.
-
4. A balloon will inflate and stretch the valve open.

TESTBANKSELLER.COM

ANS: 2

For an annuloplasty, the client will receive general anesthesia and a heart bypass machine will be used. A balloon valvuloplasty is done by inserting a catheter through the femoral vein or artery and stretching the valve open with a balloon. The client needs general anesthesia for an annuloplasty and not a local anesthetic.

PTS:1DIF:ApplyREF:Valvular Surgery

Chapter 16 Nursing Management: Patients with Arrhythmias and Conduction Problems

MULTIPLE CHOICE

1.A client is experiencing an alteration in heart rate. The nurse realizes this client is experiencing a disorder of which part of the heart?

-
1. Atrioventricular node
-
2. Bundle branches

3.	Purkinje fibers
----	-----------------

4.	Sinoatrial node
----	-----------------

ANS: 4

The sinoatrial node is the dominant pacemaker of the heart. The sinoatrial node has an inherent rate of 60 to 100 bpm. The atrioventricular node has an intrinsic rate of 40 to 60 bpm. The impulse enters the right and left bundle branches and then enters the Purkinje fibers. Impulses at this level are at 15 to 40 times per minute.

PTS: 1 DIF: Analyze REF: Anatomy and Physiology

2.A client is suspected of having cardiac damage. The nurse realizes that which of the following diagnostic tests is most commonly used to help diagnose this clients possible cardiac damage or disease?

1.	12-lead electrocardiogram
----	---------------------------

2.	Arterial blood gases
----	----------------------

3.	Cardiac angiogram
----	-------------------

4.	Cardiac enzymes
----	-----------------

ANS: 1

A 12-lead electrocardiogram is a quick and accurate diagnostic tool used to evaluate heart damage and disease. The other diagnostic tests require a longer time for results and/or are invasive procedures requiring some preparation.

PTS:1DIF:AnalyzeREF:ECG Monitoring

3.The nurse is analyzing a clients electrocardiogram tracing. Which of the following complexes is not normally seen on an electrocardiogram tracing?

1.	P wave
----	--------

2.	QRS complex
----	-------------

3.	T wave
----	--------

-
4. U wave

ANS: 4

A U wave is not always seen and can be very small. It can indicate electrolyte imbalance, medication effects, and ischemia. The P wave, QRS complex, and T wave are normally seen in the electrocardiogram tracing.

PTS:1DIF:Analyze

REF: The Normal ECG Complex; Figure 26-1 Conduction System of the Heart

4.The nurse is analyzing a clients electrocardiogram tracing and realizes that each small square on the paper is equal to:

-
1. 0.04 second.

-
2. 0.12 second.

-
3. 0.20 second.

-
4. TESTBANKSELLER.COM
0.40 second.

ANS: 1

The small square on the ECG graph paper equals 0.04 second. The large square equals 0.20 second. The PR interval is 0.12 to 0.20 second. Two large squares would be equal to 0.40 second.

PTS: 1 DIF: Analyze REF: Calculating Heart Rate

5.The nurse is reading an ECG rhythm strip and notes that there are nine QRS complexes in a 6-second strip. The heart rate is:

-
1. 36.

-
2. 54.

-
3. 81.

-
4. 90.

ANS: 4

A heart rate can be determined by multiplying the QRS complexes in a 6-second strip by 10. The heart rate is 90. This method of calculating the heart rate is the most common method used because it is quick and can be used when the heart rate is irregular.

PTS: 1 DIF: Apply REF: Calculating Heart Rate

6.The nurse notes that on a clients electrocardiogram tracing, there is one P wave for every QRS complex and a delay in the impulse transmission at the AV node. This regular rhythm is identified as:

-
- | | |
|----|------------------------|
| 1. | first-degree AV block. |
|----|------------------------|
-
- | | |
|----|--------------------------------|
| 2. | second-degree AV block type I. |
|----|--------------------------------|
-
- | | |
|----|---------------------------------|
| 3. | second-degree AV block type II. |
|----|---------------------------------|
-
- | | |
|----|-----------------------|
| 4. | complete heart block. |
|----|-----------------------|
-

ANS: 1

TESTBANKSELLER.COM

First-degree atrioventricular (AV) block occurs when there is a delay in the impulse transmission at the AV node. This delay occurs with every impulse and can be seen on every beat on the recorded rhythm strip. Second-degree and complete heart block have differences with the P wave and the associated QRS complexes.

PTS: 1 DIF: Analyze REF: First-Degree Heart Block

7.A client is unresponsive and has no pulse. The nurse notes that the electrocardiogram tracing shows continuous large and bizarre QRS complexes measured greater than 0.12 each. This rhythm is identified as:

-
- | | |
|----|----------------------------------|
| 1. | premature ventricular complexes. |
|----|----------------------------------|
-
- | | |
|----|----------------------|
| 2. | torsades de pointes. |
|----|----------------------|
-
- | | |
|----|---------------------------|
| 3. | ventricular fibrillation. |
|----|---------------------------|
-
- | | |
|----|--------------------------|
| 4. | ventricular tachycardia. |
|----|--------------------------|
-

ANS: 4

Ventricular tachycardia occurs when the patient experiences sustained consecutive premature ventricular complexes. Torsades de pointes is characterized by a wide-to-narrow pattern of the QRS complexes. Ventricular fibrillation shows a coarse wavy baseline.

PTS: 1 DIF: Analyze REF: Ventricular Tachycardia

8. An elderly client is demonstrating a change in heart rate that occurs with respirations. When planning care for the client, the nurse knows that treatment may include:

- | | |
|----|---------------------|
| 1. | Oxygen therapy |
| 2. | Analgesics |
| 3. | Antibiotics |
| 4. | Pacemaker insertion |

ANS: 4

A change in heart rate that occurs with respirations defines a sinus arrhythmia. If the client becomes symptomatic during periods of bradycardia, treatment will include atropine sulfate or pacemaker insertion. Treatment for sinus arrhythmia might include oxygen if the client is symptomatic. Treatment for this arrhythmia does not include analgesics or antibiotics.

PTS: 1 DIF: Apply REF: Sinus Arrhythmia

9. A client's electrocardiogram tracing shows a sawtooth pattern with F waves. The nurse realizes this client is demonstrating:

- | | |
|----|--------------------------------|
| 1. | atrial flutter. |
| 2. | atrial fibrillation. |
| 3. | premature atrial contractions. |
| 4. | atrial tachycardia. |

ANS: 1

Atrial flutter is characterized by F waves that occur in a characteristic sawtooth pattern. Atrial fibrillation is characterized by coarse waves with the baseline between the QRS complexes as being rough and uneven. Premature atrial contractions occur when an electrical impulse is

generated in an area of the atria outside of the SA node. Atrial tachycardia is three or more premature atrial contractions. Neither premature atrial contractions or atrial tachycardia have an F wave on the tracing.

PTS:1DIF:AnalyzeREF:Atrial Arrhythmias

10.The electrocardiogram tracing for a client shows premature junctional complexes. Which of the following should the nurse do to assist this client?

-
- | | |
|----|-------------------|
| 1. | Administer oxygen |
|----|-------------------|
-
- | | |
|----|-----------------------------|
| 2. | Increase intravenous fluids |
|----|-----------------------------|
-
- | | |
|----|----------------------------------|
| 3. | Check on the serum digoxin level |
|----|----------------------------------|
-
- | | |
|----|--|
| 4. | Assist the client to a side-lying position |
|----|--|

ANS: 3

The most common cause of premature junctional complexes is digitalis toxicity. The nurse should check on the clients serum digoxin level. Oxygen, intravenous fluids, or position changes will not help treat this rhythm. TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Premature Junctional Complexes

11.Which of the following should the nurse instruct a client who has been diagnosed with an arrhythmia?

-
- | | |
|----|----------------|
| 1. | Exercise level |
|----|----------------|
-
- | | |
|----|----------------------------------|
| 2. | Avoidance of calorie-dense foods |
|----|----------------------------------|
-
- | | |
|----|---------------------------|
| 3. | How to take his own pulse |
|----|---------------------------|
-
- | | |
|----|---------------------------------|
| 4. | Reasons why fatigue is expected |
|----|---------------------------------|

ANS: 3

Instructions for a client diagnosed with an arrhythmia include symptom management, how to take own pulse, and substances to avoid the onset of an arrhythmia. The nurse may or may not instruct on exercise level. The client does not need to avoid calorie-dense foods. Fatigue is a symptom that should be reported to a health care provider.

PTS:1DIF:Apply

REF:Table 26-2 Nursing Management for the Patient with Arrhythmias

12.A client is diagnosed with supraventricular tachycardia. The nurse should prepare to administer which of the following medications?

1.	Procainamide
2.	Amiodarone
3.	Verapamil
4.	Adenosine

ANS: 4

Adenosine has a short half-life, is given intravenous push, and is used to abruptly stop supraventricular tachycardia. Procainamide is used for tachyarrhythmias and ventricular ectopy. Amiodarone is helpful to treat ventricular fibrillation. Verapamil helps slow the heart rate with atrial fibrillation.

TESTBANKSELLER.COM

PTS:1DIF:ApplyREFPharmacology

13.A client is recovering from insertion of a pacemaker to pace the activity of the ventricles. At which point on the electrocardiogram tracing will the nurse assess pacer spikes?

1.	Before the QRS complex
2.	Before the P wave
3.	After the QRS complex
4.	After the P wave

ANS: 1

If the ventricles are being paced, there will be a pacer spike just prior to the QRS complex. If the atria are being paced, there will be a pacer spike just before the P wave. Pacer spikes that occur after the QRS complex or P wave would indicate pacemaker malfunction and should be addressed immediately.

PTS: 1 DIF: Apply REF: Permanent Pacing; Pacemaker Malfunction

MULTIPLE RESPONSE

1.A client with a heart rate of 40 who is experiencing shortness of breath and nausea is diagnosed with second-degree AV block type II. Which of the following will be included in this clients treatment? (Select all that apply.)

-
- | | |
|----|--------------------|
| 1. | Administer digoxin |
|----|--------------------|
-
- | | |
|----|-----------------------|
| 2. | Administer antiemetic |
|----|-----------------------|
-
- | | |
|----|-----------------------------|
| 3. | Administer atropine sulfate |
|----|-----------------------------|
-
- | | |
|----|---------------------------|
| 4. | Insert external pacemaker |
|----|---------------------------|
-
- | | |
|----|-----------------------------|
| 5. | Decrease intravenous fluids |
|----|-----------------------------|
-
- | | |
|----|---------------------------|
| 6. | Lower the head of the bed |
|----|---------------------------|

ANS: 3, 4

TESTBANKSELLER.COM

For second-degree AV block type II, treatment will almost always consist of external pacemaker insertion. Atropine sulfate may be used to increase the heart rate until the pacemaker can be inserted. Digitalis toxicity can cause this heart rhythm so digoxin should not be administered to this client. An antiemetic will not solve the clients underlying problem. The client may or may not need additional fluids. Lowering the head of the bed could compromise this clients respiratory status and should not be done.

PTS: 1 DIF: Apply REF: Second-Degree AV Block Type II

2.A clients electrocardiogram rhythm strip is a straight line. Which of the following should the nurse do to help this client? (Select all that apply.)

-
- | | |
|----|-------------------------|
| 1. | Assess for loose leads. |
|----|-------------------------|
-
- | | |
|----|----------------------------------|
| 2. | Assess for power to the monitor. |
|----|----------------------------------|
-
- | | |
|----|--|
| 3. | Assess the strip for possible fine ventricular fibrillation. |
|----|--|
-
- | | |
|----|--|
| 4. | Begin cardiopulmonary resuscitation once verified the client has no pulse. |
|----|--|

5. Raise the head of the bed.

6. Stop intravenous fluid infusion.

ANS: 1, 2, 3, 4

The absence of electrical activity will create the rhythm of asystole. The rhythm strip is a straight line. The nurse should confirm that the straight line is not due to another reason such as loose leads, lack of power to the monitor, or fine ventricular fibrillation. Once it is confirmed that the client has no pulse, cardiopulmonary resuscitation should be implemented. Raising the head of the bed or stopping intravenous fluid infusions is not going to help the client experiencing asystole.

PTS: 1 DIF: Apply REF: Asystole

3.The nurse is assessing a client who is diagnosed with pulseless electrical activity. Which of the following will the nurse include in this assessment? (Select all that apply.)

1. Hypovolemia

2. Hypoxia

3. Hypothermia

4. Tamponade

5. Thrombosis

6. Throat pain

ANS: 1, 2, 3, 4, 5

Assessment of pulseless electrical activity includes a review of the 5 Hs and the 5 Ts. The 5 Hs are: hypovolemia, hypoxia, hydrogen ion status, hyperkalemia/hypokalemia, and hypothermia. The 5 Ts include tablets, tamponade, tension pneumothorax, thrombosis coronary, and thrombosis pulmonary. Throat pain does not cause pulseless electrical activity.

PTS: 1 DIF: Apply REF: Pulseless Electrical Activity

4.Which of the following should be implemented to ensure the safe use of a defibrillator? (Select all that apply.)

1. Do not place over monitoring electrodes.
2. Do not place over an implanted pacemaker.
3. Place the paddles at inch from the implanted pacemaker site.
4. Apply transdermal medication to the chest before using the paddles.
5. Insert an oral airway before using the paddles.
6. Have another person hold the clients airway open while using the paddles.

ANS: 1, 2

The safe use of defibrillator paddles include: do not place over monitoring electrodes or implanted devices. Paddles should be at least 1 inch away from an implanted device. Transdermal medication should be removed from the clients chest before using the paddles. An oral airway is not needed before using the paddles. No one should be touching the client when using the paddles.

TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Red Flag: Safe Use of Defibrillator Pads

5. Which of the following interventions would be appropriate for a client recovering from a pacemaker insertion? (Select all that apply.)

1. Monitor vital signs every 15 minutes until stable.
2. Assess for chest pain.
3. Restrict movement of affected extremity.
4. Monitor electrocardiogram every 8 hours.
5. Begin intravenous fluid infusion at 150 mL/hr.
6. Reinforce dressing with excessive bleeding.

ANS: 1, 2, 3

Interventions appropriate for a client recovering from a pacemaker insertion include monitoring vital signs every 15 minutes until stable, assessing for chest pain, restricting movement of the affected extremity, monitoring electrocardiogram ongoing and post a strip every 4 hours, and report excessive bleeding from the surgical site to the health care provider. Intravenous fluids at the rate of 150 mL/hr may or may not be needed.

TESTBANKSELLER.COM

Chapter 17 Nursing Management: Patients with Vascular Disorders and Problems of Peripheral Circulation

MULTIPLE CHOICE

1. Which of the following should the nurse instruct a client in order to reduce the risk factors for developing arteriosclerosis?

-
- | | |
|----|---|
| 1. | Limit diet to contain less than 40% fat |
|----|---|
-
- | | |
|----|-------------------|
| 2. | Restrict exercise |
|----|-------------------|
-
- | | |
|----|--------------|
| 3. | Stop smoking |
|----|--------------|
-
- | | |
|----|--------------------------------|
| 4. | Avoid prescription medications |
|----|--------------------------------|

ANS: 3

To reduce the risk for arteriosclerosis, the nurse should instruct the client to stop smoking. The diet should be limited to less than 30% of fat. Exercise should be encouraged. Prescription medications are often prescribed for clients with symptoms of arteriosclerosis.

PTS:1DIF:Apply

REF: Arteriosclerosis and Atherosclerosis: Planning and Implementation: Goals

2.The nurse is concerned that an elderly client has evidence of arteriosclerosis since the clients capillary refill is greater than:

- | | |
|----|------------|
| 1. | 3 seconds. |
| 2. | 4 seconds. |
| 3. | 5 seconds. |
| 4. | 6 seconds. |

ANS: 3

Elderly patients have a greater capillary refill time due to aging. Capillary refill greater than 5 seconds is significant. Capillary refill in non-elderly clients should be 3 seconds. Capillary refill in a non-elderly client of 4 seconds would be an abnormal finding. Capillary refill of 6 seconds for all clients is an abnormal assessment finding.

PTS:1DIF:Analyze

REF: Arteriosclerosis and Atherosclerosis: Assessment with Clinical Manifestations

3.When instructing a client on ways to lower his cholesterol levels, which of the following should the nurse include?

- | | |
|----|------------------------------|
| 1. | Eat more meat and eggs. |
| 2. | Consume less meat and eggs. |
| 3. | Incorporate more vegetables. |
| 4. | Limit fruits. |

ANS: 2

Cholesterol is located in animal sources, so decreasing meat and eggs will lower cholesterol levels. The client should not be instructed to eat more meat and eggs. Vegetables and fruits do not impact the cholesterol level.

PTS:1DIF:Apply

REF: Arteriosclerosis and Atherosclerosis: Planning and Implementation: Nutrition

4.A client diagnosed with arteriosclerosis is prescribed an anticoagulant. For which of the following should the nurse assess in this client?

- | | |
|----|------------------------|
| 1. | Respiratory distress |
| 2. | Skin breakdown |
| 3. | Decreased urine output |
| 4. | Bruising and bleeding |

ANS: 4

A client who is prescribed blood-thinning medication is at a greater risk of bleeding and bruising. Anticoagulant therapy does not increase a clients risk for developing respiratory distress, skin breakdown, or decreased urine output.

PTS:1DIF:Apply

REF: Table 27-2 Pharmacology Facts: Pharmacology Therapy for Management of Arteriosclerosis and Atherosclerosis

5.The nurse is assessing a client diagnosed with an abdominal aortic aneurysm. Which of the following sounds did the nurse auscultate during the assessment?

- | | |
|----|--------------------------|
| 1. | Pleural rub |
| 2. | Hyperactive bowel sounds |
| 3. | Crackles |
| 4. | Bruit |

ANS: 4

The nurse may auscultate a bruit at the site of the aneurysm. Pleural rales and crackles are adventitious sounds heard during the assessment of the lungs. Hyperactive bowel sounds may be heard when assessing the abdomen.

PTS:1DIF:Analyze

REF: Aneurysms and Aortic Dissections: Assessment with Clinical Manifestations

6.A client is admitted with abdominal aortic aneurysm. For which of the following complications should the nurse be concerned?

- | | |
|----|----------------------|
| 1. | Hypotension |
| 2. | Cardiac arrhythmias |
| 3. | Aneurysm rupture |
| 4. | Loss of bowel sounds |

ANS: 3

Aneurysm rupture is a life-threatening occurrence and the highest risk for the client until it can be repaired. Hypotension, cardiac arrhythmias, and loss of bowel sounds are all significant potential complications; however, they are not life threatening.

PTS:1DIF:Analyze

REF: Aneurysms and Aortic Dissections: Assessment with Clinical Manifestations

7.A client who has experienced signs of Virchows triad has developed a deep vein thrombosis. Which of the following is not a part of this triad?

- | | |
|----|------------------------------|
| 1. | Venous stasis |
| 2. | Vessel wall injury |
| 3. | Alteration in blood clotting |
| 4. | Pregnancy |

ANS: 4

Pregnancy is a risk factor for thrombus, but it is not part of Virchows triad. Virchows triad includes venous stasis, vessel wall injury, and alteration of blood coagulation.

PTS: 1 DIF: Analyze REF: Thrombophlebitis: Pathophysiology

8.A client is diagnosed with Buerger's disease. Which of the following should the nurse instruct the client regarding this disorder?

-
1. It is a common disorder.

 2. It appears in women more than in men.

 3. Smoking exacerbates the disease.

 4. It is more common in African Americans.

ANS: 3

Smoking cessation halts the disease progress, but continuation of smoking exacerbates the progression of the disease. Buerger's disease is a rare disorder. It is more common in men than women. It is more common in Asians and rare among African Americans.

TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Buerger's Disease: Epidemiology; Etiology

9.A client is diagnosed with Raynaud's disease. Which of the following will the nurse most likely assess in this client?

-
1. Elevated blood pressure

 2. Pain, cyanosis, and numb, cold extremities

 3. Absent peripheral pulses

 4. Increase in varicose veins

ANS: 2

Clinical manifestations of Raynaud's disease include vasospasms; pain; cyanosis; redness; numb, cold extremities; and swelling. Elevated blood pressure, absent peripheral pulses, and varicose veins are not associated with this disorder.

PTS: 1 DIF: Apply

REF: Raynauds Phenomenon: Assessment with Clinical Manifestations

10. A client is diagnosed with acute peripheral arterial occlusion. The nurse should prepare to provide which of the following interventions for this client?

- | | |
|----|-----------------------------------|
| 1. | Administer oxygen. |
| 2. | Assist with ambulation. |
| 3. | Administer heparin as prescribed. |
| 4. | Restrict fluids. |

ANS: 3

In the treatment of acute peripheral arterial occlusion, intravenous heparin therapy is usually the first intervention. Oxygen is not the first intervention for this client. The client will most likely be on bed rest and will not ambulate. Restricting fluids would not be indicated for acute peripheral arterial occlusion.

PTS: 1 DIF: Apply

TESTBANKSELLER.COM

REF: Peripheral Arterial Occlusive Disease: Pharmacology

11. A client receiving a heparin infusion is demonstrating signs of acute bleeding. Which of the following should the nurse prepare to administer to this client?

- | | |
|----|-------------------|
| 1. | Aspirin |
| 2. | Vitamin K |
| 3. | Protamine sulfate |
| 4. | Narcan |

ANS: 3

Protamine sulfate is the heparin antagonist used for excessive bleeding. Vitamin K is the antagonist for warfarin. Aspirin and Narcan are not used for bleeding associated with a heparin infusion.

PTS: 1 DIF: Apply REF: Thrombophlebitis: Pharmacology

12. A client's blood pressure measurements have a 20 mmHg difference between the upper extremity readings. Which of the following does this assessment finding suggest to the nurse?

1. Arteriosclerosis
2. Aortic aneurysm
3. Deep vein thrombosis
4. Subclavian steal syndrome

ANS: 4

A difference of greater than 20 mmHg when assessing bilateral blood pressure measurements is considered a significant finding in the diagnosis of subclavian steal syndrome. This blood pressure discrepancy is not a finding with arteriosclerosis, aortic aneurysm, or deep vein thrombosis.

PTS: 1 DIF: Analyze

REF: Subclavian Steal Syndrome: Assessment with Clinical Manifestations

TESTBANKSELLER.COM

13. The nurse is assessing a client for risks in the development of varicose veins. Which of the following findings would increase this client's risk?

1. Normal weight
2. Prolonged standing
3. Engages in golf three times a week
4. Eats several servings of fruits and vegetables each day

ANS: 2

Risk factors for the development of varicose veins include thrombophlebitis, obesity, prolonged standing, pregnancy, and liver or pancreas dysfunction. Normal weight, activity, and balanced diet are not risk factors for the development of varicose veins.

PTS: 1 DIF: Analyze REF: Varicose Veins: Etiology

MULTIPLE RESPONSE

1.A client is having laboratory tests conducted to confirm a diagnosis of arteriosclerosis. Which of the following laboratory values would support this clients medical diagnosis? (Select all that apply.)

1.	Serum cholesterol 300 mg/dL
2.	LDL 125 mg/dL
3.	Blood glucose 90 mg/dL
4.	HDL 45 mg/dL
5.	Triglycerides 400 mg/dL
6.	Serum potassium 4.0 mEq/L

ANS: 1, 2, 4, 5

Diagnostic tests used to support the medical diagnosis of arteriosclerosis include cholesterol, LDL, HDL, and triglycerides. A serum cholesterol of 300 mg/dL, LDL of 125 mg/dL, HDL of 45 mg/dL, and triglycerides of 400 mg/dL all support the diagnosis of arteriosclerosis. Blood glucose and potassium levels are not used to diagnose arteriosclerosis.

PTS:1DIF:Analyze

REF: Table 27-1 Laboratory Tests: Recommended Cholesterol Screening Levels for Patients with Arteriosclerosis and Atherosclerosis

2.The nurse is assessing a client diagnosed with a peripheral arterial occlusion. Which of the following will the nurse assess in this client? (Select all that apply.)

1.	Pulselessness
2.	Pain
3.	Pallor
4.	Paresthesia

5. Paralysis

6. Petechiae

ANS: 1, 2, 3, 4, 5

The nurse would assess a client diagnosed with peripheral arterial disease for the six Ps: pulseless, pain, pallor, paresthesia, paralysis, and poikilocythemia. Petechiae is not a part of the six Ps assessment.

PTS:1DIF:Apply

REF: Peripheral Arterial Occlusive Disease: Assessment with Clinical Manifestations

3.The nurse is instructing a client recovering from arterial aneurysm repair. Which of the following should be included in these instructions? (Select all that apply.)

1. Do not lift anything heavier than 15 to 20 lbs.

2. Limit activity for up to 8 weeks after the surgery.

3. Use a pillow to splint when coughing.

4. Driving is permitted 1 week after surgery.

5. Notify the physician for pain, redness, or swelling around the incision.

6. Avoid pain medication.

ANS: 1, 2, 3, 5

Instructions appropriate after surgery to repair an arterial aneurysm include limit lifting to 15 to 20 lbs; limit activity for up to 8 weeks after the surgery; use a pillow to splint when coughing; and notify the physician for pain, redness, or swelling around the incision. Driving may be restricted for several weeks. Pain medication will be prescribed and encouraged to be used.

PTS:1DIF:Apply

REF:Aneurysms and Aortic Dissections: Patient and Family Teaching

4.The nurse is utilizing the Wells Scale to assess a client for deep vein thrombosis. Which of the following is assessed when using this scale? (Select all that apply.)

-
1. Treatment for cancer

 2. Recent immobility for greater than 3 days

 3. Recovery from surgery with general anesthesia within 12 weeks

 4. Entire leg edematous

 5. Pitting edema of the symptomatic leg

 6. Blood pressure 130/86 mmHg

ANS: 1, 2, 3, 4, 5

The Wells Scale is a tool used to assess a client for the presence of a deep vein thrombosis. Areas assessed include treatment or diagnosis of cancer, recent immobility for greater than 3 days, recovery from surgery during which the client received general or regional anesthesia within 12 weeks, entire leg swollen, and pitting edema confined to the symptomatic leg. Blood pressure is not a criteria used on this scale. TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Table 27-4 Modified Wells Clinical Score

5.A client is diagnosed with a venous stasis ulcer on the foot. Which of the following will be included in this clients plan of care? (Select all that apply.)

-
1. Administer oral antibiotics if infection is present.

 2. Keep the foot open to the air.

 3. Cover the foot with a hydrocolloidal dressing.

 4. Provide pain medication with debridement.

 5. Restrict fluids.

 6. Instruct the client to ambulate without shoes.

ANS: 1, 3, 4

Nursing care of a client diagnosed with a venous stasis ulcer includes provide with oral antibiotics if infection is present, cover the wound with hydrocolloidal dressing if indicated to promote the formation of granulation tissue, provide pain medication with debridement. The wound should not be kept open to the air. The client does not need a fluid restriction. The client should be instructed to never ambulate without appropriate foot protection.

PTS: 1 DIF: Apply REF: Venous Stasis Ulcer: Planning and Implementation

MULTIPLE CHOICE

1. Which of the following should the nurse instruct a client in order to reduce the risk factors for developing arteriosclerosis?

- | | |
|----|---|
| 1. | Limit diet to contain less than 40% fat |
| 2. | Restrict exercise |
| 3. | Stop smoking |
| 4. | Avoid prescription medications |

ANS: 3

TESTBANKSELLER.COM

To reduce the risk for arteriosclerosis, the nurse should instruct the client to stop smoking. The diet should be limited to less than 30% of fat. Exercise should be encouraged. Prescription medications are often prescribed for clients with symptoms of arteriosclerosis.

PTS: 1 DIF: Apply

REF: Arteriosclerosis and Atherosclerosis: Planning and Implementation: Goals

2. The nurse is concerned that an elderly client has evidence of arteriosclerosis since the client's capillary refill is greater than:

- | | |
|----|------------|
| 1. | 3 seconds. |
| 2. | 4 seconds. |
| 3. | 5 seconds. |
| 4. | 6 seconds. |

ANS: 3

Elderly patients have a greater capillary refill time due to aging. Capillary refill greater than 5 seconds is significant. Capillary refill in non-elderly clients should be 3 seconds. Capillary refill in a non-elderly client of 4 seconds would be an abnormal finding. Capillary refill of 6 seconds for all clients is an abnormal assessment finding.

PTS:1DIF:Analyze

REF: Arteriosclerosis and Atherosclerosis: Assessment with Clinical Manifestations

3. When instructing a client on ways to lower his cholesterol levels, which of the following should the nurse include?

-
- | | |
|----|-------------------------|
| 1. | Eat more meat and eggs. |
|----|-------------------------|
-
- | | |
|----|-----------------------------|
| 2. | Consume less meat and eggs. |
|----|-----------------------------|
-
- | | |
|----|------------------------------|
| 3. | Incorporate more vegetables. |
|----|------------------------------|
-
- | | |
|----|---------------|
| 4. | Limit fruits. |
|----|---------------|
-

ANS: 2

TESTBANKSELLER.COM

Cholesterol is located in animal sources, so decreasing meat and eggs will lower cholesterol levels. The client should not be instructed to eat more meat and eggs. Vegetables and fruits do not impact the cholesterol level.

PTS:1DIF:Apply

REF: Arteriosclerosis and Atherosclerosis: Planning and Implementation: Nutrition

4. A client diagnosed with arteriosclerosis is prescribed an anticoagulant. For which of the following should the nurse assess in this client?

-
- | | |
|----|----------------------|
| 1. | Respiratory distress |
|----|----------------------|
-
- | | |
|----|----------------|
| 2. | Skin breakdown |
|----|----------------|
-
- | | |
|----|------------------------|
| 3. | Decreased urine output |
|----|------------------------|
-
- | | |
|----|-----------------------|
| 4. | Bruising and bleeding |
|----|-----------------------|
-

ANS: 4

A client who is prescribed blood-thinning medication is at a greater risk of bleeding and bruising. Anticoagulant therapy does not increase a client's risk for developing respiratory distress, skin breakdown, or decreased urine output.

PTS:1DIF:Apply

REF: Table 27-2 Pharmacology Facts: Pharmacology Therapy for Management of Arteriosclerosis and Atherosclerosis

5. The nurse is assessing a client diagnosed with an abdominal aortic aneurysm. Which of the following sounds did the nurse auscultate during the assessment?

- | | |
|----|--------------------------|
| 1. | Pleural rub |
| 2. | Hyperactive bowel sounds |
| 3. | Crackles |
| 4. | Bruit |

ANS: 4

TESTBANKSELLER.COM

The nurse may auscultate a bruit at the site of the aneurysm. Pleural rub and crackles are adventitious sounds heard during the assessment of the lungs. Hyperactive bowel sounds may be heard when assessing the abdomen.

PTS:1DIF:Analyze

REF: Aneurysms and Aortic Dissections: Assessment with Clinical Manifestations

6. A client is admitted with abdominal aortic aneurysm. For which of the following complications should the nurse be concerned?

- | | |
|----|----------------------|
| 1. | Hypotension |
| 2. | Cardiac arrhythmias |
| 3. | Aneurysm rupture |
| 4. | Loss of bowel sounds |

ANS: 3

Aneurysm rupture is a life-threatening occurrence and the highest risk for the client until it can be repaired. Hypotension, cardiac arrhythmias, and loss of bowel sounds are all significant potential complications; however, they are not life threatening.

PTS:1DIF:Analyze

REF: Aneurysms and Aortic Dissections: Assessment with Clinical Manifestations

7.A client who has experienced signs of Virchows triad has developed a deep vein thrombosis. Which of the following is not a part of this triad?

-
- | | |
|----|---------------|
| 1. | Venous stasis |
|----|---------------|
-
- | | |
|----|--------------------|
| 2. | Vessel wall injury |
|----|--------------------|
-
- | | |
|----|------------------------------|
| 3. | Alteration in blood clotting |
|----|------------------------------|
-
- | | |
|----|-----------|
| 4. | Pregnancy |
|----|-----------|

ANS: 4

Pregnancy is a risk factor for thrombus, but it is not part of Virchows triad. Virchows triad includes venous stasis, vessel wall injury, and alteration of blood coagulation.

PTS: 1 DIF: Analyze REF: Thrombophlebitis: Pathophysiology

8.A client is diagnosed with Buerger's disease. Which of the following should the nurse instruct the client regarding this disorder?

-
- | | |
|----|--------------------------|
| 1. | It is a common disorder. |
|----|--------------------------|
-
- | | |
|----|---------------------------------------|
| 2. | It appears in women more than in men. |
|----|---------------------------------------|
-
- | | |
|----|----------------------------------|
| 3. | Smoking exacerbates the disease. |
|----|----------------------------------|
-
- | | |
|----|---|
| 4. | It is more common in African Americans. |
|----|---|

ANS: 3

Smoking cessation halts the disease progress, but continuation of smoking exacerbates the progression of the disease. Buerger's disease is a rare disorder. It is more common in men than women. It is more common in Asians and rare among African Americans.

PTS: 1 DIF: Apply REF: Buerger's Disease: Epidemiology; Etiology

9. A client is diagnosed with Raynaud's disease. Which of the following will the nurse most likely assess in this client?

1. Elevated blood pressure
2. Pain, cyanosis, and numb, cold extremities
3. Absent peripheral pulses
4. Increase in varicose veins

ANS: 2

Clinical manifestations of Raynaud's disease include vasospasms; pain; cyanosis; redness; numb, cold extremities; and swelling. Elevated blood pressure, absent peripheral pulses, and varicose veins are not associated with this disorder.

PTS: 1 DIF: Apply

REF: Raynaud's Phenomenon: Assessment with Clinical Manifestations

10. A client is diagnosed with acute peripheral arterial occlusion. The nurse should prepare to provide which of the following interventions for this client?

1. Administer oxygen.
2. Assist with ambulation.
3. Administer heparin as prescribed.
4. Restrict fluids.

ANS: 3

In the treatment of acute peripheral arterial occlusion, intravenous heparin therapy is usually the first intervention. Oxygen is not the first intervention for this client. The client will most likely be on bed rest and will not ambulate. Restricting fluids would not be indicated for acute peripheral arterial occlusion.

PTS: 1 DIF: Apply

REF: Peripheral Arterial Occlusive Disease: Pharmacology

11. A client receiving a heparin infusion is demonstrating signs of acute bleeding. Which of the following should the nurse prepare to administer to this client?

- | | |
|----|-------------------|
| 1. | Aspirin |
| 2. | Vitamin K |
| 3. | Protamine sulfate |
| 4. | Narcan |

ANS: 3

Protamine sulfate is the heparin antagonist used for excessive bleeding. Vitamin K is the antagonist for warfarin. Aspirin and narcan are not used for bleeding associated with a heparin infusion.

PTS: 1 DIF: Apply REF: Thrombophlebitis: Pharmacology

12. A client's blood pressure measurements have a 20 mmHg difference between the upper extremity readings. Which of the following does this assessment finding suggest to the nurse?

- | | |
|----|---------------------------|
| 1. | Arteriosclerosis |
| 2. | Aortic aneurysm |
| 3. | Deep vein thrombosis |
| 4. | Subclavian steal syndrome |

ANS: 4

A difference of greater than 20 mmHg when assessing bilateral blood pressure measurements is considered a significant finding in the diagnosis of subclavian steal syndrome. This blood pressure discrepancy is not a finding with arteriosclerosis, aortic aneurysm, or deep vein thrombosis.

PTS: 1 DIF: Analyze

REF: Subclavian Steal Syndrome: Assessment with Clinical Manifestations

13. The nurse is assessing a client for risks in the development of varicose veins. Which of the following findings would increase this client's risk?

- | | |
|----|---|
| 1. | Normal weight |
| 2. | Prolonged standing |
| 3. | Engages in golf three times a week |
| 4. | Eats several servings of fruits and vegetables each day |

ANS: 2

Risk factors for the development of varicose veins include thrombophlebitis, obesity, prolonged standing, pregnancy, and liver or pancreas dysfunction. Normal weight, activity, and balanced diet are not risk factors for the development of varicose veins.

PTS: 1 DIF: Analyze REF: Varicose Veins: Etiology

MULTIPLE RESPONSE

1. A client is having laboratory tests conducted to confirm a diagnosis of arteriosclerosis. Which of the following laboratory values would support this client's medical diagnosis? (Select all that apply.)

- | | |
|----|-----------------------------|
| 1. | Serum cholesterol 300 mg/dL |
| 2. | LDL 125 mg/dL |
| 3. | Blood glucose 90 mg/dL |
| 4. | HDL 45 mg/dL |
| 5. | Triglycerides 400 mg/dL |
| 6. | Serum potassium 4.0 mEq/L |

ANS: 1, 2, 4, 5

Diagnostic tests used to support the medical diagnosis of arteriosclerosis include cholesterol, LDL, HDL, and triglycerides. A serum cholesterol of 300 mg/dL, LDL of 125 mg/dL, HDL of

45 mg/dL, and triglycerides of 400 mg/dL all support the diagnosis of arteriosclerosis. Blood glucose and potassium levels are not used to diagnose arteriosclerosis.

PTS:1DIF:Analyze

REF: Table 27-1 Laboratory Tests: Recommended Cholesterol Screening Levels for Patients with Arteriosclerosis and Atherosclerosis

2.The nurse is assessing a client diagnosed with a peripheral arterial occlusion. Which of the following will the nurse assess in this client? (Select all that apply.)

1.	Pulselessness
2.	Pain
3.	Pallor
4.	Paresthesia
5.	Paralysis
6.	Petechiae

ANS: 1, 2, 3, 4, 5

The nurse would assess a client diagnosed with peripheral arterial disease for the six Ps: pulseless, pain, pallor, paresthesia, paralysis, and poikilocythemia. Petechiae is not a part of the six Ps assessment.

PTS:1DIF:Apply

REF: Peripheral Arterial Occlusive Disease: Assessment with Clinical Manifestations

3.The nurse is instructing a client recovering from arterial aneurysm repair. Which of the following should be included in these instructions? (Select all that apply.)

1.	Do not lift anything heavier than 15 to 20 lbs.
2.	Limit activity for up to 8 weeks after the surgery.
3.	Use a pillow to splint when coughing.

-
4. Driving is permitted 1 week after surgery.
 5. Notify the physician for pain, redness, or swelling around the incision.
 6. Avoid pain medication.
-

ANS: 1, 2, 3, 5

Instructions appropriate after surgery to repair an arterial aneurysm include limit lifting to 15 to 20 lbs; limit activity for up to 8 weeks after the surgery; use a pillow to splint when coughing; and notify the physician for pain, redness, or swelling around the incision. Driving may be restricted for several weeks. Pain medication will be prescribed and encouraged to be used.

PTS:1DIF:Apply

REF:Aneurysms and Aortic Dissections: Patient and Family Teaching

4.The nurse is utilizing the Wells Scale to assess a client for deep vein thrombosis. Which of the following is assessed when using this scale? (Select all that apply.)

-
1. Treatment for cancer
 2. Recent immobility for greater than 3 days
 3. Recovery from surgery with general anesthesia within 12 weeks
 4. Entire leg edematous
 5. Pitting edema of the symptomatic leg
 6. Blood pressure 130/86 mmHg
-

ANS: 1, 2, 3, 4, 5

The Wells Scale is a tool used to assess a client for the presence of a deep vein thrombosis. Areas assessed include treatment or diagnosis of cancer, recent immobility for greater than 3 days, recovery from surgery during which the client received general or regional anesthesia within 12 weeks, entire leg swollen, and pitting edema confined to the symptomatic leg. Blood pressure is not a criteria used on this scale.

PTS: 1 DIF: Apply REF: Table 27-4 Modified Wells Clinical Score

5. A client is diagnosed with a venous stasis ulcer on the foot. Which of the following will be included in this client's plan of care? (Select all that apply.)

-
- | | |
|----|--|
| 1. | Administer oral antibiotics if infection is present. |
|----|--|
-
- | | |
|----|--------------------------------|
| 2. | Keep the foot open to the air. |
|----|--------------------------------|
-
- | | |
|----|--|
| 3. | Cover the foot with a hydrocolloidal dressing. |
|----|--|
-
- | | |
|----|---|
| 4. | Provide pain medication with debridement. |
|----|---|
-
- | | |
|----|------------------|
| 5. | Restrict fluids. |
|----|------------------|
-
- | | |
|----|--|
| 6. | Instruct the client to ambulate without shoes. |
|----|--|
-

ANS: 1, 3, 4

Nursing care of a client diagnosed with a venous stasis ulcer includes provide with oral antibiotics if infection is present, cover the wound with hydrocolloidal dressing if indicated to promote the formation of granulation tissue, provide pain medication with debridement. The wound should not be kept open to the air. The client does not need a fluid restriction. The client should be instructed to never ambulate without appropriate foot protection.

Chapter 18 Nursing Assessment: Hematologic Function

MULTIPLE CHOICE

1. A client is diagnosed with anemia. The nurse realizes that which of the following could be the treatment for this client's disorder?

-
- | | |
|----|------------------------|
| 1. | Erythropoietin therapy |
|----|------------------------|
-
- | | |
|----|----------|
| 2. | Leukemia |
|----|----------|
-
- | | |
|----|----------------|
| 3. | Poor nutrition |
|----|----------------|
-
- | | |
|----|--------|
| 4. | Trauma |
|----|--------|
-

ANS: 1

Anemia is caused for a variety of reasons such as nutrition, chronic illness, trauma, medication therapy, immune suppression, and alterations of erythropoiesis. Erythropoietin therapy stimulates red blood cell production in the bone marrow as a treatment for anemia.

PTS: 1 DIF: Analyze REF: Anemias: Epidemiology

2.A client is diagnosed with alpha- and beta- defect thalassemia. The nurse realizes that this disease is common within which of the following cultural groups?

-
- | | |
|----|--------------------|
| 1. | Persons from China |
|----|--------------------|
-
- | | |
|----|----------------------------------|
| 2. | People of Mediterranean ancestry |
|----|----------------------------------|
-
- | | |
|----|-------------------|
| 3. | African Americans |
|----|-------------------|
-
- | | |
|----|------------------------------|
| 4. | Persons from the Philippines |
|----|------------------------------|

ANS: 3

African Americans and Africans are more likely to have both alpha- and beta-defect thalassemia. Populations of Asian descent such as those from China or the Philippines more often have alpha-defect thalassemia. Populations of Mediterranean ancestry are more susceptible to beta-defect thalassemia.

PTS: 1 DIF: Analyze REF: Thalassemia: Epidemiology

3.The mother of a newborn is concerned since the baby is jaundiced. The nurse realizes that the infant should be assessed for which of the following anemias?

-
- | | |
|----|--|
| 1. | Glucose-6-phosphate dehydrogenase (G6PD) |
|----|--|
-
- | | |
|----|--------------------------|
| 2. | Hereditary spherocytosis |
|----|--------------------------|
-
- | | |
|----|--------------------|
| 3. | Sickle-cell anemia |
|----|--------------------|
-
- | | |
|----|-------------|
| 4. | Thalassemia |
|----|-------------|

ANS: 2

Hereditary spherocytosis is also known as congenital hemolytic anemia. This anemia begins in utero and manifests as anemia and hyperbilirubinemia. A client with Glucose-6-phosphate

dehydrogenase may develop jaundice later in life but not upon birth. Thalassemia and sickle-cell anemia do not present with hyperbilirubinemia upon birth.

PTS:1DIF:Analyze

REF: Glucose-6-Phosphate Dehydrogenase Anemia: Assessment with Clinical Manifestations

4. During the health history portion of the assessment, the client states, I have sickle-cell trait. The nurse realizes that:

-
1. precautions should be taken to prevent the cell from sickling.

 2. the client is a carrier.

 3. the client will show signs of the disease as she grows older.

 4. the client will transmit the disease to any offspring.

ANS: 2

Sickle-cell anemia is an autosomal recessive disorder passed from parent to offspring in this pattern. An individual with one HbS has the sickle-cell trait and has a 50% chance of transmitting the gene to each child. There are no precautions to take to prevent the cell from sickling. The client will not demonstrate signs of the disease as she grows older. It will depend upon the other parent having the trait if any offspring will be affected with the disorder.

PTS: 1 DIF: Analyze REF: Sickle-Cell Anemia: Etiology

5. A client diagnosed with acute myeloid leukemia is recovering from a bone marrow transplant. Which of the following nursing interventions would *not* be appropriate for this client?

-
1. Assess for reactions to anesthesia.

 2. Assess vital signs.

 3. Maintain isolation precautions.

 4. Obtain a low-pressure mattress to prevent skin breakdown.

ANS: 1

The client having a bone marrow transplant does not receive anesthesia. Maintaining skin integrity, implementing isolation precautions, and monitoring vital signs are appropriate nursing measures for the client recovering from a bone marrow transplant.

PTS: 1 DIF: Apply REF: Leukemia: Planning and Implementation

6.A client diagnosed with chronic disseminated intravascular coagulation is prescribed heparin. The nurse realizes that this medication is used to:

-
- | | |
|----|---|
| 1. | increase blood flow to the circulation. |
|----|---|
-
- | | |
|----|--------------------------------|
| 2. | increase blood clot formation. |
|----|--------------------------------|
-
- | | |
|----|---|
| 3. | decrease blood flow in the circulation. |
|----|---|
-
- | | |
|----|--------------------------------|
| 4. | decrease blood clot formation. |
|----|--------------------------------|

ANS: 4

Heparin is given for its interference with the clotting processes and the chance of preventing further overuse of clotting factors. Heparin is usually only used when other methods of management are failing. Heparin does not increase or decrease blood flow in the circulation. Heparin does not increase blood clot formation.

PTS:1DIF:Analyze

REF: Disseminated Intravascular Coagulation: Pharmacology

7.The nurse should assess a client diagnosed with multiple myeloma for which of the following electrolyte imbalances?

-
- | | |
|----|---------------|
| 1. | Hypercalcemia |
|----|---------------|
-
- | | |
|----|--------------|
| 2. | Hyperkalemia |
|----|--------------|
-
- | | |
|----|-----------------|
| 3. | Hypermagnesemia |
|----|-----------------|
-
- | | |
|----|---------------|
| 4. | Hypernatremia |
|----|---------------|

ANS: 1

Destruction of the bone leads to elevated calcium levels. The other electrolyte imbalances are not characteristic of multiple myeloma.

Chapter 19 Nursing Management: Patients with Hematologic disorders**MULTIPLE CHOICE**

1.A client is receiving treatment for the diagnosis of hemophilia. Which of the following should the nurse assess in this client?

-
- | | |
|----|----------|
| 1. | Appetite |
|----|----------|
-
- | | |
|----|--------------|
| 2. | Urine output |
|----|--------------|
-
- | | |
|----|-----------------------|
| 3. | Muscle and joint pain |
|----|-----------------------|
-
- | | |
|----|------------------|
| 4. | Respiratory rate |
|----|------------------|

ANS: 3

The clinical features of hemophilia include joint and muscle hemorrhages. The weight-bearing joints are most frequently affected. The nurse should assess the client for muscle and joint pain, which occurs with bleeding. Appetite, urine output, and respiratory rate are not specifically affected by hemophilia.

PTS:1DIF:Apply

REF: Hemophilia: Assessment with Clinical Manifestations

2.A client is diagnosed with emphysema. For which of the following hematologic disorders should the nurse include in the assessment of this client?

-
- | | |
|----|------------------|
| 1. | Hemolytic anemia |
|----|------------------|

2.	Disseminated intravascular coagulation
----	--

3.	Polycythemia
----	--------------

4.	Hemophilia
----	------------

ANS: 3

One type of polycythemia is caused by an increase in the number of red blood cells in response to a reduced amount of oxygen in the body. The client with emphysema could develop this type of polycythemia. Hemolytic anemia, disseminated intravascular coagulation, and hemophilia are not associated with emphysema.

PTS: 1 DIF: Apply REF: Polycythemia: Secondary Polycythemia

3.A client, diagnosed with acute lymphoblastic leukemia, is receiving the first phase of chemotherapy. The nurse realizes this client is in which phase of treatment for the disorder?

1.	Induction
----	-----------

2.	Consolidation TESTBANKSELLER.COM
----	----------------------------------

3.	Maintenance
----	-------------

4.	Central nervous system prophylaxis
----	------------------------------------

ANS: 1

The primary goal of therapy for this type of leukemia is complete remission with restoration of normal hematopoiesis. Induction chemotherapy is administered first. Consolidation occurs afterwards. Maintenance therapy then occurs followed by central nervous system prophylaxis.

PTS: 1 DIF: Analyze REF: Acute Lymphoblastic Leukemia: Pharmacology

4.The nurse is encouraging a client diagnosed with chronic leukemia to join a support group. Which of the following would a support group address?

1.	Fatigue
----	---------

2.	Infection
----	-----------

3. Anxiety

4. Social isolation

ANS: 4

Social isolation is a common concern for clients with this diagnosis. The client should be encouraged to join a support group. A support group will not help with fatigue, infection, or anxiety.

PTS:1DIF:Apply

REF: Box 29-6 Common Problems of Patients with Chronic Lymphocytic Leukemia

5.A client is diagnosed with stage II Hodgkins lymphoma. The nurse realizes that this diagnosis means the disease is:

1. terminal.

2. limited to lymph nodes on the same side of the diaphragm.

3. in the bone marrow. TESTBANKSELLER.COM

4. easily treated.

ANS: 2

Stage II Hodgkins lymphoma means that the disease is located in two or more lymph node regions on the same side of the diaphragm. This diagnosis does not mean the client is terminal or easily treated. Stage IV of the disease would mean the disease is in the bone marrow.

PTS: 1 DIF: Analyze REF: Box 29-8 Staging Hodgkins and NHL

6.A client is diagnosed with disseminated low-grade non-Hodgkins lymphoma. Which of the following treatments would be indicated for this client?

1. Administration of CHOP

2. Radiation therapy

3. Bone marrow transplant

-
4. Watch and wait

ANS: 4

In disseminated low-grade non-Hodgkins lymphoma, early intervention does not prolong survival, so watch and wait is an acceptable approach. The reason to delay is that the client may remain stable for years without treatments that could cause adverse reactions and decrease quality of life. CHOP is standard treatment for intermediate-grade non-Hodgkins lymphoma. Radiation therapy is appropriate for both intermediate-grade and high-grade non-Hodgkins lymphoma. Bone marrow transplant is used for a client with a recurrence of the disease.

PTS:1DIF:Analyze

REF:Non-Hodgkins Lymphoma: Planning and Implementation

7.A client is diagnosed with G6PD anemia. Which of the following medications should the nurse instruct the client to avoid? (Select all that apply.)

-
1. Acetaminophen

-
2. Aspirin

-
3. Chloroquine

-
4. Nitrofurantoin

-
5. Sulfonamides

-
6. Vitamin K

ANS: 2, 3, 4, 5, 6

Medications that heighten the hemolytic affects of G6PD are antimalarial drugs (e.g., chloroquine), common coal tar analgesics (including aspirin), nitrofurantoin, oral hypoglycemics, sulfonamides, thiazides, diuretics, and vitamin K. Acetaminophen has only analgesic and antipyretic properties.

PTS:1DIF:Apply

REF: Box 29-1 Medications that Heighten the Hemolytic Affects of G6PD

8.A client diagnosed with sickle-cell anemia is experiencing vaso-occlusive crisis. Which of the following interventions would be appropriate for this client? (Select all that apply.)

-
- | | |
|----|----------------------|
| 1. | Administering oxygen |
|----|----------------------|
-
- | | |
|----|----------------------|
| 2. | Decreasing hydration |
|----|----------------------|
-
- | | |
|----|---------------|
| 3. | Managing pain |
|----|---------------|
-
- | | |
|----|--------------------|
| 4. | Promoting activity |
|----|--------------------|
-
- | | |
|----|------------------|
| 5. | Encouraging rest |
|----|------------------|
-
- | | |
|----|----------------------|
| 6. | Restricting calories |
|----|----------------------|
-

ANS: 1, 3, 5

The nursing management of sickle-cell anemia is to manage pain and prevent sickling. This type of management is accomplished by adequate hydration, oxygenation, adequate nutrition, rest, medications, management of fever and complications, and use of transfusions. Restricting fluids and calories could be detrimental to the clients recovery. The client should be encouraged to rest and not engage in activity.

TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Sickle-Cell Anemia: Planning and Implementation

9.A client is having diagnostic tests to determine the cause of anemia. The nurse realizes that these tests will focus on which of the following? (Select all that apply.)

-
- | | |
|----|----------------------|
| 1. | Presence of bleeding |
|----|----------------------|
-
- | | |
|----|---------------|
| 2. | Fluid balance |
|----|---------------|
-
- | | |
|----|---|
| 3. | Disorders that cause red blood cell destruction |
|----|---|
-
- | | |
|----|---------------------|
| 4. | Cardiac functioning |
|----|---------------------|
-
- | | |
|----|---|
| 5. | Disorders that reduce the production of red blood cells |
|----|---|
-
- | | |
|----|-----------|
| 6. | Digestion |
|----|-----------|
-

ANS: 1, 3, 5

Anemias have three causes: 1) bleeding that results in red blood cell loss, 2) conditions that cause red blood cell destruction, and 3) conditions that cause a reduction in the number of red blood cells made by the body. Diagnostic tests will focus on these three causes. Testing for anemia will not focus on fluid balance, cardiac functioning, or digestion.

PTS: 1 DIF: Analyze REF: Anemias: Pathophysiology

10.A client tells the nurse that he is anemic because of a poor diet. Which deficiencies cause nutritional anemias? (Select all that apply.)

- | | |
|----|-------------------------|
| 1. | Iron deficiency |
| 2. | Folic acid deficiency |
| 3. | Vitamin C deficiency |
| 4. | Vitamin D deficiency |
| 5. | Vitamin A deficiency |
| 6. | Vitamin B-12 deficiency |

ANS: 1, 2, 6

Nutritional anemias can be caused by deficiencies in iron, folic acid, or vitamin B-12. A vitamin D deficiency can cause osteomalacia or rickets. Vitamin C or vitamin A deficiencies do not cause anemia.

PTS:1DIF:AnalyzeREF:Nutritional Anemias

11.Which of the following should the nurse instruct a client who is receiving treatment for the diagnosis of leukemia? (Select all that apply.)

- | | |
|----|---|
| 1. | See a dentist regularly. |
| 2. | Increase fluids. |
| 3. | Report any fatigue to the physician. |
| 4. | Expect to have frequent coughs and colds. |

-
5. Use a sunblock when outdoors.
 6. Report gastrointestinal distress to the physician.
-

ANS: 1, 2, 5, 6

Nursing care for a client diagnosed with leukemia should include regular dental care, increasing fluids, using a sunblock when outdoors, and reporting gastrointestinal distress to the physician. Fatigue is common with this illness and does not need to be reported to the physician. Fre

Chapter 20 Nursing Assessment: Digestive, Gastrointestinal, and Metabolic Function

MULTIPLE CHOICE

1. A client asks the nurse what will happen to her digestion if she needs to have her appendix removed. The nurse should respond that the purpose of the appendix is:

-
1. to digest food products and another organ will take over this function.
 2. to absorb nutrients and another organ will take over this function.
 3. to secrete enzymes and another organ will take over this function.
 4. nothing, so no other organ will need to take over this function.
-

ANS: 4

The appendix is a blind-ended, tube-like structure exiting from the cecum, and it has no function in humans. The appendix is not needed to digest food, absorb nutrients, or secrete enzymes.

PTS:1DIF:ApplyREF:Cecum and Appendix

2. Which of the following questions should the nurse ask while doing an assessment of a client's digestive system?

-
1. Were you breastfed or bottle-fed as an infant?
 2. Do you have knowledge of the food pyramid?
-

3. What medication have you taken, even over-the-counter drugs?

4. Do you drink coffee or tea with meals?

ANS: 3

During the assessment, it is very important to discover what medications or over-the-counter drugs are being taken by the patient. Treatment and therapies may change because of this information. How the client was fed as an infant is not a part of this assessment. Asking if the client has knowledge of the food pyramid is not part of this assessment. If the client drinks coffee or tea with meals is not a part of this assessment.

PTS: 1 DIF: Apply REF: Assessment

3.The nurse realizes that a client diagnosed with heartburn will most likely experiencing symptoms:

1. 1 hour before eating.

2. while eating a meal.

3. 1 hour after eating.

4. first thing in the morning.

ANS: 3

Heartburn is a substernal burning sensation that is experienced within 1 hour after eating or 1 to 2 hours after reclining. Heartburn is not experienced before eating, while eating, or the first thing in the morning.

PTS: 1 DIF: Analyze REF: Heartburn

4.A client is experiencing straining at stool with the production of hard stools. The nurse realizes this client might be diagnosed with constipation if the client also has:

1. fewer than six bowel movements per week.

2. fewer than five bowel movements per week.

3. fewer than four bowel movements per week.

-
4. fewer than three bowel movements per week.

ANS: 4

The number of bowel movements a client has is very individual, but if a client has fewer than three bowel movements per week or must vigorously strain when passing stool, the client is considered to have constipation. The other choices do not fit the criteria for the diagnosis of constipation.

PTS: 1 DIF: Analyze REF: Constipation

5.The nurse is preparing to conduct an abdominal assessment on a client and realizes that this assessment should be performed in the order of:

-
1. inspection, palpation, auscultation, and percussion.
-
2. inspection, auscultation, percussion, and palpation.
-
3. auscultation, palpation, percussion, and inspection.
-
4. percussion, palpation, inspection, and auscultation.

ANS: 2

The order of abdominal assessment is inspection, auscultation, percussion, and palpation. Auscultation is performed second because palpation and percussion can alter bowel sounds. The other choices list the incorrect order for conducting this assessment.

PTS: 1 DIF: Apply REF: Assessment

6.The nurse has determined a client has absent bowel sounds because no sounds have been heard in all four quadrants for :

-
1. 1 minute.
-
2. 2 minutes.
-
3. 30 seconds.
-
4. 5 minutes.

ANS: 4

The nurse must listen for 3 to 5 minutes before concluding the patient has absent bowel sounds. Auscultating for 30 seconds or 1 or 2 minutes is not adequate to determine the absence of bowel sounds.

PTS:1DIF:Analyze

REF:Table 47-1 Assessment of Abdomen: Normal and Key Findings

7.A client scheduled for a colonoscopy should be instructed regarding the need for:

-
1. serum blood specimens.
 2. a bowel preparation.
 3. pain medications prior to the test.
 4. eating a full meal prior to the test.
-

ANS: 2

Bowel cleansing is necessary for all colonoscopy procedures. The bowel preparation selected depends on the reasons for the procedure. Serum blood specimens are not needed for a colonoscopy. Pain medication is not typically needed prior to a colonoscopy. The client should take nothing by mouth for at least 6 hours before the colonoscopy.

PTS:1DIF:ApplyREF:Colonoscopy

8.A client, scheduled for a colonoscopy, is provided with a polyethylene glycol solution to ingest the day before the test. Which of the following should the nurse instruct the client about this solution?

-
1. Keep the solution at room temperature.
 2. Sip the solution throughout the day until bowel movements begin.
 3. Drink 8 ounces of the solution every 10 minutes until totally consumed.
 4. Drink 8 ounces of the solution every hour until bowel movements begin.
-

ANS: 3

The nurse should instruct the client to refrigerate the solution and drink 8 ounces of the solution every 10 minutes until totally consumed. The solution should not be sipped throughout the day or only taken until bowel movements begin.

PTS:1DIF:ApplyREF:Colonoscopy

9. When instructing a client on the three steps of a proctosigmoidoscopy, which of the following would *not* be included?

1. Placement of a nasogastric (NG) tube for gastric deflation
2. Digital examination to dilate the anal sphincters to detect obstruction
3. Sigmoidoscope to examine the distal sigmoid colon and rectum
4. Proctoscope to examine the lower rectum and anal canal

ANS: 1

This is a diagnostic test that takes three steps: first, a digital examination; second, a sigmoidoscope; and third, a proctoscope. An NG tube is not needed for this examination.

PTS:1DIF:ApplyREFroctosigmoidoscopy

10. The nurse, planning care for a client diagnosed with severe facial trauma, realizes that which of the following will *not* be used when caring for this client?

1. Blood pressure cuff
2. Nasogastric tube
3. Indwelling urinary catheter
4. Doppler

ANS: 2

NG tube placement is contraindicated in facial trauma. Blood pressure cuffs are used for most, if not all, clients. An indwelling urinary catheter and Doppler may or may not need to be used when caring for this client.

PTS:1DIF:AnalyzeREF:Nasogastric Tubes

11. A client is prescribed to receive a nasogastric (NG) tube feeding. The nurse realizes that:

-
1. placement does not need to be checked before the feeding.
 2. an NG tube is for long-term uses.
 3. the head of the bed must be 30 degrees or greater.
 4. feeding the client is not a reason for an NG tube.
-

ANS: 3

The head of the clients bed must be at least 30 degrees to decrease the risk of aspiration during feeding. Placement must be checked prior to feeding, and NG tubes are not for long-term use.

PTS: 1 DIF: Apply REF: Feeding Tubes: Patient Preparation

12.A client is diagnosed with insufficient hydrochloric acid in the stomach. The nurse realizes this client will most likely need which of the following vitamin supplements?

-
1. A
 2. B-12
 3. C
 4. D
-

TESTBANKSELLER.COM

ANS: 2

Hydrochloric acid in the stomach triggers pepsinogen, which generates pepsin. Pepsin begins the digestion of proteins in the food and allows for the absorption of vitamin B-12. This is the vitamin supplement that the client will most likely need. Vitamins A, C, and D are not affected by a lack of hydrochloric acid in the stomach.

PTS: 1 DIF: Analyze REF: Stomach

13.A client is scheduled for an abdominal paracentesis. Which of the following should the nurse instruct the client to do before the procedure?

-
1. Empty the bladder.
-

-
- | | |
|----|-------------------------------|
| 2. | Drink a large glass of water. |
|----|-------------------------------|
-
- | | |
|----|------------------|
| 3. | Eat a full meal. |
|----|------------------|
-
- | | |
|----|--------------------------|
| 4. | Sleep for several hours. |
|----|--------------------------|
-

ANS: 1

Prior to an abdominal paracentesis, the nurse should instruct the client to empty the bladder. The client should not drink a large glass of water or eat a full meal before the procedure. Sleeping before the procedure is not helpful.

PTS: 1 DIF: Apply REF: Paracentesis

MULTIPLE RESPONSE

1.A client is being assessed for gastrointestinal system dysfunction. The nurse realizes an impairment of this function could affect which of the following? (Select all that apply.)

-
- | | |
|----|--------------------|
| 1. | Absorption of food |
|----|--------------------|
-
- | | |
|----|-------------------|
| 2. | Digestion of food |
|----|-------------------|
-
- | | |
|----|--------------------|
| 3. | Metabolism of food |
|----|--------------------|
-
- | | |
|----|-----------------------|
| 4. | Utilization of oxygen |
|----|-----------------------|
-
- | | |
|----|------------------------------|
| 5. | Synthesis of red blood cells |
|----|------------------------------|
-
- | | |
|----|--------------------|
| 6. | Filtering of water |
|----|--------------------|
-

ANS: 1, 2, 3

An impairment in a clients gastrointestinal system functioning could affect the absorption of food, the digestion of food, and the metabolism of food. An impairment in this system will not affect the utilization of oxygen, synthesis of red blood cells, or the filtering of water.

PTS: 1 DIF: Analyze REF: Anatomy and Physiology

2.While palpating the abdominal organs during a physical assessment of a client, which of the following organs will the nurse most likely *not* be able to assess? (Select all that apply.)

1.	Liver
2.	Gallbladder
3.	Pancreas
4.	Kidney
5.	Spleen
6.	Colon

ANS: 3, 5

The pancreas is not palpable, and the spleen is not normally palpable during an assessment. The other organs can be palpated during the physical assessment of the abdomen.

PTS: 1 DIF: Apply REF: Assessment

3. During the eating habits portion of an assessment of a client's gastrointestinal system, the nurse should assess which of the following? (Select all that apply.)

1.	Fluid intake
2.	Urine output
3.	Blood pressure
4.	Bowel habits
5.	Food intolerance
6.	Appetite

ANS: 4, 5, 6

When assessing a clients eating habits, the nurse should assess the clients appetite, food intolerances, and bowel habits. Fluid intake, urine output, and blood pressure are not a part of this assessment.

PTS: 1 DIF: Apply REF: Assessment

4.A client is experiencing excessive belching. Which of the following can the nurse instruct this client? (Select all that apply.)

- | | |
|----|-----------------------------|
| 1. | Eat slowly. |
| 2. | Do not drink with a straw. |
| 3. | Avoid carbonated beverages. |
| 4. | Do not smoke. |
| 5. | Do not chew gum. |
| 6. | Avoid eating onions. |

ANS: 1, 2, 3, 4, 5

Excessive belching can be resolved by instructing the client to eat slowly, do not drink with a straw, avoid carbonated beverages, do not smoke, and do not chew gum. Not eating onions might help with flatus.

PTS:1DIF:ApplyREF:Gastrointestinal Gas

5.The nurse is assessing a client for acute appendicitis. Which of the following can be done to help diagnose this disorder? (Select all that apply.)

- | | |
|----|--|
| 1. | Assess for Blumbergs sign. |
| 2. | Assess bowel sounds. |
| 3. | Perform the iliopsoas muscle test. |
| 4. | Auscultate for bowel sounds in the right lower quadrant. |

-
5. Perform the obturator muscle test.
 6. Assess for referred pain to the right shoulder.
-

ANS: 1, 3, 5

Assessment for acute appendicitis includes the use of the Blumbergs sign, iliopsoas muscle test, and the obturator muscle test. Bowel sounds are not affected by acute appendicitis. Referred pain to the right shoulder would indicate a liver dysfunction.

Chapter 21 Nursing Management: Patients with Oral and Esophageal Disorders and Patients Receiving Gastrointestinal Intubation, Enteral, and Parenteral Nutrition

MULTIPLE CHOICE

1. The nurse is instructing a client on the types of carbohydrates to include in the diet. The nurse should include that the main groups of carbohydrates are:

-
1. glucose and fructose.
 2. monosaccharides and disaccharides.
 3. fats and proteins.
 4. sucrose and cellulose.
-

ANS: 2

Carbohydrates should make up 45% to 65% of our calories. Carbohydrates are consumed in the form of monosaccharides and polysaccharides. This is what the nurse should include in the instructions to the client. Glucose, fructose, and sucrose are monosaccharides. Fats and proteins are not carbohydrates. Cellulose is a nondigestible form of a carbohydrate.

PTS:1DIF:ApplyREF:Carbohydrates

2. The nurse is instructing a client on the purpose of eating indigestible carbohydrates such as fiber. These undigestible carbohydrates are used to:

-
1. make fat.

-
- | | |
|----|-----------------|
| 2. | thin the blood. |
|----|-----------------|
-
- | | |
|----|----------------------------|
| 3. | provide bulk to the stool. |
|----|----------------------------|
-
- | | |
|----|-------------------------------|
| 4. | help with digestion of meals. |
|----|-------------------------------|
-

ANS: 3

These indigestible compounds provide bulk to the stool and assist in the process of elimination. Indigestible carbohydrates do not make fat, thin the blood, or help with the digestion of meals.

PTS:1DIF:ApplyREF:Carbohydrates

3.A client is diagnosed with a disorder that affects his ability to digest fat. The nurse realizes that the digestion of fat or lipids requires an enzyme called gastric lipase and:

-
- | | |
|----|-------------------|
| 1. | bile and insulin. |
|----|-------------------|
-
- | | |
|----|-----------------------------|
| 2. | bile and pancreatic lipase. |
|----|-----------------------------|
-
- | | |
|----|------------------------------------|
| 3. | pancreatic lipase and cholesterol. |
|----|------------------------------------|
-
- | | |
|----|-----------------------|
| 4. | bile and amino acids. |
|----|-----------------------|
-

ANS: 2

Most fat digestion takes place in the small intestines through the actions of pancreatic lipase and bile. Insulin does not digest fat. Cholesterol is a type of lipid. Amino acids are elements of protein.

PTS: 1 DIF: Analyze REF: Lipids (Fats)

4.A client is diagnosed with a protein deficiency. The nurse realizes this client may have a disorder that is affecting which of the following?

-
- | | |
|----|----------|
| 1. | Pancreas |
|----|----------|
-
- | | |
|----|-------------|
| 2. | Gallbladder |
|----|-------------|
-
- | | |
|----|------------------|
| 3. | Small intestines |
|----|------------------|
-

-
4. Colon

ANS: 3

Protein digestion begins in the stomach. Further digestion of this nutrient takes place in the small intestine. The pancreas, gallbladder, and colon do not digest protein.

PTS: 1 DIF: Analyze REF: Proteins

5.The nurse, instructing a client on the best way to maintain a healthy diet and proper nutrition, would encourage the client to:

-
1. eat foods in appropriate portion size and from all the food groups.
-
2. eat twice as much meats as grains.
-
3. eat mostly fruits.
-
4. skip milk products.

ANS: 1

TESTBANKSELLER.COM

According to the American Dietetic Association, all foods can fit in a healthy diet if the portion sizes are appropriate, foods are consumed in moderation, and regular physical activity is included. Eating twice as much meat as grains does not contribute to a healthy diet. Eating mostly fruits and skipping milk products does not contribute to a healthy diet.

PTS: 1 DIF: Apply REF: Components of a Nutritionally Adequate Diet

6.The nurse is discussing an elderly clients diet and nutritional status with the hospital dietitian because this client is at risk for:

-
1. obesity.
-
2. malnutrition.
-
3. sodium imbalance.
-
4. a blood disorder.

ANS: 2

Causes of malnutrition during a hospital stay include disease state or inadequate food intake because of pain, nausea, and the different types of foods available in the hospital. An elderly client in the hospital is not at risk for obesity. A sodium imbalance can occur both prior to or during a hospital stay. The nurse would not be discussing a clients nutritional status with a dietitian because a client is a risk for a blood disorder.

PTS: 1 DIF: Apply REF: Malnutrition: Etiology

7.The nurse, instructing a group of community members regarding diet and exercise, should instruct healthy adults and children to exercise for:

- | | |
|----|---------------------|
| 1. | 120 minutes a week. |
| 2. | 240 minutes a week. |
| 3. | 80 minutes a week. |
| 4. | 150 minutes a week. |

ANS: 4

Exercise and nutrition go hand-in-hand to prevent chronic disease. Most Americans can enhance their health by moderate aerobic exercise for at least 150 minutes each week. The other choices are either too little exercise or too much exercise to enhance health.

PTS: 1 DIF: Apply REF: Components of a Nutritionally Adequate Diet

8.While instructing a client on the bodys nutritional needs, the nurse would include that the majority of calories consumed daily should be supplied from:

- | | |
|----|----------------|
| 1. | vitamins. |
| 2. | fats. |
| 3. | carbohydrates. |
| 4. | proteins. |

ANS: 3

To meet the body's nutritional needs, 45% to 65% of calories should come from carbohydrates. Fat intake should be limited to 20% to 35% of daily calories. Protein intake should be limited to 10% to 35% of total calories. There is no percentage of caloric intake each day for vitamins.

PTS: 1 DIF: Apply REF: Components of a Nutritionally Adequate Diet

9. A client is instructed to avoid specific foods while prescribed a specific medication because of the potential for cytochrome P450 3A to be blocked, which will affect metabolism. Which of the following foods should the client be instructed to avoid?

- | | |
|----|------------------|
| 1. | Apple juice |
| 2. | Prune juice |
| 3. | Grape juice |
| 4. | Grapefruit juice |

ANS: 4

Grapefruit juice can block P-glycoprotein, and it inactivates cytochrome P450 3A for up to 24 hours. Other foods that can also block these enzymes include red wine, cyclosporine, St. John's wort, and herbal teas. Apple, prune, and grape juice do not block these enzymes.

PTS: 1 DIF: Apply REF: Cytochrome P450

10. A client who has completely eliminated fats from the diet should be assessed for a deficiency of:

- | | |
|----|--------------------------|
| 1. | bile. |
| 2. | minerals. |
| 3. | salt. |
| 4. | vitamins A, D, E, and K. |

ANS: 4

Fat is used to transport digested substances and fat-soluble vitamins. The client who has eliminated fats from the diet should be assessed for a deficiency of the fat-soluble vitamins, or vitamins A, D, E, and K.

PTS: 1 DIF: Analyze REF: Metabolism

11. The nurse should instruct a client who is prescribed warfarin to limit or avoid foods that are:

-
1. high in proteins.
 2. high in sugars.
 3. high in vitamin K.
 4. low in proteins.
-

ANS: 3

Significant changes in the intake of foods high in vitamin K can interfere with the anticoagulation properties of warfarin. These foods include bananas; celery; broccoli; green, leafy vegetables; spinach; and liver. The client who is prescribed warfarin does not need to avoid foods that are high in protein or sugar. The client does not need to avoid foods that are low in protein.

PTS: 1 DIF: Apply REF: Warfarin

TESTBANKSELLER.COM

12. While obtaining the health history, it is important for the nurse to ask the client about the use of herbal products, over-the-counter remedies, and dietary supplements because:

-
1. they should be stopped during admission to the hospital.
 2. they should be increased during a time of illness.
 3. these are not important items to talk about and should not be asked about.
 4. these products may have potential interactions with medications that are being prescribed.
-

ANS: 4

Herbal products, over-the-counter remedies, and dietary supplements may have potential interactions with medications that are being prescribed, and a history of use of these products is necessary to avoid possible drug interactions.

PTS: 1 DIF: Apply REF: Natural Products

13. A client is diagnosed as being obese. The nurse realizes that this client's body mass index (BMI) is most likely:

-
1. between 15 and 19.

 2. between 20 and 24.

 3. between 25 and 29.

 4. greater than 30.

ANS: 4

A BMI of 25 to 29 is considered overweight. A BMI greater than 30 is considered obese. A BMI less than 18.5 is considered underweight. A BMI between 18.5 and 24.9 is considered normal weight.

PTS: 1 DIF: Analyze REF: Box 48-3 BMI Categories

14.A client with a BMI of 30 has set a goal to lose 10% of her current body weight within 6 months. The nurse realizes that the safest level of weight loss for this client would be:

-
1. 1 to 2 pounds each week.

 2. 3 pounds each week.

 3. 4 to 5 pounds each week.

 4. 6 pounds each week.

ANS: 1

To reduce body weight by 10% for a person with a BMI of 30 would be to lose 1 to 2 lbs each week. The other amounts of weekly weight loss might lead to nutritional disorders.

PTS: 1 DIF: Analyze REF: Obesity: Planning and Implementation

15.While caring for a client with a gastrostomy (GT) tube, it is important for the nurse to:

-
1. clean it weekly.

 2. flush it with water before and after the feeding.

-
3. change the GT tube daily.
 4. not give medications through the tube.
-

ANS: 2

The GT site should be cleaned daily, and it is important that the nurse flush the tube with water before and after the feedings and medication administration to avoid tube obstruction. The site of the tube should be cleaned daily. The GT is not changed daily. The GT is used for medication administration.

PTS: 1 DIF: Apply REF: Gastrostomy Feedings

16.A client, diagnosed with renal failure, is prescribed enteral nutrition. The enteral food product will contain which of the following ?

-
1. Lower protein content
 2. Higher fat content
 3. Lower calorie content
 4. Lower carbohydrate content
-

ANS: 1

Enteral feedings to support a client diagnosed with renal failure will have a lower protein content. Feedings for these clients will not have a higher fat content, lower calorie content, or lower carbohydrate content.

PTS: 1 DIF: Apply REF: Table 48-2 Enteral Nutrition Formulary

MULTIPLE RESPONSE

1.The nurse is instructing a client on the advantages of following a nutritionally adequate diet. Which of the following should be included in these instructions? (Select all that apply.)

-
1. Varies during the life cycle
 2. Supports key body systems
-

- | | |
|----|--------------------------------------|
| 3. | Supports a healthy weight |
| 4. | Tastes good |
| 5. | Helps prevent chronic disease |
| 6. | Is low in carbohydrates and proteins |

ANS: 1, 2, 3, 5

A nutritionally adequate diet is one that meets the needs of the individual at that stage of his life cycle, supports the body systems, and maintains proper weight. A nutritionally adequate diet will also help prevent the onset of chronic disease. A nutritionally adequate diet is not low in carbohydrates and proteins.

PTS: 1 DIF: Apply REF: Components of a Nutritionally Adequate Diet

2.The nurse is helping a dietitian determine a clients resting energy expenditure (REE). Which of the following will influence this clients REE? (Select all that apply.)

- | | |
|----|-------------------|
| 1. | Age |
| 2. | Diet |
| 3. | BMI |
| 4. | Chronic illnesses |
| 5. | Urine output |
| 6. | Appetite |

ANS: 1, 2, 3, 4

The resting energy expenditure (REE) is the amount of calories needed to maintain body weight at rest. The REE is influenced by age, diet, BMI, and chronic illness. Urine output and appetite are not known to influence a clients REE.

PTS:1DIF:AnalyzeREF:Metabolic Rate

3.The nurse is concerned that a client with a gastrostomy feeding tube is developing a complication. Which of the following are considered complications associated with this type of feeding tube? (Select all that apply.)

1.	Nausea
2.	Vomiting
3.	Aspiration
4.	Abdominal distention
5.	Leg cramps
6.	Muscle pain

ANS: 1, 2, 3, 4

Complications of a gastrostomy tube include nausea, vomiting, malabsorption, aspiration, abdominal distention, tube obstruction, diarrhea, and constipation. Leg cramps and muscle pain are not complications of a gastrostomy tube.

Chapter 22 Nursing Management: Patients with Gastric and Duodenal Disorders

MULTIPLE CHOICE

1.Before administering an antacid, the nurse should instruct a client that this medication works in the:

1.	blood.
2.	stomach.
3.	small intestine.
4.	esophagus.

ANS: 2

Antacids work in the stomach to neutralize stomach acids. They do not work in the esophagus or small intestines. Antacids do not work in the blood.

PTS: 1 DIF: Apply REF: Gastroesophageal Reflux Disease: Pharmacology

2.The nurse is assessing a client diagnosed with gastroesophageal reflux disease. Which of the following should be included in this assessment?

-
- | | |
|----|-------------------------|
| 1. | Degree of mouth burning |
|----|-------------------------|
-
- | | |
|----|-----------------------|
| 2. | Difficulty swallowing |
|----|-----------------------|
-
- | | |
|----|---------------------|
| 3. | Presence of pyrosis |
|----|---------------------|
-
- | | |
|----|--------------------|
| 4. | Painful swallowing |
|----|--------------------|
-

ANS: 3

Mouth burning is not a symptom of gastroesophageal reflux disease. Difficulty swallowing or dysphagia is not associated with gastroesophageal reflux disease. Pain when swallowing is associated with esophagitis, not acid reflux disease. Presence of pyrosis or heartburn should be assessed in this client.

TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Gastroesophageal Reflux Disease

3.During an assessment, the nurse determines a client is at risk for ulcerative stomatitis and gum disease because the client has a history of:

-
- | | |
|----|-----------------|
| 1. | alcohol intake. |
|----|-----------------|
-
- | | |
|----|----------|
| 2. | smoking. |
|----|----------|
-
- | | |
|----|----------|
| 3. | kissing. |
|----|----------|
-
- | | |
|----|---------|
| 4. | eating. |
|----|---------|
-

ANS: 2

Clients who smoke have seven times the risk of developing gum disease. Alcohol intake increases the risk of throat cancer. Ulcerative stomatitis and gum disease is not associated with kissing or eating.

PTS: 1 DIF: Analyze REF: Disorders of the Oral Cavity

4.A client is diagnosed with a swallowing disorder. The nurse realizes that which type of diet would be indicated for this client? ?

-
- | | |
|----|--------------|
| 1. | Regular diet |
|----|--------------|
-
- | | |
|----|-------------------|
| 2. | Clear liquid diet |
|----|-------------------|
-
- | | |
|----|----------------------|
| 3. | Mechanical soft diet |
|----|----------------------|
-
- | | |
|----|--------------|
| 4. | Low-fat diet |
|----|--------------|

ANS: 3

Some clients may need a pureed diet or mechanical soft diet, especially if their swallowing difficulty is with the oral phase. Some clients may have difficulty swallowing thin liquids and foods that are tough. The client will most likely have difficulty with a regular or low-fat diet.

PTS:1DIF:AnalyzeREFysphagia: Nutrition

5.To support the nutritional needs of a client with dysphagia, the nurse realizes that all of the following are mechanisms to provide enteral feeding EXCEPT:

TESTBANKSELLER.COM

-
- | | |
|----|-------------------|
| 1. | nasogastric tube. |
|----|-------------------|
-
- | | |
|----|---|
| 2. | percutaneous endoscopic gastrostomy (PEG) tube. |
|----|---|
-
- | | |
|----|-------------------|
| 3. | jejunostomy tube. |
|----|-------------------|
-
- | | |
|----|--------------------|
| 4. | hyperalimentation. |
|----|--------------------|

ANS: 4

Hyperalimentation is associated with parenteral nutrition, not enteral nutrition. The others are forms of administration of nutrients into the gastrointestinal tract.

PTS:1DIF:AnalyzeREFysphagia: Nutrition

6.A client is scheduled for diagnostic tests to determine the ability to swallow. Which of the following diagnostic tests will provide the best information regarding this clients status?

-
- | | |
|----|---------------------------|
| 1. | Pulse oximetry with water |
|----|---------------------------|

-
- | | |
|----|---------------------------------|
| 2. | Esophageal transit scintigraphy |
|----|---------------------------------|
-
- | | |
|----|------------------|
| 3. | Videofluoroscopy |
|----|------------------|
-
- | | |
|----|----------------------|
| 4. | Esophageal manometry |
|----|----------------------|
-

ANS: 3

The gold standard for evaluation of dysphagia is videofluoroscopy or a modified barium swallow. This test demonstrates the swallowing mechanism. The other tests may be prescribed; however, they do not provide as much information as the videofluoroscopy.

PTS: 1 DIF: Analyze REF: Dysphagia: Diagnostic Tests

7.A client, diagnosed with a hiatal hernia, will experience which of the following symptoms most frequently?

-
- | | |
|----|--------|
| 1. | Nausea |
|----|--------|
-
- | | |
|----|----------|
| 2. | Vomiting |
|----|----------|
-
- | | |
|----|----------|
| 3. | Diarrhea |
|----|----------|
-
- | | |
|----|-----------|
| 4. | Heartburn |
|----|-----------|
-

ANS: 4

With a hiatal hernia, stomach acids reflux into the esophagus, causing pain and irritation that the patient will associate with heartburn. Nausea, vomiting, and diarrhea are not symptoms typically associated with a hiatal hernia.

PTS:1DIF:Analyze

REF: Hiatal Hernia: Assessment with Clinical Manifestations

8.The nurse is instructing a client diagnosed with a hiatal hernia on ways to reduce the symptoms. Which of the following should be included in these instructions?

-
- | | |
|----|---|
| 1. | Eat large meals to keep the stomach full. |
|----|---|
-
- | | |
|----|--|
| 2. | Drink lots of liquids so that the stomach does not have to work so hard. |
|----|--|
-

3. Avoid lying down after meals.

4. Lie down after eating.

ANS: 3

Sitting upright or sleeping with the head of the bed elevated helps keep the stomach contents in the stomach. The meal size should be smaller, and meals should be eaten more often so as not to overfill the stomach.

PTS:1DIF:Apply

REF: Hiatal Hernia: Planning and Implementation; Patient Playbook: Patient Education for GERD

9.A client is diagnosed with burning mouth syndrome. Which of the following interventions should be included in this clients plan of care?

1. Assess the condition of the clients teeth.

2. Collect a saliva specimen for analysis.

TESTBANKSELLER.COM

3. Tell the client to avoid vitamin supplements.

4. Teach the client how to conduct an oral self-assessment daily.

ANS: 1

Interventions for a client diagnosed with burning mouth syndrome include assessing the condition of the teeth. A saliva specimen is not used to diagnose this disorder. Vitamin supplements do not contribute to this disorder. An oral self-assessment does not need to be completed every day.

PTS:1DIF:Apply

REF: Burning Mouth Syndrome: Planning and Implementation

10.During an assessment, the nurse learns that a client is inhaling while swallowing food. Which of the following does this assessment finding suggest to the nurse?

1. The client is recovering from a stroke.

-
- | | |
|----|---------------------------------------|
| 2. | The client is at risk for aspiration. |
|----|---------------------------------------|
-
- | | |
|----|---------------------------------------|
| 3. | The client will experience dyspepsia. |
|----|---------------------------------------|
-
- | | |
|----|---|
| 4. | The client has esophageal reflux disease. |
|----|---|
-

ANS: 2

In clients with dysphagia, inspiration commonly occurs during swallowing. This increases the risk for aspiration. This assessment finding does not indicate that the client is recovering from a stroke. This assessment finding does not indicate that the client will experience dyspepsia or that the client has esophageal reflux disease.

PTS:1DIF:Analyze

REF: Dysphagia: Assessment with Clinical Manifestations

11.A client is experiencing brash water. The nurse realizes this symptom is associated with:

-
- | | |
|----|--------------|
| 1. | oral cancer. |
|----|--------------|
-
- | | |
|----|-----------------|
| 2. | gastric ulcers. |
|----|-----------------|
-
- | | |
|----|------------|
| 3. | dysphagia. |
|----|------------|
-
- | | |
|----|---------------------|
| 4. | Barretts esophagus. |
|----|---------------------|
-

ANS: 4

Brash water, or the sensation of the mouth filling with saliva because of acid backflow into the esophagus, is a symptom of Barretts esophagus. Brash water is not associated with oral cancer, gastric ulcers, or dysphagia.

PTS:1DIF:AnalyzeREF:Barretts Esophagus

12.A client has been prescribed Zantac for gastroesophageal reflux disease. The nurse realizes this medication is classified as a:

-
- | | |
|----|-----------------------------------|
| 1. | histamine H2-receptor antagonist. |
|----|-----------------------------------|
-
- | | |
|----|------------------------|
| 2. | proton pump inhibitor. |
|----|------------------------|
-

3. prokinetic agent.

4. antihistamine.

ANS: 1

Zantac is a histamine H₂-receptor antagonist. This medication is not classified as being a proton pump inhibitor, prokinetic agent, or antihistamine.

PTS: 1 DIF: Analyze REF: Gastroesophageal Reflux Disease: Pharmacology

13.A client is diagnosed with peptic ulcer disease caused by NSAID use. Which of the following would be indicated for this client?

1. Antibiotic therapy

2. Treatment similar to a client with peptic ulcer disease

3. Preparation for surgery

4. Insertion of a nasogastric tube for gastric lavage

ANS: 2

For clients diagnosed with peptic ulcer disease caused by NSAID use, the anti-inflammatory medication should be discontinued and the client should receive treatment similar to that of peptic ulcer disease. Surgery is not indicated. Antibiotics are not indicated. Gastric lavage is not indicated.

PTS:1DIF:Analyze

REF: Peptic-Ulcer Dyspepsia: Complications of PUD and the Subsequent Therapy

MULTIPLE RESPONSE

1.The nurse is instructing a client about symptoms associated with peptic ulcer disease. Which of the following should be included in these instructions? (Select all that apply.)

1. Abdominal pain

2. Pain in the middle of the night

-
- | | |
|----|-------------|
| 3. | Weight loss |
|----|-------------|
-
- | | |
|----|---------------|
| 4. | Poor appetite |
|----|---------------|
-
- | | |
|----|----------|
| 5. | Bloating |
|----|----------|
-
- | | |
|----|--------------|
| 6. | Constipation |
|----|--------------|
-

ANS: 1, 2, 3, 4, 5

Symptoms of peptic ulcer disease include abdominal pain, pain in the middle of the night; weight loss; poor appetite; and bloating. Constipation is not a symptom of peptic ulcer disease.

PTS:1DIF:Apply

REF: Peptic-Ulcer Dyspepsia: Etiology; Assessment with Clinical Manifestations

2.The nurse is planning care for a client diagnosed with oral ulcers. Which of the following should be included in this clients plan of care? (Select all that apply.)

-
- | | |
|----|---------------------------------|
| 1. | Encourage frequent oral hygiene |
|----|---------------------------------|
-
- | | |
|----|---------------------------------|
| 2. | Rinse mouth with chlorhexidine. |
|----|---------------------------------|
-
- | | |
|----|-------------------------------------|
| 3. | Increase consumption of hot fluids. |
|----|-------------------------------------|
-
- | | |
|----|---|
| 4. | Instruct in the use of topical corticosteroids. |
|----|---|
-
- | | |
|----|--|
| 5. | Encourage the client to limit smoking. |
|----|--|
-
- | | |
|----|--------------------------------|
| 6. | Avoid the use of dental floss. |
|----|--------------------------------|
-

ANS: 1, 2, 4

Good oral hygiene is essential, and rinsing the mouth with chlorhexidine is recommended. Topical corticosteroids can promote resolution of the ulcers. Drinking hot fluids and smoking may aggravate oral ulcerations and are not included in the plan of care. The client should be instructed to not smoke at all. Dental floss will not cause oral ulcers.

PTS:1DIF:Apply

REF disorders of the Oral Cavity: Planning and Implementation

3. The nurse is instructing a client on conducting an oral self-assessment. Which of the following should be included in the nurses instructions? (Select all that apply.)

-
- | | |
|----|------------------------------|
| 1. | Check the face for symmetry. |
|----|------------------------------|
-
- | | |
|----|-------------------------------------|
| 2. | Check skin on the face for changes. |
|----|-------------------------------------|
-
- | | |
|----|--|
| 3. | Check the neck for swellings or lumps. |
|----|--|
-
- | | |
|----|--|
| 4. | Check inside of cheeks for tenderness. |
|----|--|
-
- | | |
|----|-------------------------------|
| 5. | Check the tongue for changes. |
|----|-------------------------------|
-
- | | |
|----|----------------------------------|
| 6. | Check urine for change in color. |
|----|----------------------------------|

ANS: 1, 2, 3, 4, 5

When instructing a client on an oral self-assessment, the nurse should include having the client check the face for symmetry; the skin on the face for changes; the neck for swellings or lumps; the inside of the cheeks for tenderness; and the tongue for changes. The urine is not checked when doing an oral self-assessment.

PTS: 1 DIF: Apply REF: Patient Playbook: Oral Cancer Self-Assessment

4. The nurse is assisting a client with indirect techniques to improve swallowing. Which of the following are techniques included in the nurses assistance? (Select all that apply.)

-
- | | |
|----|---------------------------|
| 1. | Tongue mobility exercises |
|----|---------------------------|
-
- | | |
|----|--------------------|
| 2. | Application of ice |
|----|--------------------|
-
- | | |
|----|--------------------------------|
| 3. | Repetitive head lift exercises |
|----|--------------------------------|
-
- | | |
|----|-------------|
| 4. | Positioning |
|----|-------------|
-
- | | |
|----|--|
| 5. | Range-of-motion exercises for the neck |
|----|--|

-
6. Range-of-motion exercises for the shoulders

ANS: 1, 2, 3

Indirect techniques to improve swallowing include tongue mobility exercises, application of ice, and repetitive head lift exercises. Positioning is a compensatory mechanism. Range-of-motion exercises for the neck or shoulders does not help improve swallowing.

PTS: 1 DIF: Apply REF: Box 49-4 Techniques of Swallowing Therapy

5.A client is diagnosed with esophageal pain. Which of the following medications would be indicated for this client? (Select all that apply.)

-
1. Vasodilators
-
2. Calcium channel blockers
-
3. Isosorbide dinitrate
-
4. Antibiotics
-
5. Antipyretics
-
6. Antihistamines

ANS: 1, 2, 3

The first line of treatment for esophageal pain is often the same medications used to treat angina of cardiac origin and would include vasodilators, calcium channel blockers, and isosorbide dinitrate. Antibiotics, antipyretics, and antihistamines are not medications used to treat esophageal pain.

Chapter 23 Nursing Management: Patients with Intestinal and Rectal Disorders**MULTIPLE CHOICE**

1. In caring for a client diagnosed with a small bowel obstruction, what would the nurse expect to do first?

-
- | | |
|----|--|
| 1. | Prepare to put in a nasogastric (NG) tube. |
|----|--|
-
- | | |
|----|-----------------------|
| 2. | Give pain medication. |
|----|-----------------------|
-
- | | |
|----|----------------|
| 3. | Draw lab work. |
|----|----------------|
-
- | | |
|----|---------------------------------|
| 4. | Start an intravenous (IV) line. |
|----|---------------------------------|

ANS: 4

TESTBANKSELLER.COM

Starting an IV to give fluids and electrolytes would be the first step in caring for this client. Although an NG tube will be ordered, fluid balance is more important. Administering pain medication may make the problem worse. Drawing lab work would not be the first intervention needed for this client.

PTS: 1 REF: Surgery

2. The nurse, instructing a client about malabsorption syndrome, should include that food is absorbed in the:

-
- | | |
|----|--------|
| 1. | mouth. |
|----|--------|
-
- | | |
|----|--------------|
| 2. | bloodstream. |
|----|--------------|
-
- | | |
|----|----------|
| 3. | stomach. |
|----|----------|
-
- | | |
|----|------------------|
| 4. | small intestine. |
|----|------------------|

ANS: 4

The mouth and stomach are used mostly for digestion. The small intestine is where most of the absorption of food nutrients occurs. Food is not directly absorbed into the bloodstream.

PTS: 1 DIF: Apply REF: Small Intestine: Absorption of Nutrients

3.A client is diagnosed with appendicitis. One of the laboratory tests the nurse would expect to monitor would be:

-
1. serum sodium.
 2. white blood cell (WBC) count.
 3. hemoglobin (Hgb) and hematocrit (Hct).
 4. bilirubin level.
-

ANS: 2

Infection often accompanies the inflammation of the appendix. The nurse would be looking for an elevated WBC count. Serum sodium, hemoglobin, hematocrit, and bilirubin levels are not necessarily indicated in the care of a client diagnosed with appendicitis.

TESTBANKSELLER.COM

PTS: 1 DIF: Analyze REF: Appendicitis: Diagnostic Tests

4.When assessing the pain in a client diagnosed with appendicitis, the nurse would expect to assess:

-
1. extreme pain with slight palpation anywhere on the abdomen.
 2. pain in the upper back when the right lower quadrant is palpated.
 3. more pain when the pressure is released in the right lower quadrant.
 4. no pain when the abdomen is palpated.
-

ANS: 3

Typically rebound pain is associated with appendicitis. Rebound pain is described as more pain when pressure is released than when pressure is applied. Appendicitis pain is not associated with pain anywhere on the abdomen upon slight palpation. Appendicitis pain is not typically assessed in the upper back. Appendicitis is associated with pain.

PTS: 1 DIF: Apply REF: Box 50-3 Rovsings Sign for Appendicitis

5.A client is being evaluated for symptoms associated with diverticular disease. The nurse realizes that the best diagnostic test to be used to aid in this diagnosis would be:

1. computed tomography (CT) scan.
2. barium enema.
3. ultrasound.
4. x-ray study.

ANS: 1

A CT scan is the best method of detecting abscesses and complications evidenced in diverticulitis. Barium enema is contraindicated in acute diverticulitis because of the risk of contamination if there is an existing perforation. An ultrasound or x-rays would not adequately diagnose the presence of the disorder.

PTS: 1 DIF: Analyze REF: Diverticulitis: Diagnostic Tests

6.An elderly client has noted blood in her stool for the past few months. Which information in the medical history would strongly suggest colorectal cancer?

1. Increased bouts of vomiting
2. Change in bowel habits
3. Recent infection in the blood
4. Decrease in appetite

ANS: 2

Change in bowel habits is one of the seven danger signals for cancer. Changes in bowel habits and blood in the stool are common signs of colorectal cancer. Vomiting, decreased appetite, or recent blood infection could be symptoms of other health problems, but they are not necessarily colorectal cancer.

PTS:1DIF:Analyze

REF:Colorectal Cancer: Assessment with Clinical Manifestations

7.The nurse is caring for a client diagnosed with irritable bowel syndrome (IBS) who is experiencing diarrhea. What medication would the nurse expect to administer?

-
- | | |
|----|----------------------|
| 1. | Loperamide (Imodium) |
|----|----------------------|
-
- | | |
|----|--------------------------|
| 2. | Docusate sodium (Colace) |
|----|--------------------------|
-
- | | |
|----|--------------------|
| 3. | Lorazepam (Ativan) |
|----|--------------------|
-
- | | |
|----|----------------------|
| 4. | Haloperidol (Haldol) |
|----|----------------------|

ANS: 1

Antidiarrheal agents like Imodium can be given prophylactically or symptomatically on an as-needed basis. Docusate sodium (Colace), lorazepam (Ativan), and haloperidol (Haldol) are not indicated to treat this disorder.

PTS: 1 DIF: Apply REF: Irritable Bowel Syndrome: Pharmacology

8.A client complains of acute gastrointestinal distress. While obtaining a health history, the nurse asks about the family history. Which disorder has a familial basis?

TESTBANKSELLER.COM

-
- | | |
|----|-----------|
| 1. | Hepatitis |
|----|-----------|
-
- | | |
|----|--------------------|
| 2. | Ulcerative colitis |
|----|--------------------|
-
- | | |
|----|--------------|
| 3. | Appendicitis |
|----|--------------|
-
- | | |
|----|--------------------|
| 4. | Bowel obstructions |
|----|--------------------|

ANS: 2

Genetic factors have been identified as susceptibility factors for the development of ulcerative colitis. None of the other choices have a genetic predisposition for developing the disorder.

PTS: 1 DIF: Analyze REF: Inflammatory Bowel Disorders

9.A client diagnosed with appendicitis asks the nurse why this illness occurred. The nurse should respond that the most common cause of appendicitis is:

-
- | | |
|----|---------------------|
| 1. | ulcerative colitis. |
|----|---------------------|

2. obstruction of the appendix.

3. low-fat diet.

4. infection.

ANS: 2

An infection may occur with appendicitis, but the most common cause of infection is an obstruction of the appendix. The obstruction could be caused by lymph tissue, a fecalith, a foreign body, or worms. Ulcerative colitis, low-fat diet, or infection does not cause appendicitis.

PTS: 1 DIF: Apply REF: Appendicitis: Pathophysiology

10.A young client is experiencing acute abdominal pain. The nurse realizes that the most common cause for this type of pain would be:

1. appendicitis.

2. biliary tract disease.

TESTBANKSELLER.COM

3. kidney stones.

4. urinary tract infection.

ANS: 1

The most common cause of acute abdominal pain is appendicitis. Biliary tract disease is the most common disorder in the elderly, causing pain in the right upper quadrant. Kidney stones and urinary tract infections do not necessarily cause abdominal pain.

PTS:1DIF:AnalyzeREF:Acute Abdomen

11.A client experiencing abdominal pain and diarrhea tells the nurse that he used to smoke. Which of the following gastrointestinal disturbances is this client most likely experiencing?

1. Irritable bowel syndrome

2. Crohns disease

3. Acute appendicitis

4. Small bowel obstruction

ANS: 2

Current and former smokers appear to have a greater risk of developing Crohns disease than nonsmokers. Not smoking will not cause irritable bowel syndrome, acute appendicitis, or small bowel obstruction.

PTS:1DIF:Analyze

REF:Inflammatory Bowel Disorders: Planning and Implementation

12.A client has a history of being treated for ulcerative colitis. The nurse realizes that a life-threatening complication of this disorder is:

1. Crohns disease.

2. small bowel obstruction.

3. TESTBANKSELLER.COM
peptic ulcer disease.

4. toxic megacolon.

ANS: 4

Toxic megacolon is a life-threatening complication of ulcerative colitis, and it requires immediate surgical intervention. Crohns disease, small bowel obstruction, and peptic ulcer disease are not life-threatening complications of ulcerative colitis.

PTS: 1 DIF: Analyze REF: Ulcerative Colitis: Pathophysiology

13.The nurse assesses no bowel sounds with occasional splashing sounds over the large intestines. Which of the following do these assessment findings suggest to the nurse?

1. Ulcerative colitis

2. Irritable bowel syndrome

3. Appendicitis

-
4. Bowel obstruction

ANS: 4

Obstruction can be detected with absent bowel sounds and borborygmi or a splashing sound heard over the large intestine. Absent bowel sounds and borborygmi are not associated with ulcerative colitis, irritable bowel syndrome, or appendicitis.

PTS:1DIF:Analyze

REF:Acute Abdomen: Assessment with Clinical Manifestations

14.The nurse is instructing a client on diagnostic tests used to screen for colorectal cancer. Which of the following should be included in these instructions?

-
1. A digital rectal exam should be done annually.
-
2. A test for fecal occult blood should be done annually.
-
3. A flexible sigmoidoscopy should be done annually.
-
4. A colonoscopy should be done every 5 years after age 40.

ANS: 2

The nurse should instruct the client to have a fecal occult blood test done annually. A digital rectal exam is not a recommendation for this disease process. A flexible sigmoidoscopy should be done every 5 years after age 50. A colonoscopy should be done every 10 years after age 50.

PTS: 1 DIF: Apply REF: Table 50-8: Screening and Detection: CRC

MULTIPLE RESPONSE

1.Laparoscopic surgery is scheduled for a client diagnosed with appendicitis. Which of the following may be a result of laparoscopic surgery? (Select all that apply.)

-
1. No risk of infection
-
2. Less pain
-
3. Faster recovery times
-
4. Maybe more complications

-
- | | |
|----|------------------------|
| 5. | Shorter hospital stays |
|----|------------------------|
-
- | | |
|----|--|
| 6. | Better visualization of the abdominal organs |
|----|--|
-

ANS: 2, 3, 5

Laparoscopic surgery has less pain and faster recovery times. There are fewer complications, less bleeding, and less risk of infection so the client has a shorter hospital stay. A risk of infection is present with all surgical procedures. Laparoscopic surgery does not cause a better visualization of the abdominal organs.

PTS:1DIF:AnalyzeREF:Appendicitis: Surgery

2.The nurse is assessing a client diagnosed with diverticulitis. Which of the following are clinical manifestations associated with this disorder? (Select all that apply.)

-
- | | |
|----|--------------------------|
| 1. | Constipation or diarrhea |
|----|--------------------------|
-
- | | |
|----|------------------------------------|
| 2. | Left lower quadrant abdominal pain |
|----|------------------------------------|
-
- | | |
|----|-----------------|
| 3. | Low-grade fever |
|----|-----------------|
-
- | | |
|----|------------------------|
| 4. | Increased excitability |
|----|------------------------|
-
- | | |
|----|-----------------------------------|
| 5. | Changes in level of consciousness |
|----|-----------------------------------|
-
- | | |
|----|--------|
| 6. | Thirst |
|----|--------|
-

ANS: 1, 2, 3

In diverticulitis, there may be a chronic asymptomatic condition two-thirds of the time. If there are manifestations, they would likely be constipation or diarrhea, lower abdominal pain in the left lower quadrant, and low-grade fever. Increased excitability, changes in level of consciousness, and thirst are not clinical manifestations of diverticulitis.

PTS:1DIF:Analyze

REF: Diverticulitis: Assessment with Clinical Manifestations

3.The nurse is assessing a client diagnosed with irritable bowel syndrome (IBS). Which of the following characteristics are associated with this disorder? (Select all that apply.)

-
- | | |
|----|--------------------------|
| 1. | Recurrent abdominal pain |
|----|--------------------------|
-
- | | |
|----|--|
| 2. | Abdominal pain that improves with defecation |
|----|--|
-
- | | |
|----|--|
| 3. | Pain associated with a change in stool frequency |
|----|--|
-
- | | |
|----|---|
| 4. | Pain associated with a change in stool appearance |
|----|---|
-
- | | |
|----|---|
| 5. | Pain that occurs only during defecation |
|----|---|
-
- | | |
|----|-------------------------------------|
| 6. | Pain associated with passing flatus |
|----|-------------------------------------|
-

ANS: 1, 2, 3, 4

IBS is relatively common and is a motility disorder of the gastrointestinal tract. It is characterized by recurrent abdominal pain that improves with defecation. The pain will also appear with a change in stool frequency. The pain is also associated with a change in stool appearance. The pain of IBS does not occur only during defecation and is not associated with passing flatus.

TESTBANKSELLER.COM

PTS:1DIF:Analyze

REF: Irritable Bowel Syndrome: Assessment with Clinical Manifestations

4.A client, diagnosed with a vitamin B-12 deficiency, tells the nurse that she does not want to receive injections every month to treat the disorder. Which of the following should the nurse instruct the client regarding the effects of vitamin B-12 deficiency? (Select all that apply.)

-
- | | |
|----|---------------------------|
| 1. | Paresthesias in the hands |
|----|---------------------------|
-
- | | |
|----|--------------------------|
| 2. | Paresthesias in the feet |
|----|--------------------------|
-
- | | |
|----|--------|
| 3. | Ataxia |
|----|--------|
-
- | | |
|----|--------------------------|
| 4. | Spinal cord degeneration |
|----|--------------------------|
-
- | | |
|----|----------------|
| 5. | Loss of memory |
|----|----------------|
-

-
6. Loss of the sense of smell

ANS: 1, 2, 3, 4

Vitamin B-12 deficiency produces neurological abnormalities such as symmetrical paresthesias in the hands and feet, diminished vibratory and proprioceptive sense, ataxia, and spinal cord degeneration. Vitamin B-12 deficiency does not produce memory loss or loss of smell.

PTS: 1 DIF: Apply REF: Cobalamin

5.The nurse is planning care for a client diagnosed with an acute abdomen. Which of the following nursing diagnoses would be appropriate for this client? (Select all that apply.)

-
1. Fear
-
2. Deficient fluid volume
-
3. Ineffective coping
-
4. Acute pain
-
5. Risk of infection
-
6. Altered self-perception

ANS: 1, 2, 4, 5

Nursing diagnoses appropriate for a client diagnosed with an acute abdomen include fear, deficient fluid volume, acute pain, and risk of infection. Ineffective coping and altered self-perception would not apply to this client.

Chapter 24 Nursing Management: Patients with Hepatic and Biliary Disorders

MULTIPLE CHOICE

1.A child care worker complains of flu-like symptoms. On further assessment, hepatitis is suspected. The nurse realizes that this individual is at risk for which type of hepatitis?

-
1. Hepatitis A

2. Hepatitis B

3. Hepatitis C

4. Hepatitis D

ANS: 1

Hepatitis A virus (HAV) is spread through the fecal-oral route. Child care workers are at greater risk because of potentially poor hygiene practices. Child care workers are not at the same risk for contracting hepatitis B, C, or D.

PTS: 1 DIF: Analyze REF: Table 51-1 Comparison of the Types of Hepatitis

2. An older male is diagnosed with cirrhosis of the liver. The nurse knows that the most likely cause of this problem is:

1. being in the military.

2. traveling to a foreign country.

TESTBANKSELLER.COM

3. drinking excessive alcohol.

4. eating bad food.

ANS: 3

The destruction to the liver from alcohol often progresses from fatty liver to alcoholic hepatitis and culminates in alcoholic cirrhosis. Alcoholic cirrhosis accounts for a great number of individuals diagnosed with this disease. Cirrhosis is not associated with being in the military, traveling to a foreign country, or eating bad food.

PTS: 1 DIF: Analyze REF: Cirrhosis

3. When the liver is seriously damaged, ammonia levels can rise in the body. One of the treatments for this is:

1. administering intravenous (IV) neomycin.

2. giving vitamin K.

-
3. giving lactulose.
 4. starting the patient on insulin.
-

ANS: 3

Lactulose is a laxative that works by pulling water into the stool. It also helps pull ammonia from the blood into the colon for expulsion. IV antibiotics do not reduce serum ammonia levels. Vitamin K controls bleeding, but it does not reduce ammonia levels. Insulin is not used to reduce ammonia levels.

PTS:1DIF:Analyze

REF: Hepatic Encephalopathy: Planning and Implementation

4.A client is scheduled for a liver biopsy. The nurse realizes that the most important sign to assess for is:

-
1. infection.
 2. bleeding.
 3. pain.
 4. nausea and vomiting.
-

ANS: 2

After a liver biopsy, the client is monitored for bleeding or hemorrhage. Infection and pain are of concern, but they are not the most important signs to be monitored. Nausea and vomiting are not typically associated with a liver biopsy.

PTS: 1 DIF: Analyze REF: Nursing Strategy: Complications of a Liver Biopsy

5.The nurse realizes that the organ which is a major site for metastases, harboring and growing cancerous cells that originated in some other part of the body, is the:

-
1. spleen.
 2. gallbladder.
-

3. liver.

4. stomach.

ANS: 3

In most developed countries, this secondary type of liver cancer is more common than cancer that originates in the liver itself. The spleen, gallbladder, and stomach are not major sites for metastases.

PTS: 1 DIF: Analyze REF: Cancer of the Liver: Secondary Liver Cancer

6.A school age child is placed on a waiting list for a liver transplant. The nurse knows that the most common reason for children to need this type of transplant is because of:

1. cirrhosis due to hepatitis C.

2. biliary atresia.

3. diabetes.

TESTBANKSELLER.COM

4. Crohns disease.

ANS: 2

Biliary atresia is the most common reason for children to have a liver transplant. Cirrhosis due to hepatitis C is the reason for most adults to have a transplant. Children do not typically need a liver transplant for diabetes or Crohns disease.

PTS: 1 DIF: Analyze REF: Liver Transplantation: Etiology

7.Because health care workers are at a greater risk of hepatitis B infection, it is recommended that all health care workers:

1. wash their hands often.

2. avoid foreign travel.

3. become vaccinated.

-
4. drink bottled water only.

ANS: 3

Because of the risk of blood and body fluid exposure, it is recommended that all health care workers be vaccinated against hepatitis B virus. All health care workers should engage in frequent handwashing, but handwashing is not the primary mechanism to prevent the onset of hepatitis B. Avoiding foreign travel and drinking bottled water only will not reduce the risk of hepatitis B.

PTS: 1 DIF: Analyze REF: Table 51-1 Comparison of the Types of Hepatitis

8.A client who usually smokes a pack of cigarettes a day tells the nurse that he cannot stand the smell of smoke. The nurse realizes that this client is in which phase of hepatitis?

-
1. Preicteric
-
2. Icteric
-
3. Posticteric
-
4. Recovery

TESTBANKSELLER.COM

ANS: 1

In the preicteric phase of hepatitis, some smokers will have an aversion to smoking as a first sign of the disease. Smoking is not affected with the icteric or posticteric phases of the disease. Recovery is not a phase of hepatitis.

PTS: 1 DIF: Analyze REF: Box 51-3 Phases of Hepatitis

9.A female client is surprised to learn that she has been diagnosed with hemochromatosis. Which of the following should the nurse respond to this client?

-
1. It doesn't affect people until they are in their 50s.
-
2. I would ask the doctor if he's sure about the diagnosis.
-
3. Females often do not experience the effects of the disease until menopause.
-
4. All women have the disorder but not the symptoms.

ANS: 3

Women do not experience the effects of hemochromatosis until menopause when the regular loss of blood stops. This disorder is a genetic disorder and can affect individuals of all ages. The nurse should not doubt the physicians diagnosis. All women do not have this disorder.

PTS: 1 DIF: Apply REF: Hereditary Diseases of the Liver

10.A client is diagnosed with liver disease. Which of the following is one impact of this disorder on a clients fluid and electrolyte status?

- | | |
|----|---------------|
| 1. | Hyperkalemia |
| 2. | Hypercalcemia |
| 3. | Hypernatremia |
| 4. | Hyponatremia |

ANS: 4

Liver disease effects the fluid and electrolyte status by causing ascites, edema, hypokalemia, hypocalcemia, and hyponatremia. Liver disease does not cause hyperkalemia, hypercalcemia, or hypernatremia.

PTS: 1 DIF: Analyze REF: Box 51-7 Clinical Manifestations of Liver Disease

11.The nurse, caring for a client recovering from the placement of a shunt to treat portal hypertension, should assess the client for which of the following complications associated with this surgery?

- | | |
|----|-----------------------------|
| 1. | Myocardial infarction |
| 2. | Pulmonary emboli |
| 3. | Pulmonary edema |
| 4. | Decreased peripheral pulses |

ANS: 3

Complications after shunt surgery include the development of pulmonary edema. Myocardial infarction, pulmonary emboli, and decreased peripheral pulses are not complications associated with this type of surgery.

PTS: 1 DIF: Apply REF: Red Flag: Shunt Surgery

12.A client is diagnosed with macrovesicular fatty liver. Which of the following should the nurse instruct this client?

- | | |
|----|--|
| 1. | Expect to develop jaundice. |
| 2. | Avoid all alcohol. |
| 3. | Increase exercise. |
| 4. | Treatment includes antibiotic therapy. |

ANS: 2

The client diagnosed with macrovesicular fatty liver should be instructed to avoid all alcohol. Jaundice is a symptom of microvesicular fatty liver. The client should be instructed to rest. Antibiotic therapy is not indicated for macrovesicular fatty liver.

PTS: 1 DIF: Apply REF: Fatty Liver: Planning and Implementation

MULTIPLE RESPONSE

1.A client diagnosed with cirrhosis is experiencing the complication of ascites. Which of the following would be considered treatment for this complication? (Select all that apply.)

- | | |
|----|--------------------|
| 1. | Fluid restriction |
| 2. | Low-sodium diet |
| 3. | Increased exercise |
| 4. | Diuretic therapy |
| 5. | Pain medication |
| 6. | Bed rest |

ANS: 1, 2, 4

Ascites is the accumulation of fluid in the peritoneal cavity. Treatment strategies include fluid restriction (1000 to 1500 mL/day), low-sodium diet (200 to 500 mg/day), and diuretic therapy to remove the excessive fluid. Increased exercise, pain medication, and bed rest are not included as treatments for this complication.

PTS: 1 DIF: Apply REF: Cirrhosis: Planning and Implementation

2.A client is recovering from an endoscopic retrograde cholangiopancreatogram (ERCP). Which of the following should the nurse assess as possible complications from this procedure? (Select all that apply.)

- | | |
|----|---|
| 1. | Perforation of the stomach |
| 2. | Perforated duodenum |
| 3. | Pancreatitis |
| 4. | Aspiration of gastric contents |
| 5. | Anaphylactic reaction to the contrast dye |
| 6. | Perforated bladder |

ANS: 1, 2, 3, 4, 5

Potential complications of an ERCP are perforated stomach and duodenum, pancreatitis, anaphylactic reaction to the contrast diet, aspiration of gastric contents, and reaction to anesthesia. A perforated bladder is a possible complication from a paracentesis.

PTS:1DIF:ApplyREFDiagnostic Tests

3.A client is demonstrating yellow pigmentation of the skin and sclera. Which of the following can be used to describe this clients symptoms? (Select all that apply.)

- | | |
|----|-----------|
| 1. | Jaundice |
| 2. | Dyspepsia |
| 3. | Icterus |

4.	Sclerosis
5.	Kernicterus
6.	Cirrhosis

ANS: 1, 3, 5

Terms used to describe yellow pigmentation of the skin and sclera include jaundice, icterus, and kernicterus. Dyspepsia, sclerosis, and cirrhosis are not terms used to describe the yellow pigmentation of the skin and sclera.

PTS: 1 DIF: Analyze REF: Box 51-1 Bilirubin Labels

4.The nurse is providing dietary instruction to a client diagnosed with Wilsons disease. Which of the following should be included in these instructions? (Select all that apply.)

1.	Avoid liver.
2.	Avoid shellfish.
3.	Eat soy products.
4.	Use avocados in salads.
5.	Avoid nectarines.
6.	Avoid mushrooms.

ANS: 1, 2, 5, 6

Dietary instruction for a client diagnosed with Wilsons disease include reducing the intake of foods high in copper. This includes avoiding liver, shellfish, soy products, avocado, nectarines, and mushrooms.

PTS:1DIF:Apply

REF: Box 51-1 Dietary Recommendations for People with Wilsons Disease

5.A client is diagnosed with a disorder of the liver. The nurse realizes this client might experience which of the following? (Select all that apply.)

- | | |
|----|------------------------------|
| 1. | Low vitamin A levels |
| 2. | Increased bleeding |
| 3. | Poor digestion of fats |
| 4. | Insulin resistance |
| 5. | Elevated levels of vitamin E |
| 6. | Nerve damage |

ANS: 1, 2, 3, 4, 6

Effects of a liver disorder on a client are many. Some of the functions affected by this disorder include low levels of fat soluble vitamins, including A and E; poor synthesis of clotting factors, leading to increased bleeding; poor digestion of fats; insulin resistance; and nerve damage.

PTS: 1 DIF: Analyze REF: Cirrhosis: Pathophysiology

6.A client is diagnosed with portal hypertension. The nurse should assess the client for which of the following disorders associated with this diagnosis? (Select all that apply.)

- | | |
|----|---------------------|
| 1. | Esophageal varices |
| 2. | Splenomegaly |
| 3. | Hemorrhoids |
| 4. | Caput medusae |
| 5. | Gastritis |
| 6. | Gallstone formation |

ANS: 1, 2, 3, 4

Portal hypertension can lead to the development of esophageal varices, splenomegaly, hemorrhoids, and caput medusae. Portal hypertension does not lead to gastritis or gallstone formation.

Chapter 25 Nursing Assessment: Renal and Urinary Tract Function

MULTIPLE CHOICE

1. The nurse has provided basic information to a client about the kidneys. Which of the following client statements would indicate that additional instruction would be needed?

1. A person cannot survive without both kidneys.
2. The kidneys are approximately 4.5 inches long.
3. The kidneys are positioned in the retroperitoneal space.
4. The right kidney is lower than the left.

ANS: 1

The client statement that would indicate the need for more instruction is a person cannot survive without both kidneys. A person can easily survive with a single kidney. The other client statements would not indicate the need for additional instruction.

PTS: 1 DIF: Analyze REF: Anatomy and Physiology: Kidneys

2. The nurse is assessing the renal system of an elderly client. Which of the following is *not* an age-related change seen in the renal system?

1. Decreased glomerular filtration rate
2. Decreased muscle tone and elasticity in the ureters, bladder, and urinary sphincter
3. Prostatic hypoplasia in the male
4. Nocturia

ANS: 3

Prostatic hyperplasia, not hypoplasia, is the age-related change often seen in elderly male patients resulting in urinary retention. The other choices are age-related changes that can occur in the renal system.

PTS:1DIF:Analyze

REF: Respecting Our Differences: Age-Related Changes in the Renal System

3.A client with an alteration in the renal system is demonstrating inconsistent blood pressure control. The nurse realizes that the substance produced by the kidneys that assists in blood pressure control is:

- | | |
|----|-----------------------|
| 1. | antidiuretic hormone. |
| 2. | erythropoietin. |
| 3. | renin. |
| 4. | vitamin D. |

TESTBANKSELLER.COM

ANS: 3

Renin is produced by the kidneys and helps control blood pressure. Antidiuretic hormone is produced by the posterior pituitary. Erythropoietin stimulates the production of red blood cells. Vitamin D is activated by the kidneys and influences calcium metabolism.

PTS: 1 DIF: Analyze REF: Renin-Angiotensin System

4.A client has had a sudden 5-kg weight gain. The nurse calculates the clients fluid retention as being:

- | | |
|----|--------|
| 1. | 2.5 L. |
| 2. | 5 L. |
| 3. | 10 L. |
| 4. | 15 L. |

ANS: 2

A sudden increase of daily weight can indicate retention of body fluids. A weight gain of 1 kg would indicate retention of 1 L of fluid. The client who had a 5 kg weight gain would have a fluid retention of 5 L.

PTS: 1 DIF: Apply REF: Red Flag: Fluid Volume Excess

5.The nurse is collecting a 24-hour urine specimen from a client with an indwelling urinary catheter. How should the nurse collect this specimen?

1. Empty the catheter bag once a shift and place the urine in a collection container on ice.
2. Disconnect the catheter from the tubing and drain the urine directly into the collection container.
3. Aspirate urine from the tubing port with a sterile needle every hour and place in a collection container on ice.
4. Place the catheter bag on ice and empty regularly into the collection bottle, which is also kept on ice.

ANS: 4

When collecting a 24-hour urine specimen from a client with an indwelling catheter, the nurse should place the catheter bag on ice and empty regularly into the collection bottle which is also to be kept on ice. The other choices are incorrect and could cause inaccurate test results.

PTS: 1 DIF: Apply REF: Table 52-1 Urine Studies

6.The nurse needs to collect a urine specimen for culture from a client who does not have an indwelling urinary catheter. Which of the following instructions would the nurse provide the client regarding how to collect this sample?

1. Decrease your water intake so the sample will be more concentrated.
2. I will need to catheterize you to obtain urine.
3. Please use the wipe and cup for the sample.
4. When you use the urinal, please call so that I can get the sample.

ANS: 3

A urine specimen obtained from a non-catheterized client should be collected using a specimen cup and by using the proper cleansing technique. The nurse should not instruct the client to

reduce fluid intake. The nurse does not need to catheterize the client to obtain the specimen. The nurse should not be using urine from a urinal for this specimen.

PTS: 1 DIF: Apply REF: Table 52-1 Urine Studies

7.The nurse is reviewing the results of serum laboratory tests conducted on a client. Which of the following results should be reported?

- | | |
|----|----------------------------|
| 1. | Calcium 8.5 mg/dL |
| 2. | Potassium 6.1 mEq/L |
| 3. | Serum creatinine 1.4 mg/dL |
| 4. | Sodium 144 mEq/L |

ANS: 2

Normal potassium levels are between 3.5 and 5 mEq/L. The other values are within normal limits.

PTS: 1 DIF: Apply REF: Table 52-2 Blood Studies

8.A client, diagnosed with renal calculi, is experiencing extreme pain. The nurse explains to the client that the cause of the pain is due to the:

- | | |
|----|-------------------------------------|
| 1. | stone scratching the kidney tissue. |
| 2. | stone scraping against the bladder. |
| 3. | buildup of pressure in the ureters. |
| 4. | spasms of the urethra. |

ANS: 3

Pressure receptors in the ureters generate the extreme pain experienced during the passage of renal calculi. Pain associated with renal calculi is not caused by the stone scratching the kidney tissue or scraping against the bladder. The pain is not caused by urethral spasms.

PTS: 1 DIF: Apply REF: Ureters

9. A client has sustained trauma to the trigone portion of the bladder. The nurse realizes that which of the following will be affected in this client?

1. The ureters and urethra
2. The nephrons
3. The detrusor muscle will spasm
4. The ability to concentrate urine will be lost

ANS: 1

The trigone of the bladder accommodates the orifices of the ureters and the urethra. The nephrons are the functional unit of the kidney. Trauma to the trigone portion of the bladder may or may not cause detrusor muscle spasms. Damage to the bladder will not cause the kidney to lose the ability to concentrate urine.

PTS: 1 DIF: Analyze REF: Urinary Bladder

10. The nurse is assessing the skin of a client diagnosed with renal insufficiency. Which of the following is the nurse most likely going to assess in this client?

1. Evidence of scratching
2. Bruises
3. Flushing
4. Moist skin with good turgor

ANS: 1

Signs of persistent scratching often occurs in the client with renal disorders because of the phosphorus or calcium imbalances. Bruising and flushing are not typically associated with this disorder. The skin of a client with a renal disorder can be dry and lack turgor or be grossly edematous.

PTS: 1 DIF: Apply REF: Skin

11. A client diagnosed with a kidney disorder is scheduled for a diagnostic test that uses a contrast agent. Which of the following can be done to protect this client's kidney functioning?

-
1. Restrict fluids.

 2. Administer acetylcysteine as prescribed.

 3. Provide 0.9% normal saline through an intravenous access device.

 4. Maintain bed rest.

ANS: 2

To protect renal function in a client with a kidney disorder who needs to receive a contrast agent for a diagnostic test, the client would be provided with acetylcysteine or sodium bicarbonate. The client should not have fluids restricted. An intravenous infusion of normal saline will not protect the kidneys from possible damage from the contrast agent. Maintaining bed rest will not protect the kidneys from the contrast agent.

PTS:1DIF:Apply

REF: Red Flag: Using Contrast Agents in Renal Diagnostics

12.A client is scheduled for a renal ultrasound and a barium swallow. The nurse realizes that which of the following should be done regarding these diagnostic tests?

-
1. Complete the barium swallow first.

 2. Complete the renal ultrasound first.

 3. Complete the barium swallow and then have the renal ultrasound done immediately afterward.

 4. Wait 8 hours after the barium swallow to complete the renal ultrasound.

ANS: 2

A renal ultrasound must be done before any diagnostic tests that use barium. If this is not possible, at least 24 hours must elapse between the barium swallow and the renal ultrasound.

PTS:1DIF:Analyze

REF:Red Flag: Potential Problems Associated with Renal Ultrasounds

13.A client with chronic renal disease asks the nurse why she needs to receive erythropoietin injections. Which of the following should the nurse respond to this client?

1. It makes more vitamin D in your body.
2. It encourages your kidneys to remove more waste products.
3. It stimulates red blood cell production in the bone marrow.
4. It helps remove ammonia from your blood.

ANS: 3

Erythropoietin stimulates red blood cell production in the bone marrow, which is compromised in renal failure. This is what the nurse should respond to the client. Erythropoietin does not make vitamin D, remove waste products, or remove ammonia from the blood.

PTS: 1 DIF: Apply REF: Table 52-2 Blood Studies

MULTIPLE RESPONSE

1.A nurse is assessing a client for signs of decreased kidney function. Which of the following are symptoms of possible decreased kidney function? (Select all that apply.)

1. Increased appetite
2. Metallic taste in the mouth
3. Pruritus
4. Reduced energy level
5. Urine output of 240 mL in 8 hours
6. Weight gain

ANS: 2, 3, 4, 6

Signs of decreased kidney function are a reduced energy level, metallic taste in the mouth, anorexia, nausea, pruritus, decreased ability to concentrate, decreased urine output, and weight gain from fluid retention. Increased appetite and urine output of 240 mL in 8 hours are not seen in a client with decreased kidney function.

PTS:1DIF:AnalyzeREF:Health History

2.The nurse is reviewing a clients current medication list for those that can be nephrotoxic. Which of the following medications can be nephrotoxic? (Select all that apply.)

1.	Amphotericin B
2.	Chloroquine
3.	Erythromycin
4.	Gentamicin
5.	Tobramycin
6.	Vancomycin

ANS: 1, 4, 5, 6

Potentially nephrotoxic drugs are amikacin, gentamicin, amphotericin B, sulfonamides, tobramycin, vancomycin, chemotherapeutic agents, contrast medium, ethylene glycol, nonsteroidal anti-inflammatory drugs (NSAIDs), gold, and other heavy metals. Chloroquine and Erythromycin are not nephrotoxic medications.

PTS:1DIF:Apply

REF:Box 52-3 Potentially Nephrotoxic Drugs and Other Agents

3.A nurse is collecting a 24-hour urine sample from a client without an indwelling urinary catheter. Which of the following are steps for collecting the sample? (Select all that apply.)

1.	Discard the first void and save all subsequent urine for 24 hours.
2.	Discard the last void.
3.	Record the first void as the beginning time.
4.	Save all urine in a 24-hour period.
5.	Save the first void.

-
6. Save all urine voided except the last specimen.

ANS: 1, 3

The 24-hour urine collection procedure would include discarding the first void and recording the time as the start time. Each subsequent void would be collected and saved until the 24-hour period ends. This includes the last void. Since the first void is discarded, all urine in a 24-hour period is not saved.

PTS: 1 DIF: Apply REF: Table 52-1 Urine Studies

4.The nurse realizes that a client diagnosed with kidney disease is at risk for acid-base imbalances. Which of the following explains how the kidney contributes to acid-base balance? (Select all that apply.)

-
1. Secretes hydrogen ions
-
2. Reabsorbs bicarbonate
-
3. Generates new bicarbonate
-
4. Produces erythropoietin
-
5. Converts vitamin D
-
6. Excretes waste products from protein metabolism

ANS: 1, 2, 3

The kidneys contribute to acid-base balance by secreting hydrogen ions, reabsorbing bicarbonate, or generating new bicarbonate. The production of erythropoietin aids in the making of red blood cells. The conversion of vitamin D supports calcium metabolism. The excretion of waste products from protein metabolism does not contribute to acid-base balance.

PTS: 1 DIF: Analyze REF: Box 52-1 Functions of the Kidney

5.A client has a disorder that is affecting the reabsorption ability of the kidney. Which of the following does the renal tubule usually reabsorb to support body functions? (Select all that apply.)

-
1. Water

2.	Glucose
3.	Amino acids
4.	Vitamins
5.	Calcium
6.	Ammonia

ANS: 1, 2, 3, 4, 5

In the kidney, tubular reabsorption includes water, glucose, amino acids, vitamins, bicarbonates, calcium, magnesium, sodium, and potassium. Ammonia is secreted from the renal tubule.

PTS:1DIF:Analyze

REF: Figure 52-5 Processes and Structures of the Nephron

6.A client is recovering from a renal biopsy. After this procedure, the nurse should instruct the client to notify the nurse for which of the following? (Select all that apply.)

1.	Problems voiding
2.	Obvious blood in the urine
3.	Increased pain
4.	Fever
5.	Painful urination
6.	Constipation

ANS: 1, 2, 3, 4, 5

After a renal biopsy, the client should be instructed to notify the nurse with problems voiding, obvious blood in the urine, increased pain, fever, or painful urination. Constipation is not considered an effect of a renal biopsy.

Chapter 26 Nursing Management: Patients with Renal Disorders**MULTIPLE CHOICE**

1. The nurse, caring for a client diagnosed with pyelonephritis, realizes that common risk factors for the development of this disorder include all of the following EXCEPT:

-
- | | |
|----|--------------------|
| 1. | urinary retention. |
|----|--------------------|
-
- | | |
|----|------------------|
| 2. | urinary calculi. |
|----|------------------|
-
- | | |
|----|-----------------------------|
| 3. | prostate gland hypertrophy. |
|----|-----------------------------|
-
- | | |
|----|--------------------------|
| 4. | orthostatic hypotension. |
|----|--------------------------|

ANS: 4

One of the causes of pyelonephritis is urinary retention. Causes of urinary retention are prostate gland hypertrophy, masses, urinary calculi, or ureteral obstruction. Orthostatic hypotension does not cause pyelonephritis.

PTS: 1 DIF: Analyze REF: Pyelonephritis: Etiology

2. A client is diagnosed with autosomal dominant polycystic kidney disease. During data collection, which assessment finding would the nurse expect to discover?

-
- | | |
|----|--|
| 1. | Decreasing abdominal girth and proteinuria |
|----|--|
-
- | | |
|----|---|
| 2. | Urinary tract infection and hypotension |
|----|---|
-
- | | |
|----|--------------------|
| 3. | Pain and hematuria |
|----|--------------------|
-
- | | |
|----|--|
| 4. | Irregularly shaped kidney and glucosuria |
|----|--|

ANS: 3

Pain and hematuria are common manifestations of autosomal dominant polycystic kidney disease. Other signs may include increasing abdominal girth, proteinuria, urinary tract infection, hypertension, and enlarged, irregularly shaped kidneys.

PTS:1DIF:Apply

REF: Polycystic Kidney Disease: Assessment with Clinical Manifestations

3.A client is being prescribed medication to treat polycystic kidney disease. Which of the following medications would be indicated for this client?

- | | |
|----|------------------|
| 1. | Cephalosporin |
| 2. | Antifungal |
| 3. | Antihypertensive |
| 4. | Antiarrhythmic |

ANS: 1

Cephalosporins are considered first line antibiotics for management of cysts with polycystic kidney disease. Antifungal, antihypertensives, and antiarrhythmic medications are not used to treat this disorder.

PTS:1DIF:Apply

TESTBANKSELLER.COM

REF: Box 54-1 Antibiotic Therapy for Infected Cysts in Patients with Polycystic Kidney Disease

4.A client is diagnosed with resistant polycystic kidney disease. The medications which may be indicated for this client would be:

- | | |
|----|---------------------------------|
| 1. | penicillin and aminoglycosides. |
| 2. | clindamycin and gentamicin. |
| 3. | metolazone and amiloride. |
| 4. | pyridium and urogesic. |

ANS: 2

Clindamycin and gentamicin are lipid-soluble antibiotics used to penetrate the resistant cysts. Penicillin and aminoglycosides are part of the first-line management of polycystic kidney disease. Metolazone and amiloride are diuretics. Pyridium and Urogesic are nonopioid analgesics.

PTS:1DIF:Analyze

REF: Box 54-1 Antibiotic Therapy for Infected Cysts in Patients with Polycystic Kidney Disease

5.A client is diagnosed with rhabdomyolysis. The nurse realizes that an emergency condition that may occur with this diagnosis would be:

-
- | | |
|-------|-----------------------|
| 1. | shortness of breath. |
| <hr/> | |
| 2. | joint aches. |
| <hr/> | |
| 3. | pulmonary hemorrhage. |
| <hr/> | |
| 4. | compartment syndrome. |

ANS: 4

Rhabdomyolysis is a condition of muscle tissue destruction. Compartment syndrome may develop with extensive muscle damage. Shortness of breath, joint aches, and pulmonary hemorrhage are assessment findings of Wegeners granulomatosis.

PTS: 1 DIF: Analyze REF: Rhabdomyolysis; Etiology; Pathophysiology

6.During the admitting assessment process, a client asks, What is oliguria? Based on the nurses knowledge, the best response would be:

-
- | | |
|-------|---|
| 1. | Oliguria is a urine output less than 50 mL in 24 hours. |
| <hr/> | |
| 2. | Oliguria is a urine output less than 250 mL in 24 hours. |
| <hr/> | |
| 3. | Oliguria is a urine output less than 400 mL in 24 hours. |
| <hr/> | |
| 4. | Oliguria is a decreased urine output indicative of disease. |

ANS: 3

Oliguria is a decrease in urine output; however, this response is not specific enough. Oliguria demonstrates a urine output of 400 mL/24 hours. Anuria is a urine output of 50 mL/24 hours. Even though a urine output of less than 250 mL in 24 hours would be considered oliguria, the definition is that of less than 400 mL of urine within 24 hours.

PTS: 1 DIF: Apply REF: Acute Renal Failure: Pathophysiology

7.A client is diagnosed with acute renal failure. Which of the following diagnostic studies will the nurse be most concerned with?

- | | |
|----|---|
| 1. | Blood glucose and HbA _{1c} |
| 2. | Toxicology screening and chloride level |
| 3. | Potassium and digitalis levels |
| 4. | Chest x-ray study and magnesium level |

ANS: 3

A client who is prescribed digitalis who also has a low potassium level can experience cardiac arrest. Blood glucose and HbA_{1c} are tests usually performed on the patient with diabetes mellitus. Toxicology screening and chloride level could be assessed for a variety of health problems. Chest x-ray and magnesium level can be assessed for a variety of health problems.

PTS:1DIF:Analyze

REF: Table 54-2 Alterations in ARF/AKI and the Mechanisms of the Alterations

TESTBANKSELLER.COM

8.A client has been prescribed a restricted potassium diet. An appropriate snack for the client would be:

- | | |
|----|---------------|
| 1. | banana. |
| 2. | applesauce. |
| 3. | orange juice. |
| 4. | dried dates. |

ANS: 2

Bananas, oranges, and dried fruit are high-potassium food sources. Applesauce is the low-potassium snack.

PTS:1DIF:Apply

REF: Acute Renal Failure/Acute Kidney Injury: Nutrition

9.A client diagnosed with chronic renal failure is prescribed a diet low in protein. The rationale for this diet is that:

1. protein sources are broken down and converted to urea, which is then filtered by the kidney.
2. protein sources are of low biological value.
3. protein increases calcium and sodium levels.
4. deficit protein metabolism breaks down muscle tissue.

ANS: 1

Protein in the diet increases the amount of nitrogen waste the kidney must handle. Protein does not have low biological value. Protein does not increase calcium and sodium levels. A deficit in protein metabolism does not break down muscle tissue.

PTS: 1 DIF: Analyze REF: Chronic Renal Failure: Nutrition

10.A client diagnosed with chronic renal failure asks the nurse, Whats the difference between hemodialysis and peritoneal dialysis? Which of the following statements best explains the difference?

1. Hemodialysis is done three times a week and lasts 3 to 4 hours; peritoneal dialysis is done daily.
2. Hemodialysis uses a graft or fistula and pumps blood through a semipermeable membrane in a hemodialyzer as the filter. Peritoneal dialysis uses the peritoneal lining of the abdominal cavity as the filter.
3. Hemodialysis and peritoneal dialysis use different equipment.
4. There are different dietary requirements for hemodialysis and peritoneal dialysis.

ANS: 2

All are differences between hemodialysis and peritoneal dialysis; however, hemodialysis uses a graft or fistula and pumps blood through a semipermeable membrane in a hemodialyzer as the filter. Peritoneal dialysis uses the peritoneal lining of the abdominal cavity as the filter explains the mechanism between hemodialysis and peritoneal dialysis.

PTS:1DIF:Apply

REF: Chronic Renal Failure: Hemodialysis; Peritoneal Dialysis and Chronic Ambulatory Peritoneal Dialysis

11. The nurse would expect that a client recovering from a kidney transplant would be prescribed all the following medications EXCEPT:

- | | |
|----|---------------|
| 1. | prednisone. |
| 2. | cyclosporine. |
| 3. | azathioprine. |
| 4. | vancomycin. |

ANS: 4

Prednisone, cyclosporine, and azathioprine are common medications prescribed for renal transplant clients. Vancomycin can be nephrotoxic.

PTS: 1 DIF: Analyze REF: Chronic Renal Failure: Renal Transplantation

12. A client diagnosed with acute renal failure from an intrarenal cause should be instructed to:

- | | |
|----|--------------------------------------|
| 1. | expect blood in the urine. |
| 2. | avoid using NSAIDs. |
| 3. | increase fluids. |
| 4. | maintain a normal activity schedule. |

ANS: 2

NSAIDs contribute to intrarenal vascular constriction. Clients with this disorder should be instructed to avoid using NSAIDs. The client diagnosed with acute renal failure from an intrarenal cause should not expect blood in the urine, to increase fluids, or to maintain a normal activity schedule.

PTS: 1 DIF: Apply REF: Table 54-1 Etiology of ARF/AKI

13. A client diagnosed with acute renal failure has a magnesium level of 1.0 mg/dL. Which of the following will the nurse most likely assess in this client?

-
- | | |
|----|---------------------|
| 1. | Broad, flat T-waves |
|----|---------------------|
-
- | | |
|----|---------------|
| 2. | ST depression |
|----|---------------|
-
- | | |
|----|--------------|
| 3. | Prolonged QT |
|----|--------------|
-
- | | |
|----|-------------------|
| 4. | No clinical signs |
|----|-------------------|
-

ANS: 4

Magnesium blood levels may be low in the client diagnosed with acute renal failure, or there may be no clinical signs associated with this level. The other choices are clinical signs associated with hypokalemia.

PTS:1DIF:Apply

REF: Table 54-2 Alterations in ARF/AKI and the Mechanisms of the Alterations

14.After the nurse provides a client diagnosed with acute renal failure with Kayexalate 30 grams by mouth, the client begins to experience frequent loose bowel movements. Which of the following does this clients response indicate to the nurse?

TESTBANKSELLER.COM

-
- | | |
|----|---|
| 1. | The client needs to be treated with insulin and dextrose. |
|----|---|
-
- | | |
|----|---|
| 2. | The client needs to receive sodium bicarbonate. |
|----|---|
-
- | | |
|----|--|
| 3. | The client needs an additional dose of Kayexalate. |
|----|--|
-
- | | |
|----|---|
| 4. | The client is experiencing a response that is indicative of successful treatment. |
|----|---|
-

ANS: 4

After receiving an oral dose of Kayexalate, loose bowel movements should occur. This is indicative of successful treatment. The client does not need to be treated with insulin and dextrose. The client does not need to receive sodium bicarbonate. The client does not need an additional dose of Kayexalate unless the potassium level remains elevated.

PTS: 1 DIF: Analyze REF: Box 54-3 Management of Hyperkalemia

MULTIPLE RESPONSE

1.The nurse is assessing circulation through a clients arteriovenous shunt. Which of the following are signs of a patent site? (Select all that apply.)

- | | |
|----|----------------------------------|
| 1. | Normal blood pressure |
| 2. | Positive bruit |
| 3. | Pulse present distal to the site |
| 4. | Dry dressing |
| 5. | Palpable thrill |
| 6. | Good skin turgor |

ANS: 2, 5

A positive bruit and palpable thrill indicate potential site patency. Blood pressure and a dry dressing do not assess circulation to the shunt. Good skin turgor does not indicate good circulation through the shunt. A pulse present distal to the site is a normal finding that does not indicate site patency.

PTS:1DIF:Analyze

TESTBANKSELLER.COM

REF:Box 54-7 Management of Vascular Access Devices in Dialysis

2.For a client to be diagnosed with Anti-Glomerular Basement Membrane disease, the nurse realizes that which of the following characteristics must be present? (Select all that apply.)

- | | |
|----|---|
| 1. | Antiglomerular basement membrane (GBM) antibodies |
| 2. | Sinus infection |
| 3. | Pulmonary hemorrhage |
| 4. | Proliferative glomerulonephritis |
| 5. | Increased heart rate |
| 6. | Rapidly dropping blood pressure |

ANS: 1, 3, 4

Anti-glomerular basement membrane disease must include the three characteristics of proliferative glomerulonephritis, pulmonary hemorrhage, and the presence of anti-GBM antibodies. Sinus infection is an assessment finding of Wegeners granulomatosis. Increased heart rate and rapidly dropping blood pressure are not characteristics of this disorder.

PTS: 1 DIF: Analyze REF: Anti-GBM Disease (Goodpastures Syndrome)

3. During discharge teaching with a client diagnosed with autosomal dominant polycystic kidney disease, the nurse should stress which of the following points? (Select all that apply.)

- | | |
|----|---------------------------------|
| 1. | Take more tub baths. |
| 2. | Void frequently. |
| 3. | Practice good perineal hygiene. |
| 4. | Void after intercourse. |
| 5. | Take showers. |
| 6. | Limit fluids. |

ANS: 2, 3, 4, 5

Tub baths should be avoided for female patients. The client should not be instructed to limit fluids. Discharge teaching should include frequent voiding, good perineal hygiene, voiding after intercourse, and taking showers instead of tub baths.

PTS: 1 DIF: Apply

REF: Polycystic Kidney Disease: Patient and Family Teaching

4. The nurse is caring for a client diagnosed with pyelonephritis. Which of the following are appropriate interventions that the nurse should perform? (Select all that apply.)

- | | |
|----|--|
| 1. | Ensure adequate hydration. |
| 2. | Monitor vital signs and fluid balance. |
| 3. | Insert a urinary catheter. |

-
- | | |
|----|------------------------------|
| 4. | Provide urinary antiseptics. |
|----|------------------------------|
-
- | | |
|----|--|
| 5. | Monitor electrolytes and creatinine level. |
|----|--|
-
- | | |
|----|---------------------------|
| 6. | Monitor hemoglobin level. |
|----|---------------------------|
-

ANS: 1, 2, 4, 5

The nurse should ensure the client has adequate hydration. The nurse should monitor the clients vital signs, fluid balance, electrolytes, and creatinine level. Urinary antiseptics should be provided as prescribed. A urinary catheter is discouraged because of the risk of urinary tract infection. Monitoring of the hemoglobin level is not necessary with this disorder.

PTS:1DIF:Apply

REF: Pyelonephritis: Collaborative Management Including Nursing Intervention Classifications (NIC)

5.The nurse is caring for a client diagnosed with chronic renal failure. Which of the following would be considered expected manifestations of this disorder? (Select all that apply.)

-
- | | |
|----|------------------------------|
| 1. | Left ventricular dysfunction |
|----|------------------------------|
-
- | | |
|----|--------|
| 2. | Anemia |
|----|--------|
-
- | | |
|----|----------|
| 3. | Diarrhea |
|----|----------|
-
- | | |
|----|--------------|
| 4. | Constipation |
|----|--------------|
-
- | | |
|----|--|
| 5. | Prickly burning sensation of the extremities |
|----|--|
-
- | | |
|----|---------------|
| 6. | Restless legs |
|----|---------------|
-

ANS: 1, 2, 4, 5, 6

Clinical manifestations for a client diagnosed with chronic renal failure are many. Some of these manifestations include left ventricular dysfunction, anemia, constipation; prickly burning sensation of the extremities, and restless legs. Diarrhea is not a common clinical manifestation of this disorder.

PTS:1DIF:Analyze

REF:Table 54-5 Clinical Manifestations of Chronic Kidney Failure

6.A client is diagnosed with Wegeners granulomatosis. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

1.	Shortness of breath and cough
2.	Tinnitus
3.	Abdominal pain
4.	Conjunctivitis
5.	Muscle aches
6.	Vomiting

ANS: 1, 2, 4, 5

Assessment findings that may be present in Wegeners granulomatosis include upper and lower respiratory symptoms such as shortness of breath and cough, hearing deficit and tinnitus, visual disturbances or conjunctivitis, and joint and muscle aches. Abdominal pain and vomiting are not associated with this disorder.

Chapter 27 Nursing Management: Patients with Urinary Disorders

MULTIPLE CHOICE

1.A client is being evaluated for a lower urinary tract infection. Which of the following symptoms would the nurse expect to find?

1.	Cloudy urine
2.	Flank pain
3.	Nausea
4.	Temperature 102.9F

ANS: 1

Symptoms of a lower urinary tract infection include dysuria, frequency, urgency, hesitancy, cloudy urine, lower abdominal pain, chills, malaise, and mild fever (less than 101F). The other options are symptoms of upper urinary tract infection.

PTS:1DIF:Apply

REF:Urinary Tract Infection: Assessment with Clinical Manifestations

2.An elderly client is diagnosed with a urinary tract infection. Which of the following will the nurse most likely assess in this client?

- | | |
|----|-------------------------|
| 1. | Jaundice |
| 2. | Vomiting |
| 3. | Poor eating habits |
| 4. | Change in mental status |

ANS: 4

TESTBANKSELLER.COM

The elderly tend to have symptoms of fever or hypothermia, poor appetite, lethargy, and a change in mental status. Newborns demonstrate jaundice. Infants can experience vomiting. Children tend to have poor eating habits.

PTS:1DIF:Apply

REF:Urinary Tract Infection: Assessment with Clinical Manifestations

3.A nurse is collecting a post-void residual urine volume for a client. Which of the following volumes would be abnormal?

- | | |
|----|--------|
| 1. | 30 mL |
| 2. | 60 mL |
| 3. | 95 mL |
| 4. | 125 mL |

ANS: 4

A residual volume of greater than 100 mL is abnormal. The other volumes would be considered within normal limits.

PTS: 1 DIF: Apply REF: Box 53-1 Summary of Diagnostic Tests for UTI

4. A client is prescribed trimethoprim-sulfamethoxazole for a urinary tract infection. Which of the following instructions would *not* be appropriate for this medication?

1. Complete all the medication even if you feel better.
2. Drink extra water during the day.
3. Take on an empty stomach with water.
4. Take with an antacid.

ANS: 4

This medication does not need to be taken with an antacid. Trimethoprim-sulfamethoxazole (Bactrim) should be taken on an empty stomach with water. The client should consume extra water to prevent sedimentation in the urine and calculus formation. All medication should be taken to treat and eliminate the infection.

TESTBANKSELLER.COM

PTS: 1 DIF: Apply

REF: Box 53-2 Common Medications Used with Patients with UTI

5. A client with a urinary tract infection is being discharged with a prescription for ciprofloxacin. The nurse should include which of the following discharge instructions?

1. Do not take within 2 hours of antacid use.
2. Limit fluids.
3. Restrict activity
4. Expect to be nauseated with this medication.

ANS: 1

Ciprofloxacin should not be administered within 2 hours of taking an antacid. The client does not need to limit fluids or restrict activity. Nausea is not always a side effect of this medication.

PTS: 1 DIF: Apply

REF:Box 53-2 Common Medications Used with Patients with UTI

6.A client is recovering from a cystoscopy. The nurse would expect to assess which of the following regarding the clients urine after the procedure?

1.	Anuria
2.	Blood clots
3.	Hematuria
4.	Pink-tinged

ANS: 4

The bladder and urethra are usually irritated as a result of the procedure. This causes pink-tinged urine. Large amounts of blood in the urine, anuria, or blood clots are not expected findings after this procedure.

PTS: 1 DIF: Analyze REF: Urinary Tract Infection: Diagnostic Tests

7.A client is being treated for interstitial cystitis. Which of the following medications would *not* be prescribed for this client?

1.	Cortisone acetate (Cortone)
2.	Dimethyl sulfoxide (DMSO)
3.	Pimecrolimus (Elidel)
4.	Polysulfate sodium (Elmiron)

ANS: 3

Pimecrolimus (Elidel) is for the treatment of atopic dermatitis. The other options are medications that could be prescribed for a client diagnosed with interstitial cystitis.

PTS:1DIF:Analyze

REF:Box 53-4 Common Medications Used with Patients with IC

8.After being diagnosed, a client asks the nurse What is pyelonephritis? The nurse should respond:

-
1. Pyelonephritis is an infection of the bladder.

 2. Pyelonephritis is an infection of the urethra.

 3. Pyelonephritis is an infection of the prostate.

 4. Pyelonephritis is a common infection that needs to be treated to prevent complications.

ANS: 4

Pyelonephritis is an infection of the upper urinary tract. It may involve the ureters, the renal pelvis, and the papillary tips of the collecting ducts. Without treatment, pyelonephritis can cause renal damage. Pyelonephritis is not an infection of the bladder, urethra, or prostate.

PTS: 1 DIF: Apply REF: Pyelonephritis: Pathophysiology

9.The nurse is reviewing the health history of a client diagnosed with glomerulonephritis. Which of the medical conditions would be a risk factor for developing glomerulonephritis?

-
1. Asthma
TESTBANKSELLER.COM

 2. Hypertension

 3. Recent strep throat

 4. Renal failure

ANS: 3

Recent *Streptococcus* infection can lead to the development of glomerulonephritis. Hypertension and renal failure does not cause glomerulonephritis, but they can result from glomerulonephritis. Asthma is unrelated.

PTS: 1 DIF: Analyze REF: Glomerulonephritis: Etiology

10.The nurse is assessing a client diagnosed with glomerulonephritis. Which of the following findings is consistent with this disorder?

-
1. Brown urine

-
- | | |
|----|----------|
| 2. | Hip pain |
|----|----------|
-
- | | |
|----|-------------|
| 3. | Hypotension |
|----|-------------|
-
- | | |
|----|-------------|
| 4. | Bradycardia |
|----|-------------|

ANS: 1

Brown-, tea-, or cola-colored urine; flank pain; and periorbital edema are expected findings. Hypotension, hip pain, and bradycardia are not associated with this disorder.

PTS:1DIF:Apply

REF:Glomerulonephritis: Assessment with Clinical Manifestations

11.A client is diagnosed with nephrotic syndrome. Which of the following is the nurse most likely going to assess in this client?

-
- | | |
|----|------------|
| 1. | Glucosuria |
|----|------------|
-
- | | |
|----|-------------|
| 2. | Proteinuria |
|----|-------------|
-
- | | |
|----|-----------|
| 3. | Hematuria |
|----|-----------|
-
- | | |
|----|----------|
| 4. | Oliguria |
|----|----------|

ANS: 2

In the client diagnosed with nephrotic syndrome, there is an increase in protein in the urine. Hematuria and oliguria are uncommon assessment findings in this disorder. Glucosuria would be associated with a client diagnosed with diabetes mellitus.

PTS:1DIF:Apply

REF:Nephrotic Syndrome: Assessment with Clinical Manifestations

12.A client is surprised to learn that his acute pain is caused by a kidney stone. The nurse should instruct the client that the most common type of renal calculi is composed of:

-
- | | |
|----|----------|
| 1. | calcium. |
|----|----------|

-
- | | |
|----|----------|
| 2. | cystine. |
|----|----------|
-
- | | |
|----|-----------|
| 3. | struvite. |
|----|-----------|
-
- | | |
|----|------------|
| 4. | uric acid. |
|----|------------|
-

ANS: 1

Calcium-based stones (renal calculi) are the most common type of stone. Dietary measures should be taken to decrease the potential of developing another stone. Struvite stones are made of magnesium, phosphate, and ammonium and are usually staghorn in nature. Only 5% of renal stones are from uric acid. Cystine stones are associated with hereditary factors.

PTS:1DIF:Apply

REF: Urinary Tract Calculi (Urolithiasis): Pathophysiology

13.A client is hospitalized with kidney trauma resulting in lacerations to the parenchyma. Which of the following would be included in the management of this clients care?

-
- | | |
|----|----------------------------------|
| 1. | Bed rest with antibiotic therapy |
|----|----------------------------------|
-
- | | |
|----|-----------------|
| 2. | Restrict fluids |
|----|-----------------|
-
- | | |
|----|----------------------------|
| 3. | Encourage early ambulation |
|----|----------------------------|
-
- | | |
|----|-------------|
| 4. | Nephrectomy |
|----|-------------|
-

ANS: 1

In the case of parenchymal lacerations to the kidney, the client should be hospitalized, kept on bed rest, and provided with antibiotics until the urine clears. Restricting fluids and encouraging early ambulation would not be appropriate for this clients injuries. A nephrectomy is not indicated for this type of kidney trauma.

PTS:1DIF:Analyze

REF: Renal System Trauma: Planning and Implementation

14.The nurse is reviewing a clients risk factors for the development of renal cancer. Which of the following would be considered a risk factor for the development of this disease?

-
1. Cigarette smoking

 2. Being underweight

 3. History of hypotension

 4. History of type 2 diabetes mellitus

ANS: 1

Cigarettes smoking doubles the risk of renal cell carcinoma. Obesity, not being underweight, is a risk factor. Hypertension, not hypotension, is a risk factor. Type 2 diabetes mellitus is not a risk factor for the development of the disease.

PTS: 1 DIF: Analyze REF: Box 53-7 Causes for Renal Cancer

15.A client is scheduled for surgery to remove the bladder and create a urinary diversion. If the client has a history of complications after surgery, the type of urinary diversion that might be indicated would be:

-
1. continent diversion with a surgical opening to the abdomen.

 2. continent diversion with a replacement bladder made out of intestine.

 3. noncontinent diversion with anastomose of the ureters to the anterior wall.

 4. noncontinent diversion with anastomose of the ureters to the rectum.

ANS: 3

Noncontinent urinary diversions are considered less technically demanding and are associated with the fewest postoperative complications. This type of diversion is performed by anastomosing the ureters to the anterior body wall. The rectum is not used as a site to anastomose the ureters. Continent diversions have more postoperative complications.

PTS: 1 DIF: Analyze REF: Urinary Diversion: Surgery

MULTIPLE RESPONSE

1.The nurse is instructing a client on ways to prevent urinary tract infections. Which of the following should be included in these instructions? (Select all that apply.)

-
1. Drink cranberry juice.

 2. Drink eight glasses of water.

 3. Take baths instead of showers.

 4. Urinate before and after intercourse.

 5. In women, wipe back to front after voiding.

 6. Take the prescribed medication until the symptoms subside

ANS: 1, 2, 4

Interventions to reduce the onset of urinary tract infections include drinking cranberry juice and 6 to 8 glasses of water each day. The client should be instructed to urinate before and after intercourse. Women should wipe front to back when completing perineal care because of the close proximity of the urethra to the vagina and anus. Taking showers instead of baths helps prevent bacteria from entering the urethra while bathing. The client should be instructed to take the entire course of the prescribed medication and not just until the symptoms subside.

PTS:1DIF:Apply

REF: Patient Playbook: Considerations for Patient Teaching

2.A client is diagnosed with an upper urinary tract infection. Which structures are affected by this infection? (Select all that apply.)

-
1. Bladder

 2. Kidney

 3. Prostate

 4. Ureters

 5. Urethra

6. Rectum

ANS: 2, 4

Upper urinary tract infections are of the ureters or kidney. Lower urinary tract infections are infections of the urethra, bladder, or prostate. The rectum is not affected by an upper urinary tract infection.

PTS: 1 DIF: Apply REF: Urinary Tract Infection: Pathophysiology

3.The nurse is instructing a client on ways to reduce formation of future kidney stones. Which of the following should be included in these instructions? (Select all that apply.)

-
1. Drink plenty of fluids.
-
2. Drink soft drinks.
-
3. Limit the intake of spinach.
-
4. Take a vitamin B-12 supplement or eat foods rich in vitamin B-12.
-
5. Take a magnesium citrate supplement or eating foods rich in magnesium citrate.
-
6. Adjust calcium intake.

ANS: 1, 3, 5, 6

Instructions to reduce the formation of kidney stones in the future include: drink plenty of fluids; avoid soft drinks; limit the intake of spinach to reduce urinary oxalate levels; vitamin B6 helps reduce the formation of kidney stones; magnesium citrate helps prevent the formation of kidney stones; and calcium intake should be adjusted to prevent the formation of kidney stones.

PTS: 1 DIF: Apply REF: Patient Playbook: Self-Care Nutrition Advice

4.A client is diagnosed with renal vein thrombosis. The nurse realizes that which of the following could be indicated in this clients plan of care? (Select all that apply.)

-
1. Corticosteroids
-
2. Nephrectomy

- | | |
|----|-----------------------|
| 3. | Anticoagulants |
| 4. | Antihypertensives |
| 5. | Surgical intervention |
| 6. | Antibiotics |

ANS: 1, 3, 5

Management of the client diagnosed with renal vein thrombosis includes corticosteroids, anticoagulants, and surgical removal of the thrombi. Nephrectomy, antihypertensives, and antibiotics are not indicated in the treatment of this disorder.

PTS: 1 DIF: Analyze REF: Table 53-6 Renal Vascular Disorders

5.The nurse is assessing a client for type of urinary incontinence. Which of the following are considered types of this disorder? (Select all that apply.)

- | | |
|----|------------------------------|
| 1. | Stress
TESTBANKSELLER.COM |
| 2. | Radical |
| 3. | Urge |
| 4. | Temporary |
| 5. | Overflow |
| 6. | Functional |

ANS: 1, 3, 5, 6

The four types of incontinence are stress, urge, overflow, and functional. Radical and temporary are not types of bladder incontinence.

Chapter 28 Nursing Assessment: Endocrine Function

MULTIPLE CHOICE

1. Blood work of a female client shows an increase in the production of estradiol. The nurse realizes that this hormone is controlled by:

-
- | | |
|----|--------------------|
| 1. | positive feedback. |
|----|--------------------|
-
- | | |
|----|--------------------|
| 2. | negative feedback. |
|----|--------------------|
-
- | | |
|----|-------------------|
| 3. | nervous feedback. |
|----|-------------------|
-
- | | |
|----|-------------------|
| 4. | reverse feedback. |
|----|-------------------|

ANS: 1

Even though most of the hormones in the endocrine system are under a negative feedback mechanism, estradiol is not one of those hormones. Estradiol is controlled by a positive feedback mechanism in that when it increases, there will be an increase in the production of follicle-stimulating hormone by the anterior pituitary.

PTS: 1 DIF: Analyze REF: Positive Feedback Mechanisms

TESTBANKSELLER.COM

2. The nurse, instructing a client regarding hormones, would include which of the following in these instructions?

-
- | | |
|----|---------------------------|
| 1. | Hormones are nonspecific. |
|----|---------------------------|
-
- | | |
|----|--|
| 2. | Hormone release triggers a rapid response. |
|----|--|
-
- | | |
|----|---|
| 3. | Hormones do not influence other hormones. |
|----|---|
-
- | | |
|----|--|
| 4. | The nervous system and hormones work together to maintain homeostasis. |
|----|--|

ANS: 4

A close relationship between the endocrine and nervous systems is required to allow them to control homeostasis. The short-term rapid responses by the nervous system are balanced by the long-term responses from the endocrine system. Hormones are specific and can influence other hormones. This is what the nurse should instruct the client. The other choices are incorrect and should not be included in instructions to the client.

PTS: 1 DIF: Apply REF: Hormones

3.A client is experiencing a disorder to the anterior pituitary gland. The nurse realizes that all of the following hormones will be affected by this disorder EXCEPT:

1. adrenocorticotrophic hormone.
2. antidiuretic hormone.
3. melanocyte-stimulating hormone.
4. luteinizing hormone.

ANS: 2

Antidiuretic hormone is stored by the posterior pituitary. The other choices are under the regulation of the anterior pituitary gland and would be affected by a disorder in this area.

PTS: 1 DIF: Analyze REF: Table 55-1 Endocrine Gland Hormones

4.A client has a central nervous system disorder. The nurse realizes that the client may be experiencing alterations in hormones regulated by which of the following organs?

1. TESTBANKSELLER.COM
Hypothalamus
2. Pineal gland
3. Pituitary gland
4. Thyroid

ANS: 1

The hypothalamus is considered the major regulating organ of the body because it is the connection between the nervous system and the endocrine system. The other organs take direction from the hypothalamus through the central nervous system.

PTS:1DIF:AnalyzeREF:Hypothalamus Gland

5.A client is diagnosed with a low serum calcium level. The nurse realizes that which hormone is released when serum calcium levels are low?

1. Calcitonin

- | | |
|----|---------------------|
| 2. | Cortisol |
| 3. | Parathyroid hormone |
| 4. | Thyroxine |

ANS: 3

Parathyroid hormone is secreted when serum calcium levels are low. Calcitonin is released when serum calcium levels are high. Cortisol and thyroxine are not related to calcium.

PTS: 1 DIF: Analyze REF: Thyroid Gland; Parathyroid Glands

6.A client is diagnosed with an increased level of glucagon. The nurse realizes that the production of glucagon occurs in which of the following cells within the pancreatic islets of Langerhans?

- | | |
|----|---------|
| 1. | Alpha |
| 2. | Beta |
| 3. | Delta |
| 4. | F cells |

ANS: 1

Alpha cells produce and secrete glucagon. Beta cells produce and secrete insulin. Delta and F cells are responsible for somatostatin and pancreatic polypeptide, respectively.

PTS:1DIF:AnalyzeREFancreas Gland

7.A client is diagnosed with a low level of triiodothyronine. The nurse realizes that this hormone affects which of the following body functions?

- | | |
|----|--------------------------|
| 1. | Blood glucose regulation |
| 2. | Bone growth |
| 3. | Calcium regulation |

-
4. Metabolism

ANS: 4

Triiodothyronine (T₃) affects the metabolic rate. Bone growth is affected by growth hormone. Calcium regulation is controlled by calcitonin and parathyroid hormone. Blood glucose regulation is controlled by insulin and glucagon.

PTS:1DIF:AnalyzeREF:Thyroid Gland

8.A client is diagnosed with benign cysts on the cortex of the adrenal glands. Which of the following hormones will be affected with this health problem?

-
1. Aldosterone and cortisol
-
2. Calcitonin and parathyroid hormone
-
3. Epinephrine and norepinephrine
-
4. Prolactin and luteinizing hormone

TESTBANKSELLER.COM

ANS: 1

Aldosterone and cortisol are released by the adrenal cortex. The adrenal medulla releases epinephrine and norepinephrine. Calcitonin and parathyroid hormone are released by the thyroid and parathyroid, respectively. Prolactin and luteinizing hormone are anterior pituitary hormones.

PTS: 1 DIF: Analyze REF: Adrenal Glands: Adrenal Cortex

9.The nurse realizes that an adequate amount of which vitamin must be present for parathyroid hormone to be fully effective?

-
1. Vitamin A
-
2. Vitamin C
-
3. Vitamin D
-
4. Vitamin E

ANS: 3

Adequate vitamin D is necessary for absorption of calcium into the bloodstream. Vitamins A, C, and E do not have a role in calcium regulation.

PTS:1DIF:AnalyzeREF:Endocrine Glands

10. The nurse should suspect that a client has an endocrine disorder when which of the following findings is assessed regarding the integumentary status?

- | | |
|----|-------------------|
| 1. | Freckles |
| 2. | Presence of moles |
| 3. | Course hair |
| 4. | Even skin tone |

ANS: 3

Evidence of an endocrine disorder that can be assessed through the integumentary status would include hair loss, dry skin, course hair, brittle nails, or changes in pigmentation. Freckles, the presence of moles, or even skin tone are not evidence of an endocrine disorder.

TESTBANKSELLER.COM

PTS:1DIF:AnalyzeREF:Integumentary

11. A client is scheduled for x-rays of the long bones. The nurse realizes this diagnostic test is useful to help diagnose a disorder of the:

- | | |
|----|------------------|
| 1. | pituitary gland. |
| 2. | pancreas. |
| 3. | thyroid gland. |
| 4. | adrenal gland. |

ANS: 1

X-rays of the long bones is used to help diagnose disorders of the pituitary gland. This test is not used to diagnose disorders of the pancreas, thyroid, or adrenal glands.

PTS:1DIF:Analyze

REF: Table 55-3 Common Diagnostic Tests for Endocrine System Disorders

12. The results of a client's thyroid scan showed black and gray areas. The nurse realizes this finding is consistent with:

1. malignancies.
2. elevated phosphorus levels.
3. hyperactivity.
4. renal disease.

ANS: 3

Hyperactive areas on the thyroid scan will appear as black or gray regions or hot spots. White areas or cold spots are indicative of malignancies. Black and gray areas on the thyroid scan are not indicative of elevated phosphorus levels or renal disease.

PTS: 1 DIF: Analyze REF: Thyroid Scan

13. A client is scheduled for a thyroid scan. Which of the following should the nurse instruct the client regarding this diagnostic test?

TESTBANKSELLER.COM

1. Eliminate all salt in the diet.
2. Take nothing by mouth after midnight if I-131 is being used during the test.
3. Continue taking thyroid medication as prescribed.
4. Take nothing by mouth for 45 minutes after receiving intravenous technetium for the test.

ANS: 2

Depending upon the medium being used, instructions to a client prior to having a thyroid scan may differ; however, if the client is having an oral dose of I-131 for the test, the client should be instructed to take nothing by mouth after midnight. Clients who are prescribed medications with iodine may be instructed to stop the medications for 2 weeks prior to the scan. If receiving intravenous technetium for the scan, the client does not need to be kept on a nothing by mouth order. The client should not be instructed to eliminate all salt from the diet. The client may be instructed to discontinue all thyroid medication for 4 to 6 weeks prior to the scan.

PTS: 1 DIF: Apply REF: Thyroid Scan: Nursing Management

14. Which of the following should the nurse instruct a client who is recovering from a thyroid biopsy as an outpatient?

1. Notify the physician of any problems with breathing after the procedure.
2. Go to the emergency room with any signs of a sore throat after the procedure.
3. Expect to be admitted to the hospital if the surgeon decides to remove the thyroid after the biopsy.
4. Perform no special preparations for the test.

ANS: 1

The possibility of hematoma formation and edema post-procedure are the major complications that may present as respiratory difficulty. The client should be instructed to notify the physician of any problems with breathing after the procedure. A sore throat after a thyroid biopsy is a common experience, and the client does not need to go to the emergency room. The surgeon will not remove the thyroid gland during the same time as the biopsy. There are special preparations for the test, depending upon the level of anesthesia the client will receive.

PTS:1 DIF:Apply REF:Thyroid Biopsy

TESTBANKSELLER.COM

MULTIPLE RESPONSE

1. The nurse is assessing the endocrine system of an elderly client. Which of the following are considered age-related changes of this system? (Select all that apply.)

1. Increased estrogen in women
2. Increased production of antidiuretic hormone
3. Decreased testosterone in men
4. Increased pancreatic secretion of insulin
5. Smaller thyroid gland
6. Risk for osteoporosis

ANS: 2, 3, 5, 6

Age-related changes include a decreasing basal metabolic rate as a result of a smaller thyroid gland. There is an increased production of antidiuretic hormone, resulting in more dilute urine and polyuria. Other changes are that the pancreas secretes less insulin, estrogen decreases in women, and testosterone decreases in men. Because estrogen function decreases in females, there is an increased risk for osteoporosis.

PTS: 1 DIF: Analyze REF: Effects of Aging on the Endocrine System

2.A client is diagnosed with a thyroid storm. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

1.	Fever
2.	Tachycardia
3.	Hypotension
4.	Restlessness
5.	Cardiac arrhythmias
6.	Sweating

ANS: 1, 2, 4, 5, 6

Clinical manifestations of a thyroid storm include fever, tachycardia, restlessness, cardiac arrhythmias, and sweating. Hypotension is not a clinical manifestation of a thyroid storm.

PTS: 1 DIF: Analyze REF: Red Flag: Thyroid Crisis

3.Which of the following would indicate to the nurse that a client is experiencing an endocrine disorder that is affecting the neurological system? (Select all that apply.)

1.	Tremors
2.	Memory loss
3.	Jitteriness
4.	Nervousness

5. Loss of sensation in the feet

6. Nerve pain

ANS: 1, 2, 3, 4, 5

Common neurological findings with an endocrine disorder include tremors, memory loss, jitteriness, nervousness, and decreased sensation in the hands and feet. Nerve pain is not associated with an endocrine disorder affecting the neurological system.

PTS:1DIF:AnalyzeREF:Neurological

4.A client is scheduled for diagnostic tests to evaluate the adrenal glands. Which of the following will most likely be included in these tests? (Select all that apply.)

1. Vasopressin level

2. Urine specific gravity

3. Cortisol level

TESTBANKSELLER.COM

4. Dexamethasone suppression test

5. Progesterone assay

6. Aldosterone assay

ANS: 3, 4, 5, 6

Diagnostic tests used to evaluate the status of the adrenal glands include cortisol level, dexamethasone suppression test, progesterone assay, and aldosterone assay. Vasopressin level and urine specific gravity are used to assess the pituitary gland.

Chapter 29 Nursing Management: Diabetes Mellitus

MULTIPLE CHOICE

1.A client is diagnosed with the type of diabetes in which the plasma beta cells fail to respond to insulin. Which type of diabetes is this client experiencing?

-
- | | |
|----|----------------------|
| 1. | Gestational diabetes |
|----|----------------------|
-
- | | |
|----|----------------------------|
| 2. | Impaired glucose tolerance |
|----|----------------------------|
-
- | | |
|----|--------------------------|
| 3. | Type 1 diabetes mellitus |
|----|--------------------------|
-
- | | |
|----|--------------------------|
| 4. | Type 2 diabetes mellitus |
|----|--------------------------|

ANS: 3

Type 1 diabetes mellitus results from a defect or failure of the beta cells of the pancreas. The loss of beta cells causes a lack of insulin. The other options produce insulin.

PTS:1DIF:AnalyzeREF:Type 1 Diabetes

2.The nurse has instructed a client about type 2 diabetes mellitus. Which of the following statements would indicate the client understands the instructions?

-
- | | |
|----|---|
| 1. | It happens to everyone who has gained weight. |
|----|---|
-
- | | |
|----|--|
| 2. | I have to watch what I eat and exercise. |
|----|--|
-
- | | |
|----|------------------------------------|
| 3. | I will never have to take insulin. |
|----|------------------------------------|
-
- | | |
|----|---|
| 4. | The cells that make insulin were destroyed. |
|----|---|

ANS: 2

Persons with type 2 diabetes control their blood glucose levels with diet, exercise, and medications. Type 1 diabetes mellitus is characterized by a destruction of beta cells. Not every person who gains weight develops diabetes mellitus. Insulin is not necessary for the client diagnosed with type 2 diabetes at first, but as the beta cells continue to deteriorate, insulin may be necessary.

PTS: 1 DIF: Analyze REF: Type 2 Diabetes; Planning and Implementation

3.The nurse should instruct a client that the length of time insulin can be stored at room temperature is:

-
- | | |
|----|----------|
| 1. | 2 weeks. |
|----|----------|

2.	3 weeks.
----	----------

3.	4 weeks.
----	----------

4.	5 weeks.
----	----------

ANS: 3

An insulin vial that is currently in use can be stored at room temperature as long as 4 weeks. The other choices are incorrect lengths of time to store insulin.

PTS: 1 DIF: Apply REF: Insulin

4.The nurse is preparing short-acting and long-acting insulin for administration to a client. The purpose for the clients being prescribed these types of insulin would be to:

1.	make it easier for the client to self-administer the insulin.
----	---

2.	reduce the clients appetite.
----	------------------------------

3.	mimic the bodys own insulin pattern.
----	--------------------------------------

4.	help reduce the clients body weight.
----	--------------------------------------

ANS: 3

NPH insulin is usually given twice daily and is mixed with regular insulin to mimic the bodys own insulin pattern. Mixing two insulins is not done to make it easier for the client to administer the insulin, to reduce the clients appetite, or to help reduce the clients body weight.

PTS: 1 DIF: Analyze REF: Insulin

5.A client is prescribed insulin to be given through an intravenous access line. The nurse realizes that which of the following insulins can be administered intravenously?

1.	Glargine
----	----------

2.	Lispro
----	--------

3.	NPH
----	-----

4. Regular

ANS: 4

Regular insulin may be given intravenously or subcutaneously. All other insulins are given subcutaneously.

PTS: 1 DIF: Analyze REF: Insulin

6. Which of the following should the nurse instruct a client when teaching how to self-administer insulin?

-
1. The insulin bottle must be shaken.
-
2. The long-acting insulin is clear.
-
3. Refrigerated insulin is best for injection.
-
4. The blood glucose level should be checked prior to administration.

ANS: 4

TESTBANKSELLER.COM

Insulin bottles should not be shaken but rolled to make sure the precipitate is mixed. The long-acting insulin is cloudy. The insulin should be at room temperature for administration, and the blood glucose level should be checked prior to administration.

PTS: 1 DIF: Apply REF: Insulin

7. A client should not be prescribed tolazamide if the client is sensitive to:

-
1. penicillin.
-
2. shellfish.
-
3. strawberries.
-
4. hypoglycemia

ANS: 4

Tolazamide is a first generation sulfonylurea, and can cause a high incidence of hypoglycemia. This medication is used sparingly in the United States today because there are second-generation sulfonylureas that are more effective. Tolazamide can be used if the client is sensitive to penicillin, shellfish, or strawberries.

PTS:1DIF:ApplyREF:Oral Medications

8.A client is prescribed meglitinide as oral treatment for type 2 diabetes mellitus. Which of the following should the nurse instruct as a possible side effect of this medication?

1.	Diarrhea
2.	Constipation
3.	Flatulence
4.	Hunger

ANS: 3

The most common side effect of meglitinide is flatulence, which can cause the client minor discomfort. The nurse should instruct the client regarding this side effect. Meglitinide does not cause diarrhea, constipation, or hunger.

PTS:1DIF:ApplyREF:Oral Medications

9.A client diagnosed with type 1 diabetes mellitus administers a dose of NPH insulin at 7:00 a.m. At which of the following times would this client exhibit hypoglycemia?

1.	0800
2.	0900
3.	1000
4.	1400

ANS: 4

NPH insulin peaks in 4 to 12 hours. During these hours, the client may experience a hypoglycemic episode. The other choices identify times that are before the peak times for the insulin.

PTS: 1 DIF: Apply REF: Table 57-6 Types of Insulin

10. The nurse is instructing a client diagnosed with type 2 diabetes mellitus on dietary intake. Which of the following statements indicates that the client understands the instructions?

1. Its okay to skip a meal if I make it up later.
2. Keeping to the diet plan will keep my blood sugars at a regular level.
3. When I am in a hurry, I should take my medications without testing.
4. When I go out to dinner, its okay to share a couple of bottles of wine.

ANS: 2

The diet plan is individualized for each client. The food plan will have an emphasis on maintaining blood glucose levels, lowering blood pressure, and reducing weight since there is a high incidence of obesity in clients with type 2 diabetes. Alcohol can be part of a diet plan if in moderation. Sharing a couple of bottles of wine would not be alcohol in moderation. The food plan is combined with exercise, blood glucose testing, and medications (if needed). The client should be instructed to not skip meals. The client should be instructed to not take any medication prior to testing. The client should be instructed that alcohol intake should be in moderation.

PTS: 1 DIF: Analyze REF: Controlling Diabetes (Secondary Prevention)

11. The nurse is instructing a client diagnosed with type 2 diabetes mellitus on diagnostic tests used to evaluate the control of the disorder. The nurse should instruct the client on which of the following diagnostic tests that will provide this information?

1. Fasting plasma glucose
2. Glycosylated hemoglobin
3. Random plasma glucose
4. Two-hour oral glucose tolerance test

ANS: 2

The glycosylated hemoglobin (hemoglobin A_{1c}) test measures the amount of glucose attached to hemoglobin molecules and red blood cells over their life span of approximately 120 days. This

test provides information about long-term control. The other options give current glucose information.

PTS:1 DIF:Apply REF:Diagnostic Tests

12. The nurse is instructing a client on the speed in which some insulins take effect. During these instructions, the nurse should include that which of the following insulins has the fastest peak?

1.	Glargine
2.	Lispro
3.	NPH
4.	Regular

ANS: 2

Lispro (Humalog) is classified as an ultra-short-acting insulin that peaks in 30 to 90 minutes after subcutaneous injection. Regular is a short-acting insulin that peaks in 2 to 4 hours. NPH peaks in 4 to 12 hours. Glargine takes effect in 2 to 4 hours and has no peak.

TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Table 57-6 Types of Insulin

13. A client is instructed to rotate the sites of insulin injections because it will help prevent:

1.	a decrease in absorption.
2.	an allergic reaction.
3.	lipodystrophy.
4.	skin breakdown.

ANS: 3

The rotation of sites is used to prevent lipodystrophy, a localized complication of insulin administration characterized by changes in the subcutaneous fat at the site of the injection. The other options are not why site rotation is used.

PTS: 1 DIF: Apply REF: Insulin

14. When discussing exercise with a client diagnosed with type 2 diabetes mellitus, the client is correct in stating:

1. I will exercise when I can.
2. I will exercise once a week for 30 minutes.
3. I will try to exercise every day.
4. I should exercise for at least 60 minutes when I exercise.

ANS: 3

Clients should work toward a goal of 30 minutes of exercise daily. The intensity of exercise should allow for both breathing and talking with ease during the exercise. The other statements are incorrect and would indicate that the client needs additional instruction regarding exercise.

PTS: 1 DIF: Analyze REF: Exercise

15. A client diagnosed with type 2 diabetes mellitus becomes diaphoretic and irritable during exercise. The blood glucose level at this time is 53 mg/dL. Which of the following should the client be instructed to do when this occurs?

1. Ingest 5 to 10 g of a simple carbohydrate.
2. Ingest 10 to 15 g of a simple carbohydrate.
3. Ingest 15 to 25 g of a simple carbohydrate.
4. Call paramedics.

ANS: 2

If the client becomes hypoglycemic during exercise, the client should be instructed to stop and monitor the blood glucose level every 15 minutes until the level is greater than 89 mg/dL. The client should ingest 15 grams of a carbohydrate such as milk, juice, soft drink, or glucose tablets. The treatment can be repeated in 15 minutes if ineffective.

PTS: 1 DIF: Apply REF: Exercise

16. The nurse is instructing a client diagnosed with type 2 diabetes mellitus on daily foot care. Which of the following statements indicate the client needs further instruction?

-
- | | |
|----|---------------------------------|
| 1. | I will check my feet every day. |
|----|---------------------------------|
-
- | | |
|----|---------------------------------------|
| 2. | I will cut my toenails with scissors. |
|----|---------------------------------------|
-
- | | |
|----|---|
| 3. | I will keep my appointments with my podiatrist. |
|----|---|
-
- | | |
|----|--------------------------------|
| 4. | I will make sure my shoes fit. |
|----|--------------------------------|
-

ANS: 2

Clients and their family members knowledge and practice of foot care should be assessed regularly. Clients should be instructed to wash their feet daily with warm water and mild soap. The feet should be patted dry, particularly between the toes. The feet should be examined daily for cuts, blisters, and reddened areas. Toenails should be cut with clippers, not scissors. The shoes of a client diagnosed with type 2 diabetes mellitus should fit properly to prevent foot problems.

PTS:1DIF:Analyze

REF: Peripheral Vascular Complications of the Lower Extremities: Planning and Implementation

MULTIPLE RESPONSE TESTBANKSELLER.COM

1.The nurse is assessing a client diagnosed with type 2 diabetes mellitus for symptoms associated with diabetic ketoacidosis. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

-
- | | |
|----|-------------|
| 1. | Dehydration |
|----|-------------|
-
- | | |
|----|--------------------|
| 2. | Fruity breath odor |
|----|--------------------|
-
- | | |
|----|--------------|
| 3. | Hypertension |
|----|--------------|
-
- | | |
|----|-------------|
| 4. | Bradycardia |
|----|-------------|
-
- | | |
|----|--------------------|
| 5. | Kussmaul breathing |
|----|--------------------|
-
- | | |
|----|----------------|
| 6. | Abdominal pain |
|----|----------------|
-

ANS: 1, 2, 5, 6

The client diagnosed with diabetic ketoacidosis will experience dehydration, fruity breath odor, Kussmaul respirations, and abdominal pain. The client will also have hypotension and not hypertension. The client's heart rate will be tachycardic and not bradycardic.

PTS:1DIF:Apply

REF: Diabetic Ketoacidosis: Assessment with Clinical Manifestations

2. An elderly client being treated for type 2 diabetes mellitus begins to experience lethargy, weakness, and polyuria while recovering from cataract surgery. The nurse would suspect the client is developing hyperosmolar hyperglycemic nonketotic syndrome when which of the following is assessed? (Select all that apply.)

- | | |
|----|--------------------------------|
| 1. | Blood glucose level 450 mg/dL |
| 2. | No ketones in the urine |
| 3. | Serum sodium 145 mEq/L |
| 4. | Serum osmolality 320 mOsm/kg |
| 5. | Blood pressure 120/68 mmHg |
| 6. | Heart rate 78 beats per minute |

ANS: 1, 2, 3, 4

Assessment findings consistent with hyperosmolar hyperglycemic nonketotic syndrome include a blood glucose level greater than 400 mg/dL, absence of ketones in the urine, serum sodium greater than 140 mEq/L, and serum osmolality greater than 310 mOsm/kg. The blood pressure of 120/68 mmHg is within normal limits. The heart rate of 78 beats per minute is within normal limits.

PTS:1DIF:Analyze

REF: Hyperosmolar Hyperglycemic Nonketotic Syndrome: Assessment with Clinical Manifestations

3. The nurse is instructing a client diagnosed with type 2 diabetes mellitus on activities to reduce the onset of macrovascular complications. Which of the following should the nurse include in these instructions? (Select all that apply.)

- | | |
|----|-----------------------------|
| 1. | Attain a normal body weight |
|----|-----------------------------|

- | | |
|----|-----------------------------------|
| 2. | Stop smoking |
| 3. | Increase activity |
| 4. | Keep blood pressure under control |
| 5. | Decrease fat intake |
| 6. | Ingest alcohol every day |

ANS: 1, 2, 3, 4, 5

Macrovascular complications associated with type 2 diabetes mellitus can be controlled by addressing the modifiable risk factors. The risk factors include obesity, smoking, sedentary lifestyle, high blood pressure, and fat intake. This is what the nurse should include in the instructions to this client. The client should not be instructed to ingest alcohol every day.

PTS: 1 DIF: Apply REF: Angiopathy or Vessel Disease

4.A client is being evaluated for the diagnosis of gastroparesis. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

- | | |
|----|-------------------------|
| 1. | Constipation |
| 2. | Gastroesophageal reflux |
| 3. | Feelings of fullness |
| 4. | Vomiting |
| 5. | Nausea |
| 6. | Anorexia |

ANS: 2, 3, 4, 5, 6

Gastroparesis presents as anorexia, nausea and vomiting, feelings of fullness, and gastroesophageal reflux. Constipation is not a presenting sign of gastroparesis.

Chapter 30 Nursing Management: Patients with Endocrine Disorders**MULTIPLE CHOICE**

1. A male client is diagnosed with hyperprolactinemia. The nurse realizes that which of the following clinical manifestations occurs *less* frequently in men?

- | | |
|----|----------------------------|
| 1. | A decrease in testosterone |
| 2. | Erectile dysfunction |
| 3. | Gynecomastia |
| 4. | Infertility |

ANS: 3

In men, hyperprolactinemia causes a decrease in testosterone secondary to an inhibition of gonadotropin secretion, leading to decreased facial and body hair, erectile dysfunction, decreased libido, small testicles, and infertility. Gynecomastia occurs less frequently in men.

PTS:1DIF:Analyze

REF:Hyperprolactinemia: Assessment with Clinical Manifestations

2. A female client is admitted with hyperprolactinemia. Which of the following would *not* be a clinical manifestation of the disorder in this client?

- | | |
|----|--------------------|
| 1. | Excessive estrogen |
| 2. | Hirsutism |
| 3. | Osteoporosis |
| 4. | Weight gain |

ANS: 1

Hyperprolactinemia is associated with a decrease in estrogen, resulting in symptoms of vaginal dryness, hot flashes, osteopenia, and osteoporosis. The patient may also experience weight gain, irritability, hirsutism, anxiety, and depression.

PTS:1DIF:Analyze

REF:Hyperprolactinemia: Assessment with Clinical Manifestations

3.A client has been instructed regarding a prolactin level to be drawn the next day. Which of the following statements indicate that the client will need further instruction?

1. I will be on time, in the afternoon.
2. I will be relaxed.
3. I will make sure not to take my antihistamine.
4. I will practice another method of birth control rather than the pill.

ANS: 1

Certain medications (e.g., antihistamines and oral contraceptives) and fear can increase the prolactin level. The prolactin level is drawn in the morning.

PTS: 1 DIF: Analyze REF: Box 56-1 Prolactin Levels

4.An adult client is complaining of vision changes and difficulty speaking because the tongue is larger. The client also states that his shoes no longer fit. Based on these symptoms, the client is most likely to be diagnosed with:

1. acromegaly.
2. cretinism.
3. gigantism.
4. Graves disease.

ANS: 1

Acromegaly is caused by a hypersecretion of the pituitary growth hormone over a long period. This hypersecretion causes a coarsening of the features, including soft tissue overgrowth such as the tongue. Shoes and rings may no longer fit due to tissue and bone overgrowth. In children, hypersecretion of growth hormone causes gigantism. Cretinism and Graves disease are caused by a thyroid hormone imbalance.

PTS:1DIF:Analyze

REF: Acromegaly (Gigantism): Assessment with Clinical Manifestations

5.A client is prescribed medication after recovering from surgery to treat acromegaly. Which of the following medications would the nurse expect to see prescribed?

-
- | | |
|----|------|
| 1. | None |
|----|------|
-
- | | |
|----|---|
| 2. | Cabergoline (Dostinex) 1 mg PO twice a week |
|----|---|
-
- | | |
|----|---|
| 3. | Cortisone acetate (Cortone) 100 mg PO three times a day |
|----|---|
-
- | | |
|----|---|
| 4. | Octreotide (Sandostatin) 20 mg IM every 4 weeks |
|----|---|

ANS: 4

Sandostatin is used for residual growth hormone hypersecretion following surgery. Cortone is used to treat adrenocorticotrophic dysfunction, and Dostinex is used to treat hyperprolactinemia.

PTS: 1 DIF: Analyze REF: Acromegaly (Gigantism): Pharmacology

6.A client, complaining of weight gain, has thin extremities, a buffalo hump, and a protruding abdomen. The nurse realizes that this client is most likely to be diagnosed with which disease process?

-
- | | |
|----|------------------|
| 1. | Addisons disease |
|----|------------------|
-
- | | |
|----|-----------|
| 2. | Cretinism |
|----|-----------|
-
- | | |
|----|-------------------|
| 3. | Cushings syndrome |
|----|-------------------|
-
- | | |
|----|---------|
| 4. | Obesity |
|----|---------|

ANS: 3

Even though the client has gained weight (obesity), the distribution of that weight is characteristic for the disease process of Cushings syndrome. Cretinism and Addisons disease do not exhibit those symptoms.

PTS:1DIF:Analyze

REF: Cushings Disease (Hypercortisolism): Assessment with Clinical Manifestations

7.The nurse is providing instructions to a client receiving treatment for Cushings syndrome. Which of the following instructions would not be appropriate for this client?

1. Monitor glucose levels.
2. Implement safety precautions.
3. Wear medical identification.
4. Volunteer at the hospital to prevent depression.

ANS: 4

A client diagnosed with Cushings syndrome is predisposed to falls, injury, and increased glucose levels. The client should wear an identification bracelet indicating her disease process. The client should avoid crowds and persons with infections.

PTS:1DIF:Apply

REF: Cushings Disease (Hypercortisolism): Planning and Implementation

8.The nurse is assessing a client diagnosed with hyperaldosteronism. Which of the following would take the *least* priority during this period?

1. Assessment of breath sounds
2. Cardiac monitoring
3. Assistance with activities of daily living (ADLs)
4. Review of electrolyte levels

ANS: 3

The first priority for the nurse is to monitor cardiac and respiratory status. Cardiac status can be impaired because of changes in potassium levels, and fluid balance can be impaired because of sodium, affecting the respiratory status. After the client is stabilized, the nurse can assist the client with activities of daily living.

PTS:1DIF:Analyze

REF: Hypersecretion of the Adrenal Gland (Hyperaldosteronism): Assessment with Clinical Manifestations

9.A client is diagnosed with primary adrenal insufficiency. The nurse realizes that this disorder affects which of the following glands?

- | | |
|----|-----------------|
| 1. | Adrenal cortex |
| 2. | Adrenal medulla |
| 3. | Thyroid |
| 4. | Pituitary |

ANS: 1

Mineralocorticoids, glucocorticoids, and androgens are produced in the adrenal cortex. The principal mineralocorticoid is aldosterone. The adrenal medulla secretes the catecholamines. The thyroid and pituitary do not secrete aldosterone.

PTS:1DIF:Analyze

REF: Hyposecretion of the Adrenal Gland: Pathophysiology

10.A client tells the nurse that he is so thirsty that he has already consumed four pitchers of water. The client's urine output is 3500 mL in an 8-hour period. The client is recovering from surgery on the pituitary gland. What endocrine disorder is the client most likely experiencing?

- | | |
|----|--|
| 1. | Diabetes insipidus |
| 2. | Diabetes mellitus |
| 3. | Myxedema |
| 4. | Syndrome of inappropriate antidiuretic hormone secretion |

ANS: 1

Diabetes insipidus and diabetes mellitus both cause increased urine output, but diabetes insipidus is related to a problem with antidiuretic hormone; diabetes mellitus is a problem with glucose. Myxedema is caused by a thyroid hormone imbalance. Syndrome of inappropriate antidiuretic hormone secretion causes fluid retention.

PTS:1DIF:Analyze

REF: Diabetes Insipidus: Assessment with Clinical Manifestations

11. The nurse is planning care for a client diagnosed with Graves disease. Which of the following nursing interventions would be appropriate for this client's care?

- | | |
|----|------------------------------|
| 1. | Administer a stool softener. |
| 2. | Provide extra blankets. |
| 3. | Provide frequent meals. |
| 4. | Restrict the caloric intake. |

ANS: 3

Nursing interventions for Graves disease (hyperthyroidism) include offering frequent, high-calorie meals; medicating for diarrhea; providing a fan or decreasing the temperature on the air conditioner; and taking daily weight measurements. The client does not need a stool softener. The client does not need extra blankets. The client's metabolic rate is increased, and she should not have a restriction on caloric intake.

PTS:1 DIF: Apply

REF: Hypersecretion of the Thyroid Gland: Planning and Implementation

12. A client is hospitalized with an ongoing fever. The nurse learns that the client has had a recent infection. Currently the client is restless, diaphoretic, and agitated with the following vital signs: temperature 106°F, pulse 114, blood pressure 180/80 mmHg. Which of the following disorders is the client most likely experiencing?

- | | |
|----|-------------------|
| 1. | Addisonian crisis |
| 2. | Goiter |
| 3. | Myxedema |
| 4. | Thyroid crisis |

ANS: 4

Thyroid crisis is a serious form of hyperthyroidism that is life threatening. It is most likely to occur in persons who have been inadequately treated or undiagnosed. Infection, stress or emotional trauma, pregnancy, and medications may precipitate the event. Myxedema and

addisonian crisis would not produce a severe increase in blood pressure. Goiter tends to interfere with swallowing and breathing.

PTS: 1 DIF: Analyze REF: Thyroid Crisis (Thyroid Storm)

13.A pregnant client is receiving treatment for hyperthyroidism. Which of the following medications would the nurse expect to see?

- | | |
|----|--------------------|
| 1. | Levothyroxine |
| 2. | Methimazole |
| 3. | Propylthiouracil |
| 4. | Radioactive iodine |

ANS: 3

Propylthiouracil (PTU) is the drug of choice for treating hyperthyroidism in a pregnant or breastfeeding client. Radioactive iodine and methimazole are treatments for nonpregnant clients with hyperthyroidism. Levothyroxine is used to treat hypothyroidism.

TESTBANKSELLER.COM

PTS:1DIF:Analyze

REF: Hypersecretion of the Thyroid Gland: Pharmacology

14.A client is diagnosed with chronic lymphocytic thyroiditis. The nurse should instruct the client regarding signs and symptoms of which of the following?

- | | |
|----|--------------------------|
| 1. | Type 2 diabetes mellitus |
| 2. | Heart failure |
| 3. | Hypothyroidism |
| 4. | Renal failure |

ANS: 3

The client diagnosed with chronic lymphocytic thyroiditis will most often progress to hypothyroidism, which is permanent 95% of the time. The nurse should instruct the client regarding signs and symptoms of hypothyroidism. Chronic lymphocytic thyroiditis will not cause type 2 diabetes mellitus, heart failure, or renal failure.

PTS: 1 DIF: Apply REF: Thyroiditis

MULTIPLE RESPONSE

1. Which of the following symptoms would suggest to the nurse that a client is experiencing symptoms of pheochromocytoma? (Select all that apply.)

- | | |
|----|------------------------|
| 1. | Severe headache |
| 2. | Decreased urine output |
| 3. | Palpitations |
| 4. | Diarrhea |
| 5. | Profuse sweating |
| 6. | Weight gain |

ANS: 1, 3, 5

TESTBANKSELLER.COM

Severe headache, palpitations, and profuse sweating are the most common symptoms of pheochromocytoma. Decreased urine output, diarrhea, and weight gain are not associated with this disorder.

PTS:1DIF:Analyze

REFpheochromocytoma: Assessment with Clinical Manifestations

2. A client is receiving diagnostic tests to determine the presence of a malignant thyroid lesion. Which of the following are symptoms that are usually associated with a malignant thyroid? (Select all that apply.)

- | | |
|----|--------------------------------------|
| 1. | Hoarseness |
| 2. | Onset of dysphagia |
| 3. | Age 20; male gender |
| 4. | Thyroid scan revealing a cold nodule |

-
- | | |
|----|--------------|
| 5. | Soft nodules |
|----|--------------|
-
- | | |
|----|----------------------------------|
| 6. | Presence of a single firm nodule |
|----|----------------------------------|
-

ANS: 1, 2, 3, 4, 6

Assessment findings consistent with a malignant thyroid lesion include hoarseness, dysphagia, young adult male; thyroid scan revealing a cold nodule; and the presence of a single firm nodule. Multiple soft nodules are indicative of benign thyroid lesions.

PTS:1DIF:Analyze

REF:Table 56-5 Comparison of Benign and Malignant Thyroid Lesions

3.The nurse suspects a client is experiencing the early signs of myxedema coma when which of the following is assessed? (Select all that apply.)

-
- | | |
|----|--------------------------------|
| 1. | Reduced level of consciousness |
|----|--------------------------------|
-
- | | |
|----|-------------|
| 2. | Hypothermia |
|----|-------------|
-
- | | |
|----|-----------------|
| 3. | Hypoventilation |
|----|-----------------|
-
- | | |
|----|-------------|
| 4. | Hypotension |
|----|-------------|
-
- | | |
|----|-------------|
| 5. | Bradycardia |
|----|-------------|
-
- | | |
|----|----------------------|
| 6. | Reduced urine output |
|----|----------------------|
-

ANS: 1, 2, 3, 4, 5

Myxedema is a medical emergency. The client will present with a diminished level of consciousness, hypothermia, hypoventilation, hypotension, and bradycardia. Prior to the coma, the client may be depressed, confused, paranoid, or even manic. Reduced urine output is not associated with this disorder.

PTS:1DIF:AnalyzeREF:Myxedema Coma

4.The nurse is planning care for a client diagnosed with hypercalcemia caused by hyperparathyroidism. Which of the following should the nurse add as interventions to this clients care plan? (Select all that apply.)

1. Administer high volume intravenous fluids as prescribed.
2. Monitor arterial blood gases.
3. Calculate sodium chloride intake to achieve 400 mEq each day.
4. Provide low rates of intravenous fluids.
5. Provide thyroid replacement medication orally.
6. Monitor body temperature.

ANS: 1, 3

Management of fluid and electrolytes is the priority for a client diagnosed with hypercalcemia caused by hyperparathyroidism. The client needs intensive hydration with intravenous normal saline. The nurse also needs to ensure that the client receives greater than 400 mEq of sodium chloride each day. The other answer choices are interventions appropriate for a client diagnosed with myxedema.

TESTBANKSELLER.COM

Chapter 31 Nursing Assessment: Reproductive Function

MULTIPLE CHOICE

1. A male client asks the nurse about the purpose of the prostate gland. The nurse should respond that it is a structure that:

1. secretes an alkaline substance that neutralizes residual acidic urine in the urethra.
2. provides a milky alkaline substance that neutralizes the acidity of the male urethra and the female vagina.
3. secretes a fluid for the health and nutrition of sperm.
4. propels sperm into the ejaculatory duct.

ANS: 2

The prostate gland produces a milky alkaline fluid that helps neutralize the acidity of the male urethra and female vagina. The bulbourethral (Cowpers) gland secretes an alkaline substance that neutralizes any residual acidic urine in the urethra. The seminal vesicles secrete a fluid for the health and nutrition of sperm. The vas deferens is a duct that propels sperm into the ejaculatory duct.

PTS:1 DIF:Apply REF: Prostate Gland

2. A 50-year-old male client has had a prostate-specific antigen test. The nurse realizes that the normal range for this test would be:

- | | |
|----|---------------|
| 1. | 0 to 2 ng/mL. |
| 2. | 0 to 3 ng/mL. |
| 3. | 0 to 4 ng/mL. |
| 4. | 0 to 5 ng/mL. |

ANS: 3

The prostate-specific antigen is used to test for both benign and malignant diseases of the prostate. A PSA reading of 4 nanograms and below is considered normal.

PTS: 1 DIF: Analyze REF: Male Diagnostic Tests

3. A male client, having difficulty voiding, tells the nurse that he thinks something is wrong with his penis. The nurse reviews the structures of the penis with the client and explains that the structure that surrounds the urethra is the:

- | | |
|----|--------------------|
| 1. | corpus cavernosa. |
| 2. | corpus spongiosum. |
| 3. | glans penis. |
| 4. | prepuce. |

ANS: 2

The corpus spongiosum surrounds the urethra. The corpus cavernosa lies near the top of the penis. The glans penis is the erectile tip of the penis, and the prepuce is the foreskin.

PTS: 1 DIF: Apply REF: Penis

4.The nurse is preparing to discuss the male reproductive system with a group of adolescent school students. Which of the following would the nurse *not* include as a primary function of the male reproductive system?

1. Frequent erectile functioning and increased libido
2. Production of sperm
3. Secretion of testosterone
4. Transportation and depositing of sperm

ANS: 1

The primary functions of the male reproductive system are the production of sperm, the transportation and depositing of sperm in the female reproductive tract, and the secretion of testosterone. Frequent erectile functioning and increased libido are not primary functions.

PTS: 1 DIF: Apply REF: Male Reproductive System

5.A male client is diagnosed as being infertile. The nurse realizes which of the following structures of the clients reproductive system is affected?

1. Epididymis
2. Rete testes
3. Seminal vesicles
4. Seminiferous tubules

ANS: 4

The seminiferous tubules produce spermatozoa. The rete testes and epididymis store sperm. The seminal vesicles secrete a fluid for the health and nutrition of sperm.

PTS:1DIF:AnalyzeREF:Spermatogenesis

6.The nurse, preparing to discuss the female reproductive system with a group of adolescent females, would include that which of the following is *not* a primary function of the female reproductive system?

-
- | | |
|----|---------------|
| 1. | Breastfeeding |
|----|---------------|
-
- | | |
|----|-------------------|
| 2. | Hormone secretion |
|----|-------------------|
-
- | | |
|----|-----------|
| 3. | Pregnancy |
|----|-----------|
-
- | | |
|----|---------------------|
| 4. | Sensory innervation |
|----|---------------------|
-

ANS: 4

The primary functions of the female reproductive system are the production of ova, the secretion of hormones, pregnancy and birth of a fetus, and breastfeeding. Sensory innervation is not a primary function of the female reproductive system.

PTS: 1 DIF: Apply REF: Female Reproductive System

7.A young adult female client is concerned that she does not have enough eggs since she has not yet become pregnant. The nurse should assure her that the number of ova available to produce a pregnancy would be around:

-
- | | |
|----|------|
| 1. | 500. |
|----|------|
-
- | | |
|----|---------|
| 2. | 10,000. |
|----|---------|
-
- | | |
|----|----------|
| 3. | 300,000. |
|----|----------|
-
- | | |
|----|------------|
| 4. | 2,000,000. |
|----|------------|
-

ANS: 3

At birth the ovaries contain between 2 and 4 million ova. Most of the ova degenerate across time until there are only 300,000 to 400,000 ova present at puberty. A woman may release fewer than 500 mature ova during monthly ovulation.

PTS: 1 DIF: Apply REF: Ovaries

8.During a gynecological exam, it is noted that a client's os is in the shape of a slit. The nurse realizes that this shape means that the client has:

-
- | | |
|----|-----------------|
| 1. | borne children. |
|----|-----------------|
-

-
- | | |
|----|---------------------|
| 2. | not started menses. |
|----|---------------------|
-
- | | |
|----|-------------------------|
| 3. | not borne any children. |
|----|-------------------------|
-
- | | |
|----|-------------------------|
| 4. | gone through menopause. |
|----|-------------------------|
-

ANS: 1

The shape of the os in women who have not borne children is circular. In women who have borne children, the os is slit-like. The shape of the cervical os does not change if a client has not started menses or has gone through menopause.

PTS: 1 DIF: Analyze REF: Cervix

9.The nurse, reviewing the reproductive hormones needed to produce sperm and ova, realizes that which of the following hormones is *not* involved in the formation of sperm and ova?

-
- | | |
|----|------------------------------|
| 1. | Follicle-stimulating hormone |
|----|------------------------------|
-
- | | |
|----|--------------------------------|
| 2. | Gonadotropin-releasing hormone |
|----|--------------------------------|
-
- | | |
|----|---------------------|
| 3. | Luteinizing hormone |
|----|---------------------|
-
- | | |
|----|-----------|
| 4. | Prolactin |
|----|-----------|
-

ANS: 4

Gonadotropin-releasing hormone stimulates the release of follicle-stimulating hormone and luteinizing hormone. Follicle-stimulating hormone stimulates the production of sperm and ovum. In men, luteinizing hormone stimulates the testosterone needed for sperm production, and in women, it stimulates ovulation. Prolactin is necessary for breast formation and the production of breast milk.

PTS: 1 DIF: Analyze REF: Spermatogenesis; Menarche

10.During the examination of the male testes, the nurse should instruct the client on:

-
- | | |
|----|--|
| 1. | the importance of having an annual prostate examination. |
|----|--|
-
- | | |
|----|---------------------------------------|
| 2. | monthly testicular self-examinations. |
|----|---------------------------------------|
-

-
3. why a colonoscopy is important every 10 years after the age of 50.
 4. how a condom prevents the spread of sexually transmitted infections.
-

ANS: 2

When examining the testes, this portion of the examination can be used to teach the client about monthly testicular self-examination. This portion of the examination is not the best time to instruct the client regarding annual prostate examinations, colonoscopies, or the use of condoms.

PTS: 1 DIF: Apply REF: Physical Examination of the Male Genitalia

11.A male client has a prostate specific antigen level of 22 nanograms. The nurse realizes that this client will most likely be scheduled for a(n):

-
1. bone scan
 2. CT scan
 3. testicular biopsy
 4. duplex ultrasonography
-

TESTBANKSELLER.COM

ANS: 1

In clients with PSA levels of 20 nanograms and higher, a radionuclide bone scan is done to rule out metastasis. A CT scan detects enlarged lymph nodes, but it does not provide clear pictures of intraprostatic features. A testicular biopsy is not needed with an elevated prostate-specific antigen level. A duplex ultrasonography is used to diagnose marked arterial insufficiency as a cause of erectile dysfunction.

PTS: 1 DIF: Apply REF: Male Diagnostic Tests

12.A female clients Pap test revealed atypical results. The nurse realizes that this client will most likely be scheduled for a(n):

-
1. culdoscopy.
 2. colposcopy.
 3. loop electrosurgical excision.
-

-
4. cold-knife conization.

ANS: 2

Women with atypical Pap smear results should receive further evaluation with colposcopy. A culdoscopy is the examination of the viscera of the female pelvic cavity. Loop electrosurgical excision is a procedure to sample tissue from the cervix. Cold-knife conization is another method to take a tissue sample from the cervix.

PTS: 1 DIF: Analyze REF: Female Diagnostic Tests

MULTIPLE RESPONSE

1. A female client has an infection of the paraurethral glands. When asked by the client what these glands do, the nurse should respond: (Select all that apply.)

-
1. These glands function like the prostate gland in the male.
-
2. These glands secrete mucus near the vaginal opening.
-
3. These glands secrete mucus.
-
4. These glands are similar to the Cowpers glands in the male.
-
5. These glands are located inside the urethra.
-
6. These glands serve no real function.
-

ANS: 1, 3, 5

The paraurethral glands or Skenes glands in a female are equivalent to the prostate in the male. They are located just inside of and on the posterior area of the urethra, and they secrete mucus. The bulbourethral glands or Bartholins glands in the female secrete mucus near the vaginal opening. The bulbourethral glands are similar to the Cowpers glands in the male. The paraurethral glands do serve a purpose and function.

PTS: 1 DIF: Apply

REF: paraurethral (Skenes) Glands; Bulbourethral (Bartholins) Glands

2. The nurse is instructing a postmenopausal client in the importance of having serum lipid levels analyzed because after menopause, which of the following changes can occur? (Select all that apply.)

- | | |
|----|------------------------------------|
| 1. | Total cholesterol increases |
| 2. | Low-density lipoprotein increases |
| 3. | Triglycerides increase |
| 4. | High-density lipoprotein decreases |
| 5. | Low-density lipoprotein decreases |
| 6. | High-density lipoprotein increases |

ANS: 1, 2, 3, 4

After the age of 50 for women, total cholesterol, low-density lipoprotein cholesterol, and triglyceride levels increase after menopause. The high-density lipoprotein cholesterol levels decline, which promotes atherosclerosis. After menopause the low-density lipoprotein levels will not decrease. After menopause, the high-density lipoprotein levels will not increase.

PTS: 1 DIF: Apply REF: Menopause

TESTBANKSELLER.COM

3.The nurse is reviewing the physiological sexual response pattern within males and females and realizes that which of the following occur in both genders? (Select all that apply.)

- | | |
|----|-------------|
| 1. | Resolution |
| 2. | Orgasm |
| 3. | Erection |
| 4. | Lubrication |
| 5. | Plateau |
| 6. | Excitement |

ANS: 1, 2, 5, 6

The physiological sexual response pattern that occurs in both males and females are: excitement, plateau, orgasm, and resolution. Erection is a response in males. Lubrication is a response in females.

PTS: 1 DIF: Analyze REF: Sexual Response Cycle

4.A female client is concerned that she has not had sexual intercourse with her husband for over 2 months. Which of the following can the nurse respond as causes for an alteration in sexual functioning? (Select all that apply.)

- | | |
|----|-----------------------|
| 1. | Chronic illnesses |
| 2. | Physical disabilities |
| 3. | Negative body image |
| 4. | Medications |
| 5. | Surgical procedures |
| 6. | Employment status |

ANS: 1, 2, 3, 4, 5

There are a variety of causes for sexual dysfunction. Some reasons include chronic illnesses, physical disabilities, negative body image, medications, and surgical procedures. Employment status is not an identified cause for sexual dysfunction.

PTS:1DIF:ApplyREF:Sexual Dysfunction

5.The nurse is concerned that a female client might be experiencing intimate partner violence. Which of the following assessment questions can be used to gain more information from the client? (Select all that apply.)

- | | |
|----|--|
| 1. | In the last year have you been hit, slapped, or physically hurt by someone? |
| 2. | Are you currently sexually active? |
| 3. | Within the last year has someone made you do something sexual that you did not want to do? |

-
4. Is sex satisfying to you?
-
5. Are you afraid of your partner or anyone else?
-
6. Do you have discomfort with intercourse?

ANS: 1, 3, 5

To assess if a client might be experiencing intimate partner violence, the nurse can ask the questions: In the last year have you been hit, slapped, or physically hurt by someone?; Within the last year has someone made you do something sexual that you did not want to do?; and Are you afraid of your partner or anyone else? The other choices are questions that are used for a sexual history.

Chapter 32 Nursing Management: Patients with Female Reproductive and Breast Disorders

MULTIPLE CHOICE

1. A client tells the nurse that she experiences heavy menstrual bleeding. The nurse would document this condition as being:

-
1. dysmenorrhea.
-
2. menorrhagia.
-
3. metrorrhagia.
-
4. polymenorrhea.

ANS: 2

Menorrhagia is heavy menstrual bleeding. Metrorrhagia is bleeding between menses. Dysmenorrhea is pain during the menstrual cycle, and polymenorrhea is having menstrual cycles at 2- to 3-week intervals.

PTS: 1 DIF: Apply REF: Dysmenorrhea; Dysfunctional Uterine Bleeding

2.A client tells the nurse that she has not had menstrual cycles for 2 months since she has been training for a marathon. The nurse would document this clients lack of regular menstrual cycles as being:

- | | |
|----|-----------------------|
| 1. | dysmenorrhea. |
| 2. | primary amenorrhea. |
| 3. | oligomenorrhea. |
| 4. | secondary amenorrhea. |

ANS: 4

Secondary amenorrhea is when a woman has normal menstrual cycles but then stops. Dysmenorrhea is pain during the menstrual cycle, and oligomenorrhea is the absence of menstrual cycles for 3 months or longer. Primary amenorrhea is the lack of a menstrual cycle by age 16.

PTS: 1 DIF: Apply REF: Amenorrhea

3.The nurse is documenting that a female client is menopausal because the client has not had a menstrual cycle in:

- | | |
|----|------------|
| 1. | 6 months. |
| 2. | 9 months. |
| 3. | 12 months. |
| 4. | 15 months. |

ANS: 3

Women are considered menopausal if they have not had a menstrual cycle for 12 months. A perimenopausal state may exist prior to actual menopause.

PTS: 1 DIF: Apply REF: Menopause

4.A female client is prescribed estrogen (Alora) for hot flashes associated with menopause. Which of the following should the nurse instruct this client regarding this medication?

-
1. Hot flashes can increase.

 2. Weight gain can occur.

 3. Breast tenderness and spotting are side effects.

 4. Abdominal pain is to be expected.

ANS: 3

The nurse should instruct the client prescribed estrogen (Alora) that side effects include breast tenderness, nausea, depression, headache, and spotting (bleeding). Hot flashes do not increase with this medication. Weight gain is not a documented side effect of this medication. Abdominal pain is not an expected side effect of this medication.

PTS:1DIF:Apply

REF:Table 62-1 Medications Used to Treat Menstrual Disorders

5.The nurse is caring for a female client recovering from surgery to remove the uterus, cervix, ovaries, and fallopian tubes using a traditional approach. The nurse realizes this client has had a:

-
1. complete hysterectomy.

 2. laparoscopically assisted vaginal hysterectomy.

 3. partial hysterectomy.

 4. total abdominal hysterectomy and bilateral salpingo-oophorectomy.

ANS: 4

Removal of the uterus, ovaries, and fallopian tubes through an abdominal incision is called a total abdominal hysterectomy and bilateral salpingo-oophorectomy. A hysterectomy performed vaginally via laparoscope is a laparoscopically assisted vaginal hysterectomy. A partial hysterectomy removes the body of the uterus without the cervix, and a complete hysterectomy is the removal of the entire uterus.

PTS:1DIF:Analyze

REF: Dysfunctional Uterine Bleeding: Planning and Implementation: Surgery

6.A female client, experiencing vulvar itching and discomfort, is diagnosed with *Candida*. What would the nurse expect to find when assessing this client?

1. Foul, fishy odor
2. Gray, thin, watery discharge
3. Thick, white discharge
4. Yellow, green discharge

ANS: 3

Candida typically produces a thick, white discharge. Bacterial vaginosis causes a white or gray, thin, watery discharge and an odor. Trichomoniasis has a frothy, green/yellow/white discharge.

PTS:1DIF:ApplyREF:Infections: Vaginitis

7.A female client who has been menstruating has a temperature of 103.5F, blood pressure 88/56 mmHg, and a diffuse rash. The nurse realizes that this client is most likely experiencing:

1. pelvic inflammatory disease.
2. herpes simplex virus.
3. human papillomavirus.
4. toxic shock syndrome.

ANS: 4

Toxic shock syndrome is an acute illness associated with menstruation and tampon use. Symptoms include a high fever, a diffuse rash, falling blood pressure, nausea, vomiting, diarrhea, myalgia, disorientation, and coma. Herpes simplex virus usually results in a genital sore or ulcer. The human papillomavirus is associated with genital warts. Pelvic inflammatory disease is an inflammatory condition of the female pelvic organs.

PTS: 1 DIF: Analyze REF: Toxic Shock Syndrome

8.The nurse is teaching a group of young adults about prevention of sexually transmitted infections (STIs). Which of the following instructions would not be included during teaching?

-
1. Abstinence is the only way to completely prevent STIs.
 2. Condoms provide some protection against STIs.
 3. Make sure you and your partner finish the entire treatment regimen.
 4. Once one STI is diagnosed, you are less likely to have an infection with another STI.
-

ANS: 4

Once one STI is diagnosed, an individual is more likely to have an infection with another STI. The person should be tested for all STIs. The other choices would be appropriate for the nurse to instruct regarding STIs.

PTS:1DIF:Apply

REF: Sexually Transmitted Infection: Planning and Implementation: Population-Based Care

9.A 52-year-old female client had been treated for human papillomavirus. After 3 years of testing, the clients Pap smears are normal. The nurse realizes that the clients next Pap smear should be in:

TESTBANKSELLER.COM

-
1. 2 years
 2. 3 years
 3. 5 years
 4. 10 years
-

ANS: 2

If the client is between the ages of 30 to 70 and has three normal Pap smear results, the client does not need to have another Pap smear for 3 years. If the client is between the ages of 21 to 30 and has normal Pap smear results, the client needs another Pap smear in 2 years. If the client is over the age of 70 and the last three Pap smear results were normal, within 10 years, the Pap smears can be discontinued.

PTS: 1 DIF: Analyze REF: Table 62-3 Changes in Pap Smear Guidelines

10.A female client has had a type 1 female circumcision. The nurse realizes that which of the following has been surgically removed on the client?

-
1. Clitoris

 2. Clitoris and labia minora

 3. Clitoris, labia minora, inner surface of labia majora, and suturing of the vagina

 4. Clitoris and uterus

ANS: 1

Type 1 female circumcision is the removal of the clitoris. Type II includes the removal of the clitoris and labia minora. Type III is the removal of the clitoris, labia minora, inner surface of the labia majora, and suturing of the labia majora together to cover the urethra and vagina. There is not a type that is the removal of the clitoris and uterus.

11. The nurse is instructing a female client on what should be done if a lump is discovered while performing breast self-examination (BSE). What should the nurse instruct the client to do?

-
1. Call her physician and immediately schedule an appointment.

 2. Call to schedule an appointment next month.

 3. Take the antibiotics she has in her medicine cabinet.

 4. Wait until next month's BSE to make sure the lump is still there.

ANS: 1

Follow-up on a lump should begin immediately. The client should not wait to see if the lump remains or changes, and she should not medicate herself.

PTS: 1 DIF: Apply REF: Examination of the Breast

12. The nurse determines that a female client has a lower risk for developing breast cancer when which of the following is assessed?

-
1. Alcohol intake

 2. Breastfeeding

3. Obesity

4. Smoking

ANS: 2

Breastfeeding has consistently been shown to decrease a woman's risk of breast cancer. The other options increase a woman's risk of breast cancer.

PTS: 1 DIF: Analyze REF: Breast Cancer: Etiology

13. The nurse should instruct the client that when performing a breast self-examination, pay particular attention to which of the following areas since the greatest number of malignancies are found in this breast area?

1. Upper outer quadrant of the breast to the axilla

2. Portion of the breast closest to the xiphoid process

3. Portion of the breast closest to the abdomen

TESTBANKSELLER.COM

4. Portion of the breast closest to the neck

ANS: 1

The upper outer quadrant of the breast to the axilla is an area that needs to be evaluated since the greatest proportion of malignancies are found in this area of the breast. The other breast areas need to be examined; however, special attention should be given to the upper outer quadrant.

PTS: 1 DIF: Apply REF: Examination of the Breast

14. The nurse should instruct a client, diagnosed with mastalgia, to do which of the following?

1. Have an immediate mammogram.

2. Expect to need a biopsy.

3. Decrease the intake of caffeine.

4. Determine if breast augmentation surgery is desired.

ANS: 3

Mastalgia refers to breast pain. Pain is not generally associated with breast cancer. Wearing a well-fitting supportive brassiere during exercise and decreasing the intake of caffeine would be beneficial. The client does not need an immediate mammogram, a biopsy, or breast augmentation.

PTS: 1 DIF: Apply REF: Mastodynia and Mastalgia

15.A female client tells the nurse that she is planning on having plastic surgery to correct a minor facial defect and then have her breasts done. The nurse would identify which of the following nursing diagnoses as being appropriate for this client?

- | | |
|----|--------------------------|
| 1. | Ineffective coping |
| 2. | Anxiety |
| 3. | Hopelessness |
| 4. | Body dysmorphic disorder |

ANS: 4

TESTBANKSELLER.COM

Body dysmorphic disorder is characterized by a preoccupation with body image and the slight or imagined defect in appearance that leads to impairment or distress in functioning in social situations. Body dysmorphic disorder would be appropriate for the client who is planning on having plastic surgery for a minor facial defect and then breast augmentation surgery. The other nursing diagnoses would not be appropriate for the client at this time.

PTS: 1 DIF: Analyze REF: Breast Alterations: Psychological Aspects

16.The nurse is determining if a female client is at risk for benign breast disease. Which of the following is a risk factor for this disorder?

- | | |
|----|----------------|
| 1. | Smoking |
| 2. | Caffeine use |
| 3. | Alcohol intake |
| 4. | Age 55 |

ANS: 2

Risk factors for benign breast disease include caffeine use, imbalance between estrogen and progesterone, estrogen excess, hyperprolactemia, and age between 20 to 50 years. Smoking and alcohol intake are not risk factors for benign breast disease.

PTS: 1 DIF: Analyze REF: Red Flag: Risk Factors for Benign Breast Disease

17.A client is scheduled for a prophylactic mastectomy. The nurse should remind the client that skin flaps will be left after the surgery for:

-
1. reconstruction.

 2. suturing to the chest wall.

 3. possible use for other skin disorders.

 4. donation for someone needing a skin transplant.

ANS: 1

The goal of a mastectomy is to remove all breast tissue, including the nipple and areola, while leaving well-perfused viable skin flaps for primary closure or reconstruction. The skin flaps will not be sutured to the chest wall. The skin flaps are not for use for other skin disorders. The skin flaps are not for donation for someone needing a skin transplant.

18.When instructing a client on breast self-examination, the nurse reviews the importance of visual inspection of the breasts. Which of the following should the nurse instruct the client to focus on when doing this part of the examination? (Select all that apply.)

-
1. Contour and symmetry of the breasts

 2. Skin changes

 3. Position of the nipples

 4. Presence or absence of masses

 5. Pain

6. Size

ANS: 1, 2, 3, 4

Visual inspection of the breast self-examination focuses on the contour and symmetry of the breasts; skin changes such as scaling, puckering, dimpling, or scars; the position of the nipples; nipple discharge or retraction; and presence or absence of masses. This part of the examination does not include pain or size of the breasts.

PTS: 1 DIF: Apply REF: Examination of the Breast

19.The nurse is preparing to assess a clients nipples during a breast examination. Which of the following are considered pathological conditions that affect the nipple? (Select all that apply.)

1. Bleeding

2. Lumps

3. Discharge

4. Scars

5. Fissures

6. Large size

ANS: 1, 3, 5

The three primary pathological conditions of the nipple include bleeding, discharge, and fissures. Lumps, scars, and size are not associated with pathological conditions of the nipple.

PTS:1DIF:AnalyzeREF:Nipple Disorders

20.Which of the following should the nurse do if a female client is experiencing nipple discharge? (Select all that apply.)

1. Note the color of the discharge.

2. Determine if the discharge is from one or both breasts.

-
3. Obtain a sample of the discharge with a sterile cotton-tipped swab.
 4. Assess the nipple drainage for occult blood
 5. Apply sterile bandages over the nipple.
 6. Pad the clients bra with gauze.
-

ANS: 1, 2, 3, 4

If a female client is assessed with abnormal nipple discharge, the nurse should note the color of the discharge; determine if the discharge is from one or both breasts; obtain a sample of the discharge with a sterile cotton-tipped swab; and assess the drainage for occult blood. The nurse should not apply sterile bandages over the nipple nor pad the clients bra with gauze.

PTS: 1 DIF: Apply REF: Red Flag: Examining Nipple Discharge

21.A client is experiencing galactorrhea. Which of the following should the nurse assess in this client? (Select all that apply.)

-
1. Recent vigorous nipple stimulation
 2. Prescribed hormones, blood pressure medications, or antidepressants
 3. Intake of herbal remedies such as fennel or anise
 4. Use of street drugs such as opiates and marijuana
 5. Recent chest trauma
 6. Age of menarche
-

ANS: 1, 2, 3, 4, 5

Galactorrhea is the secretion of a milk-like fluid in a non-lactating breast. This can occur because of recent vigorous nipple stimulation, prescribed hormones, blood pressure medication, or antidepressants; intake of herbal remedies such as fennel or anise; use of street drugs such as opiates and marijuana; and recent chest trauma. Age of menarche will not help determine the cause for the disorder.

PTS:1DIF:ApplyREF:Galactorrhea

22.A client is considering breast augmentation surgery. Which of the following postoperative complications should the nurse discuss with the client regarding this surgery? (Select all that apply.)

- | | |
|----|-----------------------------------|
| 1. | Change in sensation |
| 2. | Development of a hematoma |
| 3. | Fibrous tissue around the implant |
| 4. | Heart palpitations |
| 5. | High blood pressure |
| 6. | Arm pain |

ANS: 1, 2, 3

TESTBANKSELLER.COM

Postoperative complications with breast augmentation include change in sensation, development of a hematoma; and formation of fibrous tissue around the implant. Heart palpitations, high blood pressure, and arm pain are not considered postoperative complications of breast augmentation surgery.

Chapter 33 Nursing Management: Care of the Assigned Male at Birth/Male Client with Reproductive Disorders

MULTIPLE CHOICE

1.The nurse is instructing a client diagnosed with acute prostatitis. Which of the following instructions would be the *least* beneficial to the client?

- | | |
|----|---|
| 1. | Avoid alcohol and caffeine. |
| 2. | Sex should be avoided during the acute phase. |
| 3. | Sit for as long as you can. |

-
4. Sitz baths may provide comfort.

ANS: 3

The patient should be encouraged to use sitz baths for comfort but not to sit in them for long periods of time. Caffeine, alcohol, and sex should be avoided during the acute phase.

PTS: 1 DIF: Apply REF: Prostatitis: Patient and Family Teaching

2. The nurse is documenting the health history of a client diagnosed with benign prostatic hyperplasia (BPH). In which of the following areas would the nurse take a careful history?

-
1. Bowel patterns
-
2. Eating patterns
-
3. Sleeping patterns
-
4. Urinary patterns

ANS: 4

TESTBANKSELLER.COM

A careful history on urinary patterns should be taken by the nurse. The ease with which the stream of urine is started, the strength of the stream, and the perceived amount of urine eliminated with each voiding, along with the patient's sense about whether the bladder is completely emptied and the presence of nocturia or dribbling, should be noted. The client's bowel, eating, and sleeping patterns are also important; however, they are not as important as the urinary patterns.

PTS: 1 DIF: Apply

REF: Benign Prostatic Hyperplasia: Assessment with Clinical Manifestations

3. A client, diagnosed with benign prostatic hyperplasia (BPH), should be instructed to do which of the following?

-
1. Do nothing since this disorder does not require any follow-up.
-
2. Decrease water intake to avoid dribbling.
-
3. Void every 2 to 3 hours.

-
4. Wear scrotal support.

ANS: 3

Clients with BPH should void every 2 to 3 hours to flush the urinary tract. Water should not be decreased because this will irritate the urinary mucosa. Scrotal support is not necessary, and BPH does require follow-up visits.

PTS:1DIF:Apply

REF:Benign Prostatic Hyperplasia: Patient and Family Teaching

4.A client, recovering from a transurethral resection of the prostate (TURP) with a continuous bladder irrigation system to a three-way indwelling urinary catheter, tells the nurse he has to void. What nursing intervention should the nurse perform?

-
1. Call the physician.
-
2. Increase the flow of the irrigant.
-
3. Irrigate the catheter.
-
4. Tell the client to void.

TESTBANKSELLER.COM

ANS: 3

After a TURP, clots that can occlude the catheter and create a sensation to void in the client are common. The nurse should irrigate the catheter to allow the urine to flow. The nurse does not need to phone the physician, increase the flow of the irrigant, or tell the client to void.

PTS: 1 DIF: Apply REF: Benign Prostatic Hyperplasia: Surgery

5.A client who is 12 hours postoperative after a transurethral resection of the prostate (TURP) is concerned about the blood clots in the catheter and urinary collection bag. How should the nurse respond?

-
1. I need to call your physician.
-
2. I will need to stop the bladder irrigation.
-
3. Blood clots are common during this time frame and will start to decrease in a day.

-
4. You need to stop moving and irritating the catheter.

ANS: 3

Blood clots are common during the first 36 hours following a TURP. The irrigant should not be stopped because it is flushing the clots out of the urinary system. A large amount of bright red blood would be an indication of hemorrhage. The nurse does not need to call the physician.

PTS: 1 DIF: Apply REF: Benign Prostatic Hyperplasia: Surgery

6.A client is being screened for prostate cancer. What tests would be completed at this time?

-
1. Digital rectal examination and transrectal ultrasonography
-
2. Biopsy of the prostate and magnetic resonance imagery
-
3. Complete blood cell count and prostate-specific antigen
-
4. Prostate-specific antigen (PSA) and digital rectal examination

ANS: 4

TESTBANKSELLER.COM

Early screening for prostate cancer includes a digital rectal examination and a PSA test. Other tests may be ordered later if either the PSA or digital rectal examination are abnormal.

PTS:1DIF:Analyze

REFrostate Cancer: Assessment with Clinical Manifestations

7.The nurse is instructing a client about testicular self-examination (TSE). Which of the following would *not* be included in these instructions?

-
1. The testis should feel smooth and egg-shaped.
-
2. Perform TSE after a bath or shower.
-
3. TSE should be performed monthly by every male older than age 40.
-
4. Any lumps and changes in the testicles should be reported.

ANS: 3

The highest risk group for testicular cancer is young men 15 to 35 years of age. TSE should be taught and performed monthly from the teenage years. The other choices are appropriate for the nurse to instruct the client.

PTS: 1 DIF: Apply REF: Testicular Cancer: Planning and Implementation

8.A male client is diagnosed with orchitis. The nurse should assess the client for which of the following?

-
- | | |
|----|-----------------------------|
| 1. | Recent infection with mumps |
|----|-----------------------------|
-
- | | |
|----|---------------------------------|
| 2. | Recent diagnosis of prostatitis |
|----|---------------------------------|
-
- | | |
|----|-------------------------------------|
| 3. | History of type 2 diabetes mellitus |
|----|-------------------------------------|
-
- | | |
|----|----------------------------------|
| 4. | Diagnosis of renal insufficiency |
|----|----------------------------------|

ANS: 1

Mumps is the most common viral cause of orchitis, with the orchitis occurring 4 to 7 days after the onset of mumps. Orchitis is not associated with prostatitis, type 2 diabetes mellitus, or renal insufficiency.

TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Orchitis

9.A client is diagnosed with a spermatocele. The nurse should instruct the client on which of the following?

-
- | | |
|----|---|
| 1. | The use of heat to reduce the size of the inflamed area |
|----|---|
-
- | | |
|----|--|
| 2. | The potential need for surgery to correct the disorder |
|----|--|
-
- | | |
|----|--|
| 3. | The use of ice packs to reduce the size of the inflamed area |
|----|--|
-
- | | |
|----|---|
| 4. | The importance of using antibiotics to treat the disorder |
|----|---|

ANS: 2

Spermatoceles may become significantly uncomfortable and require treatment. Surgical correction may be done if infertility is associated with the spermatocele. Surgical removal of the spermatocele is performed under local anesthesia. Heat, ice, and antibiotics are not the first line treatments for the disorder.

PTS:1DIF:Apply

REF: Hydrocele, Hematocele, and Spermatocoele: Surgery

10.A client is diagnosed with a varicocele. The nurse realizes that this client is likely to develop:

- | | |
|----|------------------|
| 1. | hydrocele. |
| 2. | prostate cancer. |
| 3. | prostatitis. |
| 4. | infertility. |

ANS: 4

Infertility or subinfertility often occurs in conjunction with varicocele because the increased blood flow in the varicocele raises the scrotal temperature about 93.2F, which is the ideal temperature for spermatogenesis. The client is not likely to develop a hydrocele, prostate cancer, or prostatitis from a varicocele.

PTS: 1 DIF: Analyze REF: Varicocele: Pathophysiology

11.A newborn male child is diagnosed with cryptorchidism. The nurse should prepare to administer which of the following to this client?

- | | |
|----|---|
| 1. | Intravenous fluids |
| 2. | Antipyretic medication |
| 3. | Human chorionic gonadotropin medication |
| 4. | Antibiotics |

ANS: 3

Human chorionic gonadotropin may be given intramuscularly to promote bilateral testicular descent. This medication is provided 2 to 3 times a week for up to 6 weeks. Intravenous fluids, antipyretics, or antibiotics are not indicated in the treatment of this disorder.

PTS: 1 DIF: Apply REF: Cryptorchidism: Planning and Implementation

12. A client is experiencing priapism. Which of the following should the nurse do first to help the client?

1. Apply ice packs to the perineum.
2. Prepare for emergency surgery.
3. Prepare for an aspiration of blood from the penis.
4. Apply heat to the perineum.

ANS: 1

The goal of treating priapism is to resolve the condition before permanent damage occurs that leaves the client unable to achieve an erection in the future. Ice packs to the perineum will resolve some cases of the disorder. This is what the nurse should do first. The client does not need emergency surgery. The client may need blood aspirated from the penis. Heat should not be applied to the perineum.

PTS: 1 DIF: Apply REF: Priapism: Planning and Implementation

MULTIPLE RESPONSE TESTBANKSELLER.COM

1. A client is diagnosed with testicular torsion. Which of the following might be indicated for this client? (Select all that apply.)

1. Manually untwist the testicle
2. Orchiopexy
3. Testicle removal
4. Pain management
5. Application of ice and a scrotal support
6. Prescribe medication

ANS: 1, 2, 3, 4, 5

The goal of the treatment for testicular torsion is to untwist the spermatic cord and reestablish normal blood flow to the testicle. The testicle may be manually untwisted to promote blood flow. If this is unsuccessful, the client may need an orchiopexy or a surgical procedure to untwist the testicle. If surgical treatment occurs within 6 hours of the onset of pain, the testicle is salvaged. If treatment is delayed for 12 hours or more, the testicle will begin to necrose and will need to be removed. Pain medication is needed for this disorder. Ice and a scrotal support are used for this disorder. No medications alone will cure this disorder.

PTS: 1 DIF: Apply REF: Testicular Torsion: Planning and Implementation

2.A client is diagnosed with epididymitis. The nurse should instruct the client on which of the following as treatment for the disorder? (Select all that apply.)

1.	Broad spectrum antibiotics
2.	NSAIDs
3.	Bed rest
4.	Elevate the scrotum
5.	Apply cold packs
6.	Apply heat

ANS: 1, 2, 3, 4, 5

Treatment for epididymitis includes broad spectrum antibiotics, NSAIDs, bed rest, elevation of the scrotum, and application of cold packs. Heat is not recommended as treatment for this disorder.

PTS: 1 DIF: Apply REF: Epididymitis

3.Which of the following should the nurse instruct a client who is recovering from a vasectomy? (Select all that apply.)

1.	Use ice packs to control postoperative bleeding.
2.	Wear cotton jockey type briefs for scrotal support.
3.	Use warm sitz baths to aid in comfort.

-
4. Recognize the signs and symptoms of postoperative infection.
 5. A vasectomy protects the client from sexually transmitted illnesses.
 6. Ejaculate will be reduced after the procedure.
-

ANS: 1, 2, 3, 4

The client recovering from a vasectomy should be instructed to use ice packs to control postoperative bleeding, wear cotton jockey type briefs for scrotal support, use warm sitz baths to aid in comfort, and recognize the signs and symptoms of postoperative infection. A vasectomy does not protect the client from sexually transmitted illnesses. Ejaculate will not be reduced after the procedure.

PTS: 1 DIF: Apply REF: Vasectomy: Patient and Family Teaching

4.The nurse is assessing a client diagnosed with balanitis and posthitis. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

1.	Penile discharge
----	------------------

2.	Hematuria
----	-----------

3.	Pain
----	------

4.	Erythema
----	----------

5.	Flank pain
----	------------

6.	Edema
----	-------

ANS: 1, 3, 4, 6

The typical manifestations for balanitis and posthitis include penile discharge, pain, erythema, and edema. Hematuria and flank pain are not associated with this disorder.

PTS:1DIF:Apply

REF:Balanitis and Posthitis: Assessment with Clinical Manifestations

5.The nurse is assessing a client who is experiencing erectile dysfunction. For which of the following should the nurse assess the client? (Select all that apply.)

1.	Diagnosis of diabetes mellitus
2.	Thyroid disease
3.	Chronic renal failure
4.	Multiple sclerosis
5.	Parkinsons disease
6.	Gastroesophageal reflux disease

ANS: 1, 2, 3, 4, 5

Erectile dysfunction has been associated with diabetes mellitus, thyroid disease, chronic renal failure, multiple sclerosis, and Parkinsons disease. Erectile dysfunction has not been associated with gastroesophageal reflux disease.

TESTBANKSELLER.COM

Chapter 34 Nursing Management: Patients with Sexually Transmitted Infections

MULTIPLE CHOICE

Identify the choice that best completes the statement or answers the question.

____ 1. During data collection the nurse notes the presence of a chancre on a male patients penis. For which sexually transmitted infection should the nurse focus additional data collection?

- a. Herpes
- b. Syphilis
- c. Gonorrhea
- d. Chlamydia

____ 2. A patient is diagnosed with a parasitic infection caused by close contact with another persons genitals. For which infection should the nurse plan care?

- a. Phthirus pubis
- b. Treponema pallidum
- c. Neisseria gonorrhoeae
- d. Chlamydia trachomatis

____ 3. It is documented in the medical record that a patient has gummas. For which sexually transmitted infection should the nurse plan care?

- a. Syphilis
- b. Gonorrhea
- c. Chlamydia
- d. Genital herpes

____ 4. The nurse is assisting with teaching a 22-year-old female patient who is diagnosed with a sexually transmitted infection (STI). She says, I don't understand. My boyfriend always wears a condom. Which understanding by the nurse should guide teaching in this situation?

- a. Condoms are a reliable source of protection against STIs.
- b. It is a myth that condoms provide any protection against STIs.
- c. Condoms can decrease the risk of STIs, but they are not foolproof.
- d. Condoms must be used with a spermicide to guarantee protection against STIs.

____ 5. The nurse is providing care for a patient with genital herpes who has vesicular lesions. What term should the nurse use to describe these lesions to the patient?

- a. Warts
- b. Rashes
- c. Blisters
- d. Papules

____ 6. Human papillomavirus (HPV) produces verrucous growths. What term should the nurse use to describe these lesions to the patient?

- a. Warts
- b. Rashes
- c. Blisters
- d. Papules

TESTBANKSELLER.COM

____ 7. The nurse is collecting data on a patient with Chlamydia. Which assessment finding should be reported immediately to the RN or physician?

- a. Painful urination
- b. Red conjunctivae
- c. Vaginal discharge
- d. Sharp pain at the base of the ribs

____ 8. Because Trichomonas is relatively large, unusually shaped, and diagnosed quickly, the nurse is asked to assist the physician obtain which type of specimen?

- a. Culture
- b. Blood test
- c. Wet mount
- d. Litmus paper

____ 9. A patient diagnosed with Trichomonas asks the nurse how the diagnosis will affect her risk for cervical cancer. Which response by the nurse is best?

- a. Wet-mount slides should be done yearly to help detect cervical cancer.
- b. Serological testing will be done to detect tumor proteins and screen for cervical cancer.
- c. Papanicolaou smears should be done more frequently because results may be altered by Trichomonas.
- d. Culture and sensitivity testing is done with Papanicolaou (Pap) smears every other year to determine if you have cervical cancer.

____ 10. A patient asks why the physician has recommended systemic interferon treatment for

genital warts. Which explanation should the nurse provide to the patient?

- a. Interferon can improve liver function.
- b. Interferons can increase your red blood cell count.
- c. Interferon treatment does not have any side effects.
- d. Interferon therapy can attack warts all over the body at the same time.

____ 11. A patient with hepatitis B virus (HBV) delivers a 6-pound 2-ounce baby boy. Which action should the nurse anticipate will be needed for the infant?

- a. Intravenous antibiotics for 12 hours
- b. Antiviral eye medication less than 2 hours after birth
- c. There is no treatment that is safe and effective for infants.
- d. HBV-immune globulin less than 12 hours after birth and then HBV vaccine series

____ 12. The nurse must bathe a patient with herpes. What is the nurse's best protection against contracting sexually transmitted infections (STIs) from patients while providing perineal hygiene?

- a. Wearing gloves at all times
- b. Washing hands following care
- c. Practicing standard precautions
- d. Avoiding touching patients who have STIs

____ 13. The nurse is caring for a pregnant woman who is fearful that her unborn child will be born blind because of having a sexually transmitted infection (STI). For which STI should the nurse plan care to prevent ophthalmia neonatorum in the newborn?

- a. Syphilis
- b. Gonorrhea
- c. Genital warts
- d. Genital herpes

TESTBANKSELLER.COM

____ 14. The nurse is caring for a young woman who is newly diagnosed with genital warts. She states, I heard you can get cancer from STIs. Is that true? Which response by the nurse is correct?

- a. No, you cannot get cancer from STIs.
- b. Yes, most STIs can lead to cancerous changes if not treated promptly.
- c. Yes, some STIs have been linked to cancer, so adequate treatment is very important.
- d. No, that is not true, but a diagnosis of cancer does increase the risk of contracting an STI.

____ 15. The nurse is identifying ways for a young adult to reduce the risk of contracting a sexually transmitted infection (STI). What should the nurse teach about the relationship between consumption of alcohol and immediate risk of contracting an STI?

- a. Alcohol may reduce inhibitions.
- b. Alcohol increases risk for liver disease.
- c. Alcohol lowers the body's resistance to infection.
- d. Alcohol impairs the integrity of the mucous membranes, providing a portal of entry for infection.

____ 16. The nurse reviews the ways to prevent condom breakage with a patient. Which patient statement indicates that more teaching is necessary?

- a. Condoms should never be reused.
- b. I should use a water-soluble lubricant.
- c. Before I use a condom, I should inflate it and check it for holes and leaks.
- d. I should make sure to leave a half inch extra space at the end of the condom.

____ 17. The nurse is assisting with the admission of a known intravenous drug abuser to a

medical unit. In addition to drug abuse, which disorder in the patients history is most consistent with a diagnosis of hepatitis?

- a. Jaundice
- b. Diabetes mellitus
- c. Bowel obstruction
- d. Chronic headaches

____ 18. The nurse is teaching a patient the importance of completing treatment for gonorrhea. On which information is the nurse basing this teaching?

- a. Gonorrhea is not treatable.
- b. Only men experience symptoms; women are usually asymptomatic.
- c. Men and women may be asymptomatic and still transmit the infection.
- d. Treatment is associated with many serious side effects, so compliance is low.

____ 19. The nurse is assisting in the preparation of a teaching seminar for adolescents to prevent the development of a sexually transmitted infection (STI). Which nonsexual activity should the nurse teach that may transmit a sexually transmitted infection (STI)?

- a. Sharing a cigarette
- b. Borrowing a hairbrush
- c. Coughing and sneezing
- d. Sharing intravenous drug equipment

____ 20. A patient asks for the best way to prevent contracting a sexually transmitted infection (STI). What response should the nurse make to this patients question?

- a. Abstinence
- b. Oral contraceptives
- c. Condom with spermicide
- d. Prophylactic oral antibiotics

____ 21. A patient diagnosed with genital warts asks how they developed. Which pathogen should the nurse explain as causing genital warts?

- a. Sarcoptes scabiei
- b. Hepatitis A and B
- c. Human papillomavirus
- d. Chlamydia trachomatis

____ 22. The nurse is caring for a 76-year-old retired man who is undergoing evaluation for dementia. What would be an important part of the mans history to report to the physician?

- a. The patient has a history of syphilis.
- b. The patient was exposed to Chlamydia.
- c. The patient has a history of hepatitis B.
- d. The patient has a history of genital warts.

____ 23. A patient is undergoing treatment that involves the burning of lesions with heat or chemical agents. The nurse recognizes that this patient most likely has which condition?

- a. Syphilis
- b. Chlamydia
- c. Hepatitis B
- d. Genital warts

____ 24. The nurse is providing care for a newborn. Which intervention should the nurse make to prevent development of ophthalmia neonatorum?

- a. Interferon injection

- b. Antibiotic eyedrops
- c. Vitamin K injection
- d. Hepatitis B virus (HBV)-immune globulin

____ 25. While reviewing a medical record the nurse notes that patient has a strawberry cervix. For which sexually transmitted infection (STI) would the nurse plan care?

- a. Gonorrhea
- b. Herpes simplex
- c. Trichomoniasis
- d. Human papillomavirus infection

____ 26. The nurse is preparing a poster presentation identifying the frequency of sexually transmitted infections (STIs) in the United States. Which STI should the nurse highlight as being the most commonly diagnosed?

- a. Gonorrhea
- b. Chlamydia
- c. Trichomoniasis
- d. Human papillomavirus

____ 27. While assisting a health care provider (HCP) conduct a pelvic examination, the patient complains of severe pain during the bimanual examination. For which health problem should the nurse suspect this patient is going to need care?

- a. Syphilis
- b. Gonorrhea
- c. Pelvic inflammatory disease
- d. Human papillomavirus infection

____ 28. While assisting with care, the nurse counsels the patient diagnosed with a sexually transmitted infection (STI) about notification of sexual partners. Which patient statement indicates the need for further teaching? (Select all that apply.)

- a. I can contact my sexual partners myself.
- b. Reporting regulations are the same throughout the country.
- c. A report form will be completed in my chart that includes a list of my sexual contacts.
- d. The public health authority can notify a list of sexual contacts without including my identity.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

____ 29. The nurse is assisting with teaching a patient who has been exposed to hepatitis B. Which symptoms should the nurse explain may occur before jaundice appears? (Select all that apply.)

- a. Rash
- b. Nausea
- c. Confusion
- d. Dark-colored urine
- e. Muscle or joint pain
- f. Elevated blood glucose

____ 30. The nurse is reviewing prescribed laboratory tests for a patient demonstrating manifestations of syphilis. What diagnostic tests should the nurse expect to be prescribed for this patient? (Select all that apply.)

- a. RPR
- b. NAT

- c. VDRL
- d. ELISA
- e. Culture
- f. CD4 counts

____ 31. A 24-year-old woman diagnosed with Chlamydia has been prescribed doxycycline. What should be included in the nurses teaching about the drug treatment? (Select all that apply.)

- a. Take this drug with a meal.
- b. Do not take with dairy products.
- c. Avoid unnecessary exposure to sunlight.
- d. Abstain from alcohol for at least 48 hours after treatment.
- e. Use birth control methods to ensure you do not become pregnant.

____ 32. The nurse is teaching a patient about the use of condoms to prevent sexually transmitted infections (STIs). Which information should the nurse include in this teaching? Select all that apply.

- a. Condoms can decrease the risk of transmitting STDs.
- b. Latex condoms are less likely to break than other types.
- c. Inflating the condom prior to use allows for effective inspection.
- d. Condoms should be used no more than twice and then discarded properly.
- e. Use of a water-soluble lubricant with a condom increases its effectiveness in preventing the spread of an STD.
- f. Use of a petroleum-based lubricant with a condom increases its effectiveness in preventing the spread of an STD.

____ 33. The nurse is providing care for a patient recently diagnosed with Chlamydia. Which information should the nurse recommend be included in patient teaching? (Select all that apply.)

- a. Women with Chlamydia may complain of a sore throat.
- b. Chlamydia is characterized by the development of chancres.
- c. Ophthalmia neonatorum is seen in infants born to women with Chlamydia.
- d. Chlamydia can be transmitted sexually and by blood and body fluid contact.
- e. The risk of ectopic pregnancy is increased in women with a history of Chlamydia.
- f. The Chlamydia virus can lie dormant in the nervous system tissues and reactivate when an individual is under stress or has a compromised immune system.

____ 34. The nurse notes that a patient is diagnosed with vulvovaginitis. What should the nurse expect when assessing this patient? (Select all that apply.)

- a. Vaginal edema
- b. Vaginal discharge
- c. Areas of ecchymosis
- d. Dark brown vaginal bleeding
- e. Complaints of vaginal itching and burning

____ 35. A patient in labor is diagnosed with mucopurulent cervicitis. For which health problems should the nurse anticipate providing care to the newborn? (Select all that apply.)

- a. Pneumonia
- b. Conjunctivitis
- c. Irregular heart rate
- d. Flaccid extremities
- e. Cyanotic extremities

____ 36. A patient diagnosed with syphilis reminds the HCP of having an allergy to penicillin.

Which medications should the nurse expect to be prescribed for this patient? (Select all that apply.)

- a. Gentamicin
- b. Amoxicillin
- c. Tetracycline
- d. Doxycycline
- e. Erythromycin

____ 37. While providing a bath the nurse suspects that an older female patient has a Trichomonas infection. What type of discharge did the nurse observe to come to this conclusion? (Select all that apply.)

- a. Frothy discharge
- b. Foul-smelling discharge
- c. Yellow-green discharge
- d. Open sores on the labia majora
- e. Wart-like growths on the labia minora

Answer Section

MULTIPLE CHOICE

1. ANS: B

The primary stage of syphilis begins with the entry of the *Treponema pallidum* spirochete through the skin or mucous membranes. Between 3 and 90 days later, a papule develops at the site of entry, then sloughs off, leaving a painless, red, ulcerated area called a chancre. A. Herpes is associated with vesicular skin lesions. C. D. Gonorrhea and Chlamydia are not associated with skin lesions.

PTS: 1 DIF: Moderate

KEY: Client Need: Safe and Effective Care Environment Safety and Infection Control | Cognitive Level: Application

2. ANS: A

Genital parasites are not a true sexually transmitted infection (STI), but they may be transmitted during close body contact. The two most commonly seen parasites are pubic lice (*Phthirus pubis*, commonly called crabs because of the shape of the lice) and scabies (*Sarcoptes scabiei*). B. C. D. *Treponema pallidum*, *Neisseria gonorrhoeae*, and *Chlamydia trachomatis* are not parasites.

PTS: 1 DIF: Moderate

KEY: Client Need: Safe and Effective Care Environment Safety and Infection Control | Cognitive Level: Application

3. ANS: A

In the tertiary stage of syphilis, the spirochete may form gummas, which are tumors of a rubbery consistency that can break down and ulcerate, leaving holes in body tissues. D. Herpes is associated with vesicular skin lesions. B. C. Gonorrhea and Chlamydia are not associated with skin lesions.

PTS: 1 DIF: Moderate

KEY: Client Need: Safe and Effective Care Environment Safety and Infection Control | Cognitive Level: Application

4. ANS: C

Condoms can greatly decrease the risk of STIs, but condoms can have tiny channels in the rubber (or other elastic material) that can allow microorganisms to pass through. Condoms can break, slip off, or be applied improperly. Petroleum-based lubricants may weaken latex condoms. A. Condoms do not provide a barrier for any area other than the penis and most of the vagina (or anus). B. Some STIs may still be transmitted by contact of surrounding uncovered tissues. D. Spermicide helps protect against pregnancy.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological Integrity Reduction of Risk Potential | Cognitive Level: Application

5. ANS: C

Vesicles are small blisters. A. B. D. Warts, rashes, and papules do not have the same characteristics.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological Integrity Reduction of Risk Potential | Cognitive Level: Application

6. ANS: A

Verrucous means wart-like. B. D. D. HPV causes wart-like growths, not rashes, blisters, or papules.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological Integrity Reduction of Risk Potential | Cognitive Level: Application

7. ANS: D

Fitz-HughCurtis syndrome, a surface inflammation of the liver, can also be caused by C. trachomatis. This inflammation may cause nausea, vomiting, and sharp pain at the base of the ribs that sometimes refers to the right shoulder and arm. A. B. C. Vaginal discharge, painful urination, and conjunctivitis are also concerns but are not as health-threatening as liver inflammation.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological Integrity Reduction of Risk Potential | Cognitive Level: Analysis |

8. ANS: C

When wet-mount slides of Trichomonas discharge are viewed under a microscope, the organisms can be identified by their motility and whip-like flagella. A. B. D. Trichomonas is not diagnosed through a culture, blood test, or litmus paper.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

9. ANS: C

Trichomonas may produce abnormal Pap smear readings, which require that more frequent Pap smears be done to provide adequate surveillance of cellular changes. A. Wet mount can identify the organism but not cellular changes. B. D. Serological testing and culture and sensitivity testing are not performed to detect cervical changes caused by Trichomonas.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

10. ANS: D

Systemic interferon treatments attack warts all over the body at the same time, rather than individually as with topical treatments. This speeds the process of treatment. A. B. C. Interferons can produce side effects of flu-like symptoms, a drop in the number of white blood cells, and changes in liver function.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityPharmacological and Parenteral Therapies | Cognitive Level: Application

11. ANS: D

It is recommended that all babies of HBV-positive mothers receive HBV immune globulin less than 12 hours after birth and then be immunized with HBV vaccine 1 week, 1 month, and 6 months after birth. A. Antibiotics are not effective against viruses. B. Eye medication may be necessary for gonorrhea or chlamydia. C. The infant needs to receive the HBV vaccination.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

12. ANS: C

A nurses best protection against catching infections from blood and body fluids of infected patients is the strict practice of standard precautions and maintaining his or her own healthy, intact skin. A. Wearing gloves at all times is not appropriate. B. Washing hands is essential but is not sufficient. D. Touching patients cannot and should not be avoided.

PTS: 1 DIF: Moderate

KEY: Client Need: Safe and Effective Care EnvironmentSafety and Infection Control | Cognitive Level: Application

13. ANS: B

Newborns born to mothers who have gonorrhea can develop ophthalmia neonatorum, which involves inflammation of the conjunctiva and deeper parts of the eye and can, ultimately, result in blindness. A. C. D. Syphilis, genital warts, and genital herpes are not associated with infant eye problems.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

14. ANS: C

Herpes, human papillomavirus (HPV), and hepatitis (not most sexually transmitted infections [STIs]) have been associated with cancers. A. Cancer has been associated with some STIs. B. Most STIs do not cause cancer. D. Having a diagnosis of cancer does not increase the risk of contracting an STI.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

15. ANS: A

Consumption of alcohol or other psychoactive drugs can reduce inhibitions and may result in unintended sexual encounters, which can transmit STIs. B. C. D. Alcohol does cause liver disease and may indirectly reduce resistance, but these are not the mechanisms by which immediate STI risk increases.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

16. ANS: C

Condoms should never be inflated to test them, because this can weaken them. B. Lubrication decreases the chances of breakage during use, but only water-soluble lubricants should be used, because substances such as petroleum jelly (Vaseline) may weaken the condom. D. Either condoms with a reservoir tip or regular condoms that have been applied while holding approximately 1/2 inch of the closed end flat between the fingertips allow room for expansion by the ejaculate without creating excessive pressure, which might break the condom. A. Condoms should not be reused.

PTS: 1 DIF: Moderate

KEY: Client Need: Health Promotion and Maintenance | Cognitive Level: Evaluation

17. ANS: A

Jaundice is a symptom of hepatitis. D. Headaches can be associated with many disorders and are not specific to hepatitis. B. C. Diabetes and bowel obstruction are not associated with hepatitis.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation| Cognitive Level: Analysis

18. ANS: C

B. Men may be asymptomatic or may have urethritis with a yellow urethral discharge. C. Women who have gonorrhea may have either no noticeable symptoms or have a sore throat, mucopurulent cervicitis (MPC), urethritis, or abnormal menstrual symptoms such as bleeding between periods. A. Gonorrhea is treatable with antibiotics, which have side effects, but not such serious side effects that compliance is affected.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

19. ANS: D

IV drug equipment can transmit some STIs. A. B. C. Sharing a cigarette or hairbrush or coughing and sneezing can spread various infections, but not generally STIs.

PTS: 1 DIF: Moderate

KEY: Client Need: Health Promotion and Maintenance | Cognitive Level: Application

20. ANS: A

Abstinence or lifelong monogamy of both sexual partners in a relationship are the only sure prevention against STIs. D. Antibiotics treat but do not prevent STIs. B. Oral contraceptives do not prevent STIs. C. Condoms may help prevent STIs, but they are not completely effective.

PTS: 1 DIF: Moderate

KEY: Client Need: Health Promotion and Maintenance | Cognitive Level: Application

21. ANS: C

Condylomata acuminatum (genital warts) is a common sexually transmitted viral infection, and their incidence is increasing rapidly. Infection with human papillomavirus (HPV) produces the condylomata. A, B, and D do not cause warts.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

22. ANS: A

Untreated syphilis can lead to neurosyphilis and neurological changes. B, C, and D do not cause neurological changes.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

23. ANS: D

There is presently no known cure for papillomavirus infection. The warts may be treated by freezing, burning, or chemically destroying them or by manipulating the patients immune system to attack the virus. Cryotherapy (freezing) of the warts may be done by touching each wart with a cryoprobe or a liquid nitrogensoaked swab. Warts may also be burned or electro-coagulated with an electrocautery or a laser. Heat causes the proteins to coagulate, resulting in death of the wart tissue. A. B. Syphilis and Chlamydia are treated with antibiotics. C. Hepatitis B virus may be treated with immune globulins as well as supportive treatment.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityPharmacological and Parenteral Therapies | Cognitive Level: Analysis

24. ANS: B

Ophthalmia neonatorum may be prevented by use of antibiotic eye preparations, which contain silver nitrate, erythromycin, or tetracycline. D. HBV immune globulin is given to prevent HPV. C. Vitamin K prevents bleeding. A. Interferon is not used.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

25. ANS: C

Visualization of the cervix during pelvic examination shows a characteristic strawberry cervix with Trichomonas infection. A, B, and D are not associated with a strawberry cervix.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

26. ANS: B

Chlamydia is the most commonly diagnosed STI in the United States. A. C. D. Gonorrhea, Trichomoniasis, and HPV are not the most commonly diagnosed STIs in the United States.

PTS: 1 DIF: Moderate

KEY: Client Need: Health Promotion and Maintenance | Cognitive Level: Application

27. ANS: C

With pelvic inflammatory disease, findings during physical examination include adnexal tenderness upon palpation, and pain in the uterus and cervix when moved during a bimanual examination. A. B. D. Pain during a bimanual examination is not associated with syphilis, gonorrhea, or human papillomavirus infection.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Analysis

28. ANS: B

The requirements for reporting STDs may vary for different states, provinces, and countries. C. The report form has spaces for listing sexual contacts who should be notified of possible STD exposure. A. Depending on the laws of the state, province, or country, HCPs may notify identified sexual contacts or patients may do so themselves. D. Contacts may also be notified by a public health authority that they have been listed as a sexual contact by an anonymous person who has tested positive for a particular STD.

PTS: 1 DIF: Moderate

KEY: Client Need: Safe and Effective Care EnvironmentSafety and Infection Control | Cognitive Level: Analysis

MULTIPLE RESPONSE

29. ANS: A, B, E

Early signs of hepatitis are loss of appetite, rashes, malaise, muscle and joint pain, headaches, nausea, and vomiting. D. As the virus affects the liver, the urine may darken and the stool color

lightens, liver enzymes may rise, and jaundice may appear. C. Confusion is a late sign of liver disease. F. Glucose abnormalities occur with pancreatic disease.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Application

30. ANS: A, C, D, E

Several tests for syphilis exist, and a combination may be used for accurate diagnosis. Cultures may be done but are difficult to grow. Serological (blood) tests include the Venereal Disease Research Laboratory (VDRL) test, the rapid plasma reagin (RPR) test, and the automated reagin test (ART). Treponemal enzyme-linked immunosorbent assay (ELISA), fluorescent treponemal antibody absorption (FTA-ABS), and polymerase chain reaction (PCR) tests for treponemal DNA are some newer methods that reduce the risk of false results. B. F. NAT is done for Chlamydia. CD4 and CD8 are used to evaluate HIV/AIDS.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

31. ANS: B, C, E

Do not administer doxycycline during pregnancy due to bone/teeth effects. Do not take with antacids or dairy products. Avoid unnecessary exposure to sunlight. A. Administer on an empty stomach. D. Alcohol should be avoided with metronidazole.

PTS: 1 DIF: Moderate

TESTBANKSELLER.COM

KEY: Client Need: Physiological IntegrityPharmacological and Parenteral Therapies | Cognitive Level: Application

32. ANS: A, B, E

Condoms can reduce (but not eradicate) the risk of STDs. Latex condoms are less likely to break during intercourse than other types. Lubrication decreases the chances of breakage during use, but only water-soluble lubricants should be used. F. Substances such as petroleum jelly (Vaseline) may weaken the condom. C. Condoms should never be inflated to test them, because this can weaken them. D/ Condoms should never be reused and should be discarded properly after use so that others will not come in contact with the contents.

PTS: 1 DIF: Moderate

KEY: Client Need: Health Promotion and Maintenance | Cognitive Level: Application

33. ANS: D, E

Chlamydia is the most commonly diagnosed sexually transmitted infection (STI) in the United States. It can be transmitted sexually and by blood and body fluid contact. Chlamydia is a frequent cause of pelvic inflammatory infection (PID) and infertility, and it increases the risk of ectopic pregnancy. A. Women who have gonorrhea may have either no noticeable symptoms or have a sore throat. C. Newborns born to mothers who have gonorrhea can develop ophthalmia neonatorum. F. Herpes viruses have an affinity for tissues of the skin and nervous system and can lie dormant in nervous system tissues and then reactivate periodically when the body undergoes stress, fever, or immune system compromise/ B. Chancres can develop with syphilis.

PTS: 1 DIF: Moderate

KEY: Client Need: Safe and Effective Care Environment Safety and Infection Control | Cognitive Level: Application

34. ANS: A, B, E

Vulvovaginitis is an inflammation of the vulva and vagina and can be asymptomatic or involve redness, itching, burning, excoriation, pain, swelling of the vagina and labia, and discharge. C. D. Ecchymosis and dark brown vaginal bleeding are not manifestations of this disorder.

PTS: 1 DIF: Moderate

KEY: Client Need: Safe and Effective Care Environment Safety and Infection Control | Cognitive Level: Application

35. ANS: A, B

Mucopurulent cervicitis (MPC) is an inflammation of the cervix that may produce a mucopurulent yellow exudate on the cervix or may have no noticeable symptoms. MPC during pregnancy can result in conjunctivitis and pneumonia in newborn infants. C. D. E. MPC does not cause irregular heart rate, or flaccid or cyanotic extremities in the newborn.

PTS: 1 DIF: Moderate

KEY: Client Need: Safe and Effective Care Environment Safety and Infection Control | Cognitive Level: Application

36. ANS: C, D

Penicillin G is the treatment of choice for patients diagnosed with syphilis. For those who are allergic to penicillin, doxycycline and tetracycline are treatment options. A. E Gentamicin and erythromycin are not antibiotics identified to treat syphilis. B. Amoxicillin is a later generation of penicillin and should not be given.

PTS: 1 DIF: Moderate

KEY: Client Need: Physiological Integrity Pharmacological and Parenteral Therapies | Cognitive Level: Application

37. ANS: A, B, C

Trichomoniasis is a sexually transmitted infection caused by a protozoan parasite. It can be also be transmitted through nonsexual contact with infected articles because it can survive for a long time outside the body. Carriers of *Trichomonas vaginalis* can be asymptomatic for several years until changes in vaginal or urethral conditions encourage an outbreak of the infection. Symptoms include redness, swelling, itching, and burning of the genital area; pain with intercourse and voiding; and a frothy, foul-smelling discharge that can be clear, white, yellowish, or greenish. D. Trichomoniasis is not associated with open sores or wart-like growths.

Chapter 35 Nursing Assessment: Immune Function**MULTIPLE CHOICE**

1. The mother of a newborn baby is concerned that the baby will develop illnesses from being around so many people. The nurse should explain that the baby has immunity that is present at birth or:

- | | |
|----|--------------------|
| 1. | acquired immunity. |
| 2. | adaptive immunity. |
| 3. | innate immunity. |
| 4. | specific immunity. |

ANS: 3

Innate immunity or natural immunity is present at birth. It is nonspecific. Acquired immunity is immunity not present at birth and can also be adaptive or specific.

PTS: 1 DIF: Apply REF: Overview of Immunity

2. The nurse instructs a client to use good handwashing and cover her nose and mouth when sneezing. These efforts will reduce others exposure to molecules that can elicit an immune response or:

- | | |
|----|-------------|
| 1. | antigens. |
| 2. | epitopes. |
| 3. | haptens. |
| 4. | immunogens. |

ANS: 4

An immunogen is any molecule that elicits an immune response. An antigen is any molecule that can bind with a specific antibody. An antigen that does not elicit an immune response by itself is called a hapten. An epitope is the reaction portion of an antigen.

PTS: 1 DIF: Apply REF: Antigen

3.The nurse is caring for a client who is experiencing an infection. The nurse knows that the body has specific cells to entrap invading organisms. Which of the following cells is not a phagocytic cell?

- | | |
|----|-----------------|
| 1. | Dendritic cells |
| 2. | Eosinophils |
| 3. | Macrophages |
| 4. | Neutrophils |

ANS: 2

The function of the eosinophils is to release toxic granules that can kill parasites and other microorganisms. Dendritic cells, macrophages, and neutrophils all have phagocytic properties.

PTS: 1 DIF: Analyze REF: Overview of Immunity

4.The nurse, after reviewing a clients immunization history, realizes that which of the following pathogen toxoids would *not* be given to an individual to develop an immune response?

- | | |
|----|-------------------|
| 1. | Attenuated polio |
| 2. | Diphtheria toxoid |
| 3. | Snake toxin |
| 4. | Tetanus toxoid |

ANS: 3

Snake toxin works too quickly for the adaptive immune system to be effective. Horses are immunized with the toxin and produce antibodies against the venom. This venom is stored until needed. The other toxoids would be provided to an individual to develop an immune response.

PTS: 1 DIF: Analyze REF: Mechanisms of Immunization

5.Which of the following test results would *not* be associated with systemic lupus erythematosus (SLE)?

- | | |
|----|--|
| 1. | Decreased level of anti-DNA antibodies |
|----|--|

-
- | | |
|----|-------------------------------------|
| 2. | Decreased level of total complement |
|----|-------------------------------------|
-
- | | |
|----|---|
| 3. | Increased level of antinuclear antibodies |
|----|---|
-
- | | |
|----|--------------------------------------|
| 4. | Increased level of rheumatoid factor |
|----|--------------------------------------|
-

ANS: 1

Increased levels of anti-DNA antibodies are associated with SLE. Decreased levels are associated with other connective tissue disorders. SLE is associated with decreased levels of total complement, increased levels of antinuclear antibodies, and increased levels of rheumatoid factor.

PTS:1DIF:AnalyzeREF:Diagnostic Tests

6.The nurse is concerned that a client will develop an overwhelming infection because which of the following laboratory values is low?

-
- | | |
|----|------------|
| 1. | Hematocrit |
|----|------------|
-
- | | |
|----|------------|
| 2. | Hemoglobin |
|----|------------|
-
- | | |
|----|-------------|
| 3. | Eosinophils |
|----|-------------|
-
- | | |
|----|------------|
| 4. | Neutropils |
|----|------------|
-

ANS: 4

Deficiency in neutrophils or neutropenia can cause an overwhelming bacterial infection. Low levels of hemoglobin and hematocrit affect the ability to supply oxygen to the client. Eosinophils are normally found in the blood in small quantities.

PTS:1DIF:AnalyzeREF:Granulocytes

7.Which of the following interventions would be appropriate for a client recovering from a splenectomy?

-
- | | |
|----|--|
| 1. | Assist with ambulation once per shift. |
|----|--|
-
- | | |
|----|--------------------|
| 2. | Medicate for pain. |
|----|--------------------|
-

-
3. Utilize strict infection control techniques.
 4. Encourage the client to deep breathe and cough every 8 hours.
-

ANS: 3

Removal of the spleen often results in life-threatening infections known as overwhelming postsplenectomy infections. The nurse should utilize strict infection control techniques when providing care to this client. Ambulation and medicating for pain would be appropriate for any client recovering from surgery. Deep breathing and coughing should be done more frequently than every 8 hours.

PTS: 1 DIF: Apply REF: Secondary Lymphoid Tissue

8. An elderly client, diagnosed with a wound infection, is not demonstrating the expected signs of inflammation. The nurse realizes this is because the:

-
1. client is prescribed medications that block this effect.
 2. client is experiencing age-related changes in immunological function.
 3. infection is localized.
 4. client has been misdiagnosed.
-

ANS: 2

One age-related change in immunological function is suppression of phagocytic activity which will cause an absence of typical signs and symptoms of infection and inflammation. The client is not demonstrating signs of inflammation because of medications, a localized infection, or misdiagnosis.

PTS: 1 DIF: Analyze

REF: Table 41-4 Age-Related Changes in Immunological Function

9. A client tells the nurse that he is allergic to Valium because he experienced nausea, vomiting, and dizziness after ingesting. How should the nurse document this information?

-
1. Client is allergic to Valium.
 2. Client does not want to be prescribed Valium.
-

-
3. Valium has caused an allergic reaction in this client.
 4. Client experiences nausea, vomiting, and dizziness after ingesting Valium.
-

ANS: 4

Many clients will say that they have allergies to medications when they are really experiencing side effects. Nausea, vomiting, and dizziness are side effects of this medication and not an allergic response. The nurse should document the clients response to the medication and not identify these responses as an allergy.

PTS: 1 DIF: Apply REF: Allergies

10.The nurse is assessing a client for a history of cancer. To aid in this assessment, the nurse can use which of the following words as a mnemonic?

-
1. CAUTION
 2. ACTION
 3. RACE
 4. OLDCART
-

ANS: 1

The word CAUTION can be used as a mnemonic to assess a client for cancer. ACTION is not used for this assessment. RACE is often used to respond to a fire. OLDCART is often used to assess for pain.

PTS: 1 DIF: Apply REF: Cancer

11.A clients social readjustment rating scale score was 325. The nurse should interpret this result as increasing the clients risk for:

-
1. disease.
 2. sleep disturbances.
 3. developing obesity.
-

-
4. inactivity.

ANS: 1

The social readjustment rating scale was developed not only as an indicator of stress but also as an indicator of disease. A score above 300 is considered high, which should indicate to the nurse that the client is at risk for developing disease. This score is not interpreted as increasing the clients risk for sleep disturbances, developing obesity, or for inactivity.

PTS: 1 DIF: Analyze REF: Stress and Social Support

12.The nurse is completing a physical assessment with a client. Which of the following findings could be caused by impaired immune function in the client?

-
1. Jugular vein distention
-
2. Neck pain
-
3. Leg rash
-
4. Hip painTESTBANKSELLER.COM

ANS: 3

Of the assessment findings provided, leg rash could be caused by impaired immune function in the client. Jugular vein distention, neck pain, and hip pain would most likely have another cause.

PTS:1DIF:Analyze

REF: Box 41-1 Common Physical Signs Associated with Impaired Immune Function

13.The nurse is reviewing the results of a laboratory test to measure the amount of immunoglobulins in a clients blood. Which of the following should have the highest value?

-
1. IgA
-
2. IgG
-
3. IgM
-
4. IgE

ANS: 2

Immunoglobulin G is the most abundant immunoglobulin. Immunoglobulin A is the second most abundant immunoglobulin. Immunoglobulin M causes the formation of natural antibodies. Immunoglobulin E is involved in inflammation and allergic responses.

PTS: 1 DIF: Analyze REF: Table 41-5 Immunoglobulin Functions

MULTIPLE RESPONSE

1. Which of the following would the nurse identify as age-related changes in immunologic function that occur in the older adult? (Select all that apply.)

-
1. Accelerated phagocytic immune response
-
2. Altered nutrition intake
-
3. Failure of immune system to differentiate self from nonself
-
4. Increased hematuria
-
5. Increased adipose tissue
-
6. Maintenance of function of the B lymphocytes
-

ANS: 2, 3, 4

A variety of changes occur as a person begins to age. These changes make the body more susceptible to infections. The phagocytic immune response is suppressed, and the B lymphocytes are impaired. Adipose tissue and skin elasticity decrease. Nutrition intake is impaired, and frequently the older adult has inadequate protein intake. Within the urinary system, one age-related change is hematuria.

PTS:1 DIF: Apply

REF: Table 41-4 Age-Related Changes in Immunological Function

2. The nurse is using a systematic approach to assessing a client's mole. Which of the following is included in this approach? (Select all that apply.)

-
1. Asymmetry
-
2. Border
-

- | | |
|----|-------------|
| 3. | Color |
| 4. | Containment |
| 5. | Density |
| 6. | Diameter |

ANS: 1, 2, 3, 6

Moles should be screened using the ABCD approach (asymmetry, border, color, and diameter). Containment and density is not a part of this assessment.

PTS: 1 DIF: Apply REF: Integumentary System

3.The nurse is providing medication to a client in order to improve the function of the clients antibodies. Which of the following are considered antibody functions? (Select all that apply.)

- | | |
|----|----------------------------|
| 1. | Neutralization |
| 2. | Agglutination |
| 3. | Opsonization |
| 4. | Activation of inflammation |
| 5. | Phagocytosis |
| 6. | Activation of complement |

ANS: 1, 3, 4, 6

Antibodies work by four basic functions: 1) neutralization, 2) opsonization, 3) activation of inflammation, and 4) activation of complement. Agglutination occurs when an antibody binds to the same epitope on a different antigen. Phagocytosis is the removal of invading organisms by specialized cells.

PTS: 1 DIF: Analyze REF: B Lymphocytes and Antibodies

4.A baby is recovering from a thymectomy. The nurse realizes that this child is at risk for developing which of the following as an adult? (Select all that apply.)

- | | |
|----|--|
| 1. | Infections |
| 2. | Increased inflammation |
| 3. | Increase in age-related chronic diseases |
| 4. | Acute otitis media |
| 5. | Gout |
| 6. | Autoimmune responses |

ANS: 1, 2, 3, 6

Immunological aging due to thymectomy in infancy can place the individual at increased risk for infections, inflammations, age-related chronic diseases, and autoimmune responses as an adult.

PTS: 1 DIF: Apply REF: Primary Lymphoid Organs

5. A client is demonstrating signs of the inflammatory response. The nurse would assess which of the following in this client? (Select all that apply.)

- | | |
|----|------------------------|
| 1. | Increased urine output |
| 2. | Thirst |
| 3. | Edema |
| 4. | Heat |
| 5. | Erythema |
| 6. | Pain |

ANS: 3, 4, 5, 6

Inflammation is characterized by localized pain, erythema, heat, and edema. Increased urine output and thirst are not signs of the inflammatory response.

Chapter 36 Nursing Management: Patients with Immunodeficiency, HIV Infection, and AIDS**MULTIPLE CHOICE**

1. A pregnant client diagnosed with human immunodeficiency virus (HIV) is asking about her baby's risk of infection. Which of the following does put the newborn at risk?

-
- | | |
|-------|------------------|
| 1. | Bottle-feeding |
| <hr/> | |
| 2. | Changing diapers |
| <hr/> | |
| 3. | Kissing the baby |
| <hr/> | |
| 4. | Vaginal birth |

ANS: 4

Breastfeeding and vaginal birth put the newborn at risk for HIV. HIV cannot be transmitted by changing diapers (feces) or kissing the baby (saliva).

PTS:1 DIF:Apply

REF: Human Immunodeficiency Virus Infection: Etiology

2. A health care provider has accidentally been stuck with a used needle. The health care provider is going to be tested for human immunodeficiency virus (HIV). Which of the following would be the testing schedule for the health care provider?

-
- | | |
|----|--|
| 1. | Tested at 2 months, 4 months, and then at 6 months |
|----|--|

-
2. Tested immediately and then again at 2 months
 3. Tested immediately and then again at 6 months
 4. Tested in 6 months and then again in 1 year
-

ANS: 3

The health care provider should be tested immediately to show if any preexisting infection exists. Seroconversion usually occurs in 1 to 3 months but can take up to 6 months. Testing at 2 months is too late to discover a preexisting infection and can be too early to detect a new infection. Testing at 6 months or 1 year would not detect a preexisting infection.

PTS:1DIF:Apply

REF: Human Immunodeficiency Virus Infection: Etiology

3. Which of the following CD4+ count would be used to confirm the diagnosis of acquired immunodeficiency syndrome (AIDS)?

-
1. 155 cells/mcL
TESTBANKSELLER.COM
 2. 255 cells/mcL
 3. 455 cells/mcL
 4. 755 cells/mcL
-

ANS: 1

A CD4+ count of less than 200 cells/mcL is used as a criterion to establish the diagnosis of AIDS. In cell counts less than 500 to 600 cells/mcL, antiviral therapy should be initiated. Cell counts greater than 600 cells/mcL are in the normal range.

PTS:1DIF:Analyze

REF: Human Immunodeficiency Virus Infection: Pathophysiology

4. The nurse, planning care for a client diagnosed with human immunodeficiency virus, realizes that the most common infection that occurs in clients with this health problem is:

-
1. cytomegalovirus infection.

-
2. *Mycobacterium tuberculosis.*
 3. *Pneumocystis carinii* pneumonia.
 4. *Streptococcus pneumoniae.*
-

ANS: 3

As the immune system becomes overpowered, opportunistic infections can occur. The most common infection is *Pneumocystis carinii* pneumonia. The other infections can also occur, but they occur less frequently.

PTS:1DIF:Analyze

REF: Human Immunodeficiency Virus Infection: Assessment with Clinical Manifestations

5.A client diagnosed with acquired immunodeficiency syndrome (AIDS) 6 years ago has a purple lesion located on the inner thigh. This lesion is most likely to be:

-
1. AIDS-related syndrome.
 2. Burkitts lymphoma.
 3. cachexia.
 4. Kaposi sarcoma.
-

ANS: 4

Kaposi sarcoma presents as abnormal lesions that appear purple or blue-red in color. They can be found anywhere but are common on the feet, arms, thighs, perineal area, and face. Cachexia is tissue wasting. Burkitts lymphoma is characterized by enlarged lymph nodes. AIDS-related syndrome is a collection of symptoms and infections resulting from the specific damage to the immune system caused by the HIV virus.

PTS:1DIF:Analyze

REF: Human Immunodeficiency Virus Infection: Assessment with Clinical Manifestations

6.The nurse realizes that which of the following tests can be used to initially identify the presence of human immunodeficiency virus (HIV) antibodies in a client?

-
1. Enzyme-linked immunosorbent assay (ELISA)
 2. Platelet count
 3. Red blood cell count
 4. Western blot
-

ANS: 1

The ELISA test detects HIV antibodies. The Western blot is used as a confirmatory test to a positive ELISA test. Red blood cell counts and platelet counts are part of standard blood studies.

PTS:1DIF:Analyze

REF:Human Immunodeficiency Virus Infection: Diagnostic Tests

7.A client diagnosed with acquired immunodeficiency syndrome (AIDS) is sitting alone crying. Which of the following is an appropriate response for the nurse to give?

-
1. Everything will be okay.
 2. Let me call your doctor about your depression.
 3. Whats wrong now?
 4. Would you like to talk?
-

ANS: 4

Asking the client if he would like to talk allows the client an opportunity to express his feelings. The other responses give the client false reassurance or put off the client.

PTS:1DIF:Apply

REF: Human Immunodeficiency Virus Infection: Planning and Implementation

8.The nurse is instructing a client on ways to reduce the risk of exposure to the human immunodeficiency virus. Which of the following activities would present the least risk of exposure to this virus?

-
- | | |
|----|--------------------------|
| 1. | Exposure to used needles |
|----|--------------------------|
-
- | | |
|----|-----------------------|
| 2. | Multiple sex partners |
|----|-----------------------|
-
- | | |
|----|--------------------|
| 3. | Perinatal exposure |
|----|--------------------|
-
- | | |
|----|----------------|
| 4. | Teeth cleaning |
|----|----------------|
-

ANS: 4

Teeth cleaning is a procedure in a dental office that routinely sterilizes its equipment and is not considered to present an increased risk of exposure to HIV. Exposure to used needles, multiple sex partners, and perinatal exposure during pregnancy and childbirth all would increase the clients risk of exposure to the virus.

PTS:1DIF:Apply

REF:Human Immunodeficiency Virus Infection: Epidemiology

9.The nurse is teaching a small group of clients about human immunodeficiency virus (HIV) at a health clinic. Which of the following statements by a group member will need further clarification?

-
- | | |
|----|---|
| 1. | Condoms should be used during sexual contact. |
|----|---|
-
- | | |
|----|--|
| 2. | Exposure can occur to a baby during pregnancy. |
|----|--|
-
- | | |
|----|---|
| 3. | HIV-infected mothers can breastfeed their babies. |
|----|---|
-
- | | |
|----|---|
| 4. | Needles should never be reused or shared. |
|----|---|
-

ANS: 3

Exposure to HIV can occur while breastfeeding an infant. This is the statement that would necessitate further clarification. The other statements are correct.

PTS:1DIF:Analyze

REF:Human Immunodeficiency Virus Infection: Epidemiology

10.The nurse is caring for a client diagnosed with human immunodeficiency virus (HIV). Which of the following precautions is best in the care of the client?

-
1. Gloves and an N-95 mask

 2. Gown, gloves, and mask if splashing with body fluids is likely

 3. Gown, gloves, mask, and placement into a negative-pressure room

 4. Only handwashing is needed

ANS: 2

Standard precautions should be followed when handling any body fluids and blood. An N-95 mask and a negative-pressure room are not necessary. Handwashing is always recommended, but it should be accompanied by other precautions if contact with body fluids or blood is likely.

PTS:1DIF:Apply

REF: Human Immunodeficiency Virus Infection: Planning and Implementation

11.A client receiving treatment for human immunodeficiency virus infection is demonstrating signs of resistance to the medication. Which of the following does this suggest to the nurse?

-
1. The medication dosages need to be increased.

 2. The client needs to be taken off all medication.

 3. The client needs additional medication to treat side effects.

 4. The client is not adhering to the prescribed medication schedule.

ANS: 4

Resistance to medication prescribed to treat human immunodeficiency virus infection can develop if the client does not adhere to the dose schedule for each drug. Resistance to the medication does not mean the dosages need to be increased. The client should not be taken off all medication. Signs of resistance to the medication are not the same as side effects.

PTS:1DIF:Analyze

REF: Human Immunodeficiency Virus Infection: Pharmacology

12.A client diagnosed with rheumatoid arthritis receives a prescription for indomethacin. Which of the following statements by the client would indicate the need for further instruction about this medication?

-
1. I have to let my doctor know if I need to start blood pressure medications.
 2. I have to make sure I get my kidneys tested as scheduled.
 3. I need to get my eyes checked regularly.
 4. This medication shouldnt upset my stomach.
-

ANS: 4

Indomethacin can cause nausea, dyspepsia, gastrointestinal pain, diarrhea, vomiting, constipation, and flatulence. This is the statement that would indicate the need for further instruction about this medication. The client should regularly have her eyes, kidneys, and liver checked for impairment.

PTS:1DIF:Analyze

REF: Table 42-2 Examples of Drugs Used for RA Therapy

13.The nurse is providing discharge instructions to a client diagnosed with systemic lupus erythematosus (SLE). Which of the following would not be including in these instructions?

TESTBANKSELLER.COM

-
1. Activity will need to be decreased during an exacerbation.
 2. Body temperature should be monitored.
 3. Corticosteroid treatment must be slowly tapered off.
 4. Sunbathing decreases symptoms.
-

ANS: 4

Exposure to sunlight exacerbates the disease process. Body temperature should be monitored for increases and possible infection. Corticosteroid treatment cannot be abruptly stopped. Activity with rest periods should be encouraged.

PTS:1DIF:Apply

REF:Systemic Lupus Erythematosus: Planning and Implementation

14.A client is scheduled for a bone marrow transplant from cells that were donated by his identical twin. The nurse realizes that the type of transplant this client is planning would be:

- | | |
|----|-------------|
| 1. | syngeneic. |
| 2. | autologous. |
| 3. | allograft. |
| 4. | apheresis |

ANS: 1

A syngeneic transplant uses bone marrow donated by an identical twin. An autologous transplant is the removal of bone marrow cells from the individual; the cells are treated and stored and then returned after the individual receives intensive chemotherapy or radiation. Allograft refers to cells and tissue obtained from the same species who has a similar type or cell compatibility. Apheresis is a procedure used to treat autoimmune disorders.

PTS: 1 DIF: Analyze REF: Graft-versus-Host Disease

MULTIPLE RESPONSE

1.The nurse is instructing a client on the modes of transmitting the human immunodeficiency virus infection. Which of the following can transmit this infection? (Select all that apply.)

- | | |
|----|-------------|
| 1. | Blood |
| 2. | Breast milk |
| 3. | Emesis |
| 4. | Saliva |
| 5. | Semen |
| 6. | Sweat |

ANS: 1, 2, 5

HIV can be transmitted only under specific conditions that permit contact with infected body fluids. Common high-risk sources are infected blood via contaminated needlestick or sharp object, contact with infected breast milk, mucous secretions (vaginal, semen), and exposure to

blood in the laboratory. HIV is not transmitted through tears, saliva, urine, emesis, sputum, feces, or sweat.

PTS:1DIF:Apply

REF:Human Immunodeficiency Virus Infection: Epidemiology

2.A client diagnosed with human immunodeficiency virus infection is prescribed antiretroviral Group 1 medications. Which medications are included in Group 1? (Select all that apply.)

1.	Enfuvirtide (Fuzeon)
2.	Ziduvudine (AZT)
3.	Didanosine (Videx)
4.	Abacavir (Ziagen)
5.	Ritonavir (Norvir)
6.	Saquinavir (Fortovase)

ANS: 2, 3, 4

Ziduvudine (AZT), didanosine (Videx), and abacavir (Ziagen) are all Group 1 medications. Ritonavir (Norvir) and Saquinavir (Fortovase) are protease inhibitors or medications within Group 2. Enfuvirtide (Fuzeon) is a fusion inhibitor or a Group 3 medication.

PTS: 1 DIF: Analyze REF: Box 42-3 Antiretroviral Drug Classifications

3.The nurse suspects a client is experiencing rheumatoid arthritis when which of the following are assessed? (Select all that apply.)

1.	Morning stiffness lasting more than 1 hour
2.	Arthritis of three or more joint areas
3.	Arthritis of the hand joints
4.	Symmetrical arthritis

5. Nodules over bony prominences

6. Bruising

ANS: 1, 2, 3, 4, 5

Findings consistent with rheumatoid arthritis include morning stiffness lasting more than 1 hour, arthritis of three or more joint areas, arthritis of the hand joints, symmetrical arthritis, nodules over bony prominences, presence of serum rheumatoid factors, and radiographic changes. Bruising is not a finding consistent with rheumatoid arthritis.

PTS:1DIF:Analyze

REF: Box 42-4 The American College of Rheumatology Criteria for Diagnosis of RA

4.The nurse is planning care for a client diagnosed with rheumatoid arthritis. Which of the following should be included in this plan of care? (Select all that apply.)

1. Muscle strengthening exercises

2. Range-of-motion exercises

TESTBANKSELLER.COM

3. Application of heat

4. Application of cold

5. Joint massage

6. Yoga

ANS: 1, 2, 3, 4, 6

Interventions proven to help clients diagnosed with rheumatoid arthritis include muscle strengthening exercises, range-of-motion exercises, application of heat, application of cold, and yoga. Actual massage of the joints can aggravate the inflammation.

PTS:1DIF:Apply

REF: Rheumatoid Arthritis: Planning and Implementation

5.A client is diagnosed with progressive systemic sclerosis. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

- | | |
|----|---|
| 1. | Telangiectasia |
| 2. | Sclerodactyly |
| 3. | Difficulty swallowing |
| 4. | Painful cold hands and fingers |
| 5. | Small white calcium deposits under the skin |
| 6. | Hematuria |

ANS: 1, 2, 3, 4, 5

In progressive systemic sclerosis, the skin, muscle, joints, lungs, esophagus, heart, digestive system, and kidneys are often affected in the diffuse subcutaneous form, often termed as CREST. Clinical manifestations include calcinosis, or small white calcium deposits under the skin; Raynauds syndrome, or painful cold hands and fingers; alteration in esophageal movement, or difficulty swallowing; sclerodactyly of the fingers and toes; and telangiectasia or permanent dilation of the capillaries, arterioles, and venules. Hematuria is not seen in this disorder.

Chapter 37 Nursing Management: Patients with Allergic Disorders

MULTIPLE CHOICE

1. The nurse is determining if a client is experiencing an immune response. Which of the following is *not* one of the four Rs of the immune response?

- | | |
|----|-----------|
| 1. | Recognize |
| 2. | Remember |
| 3. | Remove |
| 4. | Respond |

ANS: 3

The four Rs of the immune response are recognize, respond, remember, and regulate. The regulate action is turning on for the invader or turning off when the invader is destroyed. It is not about removing.

PTS:1DIF:AnalyzeREF:Allergic Dysfunction

2.A client is experiencing a transfusion reaction. The nurse realizes that a transfusion reaction is which type of hypersensitive reaction?

1.	Type 1
2.	Type 2
3.	Type 3
4.	Type 4

ANS: 2

Type 1 reactions are anaphylactic reactions. Type 2 is cytotoxic (e.g., transfusion reaction). Type 3 is immune complex. Type 4 is delayed hypersensitive.

TESTBANKSELLER.COM

PTS:1DIF:Analyze

REF:Table 43-2 Hypersensitive Reactions: Gell-Coombs Classification

3.During the health history, the clients says, I have many allergies. Of the following medications, which one would be for the treatment of allergies?

1.	Acetaminophen
2.	Docusate
3.	Diphenhydramine hydrochloride
4.	Guaifenesin

ANS: 3

Diphenhydramine hydrochloride (Benadryl) is an antihistamine that is used for allergic reactions. Guaifenesin (Robitussin) is an expectorant. Acetaminophen (Tylenol) is an antipyretic and analgesic. Docusate (Colace) is a stool softener.

PTS:1DIF:Analyze

REF: Table 43-6 Pharmacology Facts: Pharmaceutical Therapy for Symptom Relief of Chronic Allergies

4. After instructing a client about food allergies, the nurse would determine that additional instruction is needed when the client states that which of the following foods should be avoided?

1.	Carrots
2.	Peanuts
3.	Shellfish
4.	Strawberries

ANS: 1

Foods such as eggs, milk, nuts, shellfish, and soy wheat are common food allergens; carrots are not.

PTS:1DIF:Analyze

TESTBANKSELLER.COM

REF: Table 43-3 Categories of Sensitizing Agents (Allergens)

5. A client recovering from a kidney transplant needs to be assessed for a hypersensitivity reaction. Which type of hypersensitive reaction is associated with transplants?

1.	Type 1
2.	Type 2
3.	Type 3
4.	Type 4

ANS: 4

Type 1 reactions are anaphylactic reactions. Type 2 is cytotoxic. Type 3 is immune complex. Type 4 is delayed hypersensitive reaction evidenced by transplant rejection.

PTS:1DIF:Apply

REF:Table 43-2 Hypersensitive Reactions: Gell-Coombs Classification

6.An individual is petting a cat. Early response to an antigen would include all of the following EXCEPT:

- | | |
|----|---------------------|
| 1. | airway obstruction. |
| 2. | itchy eyes. |
| 3. | nasal secretions. |
| 4. | sneezing. |

ANS: 1

Early response to an antigen includes sneezing, an increase in nasal secretions, and itchy eyes. The late response can begin at the same time or be delayed hours. The late response does not occur in everyone and is characterized by more severe symptoms such as airway obstruction.

PTS:1DIF:Analyze

REF: Allergic Dysfunction: Primary and Secondary Response

7.The nurse is teaching a client how to avoid allergic reactions. Which of the following is *not* an appropriate recommendation?

- | | |
|----|---|
| 1. | Avoid the allergen. |
| 2. | Carry your prescribed medications. |
| 3. | Use corticosteroids daily to prevent reactions. |
| 4. | Wear a medical alert bracelet. |

ANS: 3

Daily corticosteroid therapy is not recommended to prevent reactions. The client should eliminate exposure to the allergen and be prepared for a reaction if one occurs.

PTS: 1 DIF: Apply REF: Anaphylaxis: Planning and Implementation

8.A client begins to sneeze and have an increase in nasal secretions. The nurse realizes that the antibody that attaches to mast cells and plays a critical role in the allergic process is:

- | | |
|----|------|
| 1. | IgA. |
| 2. | IgD. |
| 3. | IgE. |
| 4. | IgG. |

ANS: 3

IgE antibody attaches to a mast cell that subsequently releases histamine. This reaction produces allergic symptoms exhibited by clients. IgA protects against infections of the mucous membranes. IgD is found on the surface of B cells. IgG is the main immunoglobulin that is produced in response to an infection.

PTS: 1 DIF: Analyze REF: Allergic Dysfunction: Early Response

9.A client, receiving a dose of penicillin for an infection, begins to complain of difficulty breathing and has a respiratory rate of 38. This client is experiencing what type of reaction?

- | | |
|----|--------|
| 1. | Type 1 |
| 2. | Type 2 |
| 3. | Type 3 |
| 4. | Type 4 |

ANS: 1

An anaphylactic reaction is a type 1 reaction. Type 2 reactions are cytotoxic and are seen with blood transfusions. Type 3 reactions are immune related and are associated with rheumatoid arthritis or systemic lupus erythematosus. Type 4 reactions are seen with transplant rejections.

PTS:1DIF:Analyze

REF:Table 43-2 Hypersensitive Reactions: Gell-Coombs Classification

10.A client is prescribed a Leukotriene modifier as part of asthma treatment. The nurse should instruct the client that this medication:

-
1. relaxes bronchospasm.

 2. increases mucociliary clearance.

 3. reduces inflammation.

 4. inhibits the allergic process.

ANS: 4

Leukotriene modifiers inhibit the allergic process. Beta-adrenergic agonists relax bronchospasm. System bronchodilators increase mucociliary clearance. Corticosteroids reduce inflammation.

PTS:1DIF:Apply

REF: Table 43-5 Pharmacology Facts: Pharmaceutical Therapy for Asthma

11.A client is experiencing intense pruritis from contact dermatitis. Which of the following would be helpful for this client?

-
1. Administer hydroxyzine HCL as prescribed.

 2. Cover the area with a warm compress.

 3. Instruct to scratch the skin with finger pads.

 4. Increase ambulation when itching occurs.

ANS: 1

Hydroxyzine HCL is an antihistamine used to relieve pruritis associated with contact dermatitis. Covering the area with a warm compress could cause the area to itch more. The client should be instructed to avoid scratching. Ambulation will not help reduce the itchiness.

PTS:1DIF:Apply

REF: Table 43-10 Pharmacology Facts: Pharmaceutical Therapy for Urticaria

12.A client calls for the nurse and says that her mouth has been itchy since she ate lunch. Which of the following should the nurse do to assist the client?

-
1. Ask the client if she has ever been diagnosed with a food allergy.

 2. Ask the physician to prescribe an oral steroid.

 3. Assess the clients vital signs.

 4. Assess the clients tongue and throat

ANS: 1

When a person is allergic to a particular food, itching of the mouth may occur as they eat the food. The nurse should ask the client if she has ever been diagnosed with a food allergy. The client may or may not need an oral steroid. The clients vital signs will most likely not be affected so soon. No tongue or throat changes occur with a food allergy.

PTS:1DIF:ApplyREF:Food Allergy

13.The nurse begins to experience red, itchy hands after using gloves for client care. Which of the following should the nurse consider as causing these symptoms?

-
1. Seasonal allergies

 2. Influenza

 3. Latex allergy

 4. Serum sickness

ANS: 3

Latex allergy is a reaction to certain proteins in latex rubber. Increasing the exposure to latex proteins increases the risk of developing allergic symptoms. Mild reactions include skin redness and itching. Red, itchy hands are not associated with seasonal allergies, influenza, or serum sickness.

PTS:1DIF:AnalyzeREF:Latex Allergy

MULTIPLE RESPONSE

1.A client is being instructed to avoid airborne allergens. Which of the following allergens would be considered airborne? (Select all that apply.)

1.	Animal dander
2.	Eggs
3.	Fungal spores
4.	Grass pollen
5.	Insect stings
6.	Milk

ANS: 1, 3, 4

Airborne allergens include pollens (e.g., grass, plants, and trees), mold (i.e., fungal spores), dust mites, animal dander, and house dust. Eggs and milk are ingested allergens. Insect stings are injected allergens.

PTS:1DIF:Apply

TESTBANKSELLER.COM

REF: Table 43-3 Categories of Sensitizing Agents (Allergens)

2.The nurse suspects a client is experiencing atopic asthma when which of the following are assessed? (Select all that apply.)

1.	Shortness of breath with exertion
2.	Nighttime coughing
3.	Chest tightness
4.	Wheezing
5.	Rash
6.	Ear pain

ANS: 1, 2, 3, 4, 5

Clinical manifestations of atopic asthma include shortness of breath with exertion, nighttime coughing, chest tightness, wheezing, and a rash. Ear pain is not associated with this disorder.

PTS:1DIF:Analyze

REF:Atopic Asthma: Assessment with Clinical Manifestations

3.Which of the following should the nurse instruct a client diagnosed with asthma on ways to reduce asthma triggers? (Select all that apply.)

-
- | | |
|----|-----------------------------|
| 1. | Remove carpets in bedrooms. |
|----|-----------------------------|
-
- | | |
|----|------------------------------|
| 2. | Use stuffed animals as toys. |
|----|------------------------------|
-
- | | |
|----|-----------------------------|
| 3. | Keep pets off of furniture. |
|----|-----------------------------|
-
- | | |
|----|--------------------------|
| 4. | Avoid freshly cut grass. |
|----|--------------------------|
-
- | | |
|----|------------------------|
| 5. | Avoid cigarette smoke. |
|----|------------------------|
-
- | | |
|----|---|
| 6. | Use asthma medication approximately 10 minutes before exercise. |
|----|---|

ANS: 1, 3, 4, 5, 6

Interventions to reduce triggers that exacerbate asthma include removing carpets in the bedroom, keeping pets off furniture, avoiding freshly cut grass, avoiding cigarette smoke, and using asthma medication approximately 10 minutes before exercise. The client should be instructed to not use stuffed animals as toys.

PTS: 1 DIF: Apply REF: Patient Playbook: Triggers for Asthma

4.A client is scheduled for a skin-prick test to diagnose the cause for his allergic reactions. Prior to this test, which of the following medications should the client be instructed to avoid? (Select all that apply.)

-
- | | |
|----|-----------------|
| 1. | Corticosteroids |
|----|-----------------|
-
- | | |
|----|----------------|
| 2. | Antihistamines |
|----|----------------|
-
- | | |
|----|-----------------|
| 3. | Benzodiazepines |
|----|-----------------|

- | | |
|----|-----------------|
| 4. | Theophylline |
| 5. | Antidepressants |
| 6. | Aspirin |

ANS: 2, 3, 4, 5

Use of certain medications can interfere with the validity of a skin-prick response. These medications include antihistamines, benzodiazepines, theophylline, and antidepressants. Corticosteroids and aspirin have no effect on the skin-prick test.

PTS: 1 DIF: Apply REF: Atopic Asthma: Diagnostic Tests

5.A client is demonstrating signs of an anaphylactic reaction. Which of the following should the nurse do? (Select all that apply.)

- | | |
|----|--|
| 1. | Place the client in the supine position. |
| 2. | Assist the client to ambulate. |
| 3. | Maintain the airway. |
| 4. | Provide oral fluids. |
| 5. | Begin an intravenous access line. |
| 6. | Place the client in the prone position. |

ANS: 1, 3, 5

Initial medical management of a client experiencing an anaphylactic reaction includes placing the client in the supine position, maintain the airway, and begin an intravenous access line. The client should not ambulate. The client should not ingest oral fluids. The client should be supine and not prone.

Chapter 38 Nursing Management: Patients with Rheumatic Disorders

MULTIPLE CHOICE

1.The nurse, assessing a clients leukocyte level, determines the amount to be within normal limits. Which of the following would indicate a normal level of leukocytes in the clients blood?

- | | |
|----|------------------------------------|
| 1. | 14 to 18 g/dL |
| 2. | 4.6 to 6.2 million/mm ³ |
| 3. | 4500 to 11,000 mm ³ |
| 4. | 50 to 60 percent |

ANS: 3

The normal amount of leukocytes or white blood cells in the blood is 4500 to 11,000 mm³. The value of 14 to 18 g/dL is the normal hemoglobin level. The value of 4.6 to 6.2 million/mm³ represents the normal amount of red blood cells. The value of 50 to 60 percent represents a normal neutrophil level.

PTS: 1 DIF: Analyze REF: Leukocytes

2.A clients complete blood count reveals a large amount of phagocytic cells present. The nurse realizes that this type of cell is most likely:

- | | |
|----|--------------|
| 1. | basophils. |
| 2. | eosinophils. |
| 3. | monocytes. |
| 4. | neutrophils. |

ANS: 4

Monocytes are phagocytic but in a smaller amount than neutrophils. Basophils are stimulated by allergens and eosinophils by parasites. Neutrophils are the chief phagocytic cells and are present in larger numbers as a response to early inflammation.

PTS: 1 DIF: Analyze REF: Leukocytes

3.According to assessment findings, the nurse determines that a client is experiencing an inflammatory process. Which of the following did the nurse assess in this client?

-
- | | |
|----|-----------------------------------|
| 1. | Redness, swelling, heat, and pain |
|----|-----------------------------------|
-
- | | |
|----|----------------------|
| 2. | Reduced urine output |
|----|----------------------|
-
- | | |
|----|--------|
| 3. | Thirst |
|----|--------|
-
- | | |
|----|---|
| 4. | Elevated blood pressure and slow heart rate |
|----|---|
-

ANS: 1

The symptoms of the inflammatory process are redness, swelling, heat, and pain. Reduced urine output, thirst, elevated blood pressure, and slow heart rate are not symptoms of the inflammatory process.

PTS: 1 DIF: Analyze REF: Signs of Inflammation

4.A client is diagnosed with a bacterial infection. Which of the following is an example of this type of infection?

-
- | | |
|----|---------|
| 1. | Malaria |
|----|---------|
-
- | | |
|----|-----------------|
| 2. | Gastroenteritis |
|----|-----------------|
-
- | | |
|----|-------------------------|
| 3. | Urinary tract infection |
|----|-------------------------|
-
- | | |
|----|--------|
| 4. | Typhus |
|----|--------|
-

ANS: 3

Urinary tract infections are caused by bacteria. Malaria and gastroenteritis are caused by protozoa. Typhus is caused by rickettsia.

PTS: 1 DIF: Understand REF: Table 11-3 Types of Agents Causing Disease

5.A client is diagnosed with gastroenteritis. The nurse realizes that this illness occurs from which type of disease-causing organism?

-
- | | |
|----|----------|
| 1. | Bacteria |
|----|----------|
-
- | | |
|----|-------|
| 2. | Fungi |
|----|-------|
-

3.	Protozoa
----	----------

4.	Viruses
----	---------

ANS: 3

Protozoa are single-cell parasitic organisms that form cysts or spores. Diseases caused by protozoa include malaria and gastroenteritis. Hepatitis A, B, and C are examples of a disease caused by a virus. Pneumonia and urinary tract infections are examples of diseases caused by bacteria. Ringworm is an example of a disease caused by fungi.

PTS: 1 DIF: Analyze REF: Table 11-3 Types of Agents Causing Disease

6.A client has been diagnosed with Rocky Mountain spotted fever. The causative organism for this disease process is:

1.	bacteria.
----	-----------

2.	helminth.
----	-----------

3.	mycoplasma.
----	-------------

4.	rickettsia.
----	-------------

ANS: 4

Rocky Mountain spotted fever is caused by the infectious organism rickettsia. Disease processes from bacteria, helminths, and mycoplasma include urinary tract infections, tapeworm infection, and pneumonia, respectively.

PTS: 1 DIF: Understand REF: Table 11-3 Types of Agents Causing Disease

7.Which of the following will the nurse most likely assess in a client diagnosed with asthma?

1.	Wheezing and anxiety
----	----------------------

2.	Barking cough and increased blood pressure
----	--

3.	Bradycardia and restlessness
----	------------------------------

-
4. Anemia and hypoxia

ANS: 1

Common symptoms in asthma include wheezing, anxiety, cough, shortness of breath, tachycardia, restlessness, increased blood pressure, and hypoxia. Barking cough, bradycardia, and anemia are not common symptoms of asthma.

PTS: 1 DIF: Analyze REF: Asthma: An Allergic Disease

8.The nurse would expect that a client diagnosed with arthritis will be prescribed which of the following medications?

-
1. Albuterol

-
2. Furosemide

-
3. Ibuprofen

-
4. Nortriptyline
TESTBANKSELLER.COM

ANS: 3

Nonsteroidal anti-inflammatory drugs (NSAIDs) and cortisol drugs are common treatments for arthritis. Albuterol relaxes bronchial smooth muscle. Furosemide is a loop diuretic, and nortriptyline is an antidepressant.

PTS: 1 DIF: Analyze REF: Arthritis

9.A client is being admitted to a health care facility. Which type of precautions will the nurse implement at this time?

-
1. Airborne

-
2. Contact

-
3. Droplet

-
4. Standard

ANS: 4

Standard precautions are actions used with all clients. Transmission-based precautions such as airborne, contact, and droplet are used when a client is known or suspected of having a communicable disease.

PTS:1DIF:ApplyREF:Standard Precautions

10. A client diagnosed with tuberculosis is scheduled for a chest x-ray to be completed in the radiology department. Which of the following devices should be utilized when transporting this client?

-
- | | |
|----|--------------------------------|
| 1. | Face shield with mask and gown |
|----|--------------------------------|
-
- | | |
|----|-----------|
| 2. | N-95 mask |
|----|-----------|
-
- | | |
|----|---------------|
| 3. | Surgical mask |
|----|---------------|
-
- | | |
|----|--|
| 4. | Patient does not need to wear a device |
|----|--|

ANS: 3

For a client diagnosed with tuberculosis, transport out of the room should only be done when absolutely necessary and the client should wear a surgical mask during transport. A face shield, gown, or N-95 mask are not needed to transport this client.

PTS:1DIF:ApplyREF:Airborne Precautions

11. The nurse is preparing to administer medications to a client diagnosed with varicella. Which of the following personal protective equipment should the nurse use when entering the client's room?

-
- | | |
|----|--------------------------------|
| 1. | Face shield with mask and gown |
|----|--------------------------------|
-
- | | |
|----|-----------------|
| 2. | Gloves and gown |
|----|-----------------|
-
- | | |
|----|---|
| 3. | A high-efficiency particulate air filter mask |
|----|---|
-
- | | |
|----|---------------|
| 4. | Surgical mask |
|----|---------------|

ANS: 3

A high-efficiency particulate air filter mask is required personal protective equipment for the care of a client with varicella. A mask may be worn for clients on droplet precautions, and the gown and gloves are for a client on contact precautions.

PTS:1DIF:ApplyREF:Airborne Precautions

12.A client is diagnosed with venous leg ulcers. The nurse would expect that these wounds will heal by which of the following types of intention?

1.	Primary
2.	Secondary
3.	Tertiary
4.	Quaternary

ANS: 2

Primary intention type of healing occurs in wounds that are clean, and have little loss of tissue. Secondary intention occurs when a wound heals by spread of granulation tissue from the base of a wound. Venous leg ulcers heal by secondary intention. In tertiary intention, the wound must be sutured through several layers of granulation tissue in order to bring closure. Quaternary is not a type of wound healing.

PTS: 1 DIF: Analyze REF: Types of Wound Healing

13.The nurse is using the Braden Scale to determine a clients risk for developing a pressure ulcer. Which of the following areas are assessed with this scale?

1.	Home environment
2.	Finances
3.	Medications
4.	Friction and shear

ANS: 4

The Braden Scale is used to assess a client's risk for developing a pressure ulcer. This scale assesses the areas of sensory perception, moisture, activity, mobility, nutrition, and friction and shear. Home environment, finances, and medications are not assessed with the use of this scale.

PTS: 1 DIF: Apply REF: Table 11-7 Elements in Braden Pressure Scale

MULTIPLE RESPONSE

1. The nurse is identifying nursing diagnoses for a client experiencing inflammation. Which of the following diagnoses would be appropriate for this client? (Select all that apply.)

- | | |
|----|--|
| 1. | Risk for infection |
| 2. | Thermoregulation: Ineffective |
| 3. | Ineffective coping |
| 4. | Pain: Acute |
| 5. | Nutrition: Imbalanced, less than body requirements |
| 6. | Anxiety |

ANS: 1, 2, 4, 5

Nursing diagnoses appropriate for a client experiencing inflammation include risk for infection; thermoregulation: ineffective; pain: acute; and nutrition: imbalanced, less than body requirements. Ineffective coping and anxiety are not diagnoses appropriate for a client with an inflammation.

PTS: 1 DIF: Apply REF: Nursing Response: Inflammation

2. The nurse is determining the route of transmission for an infectious organism. Which of the following are types of transmission routes? (Select all that apply.)

- | | |
|----|----------------|
| 1. | Ingestion |
| 2. | Vector-borne |
| 3. | Common vehicle |

4. Airborne

5. Droplet

6. Contact

ANS: 2, 3, 4, 5, 6

There are five types of transmission routes: 1) contact, 2) droplet, 3) airborne, 4) common vehicle, and 5) vector-borne. Ingestion is not a type of transmission route.

PTS: 1 DIF: Analyze REF: Infectious Disease Control

3. The nurse is determining when gloves should be worn when providing client care. Which of the following situations would necessitate the wearing of gloves? (Select all that apply.)

1. In contact with blood

2. In contact with mucous membranes

3. Delivering a meal tray

4. Providing medications

5. Measuring urine output

6. Suctioning oral secretions

ANS: 1, 2, 5, 6

Gloves should be worn when in contact with blood, body fluids, secretions, excretions, contaminated items, mucous membranes, and nonintact skin. Gloves are not needed when delivering a meal tray or providing medications.

PTS: 1 DIF: Analyze

REF: Table 11-4 Summary of the Updated Centers for Disease Control and Prevention Isolation Guidelines

4. The nurse is concerned that a client will experience delayed wound healing when which of the following is assessed? (Select all that apply.)

- | | |
|----|--|
| 1. | Prescribed a beta-blocker medication |
| 2. | Poor appetite |
| 3. | Ambulating in the room several times a day |
| 4. | Age 85 |
| 5. | Prescribed steroids |
| 6. | Skin warm and dry |

ANS: 1, 2, 4, 5

Risk factors for delayed wound healing include ischemia, medications such as beta-blockers, smoking, exposure to cold, repetitive injury, altered nutrition infection, anti-inflammatory steroids, and older age. Ambulating in the room several times a day may encourage wound healing. Skin warm and dry will not delay wound healing.

PTS:1DIF:Analyze

TESTBANKSELLER.COM

REF: Table 11-6 Risk Factors for Delayed Wound Healing

5.The nurse is planning care for a client with a chronic wound. Which of the following principles should be reflected in this clients care?

- | | |
|----|---------------------------|
| 1. | Debridement |
| 2. | Restrict fluids |
| 3. | Provide moist environment |
| 4. | Prevent further injury |
| 5. | Maintain on bed rest |
| 6. | Nutrition |

ANS: 1, 3, 4, 6

The four principles of chronic wound management include debridement, provide moist environment, prevent further injury, and nutrition. Restricting fluids and maintaining on bed rest are not principles of chronic wound management.

Chapter 39 Nursing Assessment: Musculoskeletal Function

MULTIPLE CHOICE

1.A client is being treated for a fractured scapula. The nurse realizes that the scapula is what kind of bone type?

1.	Flat
2.	Irregular
3.	Long
4.	Short

ANS: 1

The scapula, iliac crest, and sternum are flat bone types. Metacarpals are an example of the short type. The femur is a long type bone, and the vertebra is an irregular type.

PTS: 1 DIF: Analyze REF: Table 58-1 Types of Bone

2.A school age child has sustained a fractured femur. When assessing the location of the break, the area that could cause the most concern would be the:

1.	diaphysis.
2.	epiphyseal plate.
3.	medullary cavity.
4.	metaphysis.

ANS: 2

The epiphyseal plate, or growth plate, is where active longitudinal growth occurs. If a fracture occurs in or through the epiphyseal plate, growth in that extremity may be delayed or stopped. The diaphysis is the shaft of the long bone. The medullary cavity is the bone marrow. The metaphysis is the area of transition between the diaphysis and the epiphysis.

PTS: 1 DIF: Analyze REF: Macroscopic Structure of Bone

3.A client is experiencing an increase in resorption of bone. The nurse realizes that which of the following bone cells is responsible for this function?

1.	Osteoblasts
2.	Osteoclasts
3.	Osteocytes
4.	Osteomasts

ANS: 2

Osteoclasts are responsible for resorption in the bone and work chemically through enzymatic and phagocytic action. Osteoblasts build bone. Osteocytes are mature bone cells, and osteomasts demineralize bone.

PTS: 1 DIF: Analyze REF: Microscopic Structure of Bone: Cell Types

4.A client, recovering from an extended illness, is having difficulty coordinating his movements to walk. The nurse should remind the client that skeletal muscle has which of the following normal properties?

1.	It is automatic.
2.	It is convulsive.
3.	It is involuntary.
4.	It is voluntary.

ANS: 4

Skeletal muscle is voluntary. Cardiac and smooth muscles are involuntary. Skeletal muscle is not normally convulsive in nature.

PTS: 1 DIF: Apply REF: Anatomy and Physiology of Skeletal Muscles

5.A client is experiencing unstable hip joints with ambulation. The nurse suspects that a disorder is occurring within the strong bands of connective tissue that attach bone to bone or the:

- | | |
|----|------------|
| 1. | bursae. |
| 2. | cartilage. |
| 3. | ligaments. |
| 4. | tendons. |

ANS: 3

Ligaments are strong bands of connective tissue that attach bone to bone or bone to cartilage. Ligaments help to give joints stability, guide the joint movement, and prevent excess motion within the joint. Tendons connect muscle to bone. Bursae are synovial fluid-filled sacs near joints. Cartilage covers the ends of bone to reduce friction.

PTS: 1 DIF: Analyze REF: Tendons, Ligaments, and Bursae

TESTBANKSELLER.COM

6.The nurse is moving a clients leg toward the body during range-of-motion exercises. The nurse is doing which of the following with the clients extremity?

- | | |
|----|-----------|
| 1. | Abduction |
| 2. | Adduction |
| 3. | Extension |
| 4. | Flexion |

ANS: 2

Abduction is movement away from the body, and adduction is movement toward the body. Extension is a movement that increases the angle between two joints, and flexion is a movement that decreases the angle between two joints.

PTS:1DIF:ApplyREF:Range of Motion

7.A client has an elevated C-reactive protein level. Which of the following should the nurse assess in this client?

- | | |
|----|--------------------|
| 1. | Alcohol intake |
| 2. | Cigarette use |
| 3. | Recent weight loss |
| 4. | Pregnancy status |

ANS: 2

Cigarettes can elevate levels of C-reactive protein. The nurse should assess the client for smoking. Alcohol and weight loss can falsely lower C-reactive protein levels. Pregnancy status does not affect the C-reactive protein level.

PTS:1DIF:Apply

REF: Table 58-5 Laboratory Tests (Serum) Related to Musculoskeletal Disorders

8. During the assessment of a client's muscle status, the nurse notes the client is able to complete range of motion against gravity with some resistance. The nurse would document this assessment finding as being:

TESTBANKSELLER.COM

- | | |
|----|----|
| 1. | 2. |
| 2. | 3. |
| 3. | 4. |
| 4. | 5. |

ANS: 3

Complete range of motion against gravity with some resistance is graded as being 4 or good. Complete range of motion without gravity is graded as being 2 or poor. Complete range of motion against gravity without added resistance is graded as being 3 or fair. Complete range of motion against gravity with full resistance is graded as being 5 or normal.

PTS: 1 DIF: Apply REF: Table 58-3 Graded Muscle Strength

9. A client is seeking medical attention for pain in the knee that occurred during a recent sports activity. The nurse realizes that the client will most likely have which of the following diagnostic tests?

-
- | | |
|----|--------------|
| 1. | Arthrography |
|----|--------------|
-
- | | |
|----|-----|
| 2. | MRI |
|----|-----|
-
- | | |
|----|-----------|
| 3. | Angiogram |
|----|-----------|
-
- | | |
|----|-----------|
| 4. | Bone scan |
|----|-----------|
-

ANS: 2

Magnetic resonance imaging (MRI) is now the diagnostic intervention of choice due to its noninvasive nature. Arthrography is seldom used today. Angiogram is used to confirm a diagnosis of deep vein thrombosis or pulmonary embolism. Bone scan is used to detect early bone disease.

PTS:1DIF:AnalyzeREF:Radiographic Studies

10.A middle-aged female client tells the nurse that her mother and aunts all have been diagnosed with osteoporosis. The nurse realizes that this client might benefit from having which of the following diagnostic tests?

-
- | | |
|----|--|
| 1. | Dual energy s-ray absorptiometry (DEXA) scan |
|----|--|
-
- | | |
|----|-----|
| 2. | MRI |
|----|-----|
-
- | | |
|----|---------|
| 3. | CT scan |
|----|---------|
-
- | | |
|----|-----------|
| 4. | Myelogram |
|----|-----------|
-

ANS: 1

DEXA scans measure bone density and are used in the early diagnosis of osteoporosis. MRIs, CT scans, and myelograms are not used to help diagnose osteoporosis.

PTS:1DIF:AnalyzeREF:Radiographic Studies

11.A client is recovering from surgery to repair a torn anterior cruciate ligament of the knee. The nurse should prepare the client to have which of the following diagnostic tests after the procedure?

-
- | | |
|----|-----------|
| 1. | Bone scan |
|----|-----------|
-

-
- | | |
|----|-------------|
| 2. | Arthrometry |
|----|-------------|
-
- | | |
|----|-------------|
| 3. | Arthroscopy |
|----|-------------|
-
- | | |
|----|--------------------|
| 4. | Bone marrow biopsy |
|----|--------------------|
-

ANS: 2

Arthrometry is used after surgery to repair a torn anterior cruciate ligament injury to confirm stability. Bone scans are used to identify bone disease. Arthroscopy is used to diagnose and repair specific disorders of the knee. A bone marrow biopsy is used to examine the bone marrow for abnormal tissue growth.

PTS: 1 DIF: Apply REF: Arthrometry

12. Which of the following should the nurse instruct a client who is scheduled for electromyography studies?

-
- | | |
|----|-----------------------------|
| 1. | Eat nothing after midnight. |
|----|-----------------------------|
-
- | | |
|----|--|
| 2. | Avoid taking prescribed pain medication prior to the test. |
|----|--|
-
- | | |
|----|--|
| 3. | Have an alternative form of transportation to get home after the test. |
|----|--|
-
- | | |
|----|--|
| 4. | Refrain from nicotine and caffeine for 2 to 3 hours before the test. |
|----|--|
-

ANS: 4

Clients are required to refrain from nicotine and caffeine 2 to 3 hours before an electromyography. The client does not need to eat nothing after midnight. The client can take prescribed pain medication prior to the test. The client will have no restrictions after the test and will be able to drive.

PTS: 1 DIF: Apply REF: Nerve Conduction Studies

13. A client, recovering from a fractured arm, is scheduled for an x-ray 3 weeks after the injury. The nurse explains to the client that the purpose of this x-ray is to:

-
- | | |
|----|--|
| 1. | evaluate the ossification stage of bone healing. |
|----|--|
-
- | | |
|----|-------------------------------------|
| 2. | determine the presence of a callus. |
|----|-------------------------------------|
-

-
3. assess if fibroblasts have invaded the fracture site.
 4. determine if a hematoma has formed at the fracture site.
-

ANS: 2

A hematoma will form at the fracture site within 24 hours after the injury. Fibroblasts invade the fracture site within 48 hours after the injury. The callus will reach maximum size in 2 to 3 weeks after the injury. This is the purpose of the clients x-ray 3 weeks after the injury. Ossification of bone healing will not occur until 3 to 4 months after the injury.

PTS: 1 DIF: Apply REF: Bone Repair/Fracture Healing

14. An elderly client tells the nurse that he has been having increasing difficulty walking and he has reduced range of motion in both hips. The nurse suspects that which of the following is occurring with this client?

-
1. Loss of elasticity of the ligaments and tendons
 2. Reduction in blood supply to the hips
 3. Interruption in nerve supply to the hips
 4. Hairline fractures of both hips
-

ANS: 1

Age-related changes to the muscular system include the loss of elasticity of the ligaments and tendons. The ligaments and tendons shorten, and the client experiences stiffness, loss of flexibility, and loss of range of motion. The client most likely does not have a reduction in blood and nerve supply to the hips. The client would not be able to bear weight on the legs if hairline fractures were present in the hips.

PTS: 1 DIF: Analyze REF: Aging and the Muscular System

MULTIPLE RESPONSE

1. The nurse is assessing a client's gait. Which of the following is a part of this assessment? (Select all that apply.)

-
1. The foot location during midstance

-
2. The way the client pushes off from the ball of the foot

 3. The rate and rhythm of acceleration

 4. Alignment of the head

 5. Gap between the legs

 6. Breathing pattern

ANS: 1, 2, 3, 4, 5

The gait has two phases: stance and swing. The nurse should assess the clients foot location during midstance, the way the client pushes off from the ball of the foot, and the rate and rhythm of acceleration. The nurse should also assess the clients head alignment during walking and the gap between the legs. The clients breathing pattern is not assessed during the assessment of gait.

PTS:1DIF:ApplyREF:Analysis of Gait

2.A client is scheduled for a myelogram. Which of the following should the nurse instruct the client regarding pre-procedure activities? (Select all that apply.)

-
1. Drink extra fluids the evening before the procedure.

 2. Take nothing by mouth 4 to 8 hours before the procedure.

 3. Empty the bladder before the procedure.

 4. Sleep with head elevated at a 30 degree angle.

 5. Stay in bed for 16 hours.

 6. Take pain medication as prescribed .

ANS: 1, 2, 3

Pre-procedure activities for a myelogram include drinking extra fluids the evening before the procedure; taking nothing by mouth for 4 to 8 hours before the procedure; and empty the bladder

before the procedure. Sleeping with the head of the bed elevated, staying in bed for 16 hours, and taking pain medication are all post-procedure activities.

PTS: 1 DIF: Apply REF: Myelogram

3.A client, scheduled to have joint fluid aspirated from the knee, asks the nurse why the procedure is being done. Which of the following should the nurse explain to this client? (Select all that apply.)

1. It is done to examine the synovial fluid in the joint cavity.
2. It is done to realign the knee joint.
3. It is done to relieve pain.
4. It is done to visualize the bone structure.
5. It is done to relieve edema and effusion.
6. It is done to evaluate the status of ligaments.

ANS: 1, 3, 5

Joint aspiration is performed to examine the synovial fluid in the joint cavity. It is also used to relieve pain in the joint resulting from edema and effusion. Joint aspiration is not done to realign the joint, visualize the bone structure, or evaluate the status of ligaments.

PTS:1DIF:ApplyREF:Joint Aspiration

4.A client is scheduled for somatosensory evoked potentials. The nurse realizes this test is used to help diagnose which of the following disorders? (Select all that apply.)

1. Multiple sclerosis
2. Radiculopathies
3. Myasthenia gravis
4. Peripheral nerve function

5. Charcot-Marie-Tooth disease

6. Huntingtons chorea

ANS: 2, 4, 5

Somatosensory evoked potentials are used to measure time in meters per second from the stimulation of a peripheral nerve through the response. It is useful in the evaluation of radiculopathies and peripheral nerve function and the diagnosis of Charcot-Marie-Tooth disease. This diagnostic test is not used for multiple sclerosis, myasthenia gravis, or Huntingtons chorea.

Chapter 40 Nursing Management: Patients with Musculoskeletal Disorders

MULTIPLE CHOICE TESTBANKSELLER.COM

1. A client is diagnosed with osteoarthritis. The nurse would *not* expect to find which of the following during assessment?

1. Bouchards nodes

2. Crepitus

3. Heberdens nodes

4. Symmetrical joint involvement

ANS: 4

Symmetrical joint involvement is seen with rheumatoid arthritis, not osteoarthritis. Bouchards nodes, Heberdens nodes and crepitus are all assessment findings consistent with osteoarthritis.

PTS:1DIF:Apply

REF: Osteoarthritis: Assessment with Clinical Manifestations

2. The nurse is instructing a client diagnosed with osteoarthritis. Which of the following statements indicates that the client understands these instructions?

-
1. Exercise will not be of help because it will stress my joints.
 2. I will need to lose weight; my doctor says about 20 pounds.
 3. I will take my medications only if the pain is very bad.
 4. I can still go on my marathon shopping trips with my daughter.
-

ANS: 2

Exercise and weight reduction help maintain joint mobility and muscle strength. Walking can be done at home and is low stress to the joints. Scheduled medications should be taken to relieve inflammation, and pain medications should be taken before the pain gets very bad. The client should schedule rest periods with activity so as not to do too much at one time.

PTS:1DIF:Analyze

REF: Osteoarthritis: Collaborative Management; Evaluation of Outcomes

3.A client diagnosed with gout is concerned about the formation of nodules. The nurse should explain that these nodules are called:

TESTBANKSELLER.COM

-
1. Bouchards nodes.
 2. cysts.
 3. Heberdens nodes.
 4. tophi.
-

ANS: 4

A tophus is the characteristic nodule that develops in the patient with gout. A tophus consists of uric acid crystals. Bouchards and Heberdens nodes are present in patients with osteoarthritis. Cysts are fluid-filled areas.

PTS: 1 DIF: Apply REF: Gout: Pathophysiology

4.A client is experiencing an acute attack of gout. The nurse should be prepared to provide which of the following medications as prescribed?

- | | |
|----|----------------|
| 1. | Allopurinol |
| 2. | Colchicine |
| 3. | Probenecid |
| 4. | Sulfinpyrazone |

ANS: 2

Colchicine is used for the acute attack phase. The other medications would be used for further treatment of the gout.

PTS:1DIF:ApplyREF:Gout: Pharmacology

5.A client is going to have tender points examined to determine the diagnosis of fibromyalgia. The nurse should instruct the client the number of tender points that must be positive for the diagnosis would be:

- | | |
|----|-----|
| 1. | 11. |
| 2. | 13. |
| 3. | 15. |
| 4. | 17. |

ANS: 1

The presence of at least 11 of 18 tender points is considered diagnostic for fibromyalgia. The other higher numbers would just be useful to confirm the diagnosis.

PTS:1DIF:Apply

REF: Fibromyalgia: Assessment with Clinical Manifestations

6.The nurse is teaching a group of community members at the senior center about osteoporosis. Which of the following clinical manifestations should the nurse instruct as *not* being related to the disorder?

- | | |
|----|--------------------|
| 1. | Decrease in height |
|----|--------------------|

2.	Fractures
----	-----------

3.	Kyphosis
----	----------

4.	Pain
----	------

ANS: 4

Pain is a symptom of osteoarthritis. A person with osteoporosis may not know she has osteoporosis until height and bone changes have already occurred. Kyphosis or hunchback is a symptom of osteoporosis. A person might not know that they have osteoporosis until they experience a fracture.

PTS:1DIF:Apply

REF: Osteoporosis: Assessment with Clinical Manifestations

7. During an assessment, the nurse determines that a client has a forward curvature of the thoracic spine. Which of the following terms would the nurse use to document this assessment finding?

1.	Kyphosis
----	----------

TESTBANKSELLER.COM

2.	Lordosis
----	----------

3.	Scoliosis
----	-----------

4.	Spondylolisthesis
----	-------------------

ANS: 1

Kyphosis is the forward curvature of the thoracic spine also known as humpback. Lordosis, or swayback, is an inward curvature of the lumbar spine. Scoliosis is a sideways curvature of the spine. Spondylolisthesis is when a vertebra slips over the one below.

PTS:1DIF:ApplyREF: Spinal Disorders

8. A client is being evaluated for generalized skeletal pain and pain in the hips, ribs, and feet. The nurse observes that the client walks with a waddling gait. Because of this information, which of the following should the nurse suspect is occurring with this client?

1.	Osteomalacia
----	--------------

2.	Osteomyelitis
----	---------------

3.	Osteosarcoma
----	--------------

4.	Pagets disease
----	----------------

ANS: 1

Osteomalacia is a metabolic disease that causes poor and delayed mineralization of the bone cells in mature bones. The main cause of osteomalacia is a vitamin D deficiency. Osteomalacia presents with generalized skeletal pain and pain in the hips, ribs, and feet. Pagets disease is a chronic bone disorder with no definitive cure. Osteomyelitis is a serious infection of the bone. Osteosarcoma is the most common type of primary bone cancer.

PTS: 1 DIF: Analyze REF: Osteomalacia: Pathophysiology

9.A young client has been diagnosed with a soft tissue tumor resembling striated muscle in the neck. The nurse realizes that this type of tumor would be considered:

1.	chondrosarcoma.
----	-----------------

2.	Ewings sarcoma.
----	-----------------

3.	neurofibrosarcoma.
----	--------------------

4.	rhabdomyosarcoma.
----	-------------------

ANS: 4

Rhabdomyosarcoma is a soft tissue tumor resembling striated muscle, and it is seen in the extremities, head, neck, urinary tract, or reproductive organs. Chondrosarcoma is a bone tumor most commonly seen in the knees, shoulders, and pelvis. Ewings sarcoma is a bone tumor found in the diaphysis of the femur and the flat bones of the pelvic girdle. Neurofibrosarcoma is a soft tissue tumor found in nerve sheaths.

PTS: 1 DIF: Analyze REF: Tumors of the Musculoskeletal System

10.When assessing a client diagnosed with ankylosing spondylitis, which of the following clinical manifestations will the nurse most likely assess?

1.	Small irregular pupil
----	-----------------------

-
- | | |
|----|-----------|
| 2. | Heel pain |
|----|-----------|
-
- | | |
|----|-------------|
| 3. | Onycholysis |
|----|-------------|
-
- | | |
|----|------------------------|
| 4. | Respiratory depression |
|----|------------------------|
-

ANS: 1

Other problems that occur with ankylosing spondylitis include uveitis that manifests as edema of the upper eyelid, excessive lacrimation, small irregular pupil, and swollen iris. Heel pain is a symptom of reactive arthritis. Onycholysis is seen in psoriatic arthritis. Respiratory depression is a manifestation of polymyositis.

PTS:1DIF:Analyze

REF:Ankylosing Spondylitis: Assessment with Clinical Manifestations

11.A client is surprised to learn that she has been diagnosed with osteoporosis since she does not smoke, is not underweight, and exercises. The nurse should assess if the client has any secondary conditions which could cause the disorder, including:

-
- | | |
|----|--------------------|
| 1. | diabetes mellitus. |
|----|--------------------|
-
- | | |
|----|----------------|
| 2. | heart failure. |
|----|----------------|
-
- | | |
|----|---------------|
| 3. | fibromyalgia. |
|----|---------------|
-
- | | |
|----|----------------------|
| 4. | lactose intolerance. |
|----|----------------------|
-

ANS: 4

Mild malabsorption syndrome as seen with lactose intolerance is a contributing factor to secondary osteoporosis. Secondary osteoporosis is not associated with diabetes mellitus, heart failure, or fibromyalgia.

PTS:1DIF:Apply

REF:Table 59-2 Contributing Factors to Secondary Osteoporosis

12.The nurse is assessing a client who is demonstrating clinical manifestations of Pagets disease. The nurse realizes that which of the following diagnostic tests will aid in the diagnosis of this disorder?

-
- | | |
|----|-------------|
| 1. | Chest x-ray |
|----|-------------|
-
- | | |
|----|------------|
| 2. | Hand x-ray |
|----|------------|
-
- | | |
|----|---------------------|
| 3. | Serum albumin level |
|----|---------------------|
-
- | | |
|----|----------------------------------|
| 4. | Serum alkaline phosphatase level |
|----|----------------------------------|
-

ANS: 4

An increase in serum alkaline phosphatase is often the first indication that the clients symptoms are associated with Pagets disease. Values of two to three times normal indicates the disease. X-rays of the long bones and skull aid in the diagnosis of the disorder, not x-rays of the chest or hands. Serum albumin level is not used to diagnose Pagets disease.

PTS: 1 DIF: Analyze REF: Pagets Disease: Diagnostic Tests

13.A client diagnosed with Pagets disease is prescribed alendronate (Fosamax). Which of the following should the nurse instruct the client regarding this medication?

-
- | | |
|----|---|
| 1. | Do not eat anything for 30 minutes after taking a dose. |
|----|---|
-
- | | |
|----|--|
| 2. | Take the medication on a full stomach. |
|----|--|
-
- | | |
|----|--|
| 3. | Go to the physicians office for the medication to be provided intravenously. |
|----|--|
-
- | | |
|----|--|
| 4. | Expect to stay on this medication for only 6 months. |
|----|--|
-

ANS: 1

Instructions regarding alendronate (Fosamax) should include Do not eat anything for 30 minutes after taking a dose, Take on an empty stomach with 6 to 8 ounces of water in the morning, and Do not lie down for 30 minutes after taking the medication. This medication is not to be taken on a full stomach. This medication is provided orally; the client does not need to go to the physicians office for intravenous administration of the medication. This medication can be taken for longer than 6 months.

PTS:1DIF:Apply

REF: Table 59-4 Biphosphonates used with Pagets Disease

MULTIPLE RESPONSE

1.The nurse is assessing a client who is diagnosed with gout. Which of the following findings will the nurse most likely assess in this client? (Select all that apply.)

- | | |
|----|------------------------------------|
| 1. | Decreased range of motion |
| 2. | Edema in a joint |
| 3. | Elevated uric acid levels |
| 4. | Pain that develops over many weeks |
| 5. | Fever |
| 6. | Headache |

ANS: 1, 2, 3, 5, 6

Assessment findings consistent with a client diagnosed with gout include a rapid development of pain and edema of one joint caused by increased uric acid levels. Swelling, pain, decreased range of motion in the affected joint, fever, and headache can also be present. Pain that develops over many weeks is likely to be caused by another condition.

PTS: 1 DIF: Analyze REF: Gout: Assessment with Clinical Manifestations

2.The nurse is instructing a client diagnosed with gout about a low-purine diet. Which of the following foods would be identified as those to avoid when following this diet? (Select all that apply.)

- | | |
|----|-------------|
| 1. | Avocados |
| 2. | Milk |
| 3. | Scallops |
| 4. | White bread |
| 5. | Alcohol |

-
6. Bacon

ANS: 3, 5, 6

Food high in purine include scallops and bacon. Alcohol is also high in purine. Avocados, milk, and white bread are low-purine foods.

PTS:1DIF:ApplyREF:Gout: Nutrition

3. During an assessment, the nurse determines that a client has risk factors for the development of osteoporosis. Which of the following are considered risk factors for this disorder? (Select all that apply.)

-
1. Balanced diet
-
2. Corticosteroid therapy
-
3. Low body weight
-
4. Smoker
-
5. Impaired vision
-
6. Alcohol intake is one drink per month

ANS: 2, 3, 4, 5

Risk factors for the development of osteoporosis include corticosteroid therapy, low body weight, smoking, and impaired vision. A balanced diet does increase a clients risk for developing the disorder. Alcohol intake of more than two drinks per day would increase the clients risk, but one drink per month would not.

PTS:1DIF:Analyze

REF: Red Flag: Risk Factors for Developing Osteoporosis and Related Fractures

4. A client is experiencing symptoms associated with the second-stage of Lyme disease. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

-
1. Erythema migrans

- | | |
|----|--|
| 2. | Numbness and pain in the arms and legs |
| 3. | Paralysis of facial muscles |
| 4. | Meningitis |
| 5. | Knee pain and swelling |
| 6. | Memory loss |

ANS: 2, 3, 4

Symptoms of the second-stage of Lyme disease include numbness and pain in the arms and legs, paralysis of facial muscles, and meningitis. Erythema migrans is a symptom of the first stage of the disease. Knee pain and swelling and memory loss are symptoms of the third-stage of the disease.

PTS:1DIF:Apply

REF: Lyme Disease: Assessment with Clinical Manifestations

TESTBANKSELLER.COM

5.A client has been diagnosed with fibromyalgia. The nurse realizes that which of the following categories of medications have been successful in the treatment of this disorder? (Select all that apply.)

- | | |
|----|--------------------------|
| 1. | Antiarrhythmics |
| 2. | Antibiotics |
| 3. | Antidepressants |
| 4. | Analgesics |
| 5. | Calcium channel blockers |
| 6. | Muscle relaxants |

ANS: 3, 4, 6

Analgesics, antidepressants, and muscle relaxants are medications commonly prescribed in the treatment of fibromyalgia. Antiarrhythmics, antibiotics, and calcium channel blockers are not used in the treatment of this disorder.

Chapter 41 Nursing Management: Patients with Musculoskeletal Trauma

MULTIPLE CHOICE

1. A client tells the nurse that he has pain, swelling, fatigue, and numbness of his hands. The nurse should assess the client for which of the following occupations?

-
- | | |
|----|--------------------|
| 1. | Retail store clerk |
|----|--------------------|
-
- | | |
|----|-----------|
| 2. | Lifeguard |
|----|-----------|
-
- | | |
|----|----------------------------|
| 3. | Computer keyboard operator |
|----|----------------------------|
-
- | | |
|----|------------|
| 4. | Bus driver |
|----|------------|

ANS: 3

TESTBANKSELLER.COM

Some occupations, sports, and tasks can create repetitive motion injuries or cumulative trauma. A computer keyboard operator is an occupation with a high incidence of overuse syndrome.

PTS:1DIF:Apply

REF: Box 60-2 Occupations and Sports with High Incidence of Overuse Syndrome

2. A client who plays baseball on the weekends is experiencing an arm injury. The nurse realizes this client needs to be evaluated for:

-
- | | |
|----|----------------------|
| 1. | a rotator cuff tear. |
|----|----------------------|
-
- | | |
|----|------------------------|
| 2. | lateral epicondylitis. |
|----|------------------------|
-
- | | |
|----|------------------------------|
| 3. | dislocation of the shoulder. |
|----|------------------------------|
-
- | | |
|----|------------------------|
| 4. | patellar tendinopathy. |
|----|------------------------|

ANS: 1

A rotator cuff tear can be caused by extensive overhead movements found in sports and activities such as baseball, softball, tennis, swimming, and volleyball. A dislocation of the shoulder is most commonly caused by a fall on an outstretched hand and arm. Lateral epicondylitis, or tennis elbow, is an overuse injury that involves the extensor/supinator muscles that attach to the distal humerus. Patellar tendinopathy, also known as jumpers knee, is seen in athletes who participate in activities that require a lot of jumping such as basketball.

PTS: 1 DIF: Analyze REF: Rotator Cuff Tears: Etiology

3.A client, diagnosed with an ankle sprain, is prescribed ibuprofen to control pain and inflammation. What instruction should the client receive concerning this medication?

-
1. Bleeding is not a problem with this medication.
 2. Take on an empty stomach to maximize its effect.
 3. Take with food to minimize gastrointestinal irritation.
 4. Wear sunscreen if outside to prevent a burn.
-

ANS: 3

TESTBANKSELLER.COM

Ibuprofen is a nonsteroidal anti-inflammatory drug (NSAID). NSAIDs should be taken with food to minimize gastrointestinal irritation. Ibuprofen does not increase photosensitivity; however, bleeding can be a problem when taking ibuprofen.

PTS: 1 DIF: Apply REF: Ankle Sprain: Pharmacology

4.A client, experiencing a fractured arm, asks the nurse why the splint is being applied. Which of the following should the nurse respond to this client?

-
1. It reduces the need for a cast.
 2. It reduces bleeding, swelling and pain.
 3. It prevents the need for surgery.
 4. It immobilizes the muscles and joints.
-

ANS: 2

Splinting of a fractured extremity minimizes bleeding, edema, and pain. Splinting does not reduce the need for a cast nor prevent the need for surgery. A cast immobilizes the muscles and joints.

PTS: 1 DIF: Apply REF: Fractures: Planning and Implementation

5.A client has had a cast applied to immobilize a right ulnar fracture. Which of the following nursing interventions is most important?

-
1. Calling physical therapy for a sling
 2. Checking capillary refill time
 3. Giving pain medication
 4. Starting discharge teaching
-

ANS: 2

Checking the capillary refill time determines that circulation is not compromised. The other options can be completed after ensuring that circulation to the site is still adequate.

TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Fractures: Planning and Implementation

6.A client with a right arm cast is experiencing signs of a serious complication. Which of the following would cause the nurse the most concern?

-
1. Capillary refill time less than 3 seconds
 2. Finger movement
 3. Itching under the cast
 4. Severe pain to the right arm continues after receiving pain medication
-

ANS: 4

Severe pain that continues after receiving pain medication would be considered as being disproportionate to the injury, can be a sign of compartment syndrome, and should be immediately reported. The other options are expected assessment findings for a client with a cast.

PTS:1DIF:Analyze

REF: Fractures; Compartment Syndrome: Assessment with Clinical Manifestations

7.A client is unable to pass the chair raise test. The nurse realizes this client is experiencing:

-
- | | |
|----|-------------------------|
| 1. | carpal tunnel syndrome. |
|----|-------------------------|
-
- | | |
|----|--------------------|
| 2. | rotator cuff tear. |
|----|--------------------|
-
- | | |
|----|----------------|
| 3. | fractured arm. |
|----|----------------|
-
- | | |
|----|------------------------|
| 4. | lateral epicondylitis. |
|----|------------------------|

ANS: 4

The chair raise test examines the clients ability to grip and lift. The client stands behind a chair and places the hands on the chair back. The client then attempts to raise the chair. If pain is experienced over the lateral elbows, lateral epicondylitis may be present. The chair raise test is not used to diagnose carpal tunnel syndrome, rotator cuff tear, or a fractured arm.

PTS:1DIF:Analyze

REF:Lateral Epicondylitis: Assessment with Clinical Manifestations

8.A client has been wearing a splint for carpal tunnel syndrome for 7 weeks. The nurse realizes that which of the following would be the next course of treatment for this client?

-
- | | |
|----|---------|
| 1. | Surgery |
|----|---------|
-
- | | |
|----|-----------|
| 2. | Exercises |
|----|-----------|
-
- | | |
|----|--------------------------|
| 3. | Corticosteroid injection |
|----|--------------------------|
-
- | | |
|----|---------|
| 4. | Casting |
|----|---------|

ANS: 3

If after 2 to 7 weeks of conservative treatment the carpal tunnel syndrome symptoms do not improve, corticosteroid injection is recommended. Surgery is not recommended until after corticosteroid injections have been tried. Exercises are implemented with the use of the splint. Casting is not a treatment for carpal tunnel syndrome.

PTS:1DIF:Analyze

REF: Carpal Tunnel Syndrome: Planning and Implementation

9.The nurse is planning care for a client recovering from a meniscal injury. Which of the following should be included as strategies to avoid future injuries?

1. Avoid hamstring muscle exercises.
2. Stretch before and after exercise.
3. Wear similar shoes for all activities.
4. Avoid skiing.

ANS: 2

Strategies to prevent future meniscal injuries include having strong thigh and hamstring muscles; stretching before and after exercise; wearing shoes that fit and are appropriate for the activity; and when skiing, having bindings that release the skis with a fall.

PTS: 1 DIF: Apply REF: Meniscal Injuries: Planning and Implementation

10.A client with an ankle sprain is instructed to follow RICE. Which of the following should the nurse instruct the client regarding this process?

1. Maintain your normal level of activity.
2. Apply ice to the ankle once a day.
3. Apply an elastic bandage to the site.
4. Elevate the extremity every day for 20 to 30 minutes.

ANS: 3

The nurse should instruct the client to use crutches to allow for the rest of the ankle joint and relieve pain; apply ice for 20 to 30 minutes 3 to 4 times a day; apply an elastic bandage to the site; and elevate the ankle for the first 48 hours after the injury.

PTS: 1 DIF: Apply REF: Ankle Sprain: Planning and Implementation

11.The nurse is evaluating the effectiveness of care for a client recovering from an injured Achilles tendon. Which of the following would indicate that care has been effective?

-
1. Client states steroid injections will be helpful to reduce the amount of pain.

 2. Client plans to participate in rehabilitation for 5 to 6 months after the injury.

 3. Client resumes sports activities as soon as possible.

 4. Client uses heat to decrease the inflammation and swelling from the injury.

ANS: 2

Evidence that care has been effective for a client recovering from an injured Achilles tendon would be that the client plans to participate in rehabilitation for 5 to 6 months after the injury. Steroid injections are not used for this type of injury. Sports activities should be avoided until the injury has healed and rehabilitation is completed. Cryotherapy, not heat, is used to decrease the inflammation and swelling from the injury.

PTS:1DIF:Analyze

REF: Achilles Tendon Injuries: Planning and Implementation

12.The nurse is instructing a client on ways to prevent the onset of stress fractures. Which of the following should be included in these instructions?

-
1. Avoid overtraining

 2. Increase intensity of training 10% each day

 3. Limit warm up exercises

 4. Avoid shock absorbing footwear

ANS: 1

Interventions to prevent the onset of stress fractures include: avoid overtraining; gradually increase the intensity of workouts by 10% each week; perform adequate warm up exercises; and use shock absorbing footwear and insoles.

PTS: 1 DIF: Apply REF: Box 60-4 Prevention of Stress Fractures

13.The nurse suspects a client, recovering from hip replacement surgery, is experiencing an infection when which of the following is assessed?

- | | |
|----|-------------------------------------|
| 1. | Client using crutches to ambulate |
| 2. | Blood pressure 110/68 mmHg |
| 3. | Pain with movement |
| 4. | Foot intact to sensation and motion |

ANS: 3

Evidence of an infection in the joint of a client recovering from hip replacement surgery includes erythema, edema, drainage, and tenderness over the joint; persistent pain in the joint with movement; and narrowing of the joint space upon x-ray. Using crutches to ambulate would not indicate an infection in the operative site. A blood pressure of 110/68 mmHg is within normal limits. The foot being intact to sensation and motion would indicate the limb is receiving sufficient blood and oxygenation.

PTS: 1 DIF: Analyze REF: Hip Fractures: Complications

MULTIPLE RESPONSE

1. The nurse is concerned that a client is demonstrating signs of compartment syndrome. Which of the following is considered a classical symptom of this disorder? (Select all that apply.)

- | | |
|----|---------------|
| 1. | Pain |
| 2. | Paraplegia |
| 3. | Paresthesia |
| 4. | Pink |
| 5. | Pressure |
| 6. | Pulselessness |

ANS: 1, 3, 5, 6

The classical symptoms of the six Ps of compartment syndrome are pain, paresthesia, paresis, pressure, pallor, and pulselessness. The pink color and paraplegia are not part of the classic Ps.

PTS:1DIF:Apply

REF: Compartment Syndrome: Assessment with Clinical Manifestations

2.A client is diagnosed with a pathological fracture. For which of the following disease processes should the nurse assess the client? (Select all that apply.)

- | | |
|----|---------------------------------------|
| 1. | Cushings syndrome |
| 2. | Osteomalacia |
| 3. | Pagets disease |
| 4. | Heart failure |
| 5. | Diabetes mellitus |
| 6. | Chronic obstructive pulmonary disease |

ANS: 1, 2, 3

TESTBANKSELLER.COM

Causes of pathological fractures include Cushings syndrome, osteomalacia, and Pagets disease. Pathological fractures are not associated with heart failure, diabetes mellitus, or chronic obstructive pulmonary disease.

PTS: 1 DIF: Apply REF: Box 60-5 Causes of Pathological Fractures

3.A client, recovering from a fractured pelvis, begins to have dyspnea and restlessness. The nurse is concerned that the client is experiencing a fat emboli when which of the following are assessed? (Select all that apply.)

- | | |
|----|----------------------|
| 1. | Upper body petechiae |
| 2. | Cough |
| 3. | Protein in the urine |
| 4. | Seizures |

-
- | | |
|----|------------------|
| 5. | Temperature 102F |
|----|------------------|
-
- | | |
|----|------------------------------|
| 6. | Elevated blood glucose level |
|----|------------------------------|
-

ANS: 1, 4, 5

Symptoms of fat emboli include hypoxemia, mental status changes, petechiae, seizures, and a body temperature greater than 101.3F. Cough, protein in the urine, and elevated blood glucose level are not symptoms of fat emboli.

PTS:1DIF:Analyze

REF: Fat Embolism Syndrome: Assessment with Clinical Manifestations

4.The nurse is assessing a client recovering from abdominal surgery for the development of a deep vein thrombosis. Which of the following would indicate that the client is experiencing this disorder? (Select all that apply.)

-
- | | |
|----|--|
| 1. | Pain and tenderness of the lower extremity |
|----|--|
-
- | | |
|----|--|
| 2. | Red area on a limb that is warm to the touch |
|----|--|
-
- | | |
|----|---------------------|
| 3. | Unexplained dyspnea |
|----|---------------------|
-
- | | |
|----|------------|
| 4. | Chest pain |
|----|------------|
-
- | | |
|----|------------|
| 5. | Hemoptysis |
|----|------------|
-
- | | |
|----|------------------------|
| 6. | Drop in blood pressure |
|----|------------------------|
-

ANS: 1, 2, 3, 4, 5

The client may describe limb pain as aching, cramping, sharp, dull, severe, or mild. Tenderness and pain of the lower extremity and a red area that is warm to touch are also indications that the disorder is present. Other signs and symptoms include unexplained dyspnea, chest pain, and hemoptysis. A drop in blood pressure is not an indication for a deep vein thrombosis.

PTS:1DIF:Apply

REF: Venous Thromboembolism: Assessment with Clinical Manifestations

5.The nurse is planning care for a client recovering from an amputation. Which of the following should be included in this plan of care? (Select all that apply.)

1. Provide pain medication 30 minutes before stump care.
2. Wash the stump daily with mild soap and warm water.
3. Allow the stump to dry open to the air for 10 minutes after washing.
4. Avoid massaging the stump.
5. Elevate the stump on a pillow.
6. Lie prone 10 to 20 minutes every day.

ANS: 1, 2, 3, 6

Care of the client recovering from an amputation includes providing pain medication 30 minutes before stump care; washing the stump daily with mild soap and warm water; allowing the stump to dry open to the air for 10 minutes after washing; massaging the stump daily; avoiding elevating the stump on a pillow to prevent contractures; and lying prone for 10 to 20 minutes every day to prevent contractures.

TESTBANKSELLER.COM

Chapter 42 Nursing Assessment: Neurologic Function

MULTIPLE CHOICE

1. A client is scheduled for surgery to fuse the vertebra in the lumbar region of the spine. The nurse should instruct the client that the number of vertebra being affected by this surgery would be:

- | | |
|----|-----|
| 1. | 7. |
| 2. | 12. |
| 3. | 5. |
| 4. | 4. |

ANS: 3

There are 5 vertebra in the lumbar spine region. This is what the nurse should instruct the client as being fused during the surgery. There are 7 cervical vertebra, 12 thoracic, and 4 coccygeal fused into one.

PTS: 1 DIF: Apply REF: Central Nervous System: Bones

2.A client has sustained a cerebral injury that is applying pressure to the corpus callosum. The nurse realizes that which of the following might occur with this client?

-
- | | |
|----|---------------------|
| 1. | Temporary blindness |
|----|---------------------|
-
- | | |
|----|-----------------------------|
| 2. | Temporary inability to talk |
|----|-----------------------------|
-
- | | |
|----|-----------------------------|
| 3. | Temporary inability to walk |
|----|-----------------------------|
-
- | | |
|----|---|
| 4. | Temporary miscommunication between the sides of the brain |
|----|---|

ANS: 4

The corpus callosum allows the two hemispheres of the brain to communicate. An injury to this area could cause the client to experience temporary miscommunication between the sides of the brain. Pressure on this region may or may not lead to temporary blindness, the inability to talk, or the inability to walk.

PTS: 1 DIF: Analyze REF: Brain

3.A client is recovering from an injury to the frontal lobe of the brain. The nurse realizes that which of the following will be affected by this injury?

-
- | | |
|----|---------------------------------|
| 1. | Higher intellectual functioning |
|----|---------------------------------|
-
- | | |
|----|-------------------|
| 2. | Visual perception |
|----|-------------------|
-
- | | |
|----|--------------|
| 3. | Coordination |
|----|--------------|
-
- | | |
|----|------------------|
| 4. | Respiratory rate |
|----|------------------|

ANS: 1

The major function of the frontal lobe of the cerebral hemisphere is high-level cognitive activity. This is what will be affected in the client with an injury to the frontal lobe. Visual perception

occurs in the occipital lobe. Coordination occurs from the cerebellum. Respiratory rate is controlled by the brainstem.

PTS: 1 DIF: Analyze REF: Figure 34-6 The Lobes of the Brain

4.A client is recovering from a cerebral bleed which is placing pressure on the hypothalamus. Which of the following will the nurse most likely assess in this client?

- | | |
|----|--------------------------------|
| 1. | Variations in body temperature |
| 2. | Blindness |
| 3. | Alteration in speech |
| 4. | Uncoordinated body movements |

ANS: 1

The hypothalamus regulates temperature of the body. Pressure from a cerebral bleed on the hypothalamus could cause variations in the clients body temperature. Pressure to the hypothalamus will not cause blindness, alterations in speech, or uncoordinated body movements.

TESTBANKSELLER.COM

PTS:1DIF:ApplyREFiencephalon

5.A client recovering from a cerebral vascular accident is having difficulty remembering how to chew food. The nurse realizes that which of the following cranial nerves could be affected in this client?

- | | |
|----|----|
| 1. | IX |
| 2. | X |
| 3. | XI |
| 4. | V |

ANS: 4

Cranial nerve V or Trigeminal nerve has three branches. The mandibular branch innervates the muscles for chewing. Cranial nerve IX glossopharyngeal innervates the muscles of swallowing. Cranial nerve X innervates the gastrointestinal tract through parasympathetic tracts of the nerve. Cranial nerve XI innervates the muscles of the neck for movement.

PTS: 1 DIF: Analyze REF: Table 34-2 Cranial Nerves

6. When utilizing the Glasgow Coma Scale during an assessment, the nurse identifies that the client is making incomprehensible sounds. This assessment finding would be included in which part of the assessment?

- | | |
|----|---------------------|
| 1. | Eye opening |
| 2. | Verbal response |
| 3. | Best motor response |
| 4. | Mentation |

ANS: 2

The assessment finding of incomprehensible sounds would be documented within the verbal response section of the Glasgow Coma Scale. Eye opening would assess if the client opens the eyes in response to stimuli. Best motor response would assess the stimuli needed to have the client move an extremity or body part. Mentation is not a category within the Glasgow Coma Scale.

TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Table 34-4 Glasgow Coma Scale

7. A client tells the nurse that at first she did not like to exercise but over time has grown to enjoy it and her body lets her know when she has not done enough. The nurse realizes that the client is experiencing which of the following neurological reactions to exercise?

- | | |
|----|----------------------------|
| 1. | Reduction in serotonin |
| 2. | Reduction in acetylcholine |
| 3. | Increase in endorphins |
| 4. | Reduction in dopamine |

ANS: 3

In response to exercise, the body will release endorphins from the pituitary gland, thalamus, spinal cord, and hypothalamus. This neurotransmitter aids to inhibit pain. Exercise does not reduce serotonin, acetylcholine, or dopamine.

PTS: 1 DIF: Analyze REF: Table 34-1 Neurotransmitters: Site and Action

8. Which of the following techniques should the nurse use to assess a client's pupillary response to light?

1. Briefly shine a penlight into the client's eye by passing the light from the outer edge of the eye toward the center of the eye.
2. Turn the room lights on and off quickly three times.
3. Have the client close his eyes and then quickly open them.
4. Shine the light in the center of the client's eyes for one minute then check them for movement.

ANS: 1

By briefly shining a penlight into the client's eye from the outer edge toward the center of the eye and checking for movement of the pupil, the nurse can tell if there may be brain damage or nerve damage. The other choices are not appropriate techniques to assess a client's pupillary response to light.

PTS: 1 DIF: Apply

TESTBANKSELLER.COM

REF: CN III: Oculomotor Nerve, CN IV: Trochlear Nerve, and CN VI: Abducens Nerve

9. After assessing a client's plantar reflex, the nurse documents that the finding was normal. Which of the following did the nurse assess in this client?

1. Extension of the toes
2. Flexion of the toes
3. No movement of the toes
4. Spasming of the toes

ANS: 2

The normal response is flexion of the toes. Any other response could signify neural impairment.

PTS: 1 DIF: Apply REF: Reflex Testing

10. A client is scheduled for a computed tomography scan of the brain. Which of the following should the nurse do in order to prepare this client for the diagnosed test?

1. Shave the client's head.
2. Administer a sedative.
3. Check to see if the client is allergic to shellfish or iodine.
4. Immobilize the head before movement.

ANS: 3

A CT scan commonly uses contrast agents. These contrast agents often have iodine in them. The nurse should check to see if the client is allergic to iodine or shellfish. Shellfish also have iodine in them. The nurse does not need to shave the client's head, administer a sedative, or immobilize the head before movement.

PTS: 1 DIF: Apply REF: Computed Tomography: Nursing Management

11. A client is scheduled for a diagnostic test to assess the amount of electrical activity within each of the cerebral hemispheres. The nurse realizes that the diagnostic test this client will be having is a(n):

1. myelogram.
2. electroencephalogram.
3. transcranial Doppler sonogram.
4. electromyogram.

ANS: 2

An electroencephalogram or EEG measures the electrical activity of the cerebral hemispheres. A myelogram is an invasive procedure used to visualize obstructions, compression, or herniated intervertebral discs. A transcranial Doppler sonogram measures the velocities of intracranial brain vessels. The electromyogram measures the electrical activity of the peripheral nerves.

PTS: 1 DIF: Analyze REF: Electrographic Studies

12.The nurse is assessing a client recovering from a carotid endarterectomy. Which of the following cranial nerves should the nurse include in this assessment?

1.	CN V
2.	CN VI
3.	CN X
4.	CN XII

ANS: 4

Cranial nerve XII is the hypoglossal nerve. A common cause of dysfunction of this nerve is a carotid endarterectomy. During the surgical procedure, the nerve can be stretched, causing temporary weakness, the nerve can become severed, causing permanent dysfunction. Cranial nerves V, VI, and X are not affected by a carotid endarterectomy.

PTS: 1 DIF: Apply REF: CN XII: Hypoglossal Nerve

13.The nurse assessed a clients deep tendon reflexes as being normal. Which of the following will the nurse document in the clients medical record?

1.	4+
2.	3+
3.	2+
4.	1+

ANS: 3

A deep tendon reflex of normal would be documented 2+. A deep tendon reflex that is very brisk would be documented as 4+. A deep tendon reflex being more brisk than normal would be documented as 3+. A deep tendon reflex that is sluggish would be documented as 1+.

PTS: 1 DIF: Apply REF: Table 34-7 Deep Tendon Reflex Rating Scale

MULTIPLE RESPONSE

1.During an assessment, the nurse determines that a client is experiencing sympathetic responses. Which of the following did the nurse assess in this client? (Select all that apply.)

1.	Decreased heart rate
2.	Increased bowel sounds
3.	Dilated pupils
4.	Increased heart rate
5.	Increased blood pressure
6.	Increased respiratory rate

ANS: 3, 4, 5, 6

Assessment findings consistent with a sympathetic response include dilated pupils, increased heart rate, increased blood pressure, and increased respiratory rate. Assessment findings consistent with a parasympathetic response include decreased heart rate and increased bowel sounds.

PTS:1DIF:Analyze

TESTBANKSELLER.COM

REF:Table 34-3 Sympathetic versus Parasympathetic Response

2.The nurse is planning to assess the visual acuity of a client. Which of the following tools can the nurse use to do this assessment? (Select all that apply.)

1.	Snellen chart
2.	Penlight
3.	Cotton wisp
4.	Rosenbaum pocket screener
5.	Sharp object
6.	Newspaper

ANS: 1, 4, 6

Visual acuity can be assessed by using a Snellen chart, the Rosenbaum pocket vision screener, or a newspaper. A penlight is not used to assess visual acuity. A cotton wisp is used to test for a corneal reflex. A sharp object can be used to assess cutaneous reflexes.

PTS:1DIF:ApplyREF:CN II: Optic Nerve

3.A client is assessed as having a taste abnormality. Which of the following terms can the nurse use to describe this assessment finding during documentation? (Select all that apply.)

1.	Diplopia
2.	Ageusia
3.	Hypogeusia
4.	Dysgeusia
5.	Dysphagia
6.	Ataxia

ANS: 2, 3, 4

Taste abnormalities include ageusia, or the absence of the sense of taste; hypogeusia, or diminished taste sensitivity; and dysgeusia, or a disturbed sense of taste. Diplopia is blurred or double vision. Dysphagia is difficulty swallowing. Ataxia is a lack of muscle coordination.

PTS:1DIF:ApplyREF:CN VII: Facial Nerve

4.The nurse determines that a client is experiencing an alteration in sensory functioning when which of the following are assessed? (Select all that apply.)

1.	Anesthesia
2.	Hypesthesia
3.	Parasthesia
4.	Dysesthesia

5. Hypergesia

6. Ataxia

ANS: 1, 2, 3, 4, 5

Disorders of sensory functioning can cause a variety of symptoms. Anesthesia is the absence of touch sensation. Hypesthesia is a diminished sense of touch. Parasthesia is numbness, tingling, or prickling sensations. Dysesthesia is burning or tingling. Hypergesia is increased sensitivity to pain. Ataxia described uncoordinated muscle (motor) movements most often assessed during ambulation and is not a part of the assessment of sensory functioning.

PTS:1DIF:AnalyzeREF:Sensory Function

5.The nurse is reviewing the results for a clients analysis of cerebrospinal fluid. Which of the following would be considered an abnormal finding? (Select all that apply.)

1. Opening pressure 40 mmHg

2. Cloudy

3. Elevated red blood cell count

4. Elevated white blood cell count

5. Glucose level 60 mg/dL

6. pH 7.35

ANS: 1, 2, 3, 4

Abnormal cerebrospinal fluid analysis findings include opening pressure 40 mmHg, which could indicate dehydration; cloudy in appearance would indicate an increase in white blood cells; elevated red blood cell count would indicate either a traumatic spinal tap or active bleeding; and elevated white blood cell count would indicate meningitis, tumors, or multiple sclerosis. Glucose level of 60 md/dL is a normal finding. Fluid pH of 7.35 is a normal finding.

Chapter 43 Nursing Management: Patients with Oncologic Disorders of the Brain and Spinal Cord

MULTIPLE CHOICE

1. For the client who is at risk for stroke, the most important guideline the nurse should teach is to:

-
- | | |
|----|--------------------------------|
| 1. | increase drinks with caffeine. |
|----|--------------------------------|
-
- | | |
|----|-------------------------|
| 2. | monitor blood pressure. |
|----|-------------------------|
-
- | | |
|----|---|
| 3. | increase amounts of sodium in the diet. |
|----|---|
-
- | | |
|----|------------------------------|
| 4. | monitor weight and activity. |
|----|------------------------------|

ANS: 2

Monitoring weight and activity is important, but the highest priority is monitoring the blood pressure. This is a modifiable risk factor that, when controlled, will decrease the risk of stroke.

PTS: 1 DIF: Apply

REF: Box 35-1 Modifiable Risk Factors for Stroke Development

2. The family of a client diagnosed with a stroke asks the nurse if this health problem is very common. The nurse should respond that in the United States a person has a stroke every:

-
- | | |
|----|-------------|
| 1. | 40 seconds. |
|----|-------------|
-
- | | |
|----|------------|
| 2. | 1 minutes. |
|----|------------|
-
- | | |
|----|------------|
| 3. | 2 minutes. |
|----|------------|
-
- | | |
|----|------------|
| 4. | 5 minutes. |
|----|------------|

ANS: 1

In the United States, a person has a stroke every 40 seconds, and 700,000 new or recurrent strokes each year. Strokes are the third leading cause of death in the United States behind heart disease and cancer and are the leading cause of long-term disability.

PTS: 1 DIF: Apply REF: Cerebrovascular Accidents or Strokes

3. A client is being evaluated for a stroke. The nurse knows that one of the easiest and most common diagnostic tests used to differentiate between strokes is:

-
1. computed tomography (CT).

 2. magnetic resonance imaging (MRI).

 3. electrocardiography (EEG).

 4. positron emission tomography (PET).

ANS: 1

The CT scan is widely available in most hospitals and is an important tool to differentiate between ischemic strokes and hemorrhagic stroke. It is the most common tool used to diagnose a stroke. An MRI is contraindicated in clients with metal implants or pacemakers, and it can exacerbate claustrophobia. An EEG will determine the presence of brain waves, and it is not a diagnostic test for a stroke. A PET scan determines brain tissue functioning but, it will not be able to differentiate between the types of strokes.

PTS:1DIF:AnalyzeREFDiagnostic Tests

4. While instructing a client on stroke prevention, the nurse mentions medications that are useful in stroke prevention. The following medications are effective in preventing a stroke, EXCEPT:

-
1. anticoagulants.

 2. antiplatelets.

 3. anticholinergics.

 4. neuroprotective agents.

ANS: 3

Although anticholinergic drugs have a variety of uses, stroke prevention is not one of them. All the other medications are used in a variety of ways to help with stroke prevention.

PTS:1DIF:ApplyREFPharmacology

5. A client is being seen in the emergency department experiencing symptoms of a stroke. The nurse realizes that the administration of a medication to break clots, such as tPA, should be administered within how many minutes of the client presenting to the emergency department?

-
- | | |
|----|------------|
| 1. | 30 minutes |
|----|------------|
-
- | | |
|----|------------|
| 2. | 60 minutes |
|----|------------|
-
- | | |
|----|------------|
| 3. | 90 minutes |
|----|------------|
-
- | | |
|----|-------------|
| 4. | 120 minutes |
|----|-------------|
-

ANS: 2

Medications like tPA should be given within 60 minutes of the clients arrival to the emergency department. This is why health care teams must have a plan to deal with stroke clients quickly and efficiently.

PTS: 1 DIF: Analyze REF: Emergency Management

6.The nurse, caring for a client with a traumatic brain injury, realizes that the major cause of these types of injuries is:

-
- | | |
|----|-------|
| 1. | guns. |
|----|-------|
-
- | | |
|----|---------|
| 2. | sports. |
|----|---------|
-
- | | |
|----|--------|
| 3. | falls. |
|----|--------|
-
- | | |
|----|------------------------|
| 4. | motor vehicle crashes. |
|----|------------------------|
-

ANS: 4

Although all are major causes of traumatic brain injury, motor vehicle crashes account for 20% of all traumatic brain injuries. Reasons for motor vehicle accidents causing the most traumatic brain injuries include not wearing seat belts and driving while intoxicated.

PTS: 1 DIF: Analyze REF: Brain Injuries: Etiology

7.A client is diagnosed with a mild brain injury. Which of the following is an example of a mild injury?

-
- | | |
|----|------|
| 1. | Coma |
|----|------|
-

-
- | | |
|----|--------------------|
| 2. | Locked-in syndrome |
|----|--------------------|
-
- | | |
|----|------------------|
| 3. | Vegetative state |
|----|------------------|
-
- | | |
|----|------------|
| 4. | Concussion |
|----|------------|
-

ANS: 4

A concussion is a mild form of brain trauma, and it accounts for 75% of all brain injuries. A moderate brain injury would result in the loss of consciousness ranging from a few minutes to hours and days or weeks of confusion. Coma, locked-in syndrome, and a vegetative state are all examples of severe brain injury.

PTS: 1 DIF: Analyze REF: Brain Injuries: Pathophysiology

8. The nurse, caring for a client recovering from a traumatic brain injury, knows the client and the family are eligible for specific federal programs because of the:

-
- | | |
|----|-------------------|
| 1. | Health Brain Act. |
|----|-------------------|
-
- | | |
|----|-----------------------|
| 2. | Associated Brain Act. |
|----|-----------------------|
-
- | | |
|----|-------------------------------------|
| 3. | Traumatic Brain Injury Act of 2008. |
|----|-------------------------------------|
-
- | | |
|----|-----------------------|
| 4. | Brain Protection Act. |
|----|-----------------------|
-

ANS: 3

The Traumatic Brain Injury Act of 2008 is legislation that provides a framework for prevention of, education about, and research on traumatic brain injuries. The act also supports community living for people who have sustained a traumatic brain injury and their families. The other choices are not programs to assist clients who have sustained a traumatic brain injury or their families.

PTS: 1 DIF: Analyze

REF: Law in Practice: Traumatic Brain Injury Act of 2008

9. The nurse is planning care for a client diagnosed with increased intracranial pressure after a head injury. Which of the following interventions can be used to reduce increased intracranial pressure?

-
1. Administer antibiotics as prescribed.

 2. Keep the head of the bed in the flat position.

 3. Administer corticosteroids and osmotic diuretics as prescribed.

 4. Perform range-of-motion exercises every hour.

ANS: 3

The administration of corticosteroids will decrease the swelling of the brain, and osmotic diuretics will decrease the fluid that is building up. This intervention will decrease the intracranial pressure. Antibiotics do not reduce intracranial pressure. Keeping the head of the bed in the flat position can increase intracranial pressure and not decrease it. Performing range-of-motion exercises every hour will not reduce intracranial pressure.

PTS: 1 DIF: Apply REF: Management of Head Injury

10. Which of the following should be *avoided* when caring for a client diagnosed with increased intracranial pressure?

-
1. Starting an intravenous access line

 2. Administering oxygen

 3. Placing the bed in Trendelenburg

 4. Placing the client on bed rest

ANS: 3

Intravenous access and supplemental oxygen are common interventions in the treatment of increased intracranial pressure. Placing the client on bed rest is a proper safety measure. Placing the bed in Trendelenburg position will increase blood flow to the brain and increase ICP.

PTS: 1 DIF: Apply REF: Management of Head Injury

11. A client is being instructed on treatments available for a newly diagnosed brain tumor. The nurse realizes that this client's treatment could include all of the following EXCEPT:

-
1. photo DNA therapy.

2. radiation.

3. chemotherapy.

4. surgery.

ANS: 1

Photo DNA therapy is not a therapy. The other answers are common treatment modalities for patients with brain tumors in addition to photodynamic and adjunctive medication therapy.

PTS: 1 DIF: Analyze REF: Brain Tumors: Planning and Implementation

12.A client diagnosed with an embolic stroke is not a candidate for tPA. The nurse realizes that the client might be eligible for which of the following forms of treatment?

1. Carotid stenting

2. Antiarrhythmic medication

3. Intravenous fluid therapy

4. Carotid endarterectomy

ANS: 1

In clients who are ineligible for tPA therapy, catheter-based treatment such as stenting may be an option. Carotid endarterectomy is used to prevent a stroke. Antiarrhythmic medication does not prevent a stroke. Intravenous fluid therapy does not prevent a stroke.

PTS: 1 DIF: Analyze REF: Surgery

13.A client diagnosed with a brain tumor is going to receive chemotherapy. The nurse realizes that which of the following medications would most likely be prescribed for this clients treatment?

1. Carmustine

2. Digoxin

3. Aminophylline

4. Acetaminophen

ANS: 1

One of the biggest obstacles for chemotherapeutic agents when treating brain tumors is selecting a medication that will cross the blood-brain barrier. Carmustine can cross the blood-brain barrier. The other medications are not used as chemotherapy for brain tumors.

PTS: 1 DIF: Analyze REF: Brain Tumors: Chemotherapy

MULTIPLE RESPONSE

1. A client, being tested for a stroke, is not a candidate for tPA. Which of the following would be contraindicated for the use of tPA? (Select all that apply.)

1. Minor ischemic stroke within 30 days

2. Glucose level 120 mg/dL

3. Blood pressure 190/120 mmHg

4. Lumbar puncture 2 days ago

5. Stroke onset 5 hours ago

6. INR 1.0

ANS: 1, 3, 4, 5

Contraindications of tPA to treat an embolic stroke include minor ischemic stroke within the last 30 days, blood pressure greater than 185 mmHg systolic or greater than 110 mmHg diastolic, lumbar puncture within the last 3 days, and onset of stroke greater than 3 hours. Glucose level of 120 mg/dL and INR of 1.0 would not be contraindications for tPA therapy.

PTS: 1 DIF: Analyze

REF: Table 35-2 Clinical Indications and Contraindications for tPA in Stroke Patients

2. The nurse, planning care for a client recovering from a traumatic brain injury, is including interventions to prevent sympathetic storming. Which of the following should be included in this client's plan of care? (Select all that apply.)

-
1. Medicate for pain prior to conducting a painful procedure.

 2. Elevated blood pressure indicates a sympathetic storm is ending.

 3. Continue suctioning until the clients heart rate is greater than 100 beats per minute.

 4. Cardiac arrhythmias indicate a drop in intracranial pressure.

 5. Provide beta-blockers as prescribed with symptoms of sympathetic storm.

 6. If symptoms of sympathetic storm do not appear within 24 hours, the client will not develop this health problem.

ANS: 1, 5

The nurse should medicate the client for pain prior to conducting a painful procedure and provide beta-blockers as prescribed with symptoms of a sympathetic storm. An elevated blood pressure is a symptom of sympathetic storm. An elevated heart rate is a symptom of sympathetic storming. Cardiac arrhythmias are also a symptom of a sympathetic storm and do not indicate a drop in intracranial pressure. Symptoms of sympathetic storming can occur within 24 hours after a traumatic brain injury and can reoccur periodically during the recovery process.

PTS: 1 DIF: Apply REF: Red Flag: Sympathetic Storming

3.The nurse is providing discharge instructions to a client recovering from a traumatic brain injury. Which of the following should be included in these instructions? (Select all that apply.)

-
1. Return to a full schedule of work as soon as possible.

 2. Acquire medical clearance prior to returning to work that uses heavy equipment.

 3. Avoid the use of helmets.

 4. Limit the amount of alcoholic beverages.

 5. Avoid all illicit drug use.

 6. Eat a well-balanced diet.

ANS: 2, 5, 6

Discharge instructions for a client recovering from a traumatic brain injury should include: medical clearance is needed prior to returning to work that uses heavy equipment; avoid all illicit drug use; and eat a well-balanced diet. The client should be cautioned to avoid returning to a full schedule of work as soon as possible. The client should be encouraged to use helmets or other safety equipment to protect the head. The clients should be instructed to avoid all alcoholic beverages.

PTS:1DIF:Apply

REF: Patient Playbook: Education Topics for a Patient with a Brain Injury

4.A client asks the nurse to explain symptoms that would indicate the presence of a brain tumor. Which of the following should the nurse respond to this client? (Select all that apply.)

-
1. There are no symptoms specific to a brain tumor.
 2. Dizziness is a common symptom.
 3. Ringing or buzzing in the ears can occur.
 4. Seizures may occur. TESTBANKSELLER.COM
 5. A headache that gets worse in the afternoon is specific to a brain tumor..
 6. A headache is usually experienced by 50% of all people diagnosed with a brain tumor.
-

ANS: 2, 3, 4, 6

Symptoms of a brain tumor include dizziness, ringing or buzzing in the ears, seizures, and a headache. The headache of a brain tumor is usually worse in the morning and not the afternoon. There are symptoms associated with a brain tumor.

PTS:1DIF:Apply

REF: Brain Tumors: Assessment with Clinical Manifestations

5.The nurse is instructing a client diagnosed with a brain tumor on symptoms to immediately report to her physician. Which of the following should be included in these instructions? (Select all that apply.)

-
1. New onset of seizures

2.	One-sided weakness
3.	Loss of balance
4.	Problems with vision
5.	Inability to talk
6.	Loss of appetite

ANS: 1, 2, 3, 4, 5

Brain tumor symptoms that require immediate attention include new onset of seizures, slow progressing hemiparesis, gait or balance disturbances, visual problems, hearing loss, and aphasia. Loss of appetite is not a brain tumor symptom.

PTS:1DIF:Apply

REF: Red Flag: Brain Tumor Symptoms that Require Immediate Attention

6.The nurse, caring for a client diagnosed with a brain tumor, is planning interventions to assist with swallowing and prevent aspiration. Which of the following would be appropriate for this client? (Select all that apply.)

1.	Instruct the client to tuck the chin with each swallow.
2.	Instruct the client to turn the head toward the strong side to swallow.
3.	Instruct the client to turn the head toward the weak side to swallow.
4.	Instruct the client to hold the breath while swallowing.
5.	Instruct the client to eat in a reclining position.
6.	Instruct the client to sit in an upright position when eating.

ANS: 1, 3, 4, 6

Interventions to assist a client with swallowing and prevent aspiration include have the client tuck the chin with each swallow, turn the head to the weak side to swallow, hold the breath while swallowing, and sitting in an upright position to swallow. The client should not be instructed to turn the head toward the strong side to swallow or to eat in a reclining position.

Chapter 44 Nursing Management: Patients with Neurologic Trauma

MULTIPLE CHOICE

1.The nurse is preparing a community education program on the prevention of spinal cord injuries. Which of the following individuals would most likely benefit from this education?

1. Adolescent female who plays golf
2. Adolescent male who rides horses
3. Thirty-year-old female housewife
4. Fifty-year-old male who is a computer technician

TESTBANKSELLER.COM

ANS: 2

Most spinal cord injuries are caused by motor vehicle accidents; however, falls from horses and bicycles are common causes of these types of injuries. The average age of spinal cord injuries has risen to 40.2 in 2009. The adolescent female who plays golf is at a lower risk of sustaining a spinal cord injury than the adolescent male who rides horses. The 30-year-old female housewife and 50-year-old male computer technician are also at lower risk for experiencing this type of injury.

PTS: 1 DIF: Apply REF: Spinal Cord Injury: Epidemiology

2.A client being treated for a spinal cord injury needs immediate ventilatory support. The nurse realizes that this clients level of injury is most likely:

- | | |
|----|-----|
| 1. | C3. |
| 2. | C6. |
| 3. | T3. |

4.

L3.

ANS: 1

High cervical injuries above C3 will result in loss of respiratory function and death unless ventilator support is immediately provided. Spinal cord injuries at C6, T3, or L3 do not need immediate ventilatory support.

PTS: 1 DIF: Analyze REF: Spinal Cord Injuries: Level of Injury

3.A client experienced a spinal cord injury during a football game. The paramedics applied a neck brace prior to moving the client onto a hard board for transportation. The nurse realizes that the neck brace was provided because the area of the spinal cord most vulnerable to injury is:

1. coccygeal.

2. lumbar.

3. thoracic.

4. cervical. TESTBANKSELLER.COM

ANS: 4

Because of greater movement, the cervical area is the most unstable area of the spinal cord and is the most vulnerable area for injury. The thoracic, lumbar, and coccygeal regions of the spinal cord are more stable and less vulnerable areas for injury.

PTS: 1 DIF: Analyze REF: Spinal Cord Injuries: Level of Injury

4.The nurse is caring for a client with a spinal cord injury located at T5. Which of the following should be included in this clients plan of care?

1. Use mechanical ventilation.

2. Assess blood glucose level for onset of diabetes.

3. Assist with removal of pulmonary secretions.

4. Provide assistive devices for ambulation.

ANS: 3

Because of the interruption in chest muscle innervation with spinal cord injuries located at level T7 and above, patients often need assistance with removal of secretions and have difficulty with inspiration and expiration. Mechanical ventilation is not needed for a spinal cord injury at level T5. A spinal cord injury does not precipitate the onset of diabetes. A client with a spinal cord injury at level T5 will not be able to use an assistive device to ambulate.

PTS: 1 DIF: Apply REF: Acute Management of Spinal Cord Injuries

5.The nurse is preparing to administer high-dose methylprednisolone to a client diagnosed with a spinal cord injury. The nurse realizes that for this medication to be most effective treatment should begin:

1. within 8 hours of injury.
2. between 8 and 12 hours of injury.
3. between 12 and 24 hours of injury.
4. 48 hours after the injury.

TESTBANKSELLER.COM

ANS: 1

Early treatment of spinal cord trauma with large doses of methylprednisolone have been shown to be extremely effective in the prevention of spinal cord damage after trauma occurs if administered within 8 hours of injury. Providing this medication after 8 hours following an injury is less effective in the prevention of spinal cord damage after a traumatic event.

PTS: 1 DIF: Analyze REF: Spinal Cord Injuries: Evidence-Based Care

6.A client receiving care for a spinal cord injury complains of a pounding headache, blurred vision, and has a blood pressure of 200/100 mmHg. What is the first action the nurse should take?

1. Administer pain medication.
2. Position the client on the left side.
3. Turn off the lights and decrease the noise in the room.
4. Check the bladder for distension.

ANS: 4

The symptoms suggest autonomic hyperreflexia, a medical emergency. The client should be checked for a distended bladder and be prepared for catheterization. Pain medication, positioning, or reducing environmental stimuli will not treat the underlying cause of autonomic hyperreflexia.

PTS: 1 DIF: Apply REF: Emergency Management of Complications of SCIs

7.The nurse, planning care for a client diagnosed with a spinal cord injury, would include interventions to address autonomic dysreflexia because the clients spinal cord injury is at which of the following levels?

1.	L5
2.	T12
3.	S1
4.	T4

ANS: 4

TESTBANKSELLER.COM

Clients with spinal cord injuries above the level of T6 are at the greatest risk for complications associated with autonomic dysreflexia. This complication is not common with injuries at L5, T12, or S1.

PTS: 1 DIF: Apply REF: Emergency Management of Complications of SCIs

8.A client diagnosed with a spinal cord injury has been experiencing spinal shock. Which of the following assessment findings would indicate that this shock is resolving?

1.	Blood pressure 80/55 mmHg
2.	Heart rate 48 beats per minute
3.	Reflexive emptying of the bladder
4.	Body temperature 97F

ANS: 3

Resolution of spinal shock is indicated by return of reflexes, replacement of flaccidity with hyperreflexes, and reflexive emptying of the bladder. Low blood pressure, low heart rate, and low body temperature are all indications of neurogenic shock, which is an aspect of spinal shock.

PTS: 1 DIF: Analyze REF: Emergency Management of Complications of SCIs

9.A client is recovering from a spinal cord injury at level T12. Once spinal shock has resolved and the clients status has stabilized, the client will need which of the following types of care going forward?

1.	Medical
2.	Nursing
3.	Physical therapy
4.	Spiritual

ANS: 2

Spinal cord injuries create a very intensive nursing situation for clients. It is one of the few conditions in which the need for nursing care is generally much greater than the need for medical care after the acute phase of recovery. Medical care is, of course, needed more during the acute phase of recovering from the injury. Physical therapy is one aspect of care that the client will need going forward; however, it is not as intensive as the nursing care required. Spiritual care may be needed once the client realizes that the ability to walk or function without assistance will not occur.

PTS: 1 DIF: Analyze REF: Spinal Cord Injuries: Collaborative Management

10.A client diagnosed with a spinal cord tumor is being prepared for treatment. The nurse realizes that treatment will include irradiation along with which of the following?

1.	Tylenol
2.	Vicodin
3.	Reglan
4.	Dexamethasone

ANS: 4

Irradiation and large doses of dexamethasone are the usual treatment for spinal cord tumors. This will result in reduction of edema and relief of pain. Tylenol, Vicodin, and Reglan may need to be prescribed for symptoms such as headache, pain, and nausea, but they are not considered the primary treatment for a spinal cord tumor.

PTS: 1 DIF: Analyze REF: Spinal Cord Tumors: Surgery

11.A client is diagnosed with Guillain-Barr syndrome. The nurse would assess that signs and symptoms of the disorder would appear in which of the following order?

1. In the hands and arms, progressing to the shoulders and head
2. At the top of the spine, progressing to the brain
3. In the legs, progressing up the body
4. In the face, spreading over the entire facial muscles

ANS: 3

Guillain-Barr syndrome is characterized by a weakness that starts in the lower extremities and progresses up the body to the trunk and arms and finally to the cranial nerves. Signs and symptoms associated with this disorder do not begin in the hands and arms, at the top of the spine, or in the face.

PTS: 1 DIF: Analyze REF: Guillain-Barre Syndrome: Etiology

12.The nurse is determining diagnoses appropriate for a client newly diagnosed with Guillain-Barr syndrome. Which of the following nursing diagnoses would be appropriate for this client?

1. Risk for injury
2. Ineffective breathing pattern
3. Risk for infection
4. Pain

ANS: 2

Clients diagnosed with Guillain-Barr often develop respiratory difficulties because of muscle weakness and ineffective cough. Upon diagnosis of the disorder, the client with Guillain-Barr

syndrome is most likely not at risk for injury, pain, or infection. Once the symptoms begin to resolve, the clients risk for injury, pain, or infection will increase.

PTS: 1 DIF: Apply REF: Guillain-Barre Syndrome: Nursing Diagnoses

13. Which of the following should the nurse instruct a client diagnosed with trigeminal neuralgia to help control symptoms?

1. Perform active range-of-motion exercises to all extremities.
2. Avoid extreme temperature variances in food and drink.
3. Stay in bed with little movement.
4. Drink eight glasses of water a day.

ANS: 2

Extreme temperatures in food and drink can trigger severe facial pain along the trigeminal nerve. This is what the nurse should instruct the client. Active range-of-motion exercises will not help control the symptoms of this disorder. Staying in bed with little movement or drinking eight glasses of water a day will also not control the symptoms of this disorder.

PTS: 1 DIF: Apply

REF: Trigeminal Neuralgia: Planning and Implementation

14. A client diagnosed with Bells palsy asks the nurse why eye drops have been prescribed. Which of the following should the nurse respond to this client?

1. Eyes tend to dry out because the eyelids do not close.
2. Eyes become irritated because the eye does not make tears.
3. Eyes trap dust because the eyelid does not open and the eye needs to be flushed.
4. The eye drops should not be prescribed and the physician should be notified.

ANS: 1

In Bells palsy, the eyelids do not close, and eye drops are used to keep the eye lubricated. Eye drops are not used because the eye does not make tears. Eye drops are not used to flush the eye. Eye drops will be prescribed. The physician does not need to be notified.

PTS: 1 DIF: Apply REF: Bells Palsy: Planning and Implementation

MULTIPLE RESPONSE

1.The nurse is caring for a client receiving radiation for a spinal cord tumor. The nurse realizes that interventions need to be planned to address which of the following potential complications of this treatment? (Select all that apply.)

- | | |
|----|------------------------------------|
| 1. | Increasing neurological impairment |
| 2. | Paralysis |
| 3. | Decubitus ulcers |
| 4. | Peripheral vascular disease |
| 5. | Loss of bowel and bladder function |
| 6. | Osteoporosis |

TESTBANKSELLER.COM

ANS: 1, 2, 5

Radiation complications will cause damage manifested as sensory impairments occurring after the completion of the radiation treatments. This can progress to increasing neurological impairment, paralysis, and loss of bowel and bladder function. Decubitus ulcers, peripheral vascular disease, and osteoporosis are not complications of radiation therapy for a spinal cord tumor.

PTS: 1 DIF: Apply REF: Spinal Cord Tumors: Radiation Therapy

2.The nurse realizes that most spinal cord injuries are caused by: (Select all that apply.)

- | | |
|----|------------------------|
| 1. | falls. |
| 2. | motor vehicle crashes. |
| 3. | sports injuries. |
| 4. | gunshot wounds. |

5. walking.

6. gardening.

ANS: 1, 2, 3, 4

Most spinal cord injuries are caused by motor vehicle accidents. Falls, gunshot wounds, and sports-related accidents also cause a significant number of these types of injuries. Spinal cord injuries are not caused by walking or gardening.

PTS: 1 DIF: Analyze REF: Spinal Cord Injury: Etiology

3.A client is diagnosed with Bells palsy. Which of the following will the nurse most likely assess in this client?

1. Facial weakness

2. Dry eye

3. Drooling

TESTBANKSELLER.COM

4. Impaired speech

5. Dizziness

6. Shortness of breath

ANS: 1, 2, 3, 4, 5

Symptoms of Bells palsy usually begin with facial weakness, pain, a dry eye, and dry mouth. Over time, additional symptoms include: facial twitching, hypersensitivity to sound, drooling, impaired sense of taste, impaired speech, dizziness, and the inability to blink or close the eye. Shortness of breath is not a symptom of Bells palsy.

PTS:1DIF:Apply

REF: Bells Palsy: Assessment with Clinical Manifestations

4.A client is diagnosed with a peripheral nerve injury. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

1.	Paresthesias
2.	Paralysis
3.	Hypotension
4.	Cardiac dysrhythmias
5.	Paresis
6.	Pain

ANS: 1, 2, 5, 6

Paresthesias, paralysis, paresis, and pain are common results of nerve injuries and the degree of dysfunction associated with each of these findings will vary and may change according to the injury. Hypotension and cardiac dysrhythmias are not common assessment findings with a peripheral nerve injury.

PTS:1DIF:Apply

TESTBANKSELLER.COM

REF: Peripheral Nerve Injuries: Assessment with Clinical Manifestations

5.A client is being evaluated for the presence of carpal tunnel syndrome. Which of the following diagnostic tests will be used to aid in the diagnosis of this disorder? (Select all that apply.)

1.	Electromyography
2.	Electroencephalogram
3.	Electrocardiogram
4.	Allens test
5.	Tinels sign
6.	Phalens sign

ANS: 1, 5, 6

Diagnosis of carpal tunnel syndrome is made through the use of electromyography, Tinel's sign, and Phalen's sign. Electroencephalogram, electrocardiogram, and the Allens test are not used to diagnose carpal tunnel syndrome.

Chapter 45 Nursing Management: Patients with Neurologic Disorders

MULTIPLE CHOICE

TESTBANKSELLER.COM

1. A client is diagnosed with a headache from a secondary cause. The nurse realizes this type of headache can be caused by:

- | | |
|----|-------------|
| 1. | a tumor. |
| 2. | tension. |
| 3. | a migraine. |
| 4. | cluster |

ANS: 1

Primary headaches are identified when no organic cause can be found. A tumor headache is caused by a tumor and is classified as a secondary headache.

PTS: 1 DIF: Analyze REF: Headache

2. The nurse should instruct a client diagnosed with migraine headaches to be careful not to overdose on acetaminophen (Tylenol). Which drug should the nurse tell the patient to avoid?

-
- | | |
|----|-------|
| 1. | Aleve |
|----|-------|
-
- | | |
|----|---------|
| 2. | Aspirin |
|----|---------|
-
- | | |
|----|-----------|
| 3. | Ibuprofen |
|----|-----------|
-
- | | |
|----|---------|
| 4. | Vicodin |
|----|---------|
-

ANS: 4

Vicodin, although a narcotic analgesic, also contains acetaminophen (Tylenol). It is very easy to overdose on the acetaminophen (Tylenol) component, which can lead to kidney damage. Aleve does not contain acetaminophen (Tylenol). Aspirin and ibuprofen do not contain acetaminophen (Tylenol).

PTS: 1 DIF: Apply REF: Headache: Pharmacology

3.A client is diagnosed with seizures occurring because of hepatic encephalopathy. The nurse realizes that the cause for this clients seizures would be:

-
- | | |
|----|----------------|
| 1. | physiological. |
|----|----------------|
-
- | | |
|----|-------------|
| 2. | iatrogenic. |
|----|-------------|
-
- | | |
|----|-------------|
| 3. | idiopathic. |
|----|-------------|
-
- | | |
|----|----------------|
| 4. | psychokinetic. |
|----|----------------|
-

ANS: 1

The three major causes for seizures are physiological, iatrogenic, and idiopathic. Physiological seizures include those that occur with an acquired metabolic disorder such as hepatic encephalopathy. Iatrogenic causes include new medications or drug or alcohol use. Idiopathic causes include fevers, fatigue, or strong emotions. Psychokinetic is not a cause for seizures.

PTS: 1 DIF: Analyze REF: Table 37-4 Seizure Causes

4.A client tells the nurse that he sees flashing lights that occur prior to the onset of a seizure. Which of the following phases of a seizure is this client describing to the nurse?

-
- | | |
|----|-----------------|
| 1. | Prodromal phase |
|----|-----------------|
-

-
- | | |
|----|-------------|
| 2. | Aural phase |
|----|-------------|
-
- | | |
|----|-------------|
| 3. | Ictal phase |
|----|-------------|
-
- | | |
|----|-----------------|
| 4. | Postictal phase |
|----|-----------------|
-

ANS: 2

In the aural phase a sensation or warning occurs, which the patient often remembers. This warning can be visual, auditory, gustatory, or visceral in nature. The prodromal phase of a seizure includes the signs or activity before the seizure such as a headache or feeling depressed. The ictal phase of a seizure is the actual seizure. The postictal phase is the period immediately following the seizure.

PTS: 1 DIF: Analyze REF: Seizures: Assessment with Clinical Manifestations

5.A client is experiencing a grand mal seizure. Which of the following should the nurse do during this seizure?

-
- | | |
|----|---------------------------|
| 1. | Protect the clients head. |
|----|---------------------------|
-
- | | |
|----|-------------------------|
| 2. | Leave the client alone. |
|----|-------------------------|
-
- | | |
|----|--|
| 3. | Give water to the client to avoid dehydration. |
|----|--|
-
- | | |
|----|---|
| 4. | Place a finger in the clients mouth to avoid swallowing the tongue. |
|----|---|
-

ANS: 1

One of the most important interventions for a nurse to perform during a seizure is to protect the clients head from injury. Never give a client a drink during a seizure. Placing a finger in the clients mouth could be very dangerous to the client and the nurse. Do not leave the client unattended during a seizure

PTS: 1 DIF: Apply REF: Seizures: Planning and Implementation

6.A client is prescribed phenytoin (Dilantin) for a seizure disorder. Which of the following would indicate that the client is adhering to the medication schedule?

-
- | | |
|----|-----------------------|
| 1. | The client is sleepy. |
|----|-----------------------|
-

-
2. The client is not experiencing seizures.
 3. The client no longer has headaches.
 4. The client is eating more food.
-

ANS: 2

Phenytoin (Dilantin) is a medication to control seizures. The absence of seizures indicates that the client is adhering to the medication schedule. Sleepiness, lack of headaches, or improved appetite are not indications that the medication is being used as prescribed.

PTS: 1 DIF: Analyze REF: Seizures: Table 37-6 Medications to Treat Seizures

7.The nurse is unable to insert an intravenous access line into a client who is currently experiencing a seizure. Which of the following routes can the nurse use to provide medication to the client at this time?

-
1. Oral
 2. Intranasal
 3. Rectal
 4. Intramuscular
-

ANS: 2

For a client experiencing a seizure, oral medications and sharp objects can be dangerous and should not be used. Intranasally administered drugs are rapid and effective in treating a client experiencing an acute seizure. Intranasal delivery is more effective than rectal.

PTS: 1 DIF: Apply REF: Red Flag: Intranasal Drug Delivery

8.One of the most important things a nurse can teach a client about seizure control is to:

-
1. take the medication every day as prescribed by the doctor.
 2. eat a balanced diet.
-

-
3. get lots of exercise.
 4. take naps during the day.
-

ANS: 1

Medication is effective only if it is taken as prescribed, and suddenly stopping the medication can trigger an increase in seizure activity. Diet and exercise are important to a healthy lifestyle but do little to control seizure activity.

PTS: 1 DIF: Apply REF: Seizures: Planning and Implementation

9.The nurse is instructing a client newly diagnosed with multiple sclerosis (MS). To determine the effectiveness of his teaching, the nurse would expect the client to state:

-
1. It is best for me to be in a cold environment.
 2. I should avoid taking a hot bath.
 3. I should eat foods low in salt.
 4. I should be better in a week.
-

TESTBANKSELLER.COM

ANS: 2

The clinical manifestations of MS can be exacerbated by being in a hot, humid environment or by taking a hot bath. A cold environment and low-salt foods do not impact the symptoms of multiple sclerosis. If the client states that they will improve in a week, instruction has not been effective.

PTS:1DIF:Analyze

REF:Multiple Sclerosis: Assessment with Clinical Manifestations

10.An adult female in her 30s complains of numbness and tingling in the hands, fatigue, loss of coordination, incontinence, nystagmus, and ataxia. Which of the following health problems do these symptoms suggest to the nurse?

-
1. Brain tumor
 2. Myasthenia gravis
-

3.	Multiple sclerosis
----	--------------------

4.	Diabetes
----	----------

ANS: 3

Multiple sclerosis is more common in women of this age. These are symptoms, along with the age and sex of the patient, that are common to MS. These symptoms are not necessarily associated with a brain tumor. Weakness is the primary symptom associated with myasthenia gravis. Symptoms of diabetes include weight loss, blurred vision, excessive urination, thirst, and hunger.

PTS: 1 DIF: Analyze REF: Table 37-8 Clinical Manifestations of MS

11. For a client diagnosed with Parkinsons disease, which of the following might be contraindicated?

1.	Performing range-of-motion exercises
----	--------------------------------------

2.	Drinking bottled water
----	------------------------

3.	Instituting fall precautions
----	------------------------------

4.	Taking naps
----	-------------

ANS: 2

Some clients diagnosed with Parkinsons disease develop swallowing difficulties. Powders to thicken liquids and using an upright position will help with these difficulties. Clients diagnosed with Parkinsons disease will benefit from range-of-motion exercises and resting. The client diagnosed with Parkinsons disease should be placed on fall precautions.

PTS: 1 DIF: Apply REF: Parkinsons Disease: Planning and Implementation

12. A client diagnosed with Parkinsons disease is beginning medication therapy. The nurse realizes that the goal of treatment for Parkinsons disease is to:

1.	improve sleep.
----	----------------

2.	reduce appetite.
----	------------------

-
3. control tremor and rigidity.
 4. reduce the need for joint replacement surgery.
-

ANS: 3

The goal of pharmacologic treatment for the client diagnosed with Parkinsons disease is to control tremor and rigidity and to improve the clients ability to carry out the activities of daily living. Medications for Parkinsons disease are not provided to improve sleep, reduce appetite, or reduce the need for joint replacement surgery.

PTS: 1 DIF: Analyze REF: Parkinsons Disease: Pharmacology

13.A client presents complaining of abnormal muscle weakness and fatigability. The physician suspects myasthenia gravis. Which drug can be used to test for this disease?

-
1. Pyridostigmine (Mestinon)
 2. Neostigmine (Prostigmin)
 3. Ambenonium (Mytelase)
 4. Edrophonium (Tensilon)
-

ANS: 4

Tensilon, a short-acting anticholinesterase agent, is the drug of choice for diagnosing myasthenia gravis. The clients response is a rapid improvement of manifestations within 15 to 30 seconds that last 5 minutes. The other medications are used to treat clients diagnosed with myasthenia gravis.

PTS:1DIF:Apply

REF: Myasthenia Gravis: Diagnostic Tests; Pharmacology

MULTIPLE RESPONSE

1.A client is diagnosed with tonic-clonic seizures. Which are the characteristics of these types of seizures? (Select all that apply.)

-
1. Progressing through all of the seizure phases

-
2. Beginning before age 5

 3. Lasting 2 to 3 minutes

 4. Causing injury to the client

 5. Occurring at any time, day or night

 6. Being highly variable

ANS: 1, 3, 4, 5, 6

Tonic-clonic seizures are the most common type of generalized seizure. The seizure will progress through all of the seizure phases and last 2 to 3 minutes. Because these seizures begin suddenly, there is an increased incidence of injury associated with them. These seizures can occur any time of the day or night, whether the client is awake or not. Seizure frequency is highly variable.

PTS:1DIF:AnalyzeREF:Generalized Seizures

2. Which of the following nursing interventions would be appropriate for a client diagnosed with Alzheimers disease? (Select all that apply.)

-
1. Make changes to the room often to stimulate memory function.

 2. Assign simple tasks to be completed by the client.

 3. Assist the client with any needs associated with activities of daily living (ADLs).

 4. Have personal/familiar items around the client.

 5. Do complex games and puzzles to improve memory.

ANS: 2, 3, 4

Alzheimers disease progressively alters the clients ability to function in the normal ways of living. Personal and familiar items help to keep the client oriented, and simple tasks keep the client functioning at the highest levels as long as possible.

PTS:1DIF:Apply

REF: Alzheimers Disease: Planning and Implementation; Evaluation of Outcomes

3.A client has been diagnosed with Parkinsons disease. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

- | | |
|----|------------------|
| 1. | Tremor |
| 2. | Muscle rigidity |
| 3. | Akinesia |
| 4. | Mask-like face |
| 5. | Dysphagia |
| 6. | Reduced appetite |

ANS: 1, 2, 3, 4, 5

Signs and symptoms of Parkinsons disease include tremor, muscle rigidity, akinesia, mask-like face, and dysphagia. Reduced appetite is not a sign or symptom of Parkinsons disease.

PTS:1DIF:Analyze

REF Parkinsons Disease: Assessment with Clinical Manifestations

4.The nurse is planning care for a client diagnosed with myasthenia gravis. Which of the following should be included in this clients plan of care? (Select all that apply.)

- | | |
|----|---|
| 1. | Monitor activities frequently and assist as needed. |
| 2. | Encourage progressive increase in activities. |
| 3. | Determine the best communication method. |
| 4. | Monitor weight. |
| 5. | Restrict fluids. |

-
6. Instruct in energy conservation measures.

ANS: 1, 3, 4, 6

Care for the client diagnosed with myasthenia gravis includes frequent monitoring of activities and assisting as needed, determining the best communication method, monitoring weight, and instructing in energy conservation methods. Encouraging a progressive increase in activities and restricting fluids are not appropriate interventions for a client diagnosed with myasthenia gravis.

PTS: 1 DIF: Apply REF: Myasthenia Gravis: Planning and Implementation

5.The nurse is instructing a client and family regarding the diagnosis of amyotrophic lateral sclerosis. Which of the following should be included in this teaching? (Select all that apply.)

-
1. The length of the curative treatment
-
2. That exercise and physical therapy can help the patient maximize function
-
3. The physical, emotional, and social aspects of the disease
-
4. End-of-life issues TESTBANKSELLER.COM
-
5. The use of devices to prevent aspiration pneumonia
-
6. The use of a speech therapist to aid with communication

ANS: 2, 3, 4, 5, 6

Currently, no cure for this disease exists. Because of the progressive, degenerative nature of the disease, the supportive and educative role of the nurse is important. End-of-life issues need to be discussed before an emergency situation occurs. Other topics of instruction should include the purpose of physical therapy and speech therapy; the use of devices to prevent aspiration; and the emotional and social aspects of the disease.

PTS:1DIF:Apply

REF: Amyotrophic Lateral Sclerosis: Planning and Implementation

6.The nurse is caring for a client diagnosed with Huntingtons disease. Which of the following are considered hallmark clinical manifestations of this disorder? (Select all that apply.)

-
1. Intellectual decline

2.	Weight loss
3.	Decreased appetite
4.	Reduced blood pressure
5.	Nausea
6.	Abnormal movements

ANS: 1, 6

The hallmark clinical manifestations of Huntingtons disease are intellectual decline and abnormal movements. Weight loss, decreased appetite, reduced blood pressure, and nausea are not clinical manifestations of this disorder.

Chapter 46 Nursing Management: Patients with Cerebrovascular Disorders

MULTIPLE CHOICE

Identify the choice that best completes the statement or answers the question.

____ 1. A patient with a temporary loss of motor function is diagnosed with a transient ischemic attack (TIA). What should the nurse include when assisting in the teaching about this health problem?

- a. You had a small hemorrhage in your brain.
- b. Your brain was temporarily deprived of oxygen.
- c. The neurons in your brain are tangled, so messages get mixed up.
- d. You have a vessel that is occluded, blocking the blood supply to your brain.

____ 2. The nurse is assisting with teaching a patient who has had a transient ischemic attack (TIA). On which understanding should the nurse base teaching?

- a. TIAs are not serious, and the patient should have no further problems.
- b. A TIA is predictive that the patient will have a heart attack within 1 year.
- c. A TIA is a medical emergency that requires immediate surgical intervention.
- d. A TIA is a forewarning that the patient is at risk for a cerebrovascular accident (stroke).

____ 3. The nurse is planning care for a client with right-sided weakness and aphasia from a transient ischemic attack (TIA). Which area of the brain should the nurse realize was affected in this client?

- | | |
|----|------------------|
| a. | Medulla |
| b. | Occipital lobe |
| c. | Left hemisphere |
| d. | Right hemisphere |

____ 4. A patient with a cerebrovascular accident (stroke) has left-sided flaccidity and is unable to speak but seems to understand everything the nurse says. Which term should the nurse use to document the patients communication impairment?

- | | |
|----|---------------------|
| a. | Sensory aphasia |
| b. | Motor dysphagia |
| c. | Expressive aphasia |
| d. | Receptive dysphagia |

____ 5. The nurse is documenting care provided to a patient with left-sided flaccidity caused by a stroke. Which term should the nurse use to document this patients motor status?

- | | |
|----|-----------------------------|
| a. | Ipsilateral paraplegia |
| b. | Ipsilateral hemiparesis |
| c. | Contralateral hemiplegia |
| d. | Contralateral quadriparesis |

____ 6. A patient comes into the emergency department with symptoms of a stroke. Which medication should the nurse expect may be given to the patient if diagnostic testing confirms an ischemic stroke?

- | | |
|----|---|
| a. | Heparin |
| b. | Clopidogrel (Plavix) |
| c. | Warfarin (Coumadin) |
| d. | Tissue-type plasminogen activator (tPA) |

____ 7. A patient is prescribed an antiplatelet agent to prevent strokes. Which agent was this patient most likely prescribed?

- | | |
|----|---|
| a. | Aspirin |
| b. | Warfarin (Coumadin) |
| c. | Acetaminophen (Tylenol) |
| d. | Tissue-type plasminogen activator (tPA) |

____ 8. A patient with symptoms of impending stroke is scheduled to have a cerebral angiogram. Which statement should the nurse include when assisting with patient teaching?

- a. This test is designed to detect vascular lesions in the brain.
- b. The angiogram is done to help identify swelling in the brain.
- c. We need to do this to evaluate electrical function of the brain.
- d. This test is done to examine cerebrospinal fluid for signs of bleeding.

____ 9. The nurse is caring for a hospitalized patient who has had a stroke and is waiting to be transferred to a rehabilitation facility. What nursing action can best maximize the patients rehabilitation potential while awaiting the transfer?

- a. Teach the patient what to expect at the rehabilitation facility.
- b. Keep the patient on bedrest to conserve energy for rehabilitation.
- c. Call the physical therapist for bedside rehabilitation until the transfer.
- d. Turn the patient every 2 hours to prevent pressure ulcers and contractures.

____ 10. The nurse is assisting in preparing a patient for transfer to a rehabilitation facility after a stroke. What should the nurse explain as the goal for rehabilitation?

- a. To monitor neurological status
- b. To cure any effects of the stroke
- c. To maximize remaining abilities
- d. To determine the extent of neurological deficits

____ 11. A patient is admitted to the hospital with a severe headache and photophobia. A lumbar puncture confirms a bleeding aneurysm. What nursing interventions should the nurse anticipate assisting with to prevent increased intracranial pressure (ICP) during the acute phase of illness?

- a. Morphine, dark glasses, and expectorants
- b. Quiet room, head of bed up, and stool softeners
- c. Coughing and deep breathing exercises and tranquilizers
- d. Range of motion exercises, bedside commode, and suctioning as needed

____ 12. A client with a subarachnoid bleed refuses to use a bedpan and becomes angry when denied permission to walk to the bathroom. While waiting to hear from the health care provider (HCP), which action should the nurse take?

- a. Help the patient to get up on a bedside commode
- b. Wait for the neurosurgeon to call back with orders
- c. Page security to restrain the patient from harming the nurse
- d. Administer an as-needed dose of a sedative that is ordered

____ 13. A patient is experiencing bilateral hemiparesis, dysphasia, visual changes, and altered level of consciousness, ataxia, and dysphagia. Which artery was most likely affected in this patient's stroke?

- a. Carotid
- b. Middle cerebral
- c. Posterior cerebral
- d. Vertebrobasilar/cerebellar

____ 14. The patient is diagnosed with a cerebral vascular accident that has the slowest rate of recovery and the highest probability of causing extensive neurological deficits. For which type of stroke should the nurse plan care for this patient?

- a. Thrombotic stroke
- b. Cerebral aneurysm
- c. Subarachnoid hemorrhage (SAH)
- d. Reversible ischemic neurological deficit (RIND)

____ 15. A patient enters the emergency department with right-sided weakness and vision changes. What assessment finding should be communicated to the registered nurse (RN) or HCP immediately?

- TESTBANKSELLER.COM
- a. Blood glucose 150 mg/dL
 - b. Blood pressure 148/92 mm Hg
 - c. Onset of symptoms occurred 90 minutes ago
 - d. History of transient ischemic attack (TIA) 3 months ago

____ 16. The nurse is reviewing teaching provided to a patient with transient ischemic attack (TIA). Which statement indicates that further teaching is required?

- a. The risk factors and symptoms of a TIA are just like those of a stroke.
- b. I need to stop smoking to help lower my chances of this happening again.
- c. My risk for Alzheimer's disease is increased now, so I'll have to stop driving.
- d. I recognize how important it is to take my anti-hypertension medications regularly.

____ 17. A patient began experiencing manifestations of a stroke at 0800 hours. By which time should thrombolytic medications be provided to reverse stroke symptoms?

- a. 0900 hours
- b. 1250 hours
- c. 1400 hours
- d. 1660 hours

____ 18. A patient is diagnosed with a stroke that occurred at 12 noon the previous day. When should the nurse plan to begin bedside physical therapy with this patient?

- a. After 5 days
- b. Within 2 to 3 days
- c. By 12 noon on the current day
- d. At least one week after the occurrence

____ 19. The nurse is planning care for a patient with an intracerebral hemorrhage. What should be identified as a goal for this patient?

- a. Maintain blood pressure below 120/80 mm Hg
- b. Resume activities of daily living as soon as possible
- c. Expect to experience transient numbness and tingling
- d. Receive thrombolytic medication therapy within an hour

____ 20. A patient tells the nurse that at times it seems like the mouth muscles do not want to work and the patients speech is slurred. What should the nurse realize that the patient is describing?

- a. Diplopia
- b. Dysarthria
- c. Dysphagia
- d. Dysrhythmia

____ 21. The nurse suspects that a patient has vision changes caused by a stroke. What did the nurse assess to make this determination?

- a. Patient asks that all items be placed on the right side of the bed.
- b. Patient turns head away when blood is being drawn from an arm.
- c. Patient looks down at the floor when sitting on the side of the bed.
- d. Patient does not follow with the eyes as the nurse walks around the room.

____ 22. The results of a carotid Doppler study indicate that a patient has stenosis of the left carotid artery. For which diagnostic test should the nurse prepare the patient to have completed next?

- a. MRI
- b. CT scan
- c. Echocardiogram
- d. Carotid angiography

____ 23. A patient has been prescribed pravastatin (Pravachol) to reduce cholesterol level after having a transient ischemic attack (TIA). What possible side effect should the nurse include when teaching the patient about this drug?

a.	Diarrhea
b.	Purple toe
c.	Confusion
d.	Muscle aches

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

____ 24. The nurse is assisting with a community education program about stroke prevention. Which are non-modifiable risk factors for stroke that the nurse should include? (Select all that apply.)

a.	Gender
b.	Obesity
c.	Diabetes
d.	Heredity
e.	Smoking
f.	Elevated blood lipids

____ 25. The nurse is providing care for a patient with expressive aphasia. What should the nurse expect to find in the patients plan of care? (Select all that apply.)

a.	Speak loudly.
b.	Use a picture board.
c.	Obtain an interpreter.
d.	Provide pencil and paper.
e.	Speak slowly and clearly.
f.	Gesture or pantomime the message.

____ 26. The nurse is planning information about stroke frequency as part of a community health education program. Which demographic groups should the nurse include that are at higher than average risk for stroke? (Select all that apply.)

a.	Pregnant women
b.	Asian Americans
c.	American Indians
d.	African Americans

- e. Men and women 75 years old or older
- f. Individuals who have had a transient ischemic attack (TIA)

____ 27. The nurse is involved in a blood pressure clinic in the community, and an individual with possible stroke symptoms is brought for evaluation. Which findings in the F.A.S.T. assessment indicate the need to call emergency personnel? (Select all that apply.)

- a. The patient sways when asked to stand still with eyes closed.
- b. The patient is unable to follow directions during the assessment.
- c. The patient is unable to repeat a stated phrase exactly as it was stated.
- d. The patients face shows signs of uneven symmetry when asked to smile.
- e. When asked to close the eyes and hold arms straight in front, one arm drifts downward.

____ 28. The LPN has been asked to help a patient eat who has impaired swallowing due to a stroke. What should be included in the plan of care? (Select all that apply.)

- a. Have suction equipment available.
- b. Stay with the patient during meals.
- c. Encourage the patient to eat slowly.
- d. Offer the patient a straw for liquids.
- e. Instruct the patient to try to chew on both sides of the mouth.
- f. Place the patient in high Fowlers position or in a chair for meals.

____ 29. The nurse is assisting with a community education program related to cerebral vascular accidents. What should be included in a list of symptoms that need immediate medical attention? (Select all that apply.)

- a. Sudden trouble seeing in one or both eyes
- b. Sudden severe headache with no known cause
- c. Sudden confusion, trouble speaking, or understanding
- d. Sudden loss of hearing, ringing in the ears, or stabbing ear pain
- e. Sudden trouble walking, dizziness, or loss of balance or coordination
- f. Sudden numbness or weakness of face, arm, or leg, especially on one side of the body

____ 30. A 56 year old female client asks why the nurse is assessing her for a stroke. Which manifestations did the nurse use to make this assessment decision? (Select all that apply.)

- a. Nausea
- b. Hiccups
- c. Itchy skin

- | | |
|----|--------------|
| d. | Chest pain |
| e. | Palpitations |

MULTIPLE CHOICE

1.ANS:B

TIA is a temporary impairment of the cerebral circulation causing neurological impairment that lasts less than 24 hours. A. A hemorrhage would cause a hemorrhagic stroke. D. A fully occluded vessel causes an ischemic stroke. C. Tangled messages refer to Alzheimers disease.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Application

2.ANS

About a third of patients who experience a TIA will have a stroke in the future. A. Urgent evaluation of TIA is essential in order to decrease the risk of stroke. B. There are no data related to myocardial infarction (MI) prediction. C. It is not a surgical problem.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

TESTBANKSELLER.COM

3.ANS:C

Symptom onset is sudden and generally involves one side of the bodythe side of the body opposite to the damaged area. A. B. The manifestations of right-sided weakness and aphasia would not be present if the TIA occurred in the medulla or occipital lobe. D. The client would have left-sided manifestations if the TIA occurred in the right hemisphere.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Analysis

4.ANS:C

Aphasia may be expressive, in which the patient knows what he or she wants to say but cannot speak or make sense. D. Receptive aphasia is an inability to understand spoken or written words. The patient experiencing receptive aphasia is unable to understand language. B. Dysphagia refers to difficulty swallowing. A. Sensory aphasia is not a type of communication impairment.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Application

5.ANS:C

A patient with a stroke has symptoms on the opposite side of the stroke, which is called contralateral. One-sided flaccidity is called hemiplegia. A. Ipsilateral means the same side. Para refers to the lower extremities. D. Quad refers to all four extremities; B. Hemiparesis is another term for hemiplegia.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Application

6.ANS

tPA is a thrombolytic agent that can break down the thrombus causing the occlusion, which can potentially prevent or completely reverse the symptoms of an ischemic stroke. A. B. C. Heparin, warfarin, and clopidogrel can help prevent clots but are not effective in breaking up an existing clot.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPharmacological and Parenteral Therapies | Cognitive Level: Application

7.ANS:A

Aspirin is a platelet aggregation inhibitor. C. Tylenol is an analgesic but does not affect platelet function. B. Warfarin is an anticoagulant. D. tPA is a thrombolytic agent.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPharmacological and Parenteral Therapies | Cognitive Level: Analysis

8.ANS:A

A cerebral angiogram may be completed to determine the patency of cerebral vessels and the status of any collateral circulation. D. A lumbar puncture is done to examine cerebrospinal fluid (CSF). B. Edema may be identified by radiography. C. An electroencephalogram (EEG) shows electrical function.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Application

9.ANS:C

Rehabilitation should begin as soon as the patient is stable. Waiting until the patient is at the rehabilitation facility to begin therapy wastes valuable time. A. Teaching the patient what to expect at the rehabilitation facility will not maximize the patients rehabilitation potential. B. Keeping the patient on bedrest could cause further mobility issues. D. Turning the patient every 2 hours to prevent ulcer formation and contractures will not necessarily maximize the patients rehabilitation potential.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Application

10.ANS:C

Rehabilitation can help the patient maximize remaining abilities. A. D. At this point, the patients neurological status should be stable, and all the diagnostic work has been completed. B. Cure is not realistic.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Application

11.ANS:B

A quiet room with minimal stressors, elevated head, and stool softeners can help reduce ICP. A. C. Morphine and tranquilizers are not usually recommended because they can make neurological assessment difficult. A. C. Expectorants can promote coughing, which can raise ICP. C. D. Exercises, moving, and suctioning can also raise ICP.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

TESTBANKSELLER.COM

12.ANS

Patients with subarachnoid hemorrhage are at risk for rebleeding. A. Straining to have a bowel movement and agitation both increase the risk of rebleeding. B. The patient may need to be sedated until the physician can be contacted. C. Bringing in security will be upsetting to the patient and can also increase the risk of raising the BP and bleeding.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Application

13.ANS

These are symptoms of vertebrobasilar/cerebellar occlusion. A. B. C. Carotid and middle or posterior cerebral occlusions are not associated with ataxia or dysphagia.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Analysis

14.ANS:C

SAH is caused by rupture of blood vessels on the surface of the brain. This type of infarct has the slowest rate of recovery and the highest probability of leaving the patient with extensive

neurological deficits. B. Aneurysms are often asymptomatic if they do not bleed. D. RIND is reversible. A. A thrombotic stroke does not have the slowest rate of recovery.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Application

15.ANS:C

All the data are significant. However, the onset of symptoms is within the time frame for the patient to receive a thrombolytic. If the nurse acts quickly, the patients stroke may be able to be reversed.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

16.ANS:C

There is no association between TIA and the development of Alzheimers disease. A. The risk factors, causes, and symptoms of a TIA are identical to a cerebrovascular accident (CVA). Patients who have had a TIA have an increased risk of having a stroke. Treatment, therefore, is mostly focused on minimizing the patients risk factors for a stroke. B. D. Modifiable risk factors are those risks that can be changed by treatment, such as treating high blood pressure, or by lifestyle modification, such as stopping smoking.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Analysis

17.ANS:B

If a patient experiencing ischemic stroke symptoms receives treatment within 4.5 hours of symptom onset, medication can be provided to resolve the deficits. A. A patient needs to be treated within 4.5 hours and not 1 hour. C. D. This is too long to wait to provide medication to treat the symptoms of a stroke.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Analysis

18.ANS:C

Patients should be mobilized within 24 hours if possible to prevent complications of immobility. Physical and occupational therapy are provided to maximize functioning and to progress the patient toward a return to baseline functioning. A. B. D. Waiting to begin physical therapy could reduce the patients success with physical rehabilitation.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

19.ANS:A

An intracerebral hemorrhage is usually caused by uncontrolled hypertension. Maintaining blood pressure below 120/80 mm Hg should be the goal for these patients. B. These patient cannot resume activities of daily living until the bleeding is controlled within the brain. C. Transient numbness and tingling could indicate additional brain damage from bleeding. D. Thrombolytic therapy is not indicated for an intracerebral hemorrhage.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

20.ANS:B

Slurred or indistinct speech because of a motor problem or lack of coordination is referred to as dysarthria. A. Diplopia is double vision. C. Dysphasia refers to difficulty swallowing. D. Dysrhythmia is an irregular heartbeat.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Analysis

TESTBANKSELLER.COM

21.ANS

If the patients eyes do not follow the nurse when moving around the room, there is a good chance that the patient has a deficit in that visual field. A. B. C. Placing items on the right side of the bed, turning the head away while blood is being drawn, and looking down at the floor when sitting on the side of the bed do not indicate vision changes caused by a stroke.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Analysis

22.ANS

Carotid Doppler testing uses ultrasound to detect stenosis of the carotid arteries. Carotid angiography can be done to further determine degree of blockage and help guide treatment. A. B. C. After a carotid Doppler, an MRI, CT scan, or echocardiogram are not indicated.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

23.ANS

Muscle pain or aches can signal a serious side effect (rhabdomyolysis) and should be reported.
A. B. C. Diarrhea, purple toe, and confusion are not side effects of statins.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPharmacological and Parenteral Therapies | Cognitive Level: Application

MULTIPLE RESPONSE

24.ANS:A, D

Gender and heredity are not modifiable. B. C. E. F. The patient can control diabetes, heart disease, diet, exercise, lipids, and smoking to some degree.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

25.ANS:B, D

For expressive aphasia, pencil and paper or a picture board can help with communication. A. Speaking loudly is not helpful unless the patient has a hearing deficit also. E. F. Speaking slowly and pantomiming may be helpful for receptive aphasia, not expressive. C. Interpreters are used for language barriers, not for aphasia.

PTS:1DIF:Moderate

TESTBANKSELLER.COM

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Application

26.ANS:A, C, D, E, F

Some population groups, such as African Americans, American Indians, Alaskan natives, and Mexican Americans, have a higher than average risk. Recent studies indicate that the risk of stroke may be higher in women during pregnancy and the 6 weeks following childbirth. Patients who have had a TIA have an increased risk of having a stroke; about 24% to 29% of patients who experience a TIA will have a stroke within 5 years. Strokes are most common in people over the age of 75. B. Asian Americans are not as high risk.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

27.ANS:C, D, E

The acronym F.A.S.T. can help identify a stroke. Ask the person to smile: If the face droops or is uneven on one side, it is abnormal. Ask the person to close his or her eyes and hold arms out in front of him or her: If an arm cannot be raised or drifts downward, it is abnormal. Ask the person to say It is a bright and sunny day: Any difficulty understanding or speaking is abnormal. Call

911 immediately for any abnormal findings. A. These are all indicators of a possible stroke. Brain cells may be dying. B. Inability to follow instructions is a concern but is not part of the F.A.S.T. assessment.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityPhysiological Adaptation | Cognitive Level: Analysis

28.ANS:A, B, C, F

High Fowlers position can help reduce aspiration risk. The nurse should stay with the patient and be prepared to use suction in case aspiration does occur. D. Straws and thin liquids increase risk of aspiration. E. The patient should chew on the unaffected side; if he or she chews on the affected side, it is difficult to sense the food, and pocketing may occur. Patients should be encouraged to eat slowly and chew his or her food thoroughly.

PTS:1DIF:Moderate

KEY: Client Need: Physiological IntegrityReduction of Risk Potential | Cognitive Level: Application

29.ANS:A, B, C, E, F

The five signs or symptoms recognized by the American Heart Association/American Stroke Association include sudden numbness or weakness of face, arm, or leg, especially on one side of the body; sudden confusion, trouble speaking, or understanding; sudden trouble seeing in one or both eyes; sudden trouble walking, dizziness, or loss of balance or coordination; and sudden severe headache with no known cause. D. Sudden loss of hearing, ringing in the ears or ear pain are not manifestations of a stroke.

PTS:1DIF:Moderate

KEY: Client Need: Health Promotion and Maintenance | Cognitive Level: Application

30.ANS:A, B, D, E

Women have unique symptoms of a stroke including a sudden onset of nausea, hiccups, chest pain, and palpitations. C. Itchy skin is not a manifestation of a stroke.

Chapter 47 Nursing Assessment: Sensorineural Function

MULTIPLE CHOICE

1.A client is diagnosed with an inability to recognize visual information. The nurse realizes that which of the following cranial nerves is involved in the transmitting of visual stimuli to the brain for interpretation?

1.

CN II

2.	CN III
----	--------

3.	CN IV
----	-------

4.	CN VI
----	-------

ANS: 1

The optic nerve is the second cranial nerve and is responsible for the transmitting of visual stimuli. Cranial Nerves III, IV, and VI control extraocular eye movements.

PTS: 1 DIF: Analyze REF: Anatomy and Physiology of the Eye

2.A client is diagnosed with a vision disorder. The nurse realizes that the client will experience an alteration in sensory information because the eyes transmit what percentage of all sensory information to the brain?

1.	30%
----	-----

2.	50%
----	-----

TESTBANKSELLER.COM

3.	70%
----	-----

4.	90%
----	-----

ANS: 3

Approximately 70% of all sensory information reaches the brain through the eyes.The other percentages are incorrect.

PTS: 1 DIF: Analyze REF: Anatomy and Physiology of the Eye

3.The nurse is performing an assessment on a client. To test the optic nerves function, what should the nurse do?

1.	Check for extraocular movement.
----	---------------------------------

2.	Check the pupils for reaction to light.
----	---

3.	Check to see if the patient can blink.
----	--

-
4. Use a Snellen chart.

ANS: 4

A Snellen chart is used to assess visual acuity of the optic nerve. Extraocular movements assess cranial nerves III, IV, and VI. Pupil reaction to light and eye blinking are not functions of the optic nerve.

PTS: 1 DIF: Apply REF: Assessment of the Eye

4.The nurse realizes that a client, diagnosed with chronic dry eyes, may have a disorder of the lacrimal gland because it:

-
1. covers the eye for protection.
-
2. produces tears to lubricate the eye.
-
3. helps the eye keep its shape.
-
4. provides blood to the eye.

TESTBANKSELLER.COM

ANS: 2

The lacrimal gland moistens the eye by producing and distributing tears to lubricate the eye. The lacrimal gland does not cover the eye for protection, help the eye keep its shape, or provide blood to the eye.

PTS:1DIF:AnalyzeREF:External Eye

5.When assessing the corneal reflex, the nurse realizes this reflex is a function of which cranial nerve (CN)?

-
1. CN II
-
2. CN III
-
3. CN IV
-
4. CN V

ANS: 4

The stimulation of the trigeminal nerve (CN V) causes the corneal reflex, a protective blink. Cranial nerves II, III, or IV do not control the corneal reflex.

PTS: 1 DIF: Analyze REF: Internal Eye

6.A client is having difficulty perceiving different colors. The nurse realizes the client may have a disorder that affects the photosensitive receptor cells of the retina, which makes the perception of color possible, or a disorder that affects the:

- | | |
|----|--------------|
| 1. | rods. |
| 2. | cones. |
| 3. | optic discs. |
| 4. | irises. |

ANS: 2

Other neurosensory elements located in the retina are cones, which mediate color vision. Rods mediate black-and-white vision. The optic disc and iris are not responsible for color vision.

TESTBANKSELLER.COM

PTS: 1 DIF: Analyze REF: Internal Eye

7.A client was assessed as having normal intraocular pressure. The nurse would document this clients pressure as being:

- | | |
|----|-----------------|
| 1. | 5 mmHg 3 mmHg. |
| 2. | 15 mmHg 3 mmHg. |
| 3. | 30 mmHg 3 mmHg. |
| 4. | 50 mmHg 3 mmHg. |

ANS: 2

Normal intraocular pressure is about 15 mmHg 3 mmHg. An intraocular pressure of 5 mmHg would be too low. A pressure of 30 to 50 mmHg would be considered critical.

PTS: 1 DIF: Apply REF: Internal Eye

8.A client tells the nurse that she has to swallow to improve her hearing. The nurse realizes that this action:

-
- | | |
|----|--|
| 1. | causes the tympanic membrane to vibrate. |
|----|--|
-
- | | |
|----|---------------------------|
| 2. | makes the hammer vibrate. |
|----|---------------------------|
-
- | | |
|----|-------------------------|
| 3. | stabilizes equilibrium. |
|----|-------------------------|
-
- | | |
|----|---------------------|
| 4. | equalizes pressure. |
|----|---------------------|

ANS: 4

The middle ear is connected to the nasopharynx by the eustachian tubes, which serve as a channel to equalize pressure. The equalization of pressure is aided by yawning or swallowing. Swallowing does not cause the tympanic membrane to vibrate. This action does not cause the hammer to vibrate. This action also does not stabilize equilibrium.

PTS: 1 DIF: Analyze REF: Middle Ear

9.The nurse who assesses a hematoma behind a clients left ear over the mastoid bone would document this finding as being: TESTBANKSELLER.COM

-
- | | |
|----|---------|
| 1. | normal. |
|----|---------|
-
- | | |
|----|---------------|
| 2. | Battles sign. |
|----|---------------|
-
- | | |
|----|-------------------------|
| 3. | caused by sun exposure. |
|----|-------------------------|
-
- | | |
|----|-----------------|
| 4. | perichondritis. |
|----|-----------------|

ANS: 2

A hematoma behind the ear over the mastoid bone would be documented as Battles sign and is an indication of head trauma to the temporal bone. This is not a normal finding. Battles sign is caused by head trauma to the temporal bone and not by sun exposure. Perichondritis is inflamed connective tissue of the ear cartilage.

PTS:1DIF:Apply

REF: Table 38-4 External Ear Assessment Findings in the Inspection of the Ear

10. During the assessment of a client's external ear canal, the nurse identifies a painful pustule. The nurse realizes this assessment finding could be caused by:

- | | |
|----|---------------------|
| 1. | furunculosis. |
| 2. | exostoses. |
| 3. | hemotympanum. |
| 4. | acute otitis media. |

ANS: 1

Infection of the hair follicle in the ear, or furunculosis, is caused by a painful boil-like pustule in the external ear canal. Exostoses are hard, bony lesions deep in the external ear canal. Hemotympanum is blood in the middle ear. Acute otitis media causes inflammation of the middle ear that is diagnosed by assessing the tympanic membrane.

PTS: 1 DIF: Analyze REF: Table 38-5 Otoscopic Assessment of the Ear

MULTIPLE RESPONSE

TESTBANKSELLER.COM

1. Which of the following tests can be done by the nurse to assess a client's hearing? (Select all that apply.)

- | | |
|----|--------------------|
| 1. | Voice-whisper test |
| 2. | Allens test |
| 3. | Weber test |
| 4. | Cochlear test |
| 5. | Rinne test |
| 6. | Stapes test |

ANS: 1, 3, 5

Tests that can be conducted by the nurse to assess for hearing include the voice-whisper test, the Weber test, and the Rinne test. The Allens test is used to assess blood flow to the hand. The cochlear test and stapes test do not exist.

PTS: 1 DIF: Apply REF: Ear: Examination and Findings

2.The nurse, assisting with the examination of a clients eyes, is preparing the ophthalmoscope. Which of the following apertures might be needed for this examination? (Select all that apply.)

1.	Small round light
2.	Large round light
3.	Grid
4.	Slit light
5.	Green light
6.	Black light

ANS: 1, 2, 3, 4, 5

Apertures of the ophthalmoscope include small round light, large round light, grid, slit light, and green light. A black light is not an aperture of the ophthalmoscope.

PTS: 1 DIF: Apply REF: Table 38-1 Apertures of the Ophthalmoscope

3.Which of the following would the nurse include when assessing a clients ears? (Select all that apply.)

1.	Onset of dizziness
2.	Changes in hearing
3.	Presence of otorrhea
4.	Duration of otalgia

5. Swallowing difficulties

6. Degree of neck pain

ANS: 1, 2, 3, 4

When assessing a client's ears, the nurse should include onset of dizziness, changes in hearing, presence of otorrhea, and duration of otalgia. Swallowing difficulties and degree of neck pain are not a part of this assessment.

PTS: 1 DIF: Apply REF: Patient Playbook: Assessing Ear Problems

4. The nurse assesses a client as having many risk factors for otitis media. Which of the following would increase the client's risk for developing this disorder? (Select all that apply.)

1. Frequent upper respiratory tract infections

2. Attends daycare

3. Male gender

4. Female gender

5. Age 15

6. Rides a bicycle

ANS: 1, 2, 3

Risk factors for the development of otitis media include age less than 2 years; history of frequent upper respiratory infections; lives in cold weather; male gender; is Caucasian, Native American, or an Alaskan native; has a strong positive family history for the disorder; used a pacifier past the age of 6 months; lives in a smoky environment; attends daycare; was bottle fed; has been diagnosed with Down syndrome; or has craniofacial disorders. Female gender, age 15, and riding a bicycle are not risk factors for the development of otitis media.

PTS: 1 DIF: Analyze REF: Red Flag: Risk Factors for Otitis Media

5. A client tells the nurse that he has noticed a decrease in hearing. Which of the following would be risk factors for the client to have a change in hearing? (Select all that apply.)

1.	Smoking
2.	Neck trauma
3.	Cardiovascular disease
4.	Aging
5.	Diabetes
6.	Chronic infection

ANS: 1, 3, 4, 6

Risk factors for hearing loss include noise exposure, smoking, ototoxic drugs, congenital or hereditary factors, cardiovascular disease, aging, tumors, trauma, chronic infections, systemic disease, tympanic membrane perforation, Mniere disease, and barotrauma. Neck trauma and diabetes are not risk factors for a hearing loss.

TESTBANKSELLER.COM

Chapter 48 Nursing Management: Patients with Eye and Vision Disorders

MULTIPLE CHOICE

1.A client is diagnosed with strabismus. Which of the following will the client most likely experience with this disorder?

1.	Nystagmus
2.	Diplopia
3.	Aphakic vision
4.	Ptosis

ANS: 2

Diplopia, or double vision, is the primary symptom of strabismus. Nystagmus is a disorder that causes involuntarily rhythmic movements in the eye. Aphakic vision occurs when the lens of the eye is removed. Ptosis is drooping of the eyelid.

PTS: 1 DIF: Analyze REF: Ocular Movement Disorders: Strabismus

2.A client is experiencing a gradual blurring of vision in both eyes not associated with any pain. The nurse suspects the client is experiencing:

-
- | | |
|----|-----------|
| 1. | glaucoma. |
|----|-----------|
-
- | | |
|----|------------|
| 2. | cataracts. |
|----|------------|
-
- | | |
|----|-----------------------|
| 3. | macular degeneration. |
|----|-----------------------|
-
- | | |
|----|---------------------|
| 4. | retinal detachment. |
|----|---------------------|

ANS: 2

Cataracts occur as the opacity of the lens becomes cloudy, blurring the vision. It occurs in both eyes but is usually worse in one eye. Gradual eye blurring is not associated with glaucoma, macular degeneration, or retinal detachment.

PTS: 1 DIF: Analyze REF: Cataracts: Pathophysiology

3.The nurse should instruct a client, diagnosed with glaucoma, that the purpose of medication is to:

-
- | | |
|----|--------------------------------|
| 1. | help dry up excess secretions. |
|----|--------------------------------|
-
- | | |
|----|---------------------------------|
| 2. | lower the intraocular pressure. |
|----|---------------------------------|
-
- | | |
|----|------------------------------------|
| 3. | strengthen the muscles of the eye. |
|----|------------------------------------|
-
- | | |
|----|--------------------------------|
| 4. | improve the vision in the eye. |
|----|--------------------------------|

ANS: 2

Glaucoma is a disease that relates to the increase of intraocular pressure. The medication given will decrease this intraocular pressure. Medication for glaucoma is not used to help dry up excess secretions, strengthen the eye muscles, or improve vision.

PTS: 1 DIF: Apply REF: Glaucoma: Pharmacology

4. After surgery to remove a cataract, which of the following should the nurse instruct the client?

1. Be sure to follow the schedule for the prescribed eyedrop medication.
2. Sleep on the right side to promote drainage.
3. It is okay to rub the eye because the surgery was on the inside.
4. This is an outpatient procedure, and there are no instructions for the patient.

ANS: 1

Client education is extremely important in the aftercare of cataract surgery. There is a need to emphasize the postoperative care of eyedrop instillation. The client should not place any pressure near or on the eye. Postoperative instructions are highly important for the client having an outpatient surgical procedure.

PTS: 1 DIF: Apply REF: Cataracts: Planning and Implementation

5. A tonometry test has been performed with a client and the results are 25 mmHg. The nurse knows that:

TESTBANKSELLER.COM

1. the reading is low and there is no problem.
2. the reading is normal and nothing needs to be done at this time.
3. the results are high and follow-up readings and tests are needed.
4. the results are high and there is no cure to bring the pressure down.

ANS: 3

Several readings need to be taken throughout the day to establish the highest reading to be the treated pressure. Normal intraocular pressure ranges from 12 to 16 mmHg. The reading of 25 mmHg is not low or normal. Medication can be prescribed to reduce the pressure.

PTS: 1 DIF: Analyze

REF: Glaucoma: Assessment with Clinical Manifestations and Diagnostic Tests

6. A client has been diagnosed with cataracts. The nurse realizes that the only treatment for this disorder is?

-
- | | |
|----|----------------------------------|
| 1. | Medical management with eyedrops |
|----|----------------------------------|
-
- | | |
|----|------------------------------|
| 2. | Surgical removal of the lens |
|----|------------------------------|
-
- | | |
|----|----------|
| 3. | Cryopexy |
|----|----------|
-
- | | |
|----|--------------|
| 4. | Phototherapy |
|----|--------------|
-

ANS: 2

Surgical treatment for cataracts begins when vision is sufficiently impaired. The lens is removed and the replacement artificial intraocular lens is put in place. Cataracts cannot be treated with medication alone. Cryopexy and phototherapy are not used to treat cataracts.

PTS:1DIF:AnalyzeREF:Cataracts: Surgery

7.Which of the following should the nurse assess in a client diagnosed with open-angle glaucoma?

-
- | | |
|----|-----------------------|
| 1. | Degree of lost vision |
|----|-----------------------|
-
- | | |
|----|-----------------------|
| 2. | Severity of headaches |
|----|-----------------------|
-
- | | |
|----|--------------------------|
| 3. | Amount of blurred vision |
|----|--------------------------|
-
- | | |
|----|---------------|
| 4. | Date of onset |
|----|---------------|
-

ANS: 1

Open-angle glaucoma is characterized by a gradual increase in pressure and a gradual loss of vision. Closed-angle glaucoma presents with a sudden onset causing headache, blurred vision, and eye pain.

PTS: 1 DIF: Apply REF: Glaucoma: Pathophysiology

8.A client is experiencing little flashes of lights and things floating in the visual field. The nurse suspects:

-
- | | |
|----|------------|
| 1. | cataracts. |
|----|------------|
-

-
2. glaucoma.
 3. conjunctivitis.
 4. retinal detachment.
-

ANS: 4

Retinal detachment is clinically manifested by flashes and floaters in the visual field. Flashes of light and floaters are not associated with cataracts, glaucoma, or conjunctivitis.

PTS:1DIF:Analyze

REF:Retinal Detachment: Assessment with Clinical Manifestations

9.A client tells the nurse that she sees a shadow that is slowly getting worse in her left eye. Which of the following should the nurse do?

-
1. Instruct the client to return home to rest in bed.
 2. Encourage the client to continue with normal daily activities.
 3. Notify an ophthalmologist.
 4. Encourage fluids and normal saline eyedrops.
-

ANS: 3

The nurse should notify an ophthalmologist with the client's symptoms. The onset of a shadow in the field of vision that will not dissipate is an indication of a detached retina. Retinal detachments rarely self-repair, and the client will need surgery. The nurse should not instruct the client to return home to rest in bed. The client should not be encouraged to continue with normal daily activities. Fluids and saline eyedrops will not help a detached retina.

PTS:1DIF:Apply

REF:Retinal Detachment: Assessment with Clinical Manifestations

10.A client is experiencing a loss of central vision but not a loss of peripheral vision. The nurse realizes the client should be evaluated for:

-
1. detached retina syndrome.
-

-
- | | |
|----|------------|
| 2. | nystagmus. |
|----|------------|
-
- | | |
|----|-----------------------|
| 3. | macular degeneration. |
|----|-----------------------|
-
- | | |
|----|-----------------|
| 4. | conjunctivitis. |
|----|-----------------|
-

ANS: 3

Macular degeneration is a deterioration of part of the retina, causing loss of central vision but not affecting peripheral vision. The loss of central vision is not typically seen in a detached retina, nystagmus, or conjunctivitis.

PTS: 1 DIF: Analyze REF: Macular Degeneration: Pathophysiology

11.A client is experiencing redness, burning, itching, and pain of the eyes. The nurse suspects the client is experiencing:

-
- | | |
|----|--------------|
| 1. | blepharitis. |
|----|--------------|
-
- | | |
|----|-----------------|
| 2. | conjunctivitis. |
|----|-----------------|
-
- | | |
|----|------------|
| 3. | keratitis. |
|----|------------|
-
- | | |
|----|---------|
| 4. | iritis. |
|----|---------|
-

TESTBANKSELLER.COM

ANS: 2

Clinical manifestations of conjunctivitis (pink eye) include watery eyes, redness, itching, and burning pain. Blepharitis is associated with a sticky exudate. Keratitis is associated with photophobia. Iritis is associated with blurred vision and photophobia.

PTS: 1 DIF: Analyze REF: Inflammatory and Infectious Eye Conditions

12.A client has been diagnosed as being legally blind. The nurse realizes this clients vision is:

-
- | | |
|----|---|
| 1. | 20/200 or less in the better eye with correction. |
|----|---|
-
- | | |
|----|---|
| 2. | 20/200 or less in the worse eye without correction. |
|----|---|
-
- | | |
|----|--|
| 3. | 20/100 or less in the better eye without correction. |
|----|--|
-

-
4. 20/100 or less in the worse eye with correction.

ANS: 1

Legal blindness is defined as vision of 20/200 or less on a Snellen chart in the better eye with correction. The eye needs to have correction in order to be diagnosed as legally blind; therefore, the choice of 20/200 in the worse eye without correction would be incorrect. The vision measurements of the other choices can be corrected with lenses and would not be categorized as legal blindness.

PTS: 1 DIF: Analyze REF: Low Vision and Blindness

13.The nurse realizes that the best medication treatment for open-angle glaucoma would be:

-
1. timolol (Timoptic) eyedrops.
-
2. latanoprost (Xalatan) eyedrops.
-
3. timolol (Timoptic) and Latanoprost (Xalatan) eyedrops.
-
4. metoprolol oral medication.

ANS: 3

For the best effect in the treatment of open-angle glaucoma, timolol (Timoptic) and latanoprost (Xalatan) should be prescribed together. Metoprolol is not prescribed for open-angle glaucoma.

PTS: 1 DIF: Analyze REF: Glaucoma: Pharmacology

MULTIPLE RESPONSE

1.A client tells the nurse that he does not want to develop macular degeneration like his mother. Which of the following should the nurse instruct the client as being risk factors for the development of this disorder? (Select all that apply.)

-
1. There is greater risk as people age.
-
2. Women are at greater risk than men.
-
3. African Americans are at greater risk than Caucasians.

-
4. Family history of macular degeneration increases risk.
-
5. Smoking does not increase risk.
-
6. Alcohol prevents the onset of this disorder.

ANS: 1, 2, 4

Recent statistics show that macular degeneration is age related and that women are at greater risk than men. Family history and smoking are also significant risk factors. Caucasians are at greater risk than African Americans. Alcohol does not prevent the onset of this disorder.

PTS: 1 DIF: Apply REF: Macular Degeneration

2.A client is receiving tests to diagnose glaucoma. Which of the following diagnostic tests will be used to identify this disorder in the client? (Select all that apply.)

-
1. Visual acuity
-
2. Visual field test
TESTBANKSELLER.COM
-
3. Tonometry
-
4. Weber test
-
5. Rinne test
-
6. Electroencephalogram

ANS: 1, 2, 3

Glaucoma is determined through a comprehensive eye exam including a visual acuity test, visual fields test, dilated eye exam, and tonometry. The Weber and Rinne tests are used in an ear assessment. An electroencephalogram is not used to diagnose glaucoma.

PTS:1DIF:Analyze

REF: Glaucoma: Assessment with Clinical Manifestations and Diagnostic Tests

3.A client is diagnosed with ocular cancer. The nurse realizes this client could be treated with: (Select all that apply.)

1.	Enucleation
2.	Laser surgery
3.	Plaque brachytherapy
4.	Block incision
5.	Trabeculoplasty
6.	Trabeculectomy

ANS: 1, 3, 4

Surgical options for a client diagnosed with ocular cancer include enucleation, plaque brachytherapy, or block incision. Laser surgery, trabeculoplasty, and trabeculectomy would be used to treat glaucoma.

PTS: 1 DIF: Analyze REF: Ocular Cancer: Surgery

TESTBANKSELLER.COM

4.A client, diagnosed with keratoconus, asks the nurse what caused the disorder to develop. The nurse should instruct the client on which of the following as risk factors for the development of this disorder? (Select all that apply.)

1.	Sun exposure
2.	Ocular allergies
3.	Wearing rigid contact lenses
4.	Vigorous eye rubbing
5.	Herpes simplex virus
6.	Dry eyes

ANS: 2, 3, 4

Risk factors for the development of keratoconus include ocular allergies, rigid contact lens wear, and vigorous eye rubbing. Sun exposure, herpes simplex virus, and dry eyes are not risk factors for this disorder.

PTS: 1 DIF: Apply REF: Corneal Disorders: Keratoconus

5.The nurse is planning instruction for a client experiencing dry eyes. Which of the following should be included in these instructions? (Select all that apply.)

1. Drink 8 to 10 glasses of water each day.

2. Apply petroleum jelly to the eyelids.

3. Blink more frequently.

4. Avoid sun exposure.

5. Avoid rubbing the eyes.

6. Avoid dry air. TESTBANKSELLER.COM

ANS: 1, 3, 5, 6

Interventions to improve dry eyes include drink 8 to 10 glasses of water each day; blink more frequently; avoid rubbing the eyes; and know that dry air makes the condition worse. Petroleum jelly is not a treatment for dry eyes. Avoiding the sun is good advice; however, it is not proven to help with dry eyes.

PTS: 1 DIF: Apply REF: Patient Playbook: Treatment of Dry Eyes

6.Which of the following should the nurse instruct a client diagnosed with type 2 diabetes mellitus regarding vision care? (Select all that apply.)

1. Maintain good glucose control.

2. Stop smoking.

3. Limit exercise.

4. Reduce reading.

5. Frequently rest the eyes.

6. Rub eyes daily.

ANS: 1, 2

To preserve vision and reduce the onset of diabetic retinopathy, the nurse should instruct the client to control blood glucose level, manage other complications, and stop smoking. The client should not be instructed to limit exercise, reduce reading, rest the eyes, or rub the eyes to prevent the onset of diabetic retinopathy.

Chapter 49 Nursing Management: Patients with Hearing and Balance Disorders

MULTIPLE CHOICE

1. A client is not able to successfully pass the whisper test. Which of the following would be indicated for this client?

1. Head CT scan

2. Audiometry

3. MRI of the brain

4. Electroencephalogram

ANS: 2

Failure to pass the whisper test would indicate the need for formal audiometry testing. The client would not need a head CT or MRI at this time. An electroencephalogram is not necessary.

PTS:1DIF:Analyze

REF:Auditory Dysfunction: Assessment with Clinical Manifestations

2. A client is prescribed a medication that is ototoxic. The nurse realizes that this medication may cause:

-
1. permanent or temporary vision loss.

 2. permanent or temporary hearing loss.

 3. nausea and vomiting.

 4. central nervous system (CNS) depression.

ANS: 2

Although many drugs cause nausea and vomiting and central nervous system (CNS) depression, ototoxic drugs cause hearing loss and the risks must be considered prior to suggesting these types of medications.

PTS:1DIF:Analyze

REF: Auditory Dysfunction: Ototoxic Medications and Auditory Dysfunction

3.The nurse is trying to communicate with a hearing-impaired client. The best way to do this is to:

-
1. write down all of the message.

 2. shout in the impaired ear.

 3. speak slowly and clearly while facing the client.

 4. talk in a regular voice in the good ear.

ANS: 3

When trying to communicate with the hearing-impaired client, the nurse should speak slowly and clearly while facing the client to give her the opportunity to see and hear the words being spoken. The nurse should not write down all of the messages. Shouting in the impaired ear will not improve the clients hearing. Talking in a regular voice into the good ear will not improve hearing.

PTS:1DIF:Apply

REF:Nursing Strategy: Communicating with the Hearing Impaired

4.A client is diagnosed with a conductive hearing loss. The nurse realizes type of hearing loss is *not* associated with:

-
- | | |
|----|----------|
| 1. | cerumen. |
|----|----------|
-
- | | |
|----|---------------|
| 2. | brain damage. |
|----|---------------|
-
- | | |
|----|---------------|
| 3. | otitis media. |
|----|---------------|
-
- | | |
|----|---------------|
| 4. | otosclerosis. |
|----|---------------|
-

ANS: 2

Conductive hearing loss results in a blockage of sound waves in the external or middle portions of the ear. Wax (cerumen) buildup and infections are a large part of conductive hearing loss. Otosclerosis is associated with conductive hearing loss. Brain damage is not a cause of conductive hearing loss.

PTS: 1 DIF: Analyze REF: Conductive Hearing Loss

5.A client is complaining of dizziness, unilateral ringing in the ear, feeling of pressure or fullness in the ear, and unilateral hearing loss. The nurse would suspect the client is experiencing:

-
- | | |
|----|-----------------|
| 1. | Mnires disease. |
|----|-----------------|
-
- | | |
|----|-----------------|
| 2. | osteosclerosis. |
|----|-----------------|
-
- | | |
|----|---------------|
| 3. | otitis media. |
|----|---------------|
-
- | | |
|----|--------------|
| 4. | mastoiditis. |
|----|--------------|
-

ANS: 1

All of the clients complaints are signs and symptoms of Mnires disease. Although hearing disorders may have similar signs and symptoms, they do not include all of them.

PTS:1DIF:AnalyzeREF:Menieres Disease

6.A client complains of a slight itching, slight pain, and a scratching sound in the ear. The nurse suspects that an insect may have entered the ear. Which of the following should *not* be done?

-
- | | |
|----|------------------------------------|
| 1. | Add water to flush out the insect. |
|----|------------------------------------|
-
- | | |
|----|-------------------------------------|
| 2. | Add mineral oil to kill the insect. |
|----|-------------------------------------|
-

3. Add lidocaine to kill the insect.

4. Call an otologist for a referral.

ANS: 1

Avoid placing water in the ear canal, which will only make the insect swell, thereby making it more difficult to remove. An otologist should be called for the removal. The audiologist may prescribe mineral oil or lidocaine to be applied to the ear canal.

PTS:1DIF:ApplyREF:Foreign Body

7.The hearing of an unresponsive client needs to be assessed. Which of the following will be used to assess the hearing of this client?

1. Audiometer

2. Brainstem auditory evoked responses (BAER) test

3. Rinne test

TESTBANKSELLER.COM

4. Weber test

ANS: 2

The BAER test calculates the ability to hear in a client who is unresponsive. The BAER measures the sound impulse needed to evoke a brain response, which will indicate the clients ability to hear. The other tests need the cooperation of the client and cannot be done at this time.

PTS: 1 DIF: Apply REF: Brainstem Auditory Evoked Response Test

8.The nurse is planning to assess a client diagnosed with conductive hearing loss. When performing the Weber test, the nurse would expect which of the following findings?

1. The sound will be louder in the affected ear.

2. The sound will be louder in the good ear.

3. Air conduction is shorter than bone conduction.

-
4. No sounds will be heard.

ANS: 1

During a Weber test, which tests bone conduction, a client with a conductive hearing loss hears louder sounds on the affected side. Hearing louder sounds on the unaffected side is sensorineural loss. The Rinne test compares bone with air conduction. The client will hear sounds louder in the affected ear.

PTS: 1 DIF: Apply REF: Rinne and Weber Tests

9.The nurse is performing postoperative teaching with a client recovering from a stapedectomy. Which of the following instructions would the nurse want to include in the teaching?

-
1. It is okay to resume exercise the next day.
-
2. It is okay to resume work the same day.
-
3. It is okay to shower and shampoo the next day.
-
4. It is okay to blow the nose gently one side at a time.

ANS: 4

Care must be taken not to disturb the ossicles from their position, so exercise and work should not be resumed until healing is complete. It is also important to keep the ear dry. The client should be taught to blow the nose gently on one side at a time so as not to increase the pressure in the ear.

PTS: 1 DIF: Apply REF: Repair of Inner Ear Disorders

10.After a mastoidectomy, the most important complication for the nurse to assess for is:

-
1. vomiting.
-
2. headache.
-
3. fever.
-
4. stiff neck.

ANS: 3

All are complications that can occur following this type of surgery. Fever is of extra importance because of its possible link to infection. The mastoid bone is in direct contact with the brain, and therefore any infection can travel to the brain.

PTS: 1 DIF: Analyze REF: Repair of Inner Ear Disorders

11. When instructing a client on cleaning the ear, the nurse should instruct the client to clean:

-
1. only the outer ear.

 2. all the way to the middle ear.

 3. all parts of the ear outer, middle, and inner ear.

 4. just the tympanic membrane.

ANS: 1

Only the outer portion of the ear should be cleaned. Inserting different objects into the ear canal may result in injury and damage. [TESTBANKSELLER.COM](https://www.testbankseller.com)

PTS: 1 DIF: Apply REF: Planning and Implementation

12. Which of the following would prohibit an elderly client from wanting to obtain and use a hearing aid?

-
1. Fears sounds will be too loud

 2. Thinks not necessary for a temporary problem

 3. Fears the cost

 4. Prefers silence

ANS: 3

Some of the problems encountered by clients obtaining hearing aids include appearance, cost, education, unrealistic expectations, and difficulty with the care and maintenance of the hearing aids. The other choices are not problems encountered by clients obtaining hearing aids.

PTS: 1 DIF: Analyze REF: Hearing Aids

13. Which of the following should the nurse instruct a client who is being fitted for a hearing aid?

1. Keep the appliance turned on at all times.
2. Store the hearing aid in a warm, moist place.
3. Batteries last for at least 1 month.
4. Clean ear molds at least once a week.

ANS: 4

The nurse should instruct the client to turn off the appliance when not in use; store in a cool, dry place; change the batteries at least once per week; and clean ear molds at least once per week.

PTS: 1 DIF: Apply REF: Patient Playbook: Care of Hearing Aids

MULTIPLE RESPONSE

1. The nurse is instructing a client diagnosed with otitis media on management during the acute phase. Which of the following should the nurse include in the teaching? (Select all that apply.)

TESTBANKSELLER.COM

1. Take the antibiotics as ordered.
2. Take over-the-counter analgesics for mild pain as recommended.
3. It is okay to go swimming.
4. It is okay to go on vacation and trips that require flying.
5. If excruciating pain develops, seek medical care.
6. Limit fluids.

ANS: 1, 2, 5

Clients must complete the medication as ordered to kill the infection. Mild analgesics for pain are often needed. If excruciating ear pain develops, the client should seek medical care to rule out perforation of the eardrum. It is important to keep the ear dry, so the client should not swim at this time. Flying is not recommended at this time. Limiting fluids is not necessary with otitis media.

PTS: 1 DIF: Apply REF: Otitis Media

2. When caring for a client with total hearing loss, the nurse is instructing the client about the many options that are available to function in a hearing world. Which of the following should the nurse include? (Select all that apply.)

- | | |
|----|-------------------------------|
| 1. | Flashing lights for alarms |
| 2. | TV with closed captions |
| 3. | Talking computer |
| 4. | Lip reading and sign language |
| 5. | Cell phones with headsets |
| 6. | Loud ringers on telephones |

ANS: 1, 2, 4

TESTBANKSELLER.COM

Patients who have no hearing have access to various mechanisms to alert them to various sounds. Flashing lights for alarms to phones and doorbells, TV with closed captions for the hearing impaired, and classes in lip reading and sign language are some options. Talking computers and cell phones with headsets are advancements for the hearing, not for the hearing impaired. Loud ringers on telephones would also be helpful to the client with some hearing and not a total hearing loss.

PTS: 1 DIF: Apply REF: Communication Tools

3. A client is diagnosed with a congenital hearing loss. Which causes does the nurse realize are reasons for this type of hearing loss? (Select all that apply.)

- | | |
|----|----------------------|
| 1. | Genetics |
| 2. | Natal infections |
| 3. | Physical deformities |
| 4. | Noise levels |

-
- | | |
|----|-------------------------|
| 5. | Maternal ototoxic drugs |
|----|-------------------------|
-
- | | |
|----|---------------------------|
| 6. | Maternal TORCH infections |
|----|---------------------------|
-

ANS: 1, 2, 3, 5, 6

Congenital hearing loss can be derived from genetics, natal infections, or physical deformities of the ear in addition to maternal ototoxic drug use and maternal TORCH infections that include toxoplasmosis, rubella, cytomegalovirus, and herpes virus type 2. Noise levels do not cause a congenital hearing loss.

PTS: 1 DIF: Analyze REF: Auditory Dysfunction: Genetics

4.A client with a family history of hearing loss asks the nurse what he can do to prevent this disorder as he ages. Which of the following should the nurse instruct this client? (Select all that apply.)

-
- | | |
|----|--|
| 1. | Turn down radio and television volume. |
|----|--|
-
- | | |
|----|--|
| 2. | Avoid noisy areas such as rock concerts. |
|----|--|
-
- | | |
|----|--------------------------|
| 3. | Wear protective devices. |
|----|--------------------------|
-
- | | |
|----|-------------------------------------|
| 4. | Use plain cotton balls in the ears. |
|----|-------------------------------------|
-
- | | |
|----|---------------------|
| 5. | Avoid sun exposure. |
|----|---------------------|
-
- | | |
|----|--|
| 6. | Flush the ears daily with mineral oil. |
|----|--|
-

ANS: 1, 2, 3

Measures to prevent hearing loss include turning down the volume on the radio and television, avoiding noisy areas such as rock concerts, and wearing protective devices. Using cotton balls in the ears does not decrease noise from reaching the middle ear. Sun exposure does not impact hearing. Flushing the ears daily with mineral oil might decrease the buildup of cerumen; however, it will not improve hearing.

PTS:1DIF:ApplyREF:Primary Prevention

5.Which of the following are indications that a client has been exposed to excessive noise? (Select all that apply.)

-
1. Raising the voice to talk in normal conversation
 2. Clear drainage from the ears
 3. Inability to hear a conversation 2 feet away
 4. Sounds are muffled
 5. Ringing of the ears
 6. Short periods of pain in the ears
-

ANS: 1, 3, 4, 5, 6

Warning signs of excessive noise exposure include raising the voice to talk in normal conversation, inability to hear a conversation 2 feet away, muffled sounds, ear ringing, and short periods of ear pain. Clear drainage from the ears does not occur with excessive noise exposure.

TESTBANKSELLER.COM

Chapter 50 Nursing Assessment: Integumentary Function

MULTIPLE CHOICE

1. A client has sustained an injury to the skin that extends into the innermost layer, which is called the:

-
1. stratum germinativum.
 2. stratum granulosum.
 3. stratum lucidum.
 4. stratum spinosum.
-

ANS: 1

The epidermis is composed of five layers of stratified squamous epithelial cells named (outer to inner): the stratum corneum, stratum lucidum, stratum granulosum, stratum spinosum, and stratum germinativum.

PTS: 1 REF: Epidermis

2.A client has a yellow tone to the skin. The nurse realizes that which of the following cells are responsible for the yellow tone of the skin?

1.	Carotenoids
2.	Langerhans cells
3.	Melanocytes
4.	Merkel cells

ANS: 1

Carotenoids are responsible for the yellow tone of the skin in some individuals. Langerhans cells are responsible for foreign antigen recognition, and they provide immune protection. Melanocytes are responsible for skin pigmentation. Merkel cells are thought to be involved in sensing touch and pressure.

PTS: 1 DIF: Analyze REF: Cells in the Epidermis

TESTBANKSELLER.COM

3.A client has a health condition that affects her ability to sweat. The nurse realizes that the sweat glands are epidermal appendages known as:

1.	apocrine glands.
2.	eccrine glands.
3.	hydriatric glands.
4.	sebaceous glands.

ANS: 2

The eccrine glands are sweat-producing glands that play a role in thermoregulation. The apocrine glands produce odorless and milky secretions. Sebaceous glands produce sebum, which functions as a waterproofing layer. Hydriatric is a term meaning treatment with water.

PTS: 1 DIF: Analyze REF: Table 44-1 Structures and Functions of the Skin

4.A client sustained an injury to the crescent-shaped area at the proximal end of the nail of one finger or the:

-
- | | |
|----|---------|
| 1. | lunula. |
|----|---------|
-
- | | |
|----|--------------|
| 2. | nail matrix. |
|----|--------------|
-
- | | |
|----|-------------|
| 3. | paronychia. |
|----|-------------|
-
- | | |
|----|-------------------|
| 4. | periungual tissue |
|----|-------------------|

ANS: 1

The lunula is the pale, crescent-shaped area at the proximal end of each nail. The periungual tissue surrounds the nail plate and the free edge of the nail. Paronychia is inflammation at the base of the nail plate. The nail matrix is undifferentiated epithelial tissue from which the nails arise.

PTS: 1 DIF: Analyze REF: Epidermal Appendages

5.The nurse is assessing the skin of an elderly client. Which of the following would *not* be assessed in this client?

-
- | | |
|----|----------------------|
| 1. | Decreased elasticity |
|----|----------------------|
-
- | | |
|----|--------------------------|
| 2. | Increased skin hydration |
|----|--------------------------|
-
- | | |
|----|--------------------|
| 3. | Slow wound healing |
|----|--------------------|
-
- | | |
|----|---------------|
| 4. | Thinning skin |
|----|---------------|

ANS: 2

As a person ages, there is a reduction in the density of sweat glands, sebaceous glands, and blood vessels, which results in a decrease in skin hydration. As a person ages, skin loses elasticity. Reduced blood vessels lead to slower wound healing. Reduction in subcutaneous tissue causes the skin to thin.

PTS: 1 DIF: Analyze REF: Cellular Effects of Aging on the Skin

6.A client is experiencing elevated fluid-filled lesions on the skin. The nurse would document these lesions as being:

-
- | | |
|----|----------|
| 1. | macules. |
|----|----------|

2. nodules.

3. vesicles.

4. wheals.

ANS: 3

Vesicles are elevated, fluid-containing lesions. Macules are flat, circumscribed changes of the skin. Nodules are elevated, solid lesions. Wheals are solid elevations formed by local, superficial, transient edema, usually in response to a pruritic condition.

PTS: 1 DIF: Analyze REF: Box 44-1 Primary Lesions

7.A client is experiencing a change in skin status. During the assessment, the nurse asks about any changes in laundry products as part of which of the following systems review?

1. Environmental changes

2. Nutritional patterns

TESTBANKSELLER.COM

3. Activities and exercise patterns

4. Cultural influences

ANS: 1

When assessing environmental changes during the assessment of the skin, the nurse would assess for any changes in laundry products. Nutritional patterns would be assessed by changes in diet or food supplements. Activities and exercise patterns would be assessed by any use of protective skin preparations. Cultural influences would be assessed by factors that may influence the choice of treatment options.

PTS:1DIF:Apply

REF: Table 44-2 Review of Systems Related to Skin Disorders

8.A client is recovering from burns located on both arms, anterior chest, and both legs. The nurse realizes that this client may need which of the following dietary supplements?

1. Vitamin B

2.	Vitamin C
----	-----------

3.	Vitamin D
----	-----------

4.	Vitamin E
----	-----------

ANS: 3

The skin synthesizes vitamin D by using ultraviolet light to convert 7-dehydrocholesterol in the epidermis. In the event of a large burn, the client may need vitamin D supplementation. The other vitamins are essential to normal body functioning; however, they may not be affected because of the burn.

PTS: 1 DIF: Analyze REF: Synthesis

9.The nurse, assessing a clients skin as being overly dry and scaly, would document this finding as being:

1.	poor turgor.
----	--------------

2.	ichthyosis.
----	-------------

3.	edematous.
----	------------

4.	anasarca.
----	-----------

ANS: 2

Ichthyosis is dry, scaly skin. Poor turgor is the term used if the skin shows evidence of poor hydration. Edematous is the term used if there is an abnormal accumulation of fluid under the skin. Anasarca is a term used for generalized edema.

PTS: 1 DIF: Apply REF: Assessment of the Skin

10.A client has a nonpalpable skin lesion that is causing a change in skin color greater than 1 cm in diameter. The nurse would document this finding as being a(n):

1.	patch.
----	--------

2.	macule.
----	---------

3.	wheel.
----	--------

4.	vesicle.
----	----------

ANS: 1

A patch is a localized change in skin color of greater than 1 cm in diameter. A macule is a localized change in skin color of less than 1 cm in diameter. A wheal is localized edema in the epidermis causing irregular elevation that may be red or pale. A vesicle is an accumulation of fluid between the upper layers of the skin.

PTS: 1 DIF: Apply REF: Figure 44-6 Morphology of Primary Lesions

11.A client has an enlarged scar as a result of abdominal surgery. The nurse realizes this scar would be considered a(n):

1.	erosion.
----	----------

2.	fissure.
----	----------

3.	excoration.
----	-------------

4.	keloid.
----	---------

ANS: 4

A keloid is an enlarged scar that extends beyond the wound edges. An erosion is caused by a loss of epidermis. A fissure is a linear crack in the epidermis. An excoriation is the loss of epidermal layers of skin, exposing the dermis.

PTS: 1 DIF: Analyze REF: Figure 44-7 Morphology of Secondary Lesions

12.A client has several patches of horny, thickened skin on the palmar surface of the hands and feet. The nurse would document this finding as being:

1.	keratosis.
----	------------

2.	linear.
----	---------

3.	serpiginous.
----	--------------

-
4. dermatomal.

ANS: 1

Keratosis are patches of horny, thickened skin associated with callus formation. Linear lesions have a band-like configuration. Serpiginous lesions are serpent shaped. Dermal lesions occur in the location of the dermatome supplied by one or more dorsal ganglia.

PTS:1DIF:Apply

REF: Box 44-2 Description and Configuration of Selected Group of Lesions

13.A school age child is diagnosed with many individual and separate skin lesions. The nurse realizes this client is experiencing the result of:

-
1. measles.

-
2. poison ivy.

-
3. herpes zoster.

-
4. TESTBANKSELLER.COM
insect bites.

ANS: 4

Insect bites create individual and separate lesions. Measles lesions are generalized and scattered all over the body. Poison ivy causes a linear lesion. Herpes zoster creates a lesion that is linear along a nerve root.

PTS: 1 DIF: Analyze REF: Figure 44-5 Arrangement of Lesions

MULTIPLE RESPONSE

1.The nurse is assessing the skin status of a client. Which of the following will be included in this assessment? (Select all that apply.)

-
1. Personal perception of the skin problem

-
2. Nutritional pattern

-
3. Elimination pattern

- | | |
|----|-------------------|
| 4. | Self-concept |
| 5. | Self-image |
| 6. | Employment status |

ANS: 1, 2, 3, 4, 5

The assessment of the skin should include personal perception of the skin problem, nutritional pattern, elimination pattern, self-concept, and self-image. Employment status is not a part of this assessment.

PTS:1DIF:Apply

REF: Table 44-2 Review of Systems Related to Skin Disorders

2.The nurse determines that a client has skin changes consistent with sun exposure. Which of the following did the nurse assess in this client? (Select all that apply.)

- | | |
|----|-------------------|
| 1. | Age spots |
| 2. | Actinic keratoses |
| 3. | Telangiectasias |
| 4. | Lentigines |
| 5. | Freckles |
| 6. | Burrows |

ANS: 1, 2, 3, 4, 5

Skin changes consistent with sun exposure include age spots, actinic keratoses, telangiectasias, lentigines, and freckles. Burrows are lesions seen with animal parasites.

PTS: 1 DIF: Analyze REF: Skin Changes Throughout the Life Span

3.The nurse is assessing a client for primary skin lesions. Which of the following would be considered primary lesions of the skin? (Select all that apply.)

1.	Crust
2.	Scales
3.	Tumors
4.	Nodules
5.	Macules
6.	Plaques

ANS: 3, 4, 5, 6

Primary skin lesions include tumors, nodules, macules, and plaques. Secondary skin lesions include crust and scales.

PTS:1DIF:Analyze

REF: Box 44-1 Primary Lesions; Figure 44-7 Morphology of secondary lesions

4.The nurse is describing the distribution and configuration of lesions. Which of the following can be used for this description? (Select all that apply.)

1.	Iris
2.	Annular
3.	Linear
4.	Keratosis
5.	Wheal
6.	Bullae

ANS: 1, 2, 3, 4

When describing the distribution and configuration of lesions, the terms iris, annular, linear, and keratosis can be used. Wheal and bullae describe primary lesions.

PTS:1DIF:Apply

REF: Box 44-2 Description and Configuration of Selected Group of Lesions

5.The nurse assesses a linear lesion along the length of a clients leg. Which diagnosis does the nurse realize is associated with linear lesions? (Select all that apply.)

1.	Drug reaction
2.	Herpes zoster
3.	Herpes simplex
4.	Hookworm
5.	Dermatitis
6.	Poison ivy

ANS: 4, 5, 6

Linear lesions are associated with poison ivy, dermatitis, or hookworm. Polycyclic lesions are associated with drug reactions. Linear lesions along a nerve root are associated with herpes zoster. Grouped or clustered lesions are associated with herpes simplex.

Chapter 51 Nursing Management: Patients with Dermatologic Problems

MULTIPLE CHOICE

1.A clients wound is being debrided by letting a wet-to-dry dressing that is packed into the wound dry. This type of debridement is called:

1	autolytic debridement.
2	enzymatic debridement.

3	mechanical debridement.
---	-------------------------

4	sharp debridement.
---	--------------------

ANS: 3

Mechanical debridement makes use of gauze dressing to remove necrotic or devitalized tissue from wounds. Autolytic debridement makes use of the normal phagocytic action of the macrophages and leukocytes present in the wound. Enzymatic debridement is accomplished by using a chemical debriding agent. Sharp debridement is cutting away necrotic tissue from the wound.

PTS:1DIF:AnalyzeREF:Debridement

2.A client is experiencing a circular lesion with an advancing, red, scaly border on the abdomen. The nurse recognizes this lesion as being:

1	tinea capitis.
---	----------------

2	tinea corporis.
---	-----------------

TESTBANKSELLER.COM

3	tinea cruris.
---	---------------

4	tinea pedis.
---	--------------

ANS: 2

Tinea corporis is a fungal infection that involves the face, trunk, and limbs. Tinea pedis is a common infection of the feet. Tinea cruris occurs in the groin and inner thigh, and tinea capitis involves the scalp.

PTS:1DIF:AnalyzeREF:Fungal Infections

3.A client is complaining of pain and drainage coming from an area on his back. The nurse assesses the area and finds a large erythematous, swollen mass with multiple areas of purulent drainage. The nurse suspects the client has a(n):

1	abscess.
---	----------

2	carbuncle.
---	------------

3	furuncle.
---	-----------

4	papule.
---	---------

ANS: 2

Carbuncles are an aggregate of infected follicles originating deep in the dermis and subcutaneous tissue. Carbuncles are many furuncles, and they form an erythematous, swollen, broad, and slowly evolving mass that can ulcerate and drain from multiple openings. A furuncle is a single localized induration that is painful. An abscess is a cavity containing pus, and a papule is a small, raised lesion.

PTS: 1 DIF: Analyze REF: Furuncles and Carbuncles

4.A client has what appears to be a bacterial infection or warts on her fingertips. This can be a sign of:

1	herpes gladiatorum.
---	---------------------

2	herpes simplex.
---	-----------------

TESTBANKSELLER.COM

3	herpes zoster.
---	----------------

4	herpetic whitlow.
---	-------------------

ANS: 4

Herpetic whitlow usually occurs on the fingertips and can resemble a bacterial infection or warts. Herpes gladiatorum is most frequently found in athletes who participate in contact sports. The appearance of herpes zoster is usually down a single dermatome. Herpes simplex is usually seen orally or on the genitals.

PTS: 1 DIF: Analyze REF: Cutaneous Herpes Simplex

5.A school-age child is experiencing pruritic vesicles around the mouth. The lesions have a honey-colored crust. The nurse realizes that the child is most likely experiencing:

1	candidiasis.
---	--------------

2	herpes simplex.
---	-----------------

3	impetigo.
---	-----------

4	tinea corporis.
---	-----------------

ANS: 3

Impetigo is a common, superficial skin infection beginning as a focal erythema and progressing to pruritic vesicles, erosions, and honey-colored crusts. Oral herpes simplex would look like a cold sore. Tinea corporis has a circular, red, scaly border, and candidiasis is a proliferation of the normal yeast flora.

PTS: 1 DIF: Analyze REF: Impetigo

6.A client is being treated for lice. Which of the following medications would the nurse expect to see prescribed for this client?

1	Acyclovir
---	-----------

2	Diphenhydramine
---	-----------------

3	Mupirocin
---	-----------

4	Permethrin
---	------------

ANS: 4

Permethrin is applied for treatment of head lice. Diphenhydramine controls the itching but does not treat the infestation. Mupirocin treats impetigo, and acyclovir is for herpes simplex virus.

PTS: 1 DIF: Apply REF: Pediculosis

7.A client is diagnosed with genital herpes simplex virus. The nurse know that symptoms of the primary infection occur:

1	1 to 4 days after exposure.
---	-----------------------------

2	3 to 7 days after exposure.
---	-----------------------------

3	5 to 9 days after exposure.
---	-----------------------------

4 7 to 11 days after exposure.

ANS: 2

Symptoms of the primary herpes simplex infection occur 3 to 7 days after exposure. The other choices do not describe the length of time before symptoms of the primary herpes simplex infection occur.

PTS:1DIF:AnalyzeREF:Herpes Infections

8.A client is demonstrating patches of thick, red skin with silvery scales on the elbows and knees. The nurse suspects that this client is experiencing:

1 psoriasis.

2 rosacea.

3 scabies.

4 stasis dermatitis.

TESTBANKSELLER.COM

ANS: 1

Psoriasis is characterized by patches of thick, red skin with silvery scales, usually on the scalp, elbows, knees, and lumbosacral areas. Rosacea is a chronic, inflammatory condition characterized by erythema, papules, pustules, and telangiectasis. Scabies is a highly contagious, pruritic skin infection caused by a mite. Stasis dermatitis is a condition that occurs on the lower extremities of patients with venous insufficiency.

PTS: 1 DIF: Analyze REF: Psoriasis

9.A middle-aged construction worker has a raised lesion with a pearly border on his arm that bleeds easily. The nurse realizes that this client most likely is experiencing a(n):

1 actinic keratosis.

2 basal cell carcinoma.

3 malignant melanoma.

4 melanoma in situ.

ANS: 2

Basal cell carcinoma in its nodular form appears as a pearly, translucent bump that bleeds easily. Actinic keratosis is seen or palpated on the face, scalp, arms, and ears. It can have a color from tan to red or have the patient's normal skin tone. Malignant melanoma is a lesion that has changed its color and shape, has gotten bigger, or has an irregular border. Melanoma in situ presents with flat or raised lesions with histologic features of melanoma.

PTS: 1 DIF: Analyze REF: Nonmelanoma Skin Cancers

10. A client periodically experiences pseudofolliculitis barbae. Which of the following should the nurse instruct this client?

-
- | | |
|---|----------------------|
| 1 | Avoid close shaving. |
|---|----------------------|
-
- | | |
|---|--|
| 2 | Avoid washing the hair prior to shaving. |
|---|--|
-
- | | |
|---|-----------------------------|
| 3 | Apply a topical antibiotic. |
|---|-----------------------------|
-
- | | |
|---|---|
| 4 | Contact the physician since the client needs a prescription for an oral antibiotic. |
|---|---|
-

ANS: 1

TESTBANKSELLER.COM

Pseudofolliculitis barbae is a foreign body reaction to hair in individuals with a genetic inclination for curly, spiral-shaped hair. Prevention requires the client to avoid close shaving. This is what the nurse should instruct this client. This client is to wash the hair prior to shaving. This condition does not need an oral or topical antibiotic.

PTS: 1 DIF: Apply REF: Folliculitis

11. A client is diagnosed with tinea versicolor. Which of the following should the nurse instruct this client regarding the care for this skin condition?

-
- | | |
|---|---|
| 1 | Do nothing since there is no treatment. |
|---|---|
-
- | | |
|---|--------------------------------|
| 2 | Utilize shampoo with selenium. |
|---|--------------------------------|
-
- | | |
|---|---|
| 3 | Utilize an oral antifungal preparation as prescribed. |
|---|---|
-
- | | |
|---|--|
| 4 | Apply warm compresses to the affected areas. |
|---|--|
-

ANS: 2

Treatment for tinea versicolor includes the use of selenium shampoo. The nurse should not instruct the client to do nothing since treatment does exist for this condition. Oral antifungal preparations are not necessary for this condition. Warm compresses will not help this condition.

PTS:1DIF:ApplyREF:Fungal Infections

12. Which of the following should the nurse instruct a client who is prescribed a topical medication for a skin condition?

-
- | | |
|---|---|
| 1 | Apply directly to broken or irritated skin. |
|---|---|
-
- | | |
|---|-----------------------|
| 2 | Apply before bathing. |
|---|-----------------------|
-
- | | |
|---|----------------------|
| 3 | Apply after bathing. |
|---|----------------------|
-
- | | |
|---|--|
| 4 | Cover the area with an occlusive dressing. |
|---|--|

ANS: 3

The client should be instructed to apply the medication to the skin after bathing since hydration of the area will increase absorption of the medication. The medication should not be applied directly to broken or irritated skin. The medication should not be applied before bathing. The area should not be covered with an occlusive dressing.

PTS:1DIF:Apply

REF: Table 45-2 What Every Patient Needs to Know About Topical Medications

13. A client is diagnosed with a dermatologic condition causing pruritis and inflammation. Which of the following should the nurse instruct this client?

-
- | | |
|---|---|
| 1 | Use regular perfumed lotion to moisturize the skin. |
|---|---|
-
- | | |
|---|---|
| 2 | Use scented soap to bathe the skin daily. |
|---|---|
-
- | | |
|---|-----------------------|
| 3 | Apply skin oil daily. |
|---|-----------------------|
-
- | | |
|---|---|
| 4 | Apply a body moisturizer to the skin within 3 to 5 minutes after bathing. |
|---|---|

ANS: 4

Regular usage of body moisturizers, particularly within 3 to 5 minutes after bathing or showering, will aid in the prevention of dry, flaking, and itching skin. Perfumed lotions and

scented soaps contain alcohol, which will exacerbate pruritis and inflammation. Skin oil does not penetrate into the skin.

PTS: 1 DIF: Apply REF: Moisturizers and Lubricants

MULTIPLE RESPONSE

1. The nurse is assessing a client's skin for signs of normal aging. Which of the following are skin changes seen with aging? (Select all that apply.)

1	Lentigo
2	Loss of subcutaneous tissue
3	Telangiectasias
4	Thickened, wrinkled, yellowish skin
5	Thin, fragile, and inelastic skin
6	Seborrheic keratosis

ANS: 2, 5

Thin, fragile, inelastic skin that has the loss of subcutaneous tissue is the result of normal aging. Skin that has aged as a result of sun damage exhibits a thickened, wrinkled, yellow appearance. It may also have telangiectasias, lentigo, or seborrheic keratosis.

PTS: 1 DIF: Analyze REF: Table 45-3 Normal Aging versus Photoaging Skin

2. A client is diagnosed with severe nodulocystic acne. The nurse should instruct the client on which of the following types of treatments? (Select all that apply.)

1	Oral antibiotics
2	Benzoyl peroxide
3	Sulfur
4	Intralesional injections

5	Soap and water
---	----------------

6	Topical therapy
---	-----------------

ANS: 1, 4, 6

Treatment for severe nodulocystic acne includes oral antibiotics, intralesional injections, and topical therapy. Benzoyl peroxide is indicated for mild and moderate acne. Sulfur is indicated for moderate acne. Soap and water is indicated for mild acne.

PTS: 1 DIF: Apply REF: Acne

3.A client is diagnosed with cellulitis. Which of the following will the nurse most likely assess in this client? (Select all that apply.)

1	Heat
---	------

2	Redness
---	---------

3	Swelling
---	----------

TESTBANKSELLER.COM

4	Pain
---	------

5	Glossy, stretched skin appearance
---	-----------------------------------

6	Thirst
---	--------

ANS: 1, 2, 3, 4, 5

Cardinal signs of cellulitis include heat, redness, swelling, pain, and a glossy, stretched appearance of the skin. Thirst is not associated with cellulitis.

PTS: 1 DIF: Apply REF: Cellulitis

4.A client is diagnosed with rosacea. Which of the following should the nurse instruct the client regarding this condition? (Select all that apply.)

1	Avoid sunlight.
---	-----------------

2	Avoid alcohol.
---	----------------

-
- | | |
|---|-------------------|
| 3 | Avoid spicy food. |
|---|-------------------|
-
- | | |
|---|---------------------------------|
| 4 | Wash the face five times a day. |
|---|---------------------------------|
-
- | | |
|---|---------------------------------------|
| 5 | Wash the face with a clean washcloth. |
|---|---------------------------------------|
-
- | | |
|---|--|
| 6 | Apply medication to affected areas immediately after washing the face. |
|---|--|
-

ANS: 1, 2, 3

Treatment of rosacea includes avoiding sunlight, alcohol, and spicy food. The client should be instructed to not overwash the face. Washcloths should not be used. Medication should be applied to the entire face, waiting 15 to 20 minutes after washing to apply.

PTS: 1 DIF: Apply REF: Rosacea

5.A client is diagnosed with a viral skin infection. The nurse realizes that which of the following medications may be prescribed for this client? (Select all that apply.)

-
- | | |
|---|-----------------------|
| 1 | Nystatin (Mycostatin) |
|---|-----------------------|
-
- | | |
|---|--------------------|
| 2 | Docosanol (Abreva) |
|---|--------------------|
-
- | | |
|---|------------|
| 3 | Boric acid |
|---|------------|
-
- | | |
|---|-----------------------|
| 4 | Penciclovir (Denavir) |
|---|-----------------------|
-
- | | |
|---|-------------------|
| 5 | Hydrogen peroxide |
|---|-------------------|
-
- | | |
|---|---------------------|
| 6 | Acyclovir (Zovirax) |
|---|---------------------|
-

ANS: 2, 4, 6

Antiviral medications include docosanol (Abreva), penciclovir (Denavir), and acyclovir (Zovirax). Nystatin (Mycostatin) is an antifungal medication. Boric acid is an antipruritic solution. Hydrogen peroxide is an antiseptic solution.

Chapter 52 Nursing Management: Patients with Burn Injury**MULTIPLE CHOICE**

1. The nurse is assessing a client's burn for the zones of injury. Which of the following will the nurse *not* assess in the client at this time?

1. Zone of coagulation

2. Zone of eschar

3. Zone of hyperemia

4. Zone of stasis

ANS: 2

There is no zone of eschar. The zones of injury are the zone of coagulation, the zone of stasis, and the zone of hyperemia.

PTS: 1 DIF: Analyze REF: Local Tissue Response

2. Which of the following will the nurse most likely assess in a client diagnosed with a second-degree burn?

1. No pain and necrotic areas

2. No pain and scarring

3. Pain and blisters

-
4. Pain and peeling after 2 to 5 days

ANS: 3

A second-degree burn is a partial-thickness burn. This type of burn is very painful, has blisters that increase in size, blanches with pressure, and may or may not require grafting. The other characteristics are for first- or third-degree burns.

PTS:1DIF:ApplyREFathophysiology

3.The nurse, caring for a client with severe burns, realizes that the clients care will progress through specific periods of treatment EXCEPT:

-
1. acute period.
-
2. emergent period.
-
3. rehabilitation period.
-
4. stabilization period.

TESTBANKSELLER.COM

ANS: 4

The three periods of treatment in the care of the seriously burned client are the emergent period, the acute period, and the rehabilitation period. There is no stabilization period of burn care.

PTS: 1 DIF: Analyze REF: Planning and Implementation

4.A client who has experienced a burn is in the emergent phase of treatment that usually occurs during which of the following periods?

-
1. 24 to 48 hours
-
2. 36 to 72 hours
-
3. 48 to 96 hours
-
4. 1 to 7 days

ANS: 1

The emergent period is the first 24 to 48 hours after a burn. The acute phase begins after the emergent period ends and lasts until the burn is healed. The rehabilitation period begins with wound closure and continues until the patient has reached the highest level of functioning.

PTS: 1 DIF: Analyze REF: Planning and Implementation

5.A client is beginning the initial treatment of a major burn in the emergency room. Which of the following interventions would *not* be completed?

1. Inserting an indwelling urinary catheter
2. Intubating the patient
3. Giving oral medications for pain management
4. Starting an intravenous solution of Ringers lactate

ANS: 3

Large burns cause decreased peristalsis, and therefore nothing should be given by mouth. Clients with large burns may vomit, and attention is needed to prevent them from aspirating vomitus. Prophylactic intubation may be initiated if any heat or smoke has been inhaled. Fluid replacement and urine output are necessary for the treatment of the burn client.

PTS: 1 DIF: Apply REF: Emergency Department Management

6.The formula used to calculate the volume of intravenous (IV) fluid required for fluid resuscitation of a client receiving care in the first 24 hours after a burn is:

1. 1 to 2 mL of lactated Ringers solution body weight percent burn.
2. 2 to 3 mL of lactated Ringers solution body weight percent burn.
3. 2 to 4 mL of lactated Ringers solution body weight percent burn.
4. 3 to 6 mL of lactated Ringers solution body weight percent burn.

ANS: 3

The formula used to calculate the volume of IV fluid required for fluid resuscitation is based on the Parkland formula. Using this formula, the clients fluid requirements for the first 24 hours after injury are estimated. For adults, the formula is 2 to 4 mL of lactated Ringers solution body weight (in kg) percent burn. The other amounts of fluid are incorrect for the adult client.

PTS: 1 DIF: Apply REF: Planning and Implementation

7.A client is being evaluated in the emergency department following a burn injury at home. The client has second- and third-degree burns to the right and left arms, back, and both posterior legs. Using the rule of nines, the nurse would calculate this clients burn as being:

- | | |
|----|------|
| 1. | 36%. |
| 2. | 45%. |
| 3. | 54%. |
| 4. | 63%. |

ANS: 3

The right and left arms are 18%, the back is 18%, and the posterior legs are 18%; this equals 54%. The other calculations are incorrect.

PTS: 1 DIF: Apply REF: Figure 46-3 Rule of Nines

8.Health care professionals are required to report suspected abuse or neglect. Which of the following is not a typical sign of abuse with a burn injury?

- | | |
|----|--|
| 1. | Emergency management notification of a burn injury within 1 hour of occurrence |
| 2. | A burn injury accompanied by fracture and bruises |
| 3. | Differing accounts of how the injury occurred with each new interview |
| 4. | Treatment sought by a non-relation |

ANS: 1

Notifying emergency management of a burn injury within 1 hour of occurrence is not a typical sign of abuse. The other options are typical signs of abuse.

PTS: 1 DIF: Analyze REF: Law in Practice: Burns as a Result of Abuse

9.The nurse is assessing a client diagnosed with second- and third-degree burns. Which of the following assessment signs would *not* need to be reported by the nurse?

-
- | | |
|----|--------------|
| 1. | Brassy cough |
|----|--------------|
-
- | | |
|----|------------|
| 2. | Hoarseness |
|----|------------|
-
- | | |
|----|------------------------|
| 3. | Respiratory rate of 36 |
|----|------------------------|
-
- | | |
|----|---|
| 4. | Urine output of 30 mL in the first hour |
|----|---|
-

ANS: 4

A urine output of 30 to 50 mL per hour is a sign of adequate fluid hydration. A brassy cough, hoarseness, or an increasing respiratory rate can be signs of potential airway obstruction and respiratory distress.

PTS: 1 DIF: Apply REF: Planning and Implementation

10.A nurse is managing the fluid status of a client being treated for a burn. Which of following is an indicator of adequate fluid resuscitation?

-
- | | |
|----|---------------------------|
| 1. | Blood pressure 95/60 mmHg |
|----|---------------------------|
-
- | | |
|----|---------------|
| 2. | Pulse 115 bpm |
|----|---------------|
-
- | | |
|----|-------------------|
| 3. | Patient confusion |
|----|-------------------|
-
- | | |
|----|-----------------------|
| 4. | Urine output 30 mL/hr |
|----|-----------------------|
-

ANS: 4

Fluid should be titrated to ensure a urine output of 30 to 50 mL/hr. The other assessment values are not indicators of adequate fluid hydration.

PTS: 1 DIF: Analyze REF: Planning and Implementation

11.The nurse is initiating care for a client diagnosed with burns to the chest, back, neck, and face. For this client, which of the following nursing diagnoses would receive the highest priority?

-
- | | |
|----|----------------------|
| 1. | Disturbed body image |
|----|----------------------|
-
- | | |
|----|-------------------------|
| 2. | Impaired skin integrity |
|----|-------------------------|
-

3. Ineffective airway clearance

4. Risk for infection

ANS: 3

The highest priority would be to maintain the airway with adequate oxygenation and ventilation. The other nursing diagnoses would not be the first priority.

PTS: 1 DIF: Apply REF: Planning and Implementation

12.A client is scheduled to receive a skin graft from another species as part of the treatment for a burn wound. Which of the following is a graft of skin obtained from another species?

1. Allograft

2. Autograft

3. Heterograft

4. TESTBANKSELLER.COM
Homograft

ANS: 3

A heterograft (xenograft) is a graft of skin obtained from another species, such as a pig. An autograft is a permanent graft. A homograft, or cryopreserved cadaveric allograft, is a graft of skin obtained from a cadaver 6 to 24 hours after death that is used as a temporary graft. An allograft is a graft of skin from someone of the same species.

PTS:1DIF:AnalyzeREF:Acute Phase: Surgery

13.A client is recovering from a skin graft to her right arm. Which of the following nursing interventions would *not* be indicated for this client?

1. Assess for bleeding.

2. Assess for drainage underneath the graft site.

3. Encourage exercise of the right arm.

4. Remove dressings slowly.

ANS: 3

Exercise of the site may cause dislodgement of the skin graft. The graft should be examined every 24 hours because drainage or blood can accumulate under the graft and cause nonadherence. Dressings should be removed slowly and carefully so that the graft is not disturbed.

PTS:1DIF:ApplyREF:Acute Phase: Surgery

14.A client diagnosed with a major burn is being prescribed medication for pain. The nurse realizes that the drug of choice for this client will be:

- | | |
|----|-------------------|
| 1. | morphine sulfate. |
| 2. | acetaminophen. |
| 3. | aspirin. |
| 4. | meperidine. |

ANS: 1

TESTBANKSELLER.COM

Morphine sulfate is the drug of choice for pain relief for the client experiencing a burn. Acetaminophen and aspirin are not strong enough to control the pain for this client. Meperidine is not provided as the drug of choice.

PTS:1DIF:AnalyzeREFPharmacology

15.To support the nutritional needs of a client recovering from a burn injury, the nurse will prepare to administer which of the following?

- | | |
|----|------------------------------|
| 1. | High carbohydrate diet |
| 2. | High fat diet |
| 3. | Low protein diet |
| 4. | Vitamins C and A supplements |

ANS: 4

Vitamins C and A are provided at doses higher than recommended because of the role they play as cellular antioxidants, and they are required for collagen synthesis. A high carbohydrate diet

should be avoided to prevent carbon dioxide production and hyperglycemia. Protein is needed for wound healing and should be calculated according to the clients weight. Fat intake should not exceed 30% of total daily calories.

PTS: 1 DIF: Apply REF: Nutrition

MULTIPLE RESPONSE

1.A client has been diagnosed with a full-thickness burn injury to the hands and arms. Which of the following characteristics would the nurse expect to find? (Select all that apply.)

1.	Blanches with fingertip pressure
2.	Charred vessels visible under eschar
3.	Many blisters that increase in size
4.	Nerve endings dead
5.	No edema
6.	Very painful

TESTBANKSELLER.COM

ANS: 2, 4

A full-thickness burn is a third-degree burn. A third-degree burn exhibits charred vessels visible under eschar, and the nerve endings are dead. There is no pain, and blisters are rare and do not increase in size. There is no blanching with pressure. Edema is not present with this type of burn injury.

PTS:1DIF:AnalyzeREFathophysiology

2.The goals of management during the emergent period after a burn include which of the following? (Select all that apply.)

1.	Airway management
2.	Aseptic technique
3.	Emotional support

-
- | | |
|----|-------------------|
| 4. | Fluid replacement |
|----|-------------------|
-
- | | |
|----|-----------------|
| 5. | Pain management |
|----|-----------------|
-
- | | |
|----|----------------|
| 6. | Rehabilitation |
|----|----------------|
-

ANS: 1, 2, 3, 4, 5

The goals of management during the emergent period the first 24 to 48 hours after a burn are to secure the airway, support circulation by fluid replacement, keep the client comfortable with analgesics, prevent infection through careful wound care, maintain the body temperature, and provide emotional support. Rehabilitation of the client is not an immediate concern during the emergent period.

PTS:1DIF:ApplyREF:Emergent Phase

3.An individuals sleeve catches on fire while cooking. He runs through the kitchen and out the back door. Which of the following interventions should the family perform? (Select all that apply.)

-
- | | |
|----|--|
| 1. | Have the individual stand for easy access. |
|----|--|
-
- | | |
|----|--------------------------|
| 2. | Remove any loose debris. |
|----|--------------------------|
-
- | | |
|----|---|
| 3. | Remove clothing adhered to the burned area. |
|----|---|
-
- | | |
|----|-----------------|
| 4. | Remove jewelry. |
|----|-----------------|
-
- | | |
|----|--------------------------------------|
| 5. | Use the water hose to cool the burn. |
|----|--------------------------------------|
-
- | | |
|----|---|
| 6. | Cover the burned areas with a clean dry material. |
|----|---|
-

ANS: 2, 4, 5, 6

The individual should not stand. Standing will cause the flames and smoke to engulf the facial area. The best intervention is to stop the person; wrap him in a blanket, coat, sheet, or towel; and roll him on the ground to exclude oxygen and thereby put out the fire. Any water source can be used to extinguish flames, cool the burn, or dilute the chemical area. Once all the flame is extinguished, clothing (except clothing adhered to burned skin), jewelry, and debris are carefully removed. The burned areas should be covered with a dry clean material.

PTS: 1 DIF: Apply REF: Prehospital Care and First Aid

4.The nurse is determining if a client who sustained a burn should be referred to a burn unit for care. Which of the following types of burn injuries should be referred to this type of care area? (Select all that apply.)

- | | |
|----|---|
| 1. | Burn on the face |
| 2. | Burn to the genitalia |
| 3. | Burn to a fractured limb |
| 4. | Sunburn |
| 5. | Burn caused by hot water to approximately 5 inches of the forearm |
| 6. | Burn caused by chemicals |

ANS: 1, 2, 3, 6

TESTBANKSELLER.COM

The types of burn injuries that should be referred to a burn unit for care include burns to the face and genitalia; burns to a fractured limb; and burns caused by chemicals. Sunburn and a small burn from hot water do not need to be referred to a burn unit for care.

PTS: 1 DIF: Analyze REF: Box 46-2 Burn Center Referral Criteria

5.The nurse is preparing to provide wound care to a client newly diagnosed with a burn. Which of the following are goals of this initial wound care? (Select all that apply.)

- | | |
|----|-----------------------------------|
| 1. | Hydrate the skin. |
| 2. | Cleanse the skin. |
| 3. | Prevent further skin destruction. |
| 4. | Provide comfort. |
| 5. | Prevent nutritional deficits. |

-
6. Prevent infection.

ANS: 2, 3, 4, 6

Goals for initial wound care for a client newly diagnosed with a burn include cleansing to prevent infection, prevent further skin destruction, and provide comfort. Skin hydration and prevention of nutritional deficits are not goals of initial wound care.

Chapter 53 Nursing Management: Shock and Multisystem Failure

MULTIPLE CHOICE

1.The nurse is caring for a client who has sustained multiple injuries from a motor vehicle accident. The nurse realizes that the client will have a release of stress hormones that are useful for all of the following EXCEPT:

-
1. preventing loss of fluids.
-
2. preventing hypotension.
-
3. preventing infection.
-
4. preventing ingestion of food.

ANS: 4

The purpose of the release of stress hormones following an insult to the body is to aid in restoring balance to the system and to prevent secondary complications, including loss of fluids, hypotension, and infection. The stress hormones are not released to prevent the ingestion of food.

PTS: 1 DIF: Analyze REF: Neuroendocrine System Response

2.The nurse is planning care for a client diagnosed with acute respiratory distress syndrome (ARDS)? Which of the following is *not* included in the management of this disorder?

-
1. Treating the underlying cause
-
2. Promoting gas exchange
-
3. Providing oxygen therapy

-
4. Promoting urine output

ANS: 4

The nursing management of ARDS includes facilitating oxygenation and ventilation, which is accomplished by treating the underlying cause, promoting gas exchange, and providing oxygen therapy. Promoting urine output is not part of the management of this disorder.

PTS:1DIF:Apply

REF: Acute Respiratory Distress Syndrome: Planning and Implementation

3.The nurse, caring for a client diagnosed with shock, realizes that the stage in which the body attempts to remedy the problem by initiating the homeostatic mechanism would be?

-
1. Initial stage
-
2. Compensatory stage
-
3. Multiple organ failure stage
-
4. Refractory stage

TESTBANKSELLER.COM

ANS: 2

During the compensatory stage, the body tries to remedy the problem. If it cannot, cellular damage will occur and organ failure and death may follow. During the initial stage, there is a decrease in cardiac output and impaired tissue perfusion. In multiple organ failure, every system in the body is affected. In the refractory stage, the body can no longer respond to therapy and the shock condition is considered irreversible.

PTS: 1 DIF: Analyze REF: The Four Stages of the Shock Syndrome

4.A client experiences a bee sting, complains of difficulty breathing, and shows sign of hypoxia and hypotension. The nurse realizes these are signs of anaphylactic shock, and she should do which of the following first?

-
1. Get a medical alert bracelet for the patient.
-
2. Give epinephrine intravenously or via endotracheal tube.
-
3. Check with the family for a history.

-
4. Admit the client through the admitting department.

ANS: 2

Anaphylactic shock is a medical emergency, and treatment is needed immediately. The nurse should expect to give epinephrine to promote bronchodilation and vasoconstriction. The other choices can be done after the client's airway and ventilation are stabilized.

PTS: 1 DIF: Apply REF: Anaphylactic Shock: Pharmacology

5. A client is diagnosed with failure of the left ventricle to provide adequate delivery of oxygen to the body tissues due to a weakened forward pumping function of the heart. The nurse realizes this client is experiencing:

-
1. anaphylactic reaction.
-
2. cardiogenic shock.
-
3. hypovolemia.
-
4. metabolic acidosis.

ANS: 2

In cardiogenic shock, there is an impaired forward pumping function with decreased stroke volume and decreased cardiac output. This dysfunction results in a backup of blood into the pulmonary system, and it can cause metabolic acidosis. Anaphylactic shock is a systemic reaction to an antigen. Hypovolemia is a loss of circulating blood. Metabolic acidosis is an acid-base imbalance that can occur from a variety of health conditions or disease processes.

PTS: 1 DIF: Analyze REF: Cardiogenic Shock

6. To assess if the renal system in a client diagnosed with multisystem failure is functioning properly, the nurse would expect to see urine output of:

-
1. 10 mL per hour.
-
2. 20 mL per hour.
-
3. 30 mL per hour.

-
4. 40 mL per hour.

ANS: 3

Elimination of 30 mL per hour of urine is considered to be an approximate estimate of renal function. A urine output less than 30 mL per hour indicates renal failure. A urine output of greater than 40 mL per hour is considered within normal limits.

PTS:1DIF:Apply

REF: Box 65-6 Selected Manifestations of Cardiogenic Shock

7.The nurse realizes that a client, diagnosed with neurogenic shock, is at risk for developing:

-
1. skin breakdown.
-
2. sweating.
-
3. deep vein thrombosis.
-
4. infection.

TESTBANKSELLER.COM

ANS: 3

The client is at a greater risk for deep vein thrombosis (DVT) because of the pooling of blood in the lower extremities. The client is at risk for skin breakdown, sweating, and infection; however, the risk for a DVT is a priority during the shock phase.

PTS: 1 DIF: Analyze REF: Neurogenic Shock: Pathophysiology

8.A client is diagnosed with septic shock. The nurse realizes that the major cause of this type of shock is:

-
1. gram-negative bacteria.
-
2. gram-positive bacteria.
-
3. fungi.
-
4. viruses.

ANS: 1

All are potential causes of septic shock, but gram-negative bacteria are considered the major cause of septic shock.

PTS: 1 DIF: Analyze REF: Septic Shock: Pathophysiology

9.A client is diagnosed with cardiogenic shock. The nurse should plan interventions to address which of the following potential complications of this disorder?

- | | |
|----|-----------------------|
| 1. | Pulmonary embolism |
| 2. | Deep vein thrombosis |
| 3. | Renal failure |
| 4. | Myocardial infarction |

ANS: 4

In cardiogenic shock, there is a reduction in oxygenated arterial blood that decreases perfusion throughout the body. The most serious complication of cardiogenic shock is myocardial infarction. Pulmonary embolism, deep vein thrombosis, and renal failure are not considered the most serious complications of cardiogenic shock.

PTS: 1 DIF: Apply REF: Cardiogenic Shock: Pathophysiology

10.The nurse is concerned that a client will develop neurogenic shock when which of the following is assessed?

- | | |
|----|--------------------------------|
| 1. | Fractured left lower extremity |
| 2. | Spinal cord injury at T1 |
| 3. | Jugular vein distention |
| 4. | Sluggish bowel sounds |

ANS: 2

Neurogenic shock is common following a spinal cord injury at or about T6. The client with a spinal cord injury at T1 is at risk for developing neurogenic shock. The other assessment findings do not place the client at risk for developing this type of shock.

PTS:1DIF:AnalyzeREF:Neurogenic Shock

11.The nurse is planning care for a client diagnosed with disseminated intravascular coagulation. Which of the following should be included in this plan of care?

-
1. Avoid intramuscular injections.

 2. Provide adequate daily caloric intake.

 3. Perform range-of-motion exercises to all extremities twice a day.

 4. Restrict fluids.

ANS: 1

When planning care for a client diagnosed with disseminated intravascular coagulation, the nurse needs to incorporate careful handling of the client since bleeding occurs easily. Sharp objects are to be avoided such as razors and intramuscular injections. Dietary intake and range-of-motion exercises are important for all critically ill clients; however, avoiding sharps is the priority for a client with disseminated intravascular coagulation. There is no physiological reason to restrict fluids in this client.

TESTBANKSELLER.COM

PTS: 1 DIF: Apply REF: Disseminated Intravascular Coagulation

12.A firefighter is brought to the emergency department with complaints of headache, weakness, shortness of breath, cough, and chest pain. During the assessment of this client, the nurse realizes the client should be evaluated for:

-
1. cyanide poisoning.

 2. pulmonary emboli.

 3. pneumonia.

 4. stroke.

ANS: 1

Smoke inhalation poses a risk of hydrogen cyanide poisoning, especially for firefighters. Clinical manifestations of this disorder include headache, weakness, shortness of breath, cough, and chest pain. The risk for pulmonary emboli, pneumonia, or stroke are not associated with fire or smoke inhalation.

PTS:1DIF:Analyze

REF:Red Flag: Fires, Smoke Inhalation and Cyanide Poisoning

MULTIPLE RESPONSE

1.The nurse is assessing a client for the acute phase of the inflammatory immune response. Which of the following are considered cardinal signs of this response? (Select all that apply.)

1.	Rubor
2.	Tumor
3.	Dolor
4.	Scarring
5.	Calor
6.	Loss of function

TESTBANKSELLER.COM

ANS: 1, 2, 3, 5

The cardinal signs of the inflammatory response include: rubor or redness; tumor or swelling; dolor or pain; and calor or heat. Loss of function is no longer considered a cardinal sign of the inflammatory response. Scarring is part of the healing response.

PTS:1DIF:Apply

REF: Inflammatory Immune Response; Nursing Strategy Feature: Cardinal Signs of Infection

2.The nurse is determining if a client is experiencing chronic inflammation. Which of the following are indications for this type of inflammation? (Select all that apply.)

1.	Chronic elevation of white blood cells
2.	Low-grade fever
3.	Pain
4.	Scar formation

5. Low blood pressure

6. Hematuria

ANS: 1, 2, 3, 4

Chronic inflammation is characterized by chronic elevation of white blood cells, low-grade fever, pain, and scar formation. Low blood pressure and hematuria are not characteristics of chronic inflammation.

PTS:1DIF:Analyze

REF: Box 65-1 Characterizations of Chronic Inflammation

3.The nurse is assessing a client for systemic inflammatory response syndrome (SIRS). Which of the following disease processes are associated with this syndrome? (Select all that apply.)

1. Infection

2. Pancreatitis

3. Ischemia

4. Trauma

5. Massive transfusions

6. Heart failure

ANS: 1, 2, 3, 4, 5

Disease processes associated with systemic inflammatory response syndrome include infection, pancreatitis, ischemia, trauma, and massive transfusions. Heart failure is associated with cardiogenic shock.

PTS: 1 DIF: Analyze REF: Systemic Inflammatory Response Syndrome

4.A client is experiencing symptoms associated with distributive shock. What types of shock are included in this category? (Select all that apply.)

1. Neurogenic

2.	Hypovolemic
3.	Anaphylactic
4.	Cardiogenic
5.	Septic
6.	Chronic

ANS: 1, 3, 5

There are three basic categories of shock syndrome: 1) hypovolemic, 2) cardiogenic, and 3) distributive. Subcategories of distributive shock include neurogenic, anaphylactic, and septic. Chronic is not a type of shock.

PTS:1DIF:AnalyzeREF:Shock Syndrome

5.The nurse is caring for a client experiencing hypovolemic shock. Which of the following interventions would be appropriate for this client? (Select all that apply.)

1.	Monitor intravenous fluid replacement
2.	Monitor vital signs
3.	Assess for manifestations of fluid overload
4.	Monitor white blood cell count and hemoglobin and hematocrit levels
5.	Position for comfort
6.	Assist to a sitting position

ANS: 1, 2, 3, 4, 5

Interventions appropriate for a client experiencing hypovolemic shock include monitoring intravenous fluid replacement, monitoring vital signs, assessing for manifestations of fluid overload, monitoring white blood cell count and hemoglobin and hematocrit levels, and position for comfort. The client should not be in a sitting position.

Chapter 54 Nursing Management: Critical Care**MULTIPLE CHOICE**

1.The nurse determines that a clients cardiac output is normal. Which of the following values would be considered normal?

- | | |
|----|-----------|
| 1. | 1 L/min. |
| 2. | 3 L/min. |
| 3. | 6 L/min. |
| 4. | 15 L/min. |

ANS: 3

Normal cardiac output is 4 to 8 L/min. This variation is due to the differences in body sizes of individuals. The values less than 4 L/min are low and the value of 15 L/min is extremely high.

PTS:1DIF:AnalyzeREF:Cardiac Output

2.A clients cardiac index is calculated to be 3.1 L/minute/m². Which of the following could explain this clients cardiac index value?

- | | |
|----|-----------------------------|
| 1. | Acute myocardial infarction |
| 2. | Cardiogenic shock |
| 3. | This is a normal value |
| 4. | Fever |

ANS: 3

The normal cardiac index range is 2.5 to 4.2 L/min/m². An index of 3.1 L/min/m² is considered a normal value. Cardiac index is low with the diagnosis of acute myocardial infarction or cardiogenic shock. Cardiac index is elevated with a fever.

PTS:1DIF:AnalyzeREF:Cardiac Output

3. A client with an elevated cardiac index has received diuretic medication as prescribed. The nurse realizes that this medication will affect which cardiac parameter?

- | | |
|----|---------------|
| 1. | Afterload |
| 2. | Contractility |
| 3. | Preload |
| 4. | Stroke volume |

ANS: 3

Preload is characterized by the amount of cardiac muscle fiber stretch preceding each contraction. Preload will decrease with the use of diuretics. Contractility is the force of ventricular contraction. Afterload is the pressure and forces opposing ventricular contraction. The stroke volume is the amount of blood pumped by the left ventricle in one contraction. Diuretics do not specifically impact contractility, afterload, or stroke volume.

PTS: 1 DIF: Analyze REF: Hemodynamic Monitoring: Preload

4. The nurse is caring for a client who is experiencing an increase in cardiac contractility. Which of the following will decrease contractility and reduce myocardial oxygen demand for this client?

- | | |
|----|--|
| 1. | Administer Primacor as prescribed. |
| 2. | Administer Digoxin as prescribed. |
| 3. | Administer beta-blocker as prescribed. |
| 4. | Administer potassium chloride as prescribed. |

ANS: 3

Medications such as beta-blockers and calcium channel blockers will reduce myocardial oxygen demand, decreasing cardiac contractility. Medications such as Primacor, Digoxin, and potassium chloride will increase cardiac contractility.

PTS: 1 DIF: Analyze REF: Contractility

5. Which of the following interventions would ensure the accuracy of hemodynamic parameters and ensure an air-free system?

1. Slowly flush the system after taking a blood sample.
2. Loosen connections.
3. Keep pressure bag inflated to 300 mmHg.
4. Add extensions to the line.

ANS: 3

Interventions to ensure the accuracy of hemodynamic parameters and an air-free system include fast-flushing the system after taking a blood sample, keeping connections tight, keeping the pressure bag inflated to 300 mmHg, not adding extensions to the line, and periodically performing flick and flush to the tubing system.

PTS: 1 DIF: Apply REF: Measurement of Hemodynamic Parameters

6. A client is prescribed a vasoactive intravenous medication to maintain a normal blood pressure. Which of the following is not a vasoactive medication?

1. Amiodarone
2. Dopamine hydrochloride
3. Nitroprusside sodium
4. Norepinephrine

ANS: 1

Amiodarone is an antiarrhythmic medication that is used for life-threatening ventricular arrhythmias and management of supraventricular tachyarrhythmias. All of the other medications are classified as vasoactive.

PTS: 1 DIF: Analyze REF: Intra-Arterial Monitoring: Indications

7. A client's right atrial pressure is measured as being 6 mmHg. The nurse would document which of the following regarding this client's pressure?

-
1. Right arterial pressure within normal limits
 2. Right arterial pressure below normal limits
 3. Right arterial pressure above normal limits
 4. Right arterial pressure unable to obtain
-

ANS: 1

The normal range for right atrial pressure or central venous pressure is 0 to 8 mmHg. The nurse would document right arterial pressure within normal limits. The other choices are incorrect interpretations of the client's right arterial pressure measurement.

PTS: 1 DIF: Apply REF: Pulmonary Artery Parameters

8. A client has an intraparenchymal probe inserted into his brain tissue. The nurse realizes an advantage of this type of intracranial monitoring device would be:

-
1. low risk of intracerebral bleeding.
TESTBANKSELLER.COM
 2. low risk of infection.
 3. sturdy and will not break.
 4. an inexpensive method to monitor.
-

ANS: 2

The intraparenchymal probe is easy to insert and has a low risk of infection. The probe does have a risk of causing intracerebral bleeding. The catheter is fragile and may break. The equipment is expensive for this type of probe.

PTS: 1 DIF: Analyze REF: Types of Intracranial Pressure Monitoring

9. A client is diagnosed with increased intracranial pressure. Which of the following interventions can be used to reduce this pressure?

-
1. Administer hypotonic intravenous fluids.
-

-
2. Administer Mannitol.
 3. Keep head of the bed flat.
 4. Keep PaCO₂ level above normal.
-

ANS: 2

Interventions to reduce increased intracranial pressure include administering an osmotic diuretic such as Mannitol. Hypotonic fluids should be avoided since cerebral edema could occur. The head of the bed should be raised to 30 to 45 degrees. A lower PCO₂ level will reduce intracranial pressure.

PTS:1DIF:Apply

REF:Management of Patients with Increased Intracranial Pressure

10.A client who has an endotracheal tube is being considered for a tracheostomy. Which of the following criteria would support the placement of a tracheostomy in this client?

-
1. Client is unable to maintain airway when extubated.
 2. Client has a history of diabetes mellitus.
 3. Client has been diagnosed with hypertension.
 4. Client is coughing and bucking the endotracheal tube.
-

ANS: 1

A tracheostomy tube would be indicated for clients who fail to wean rapidly, are unable to protect the airway for a prolonged period of time because of neurological problems, and have problems with oropharyngeal trauma. A history of diabetes mellitus or hypertension are not indications for the placement of a tracheostomy. Client coughing and bucking the endotracheal tube could be indications that the client can maintain the airway.

PTS:1DIF:AnalyzeREF:Tracheostomy

11.A client who is being mechanically ventilated has positive end expiratory pressure set at 20 cm of water. Which of the following should the nurse assess in this client?

-
1. Hemothorax
-

2. Pneumothorax

3. Increased venous return

4. Hypertension

ANS: 2

High levels of positive end expiratory pressure can cause pneumothorax or decreased venous return. Positive end expiratory pressure does not cause a hemothorax or hypertension.

PTS: 1 DIF: Apply REF: Additional Ventilator Settings and Modes

12.The nurse, planning care for a client who is mechanically ventilated, would plan to administer medication to prevent the onset of which of the following complications?

1. Hyperglycemia

2. Hypertension

3. TESTBANKSELLER.COM
Stress ulcers

4. Thrombophlebitis

ANS: 3

Stress ulcers are a complication of mechanical ventilation and often occur within 72 hours of illness. Prevention of stress ulcers in the mechanically ventilated client includes the prophylactic administration of proton pump inhibitors, histamine blockers, or sucralfate. Hyperglycemia and hypertension are not complications of receiving mechanical ventilation. Thrombophlebitis can occur in any client who is maintained on strict bed rest and is not necessarily being mechanically ventilated.

PTS: 1 DIF: Apply REF: Stress-Related Mucosal Disease

13.A client who is being mechanically ventilated has been receiving propofol (Diprivan) to enhance compliance. Which of the following will occur when the medication is discontinued and the client is removed from the ventilator?

1. The client will demonstrate hypotension.

-
2. The nurse will need to administer Narcan.
 3. The client will develop a dysrhythmia.
 4. The client will quickly gain consciousness.
-

ANS: 4

Propofol (Diprivan) is a sedative-hypnotic anesthetic used to enhance compliance with mechanical ventilation. When discontinued, the client will wake up quickly. Diprivan does not cause hypotension. Narcan is not needed to reverse Diprivan. Diprivan does not cause dysrhythmias.

PTS:1DIF:Analyze

REF: Table 19-7 Medications Commonly Used to Enhance Compliance with Mechanical Ventilation

14.A client has been participating in weaning from the ventilator for several days. Which of the following should be done to assist this client to regain strength?

-
1. Provide full ventilatory support during the overnight hours.
 2. Withdraw medications to enhance compliance with mechanical ventilation.
 3. Conduct passive range of motion to all extremities.
 4. Assist to a sitting position and dangle legs off the side of the bed.
-

ANS: 1

Discontinuation of mechanical ventilation will not be successful if the client is exhausted from the weaning process. The client who requires several days of weaning should be allowed to rest on full ventilatory support during the overnight hours. The client should be weaned from medication used to enhance compliance with mechanical ventilation. Passive range of motion will not help increase the clients strength for the weaning process. Having the client in a sitting position with legs dangling off the side of the bed will not enhance the weaning process.

PTS: 1 DIF: Apply REF: Ventilatory Weaning: Indications

15.Which of the following nursing interventions would be appropriate for a client who has had an endotracheal tube removed?

-
1. Evaluate arterial blood gas analysis 2 hours after extubation.

 2. Encourage the client to limit movement for the first day after extubation.

 3. Provide oral care immediately after extubation.

 4. Instruct the client that hoarseness will be present for at least 3 months.

ANS: 3

Interventions for a client who has been extubated include providing oral care immediately after extubation. Arterial blood gases should be analyzed within 30 to 60 minutes after extubation. The client should be encouraged to deep breathe, cough, and turn in bed frequently to mobilize secretions. Hoarseness should resolve within a few days and not in 3 months.

PTS:1DIF:ApplyREF:Nursing Management

MULTIPLE RESPONSE

1.The nurse is preparing the equipment needed for hemodynamic monitoring. Which of the following equipment should be included in preparation for monitoring? (Select all that apply.)

TESTBANKSELLER.COM

-
1. Amplifier

 2. Flow and pressure system

 3. Monitor

 4. Pressure bag, fluid flush system, and tubing

 5. Transducer

 6. Arm board

ANS: 3, 4, 5

All hemodynamic monitoring of patients requires three necessary pieces of equipment: 1) a monitor; 2) a transducer; and 3) a combination of pressure bag, fluid flush device, and tubing system. The other items listed are not equipment needed for hemodynamic monitoring.

PTS: 1 DIF: Apply REF: Equipment for Hemodynamic Monitoring

2.The nurse is trying to determine the cause of a dampened waveform. Which of the following could cause a dampened waveform. (Select all that apply.)

1.	Air bubbles
2.	Blood clots
3.	Kinks
4.	Loose connections
5.	Use of shorter tubing
6.	Low blood pressure

ANS: 1, 2, 3, 4

Air bubbles, blood clots, kinks, and loose connections are all reasons for a dampened waveform. When a system is underdamped, one of the ways to correct the system is to obtain shorter tubing. Low blood pressure will not cause the waveform to be dampened.

PTS: 1 DIF: Analyze REF: Measurement of Hemodynamic Parameters

3.A clients pulmonary artery wedge pressure is 1 mmHg. Which of the following health problems can cause this low pressure? (Select all that apply.)

1.	Altered left ventricular function
2.	Dehydration
3.	Elevations in blood volume
4.	Hemorrhage
5.	Hypovolemia
6.	Peripheral edema

ANS: 2, 4, 5

The pulmonary artery wedge pressure will be abnormally low in patients who are hypovolemic, dehydrated, or hemorrhaging. Elevated levels occur in patients who have altered left ventricular function or elevations in blood volume. Peripheral edema does not impact the clients pulmonary artery wedge pressure.

PTS: 1 DIF: Analyze REF: Pulmonary Artery Pressure Monitoring

4.A client is determined to not be a candidate for intra-aortic balloon pump monitoring. Which of the following client conditions is the balloon pump contraindicated? (Select all that apply.)

-
- | | |
|----|----------------------|
| 1. | Aortic insufficiency |
|----|----------------------|
-
- | | |
|----|----------------|
| 2. | Gastric ulcers |
|----|----------------|
-
- | | |
|----|--------------------------------------|
| 3. | Dissecting abdominal aortic aneurysm |
|----|--------------------------------------|
-
- | | |
|----|------------------------------------|
| 4. | Severe peripheral vascular disease |
|----|------------------------------------|
-
- | | |
|----|-----------------------|
| 5. | Absent femoral pulses |
|----|-----------------------|
-
- | | |
|----|-----------------------|
| 6. | Uncontrolled bleeding |
|----|-----------------------|

ANS: 1, 3, 4, 5, 6

Contraindications for the use of an intra-aortic balloon pump include aortic insufficiency, dissecting abdominal aortic aneurysm, severe peripheral vascular disease, absent femoral pulses, and uncontrolled bleeding. Gastric ulcers is not a contraindication for the use of this pump.

PTS: 1 DIF: Analyze REF: Cardiac Assist Devices

5.Which of the following interventions would be appropriate for a client who is being mechanically ventilated? (Select all that apply.)

-
- | | |
|----|---------------------------|
| 1. | Secure artificial airway. |
|----|---------------------------|
-
- | | |
|----|---|
| 2. | Auscultate lungs every 4 hours and as needed. |
|----|---|
-
- | | |
|----|---|
| 3. | Monitor endotracheal tube cuff pressure once per shift. |
|----|---|

-
4. Provide alternative form of communication.
 5. Apply vest and wrist restraints.
 6. Monitor arterial blood gas analysis results and adjust ventilator as needed.
-

ANS: 1, 2, 3, 4, 6

Nursing interventions appropriate for a client being mechanically ventilated include secure artificial airway, auscultate lungs every 4 hours and as needed, monitor endotracheal tube cuff pressure once per shift, provide alternative form of communication, monitor arterial blood gas analysis results, and adjust ventilator as needed. Soft wrist restraints can be applied if necessary. A vest restraint is not necessary.

Chapter 55 Nursing Management: Emergencies and Disasters

MULTIPLE CHOICE

1. The nurse in the emergency department is using a triage system because this system ranks clients by:

TESTBANKSELLER.COM

-
1. severity of illness or injury.
 2. body systems involved.
 3. name.
 4. age.
-

ANS: 1

No one can predict when the next patient will arrive or the severity of their injury or illness; this is why emergency departments utilize a triage system, which is a method to rank or classify patients illnesses and the severity of their injuries. The triage system does not rank clients by body systems involved, name, or age.

PTS: 1 DIF: Apply REF: Triage

2. In the event of a mass casualty situation, the best triage nurse is:

-
1. the recently graduated registered nurse (RN).

 2. the licensed vocational nurse (LVN) with 5 years experience.

 3. the registered nurse (RN) with the most experience and best assessment skills.

 4. the recently graduated licensed vocational nurse (LVN).

ANS: 3

Triage is usually performed by an RN who is experienced and can complete a rapid assessment. Triage nurses are challenged with assessing the order in which clients need to be evaluated by an emergency department doctor. The recently graduated registered nurse does not have the experience necessary to perform adequate triage. The recently graduated licensed vocational nurse and the licensed vocational nurse with 5 years experience do not have the appropriate education on client assessment to serve as a triage nurse.

PTS: 1 DIF: Analyze REF: Triage

3. The nurse, triaging victims of a mass casualty incident, will focus attention on the victims who are color coded as:

TESTBANKSELLER.COM

-
1. green.

 2. yellow.

 3. red.

 4. black.

ANS: 3

Victims coded green will do well with minimal care, and victims coded black will most likely die even with care. The focus is on the red-coded victims, then the yellow, using immediate resources and rapid intervention for those who will benefit the most.

PTS: 1 DIF: Apply REF: START Method of Triage

4. The emergency room nurse is utilizing a triage approach for a mass casualty incident that is different from traditional triage. The difference between these two triage approaches is that:

-
1. mass casualty is first come, first served; traditional is most critical first.
 2. mass casualty is most likely to survive first; traditional is first come, first served.
 3. mass casualty is most critical first; traditional is most likely to survive first.
 4. mass casualty is most likely to survive first; traditional is most critical first.
-

ANS: 4

In traditional triaging the most critical come first, but in mass casualties the number of people and limits of supplies have to be taken into consideration. This means the clients who are most likely to survive are treated first and the most critical are treated last.

PTS: 1 DIF: Apply REF: START Method of Triage

5.The emergency department nurse is preparing to triage victims of an internal event. Which of the following would be considered an internal event?

-
1. Bus crash in front of the hospital
TESTBANKSELLER.COM
 2. Train crash 5 miles away
 3. Fire in the hospital
 4. Explosion in a nearby oil station
-

ANS: 3

An internal event is an event inside the facility, such as a fire in the building or a water pipe breaking. All the other events are outside the facility and would be external.

PTS:1DIF:AnalyzeREF:HEICS Activator

6.The Emergency Preparedness Committee in a health care organization is reviewing available supplies for a mass casualty event since supplies need to be able to support the organizations functioning for:

-
1. 24 hours.

2.	48 hours.
----	-----------

3.	96 hours.
----	-----------

4.	1 week.
----	---------

ANS: 3

The plan must incorporate strategies to care for a large influx of clients for up to 96 hours because it may be this long before assistance from the government can arrive. The other choices are incorrect lengths of time for the organization to prepare for supplies.

PTS: 1 DIF: Apply REF: Hospital Operations Plan

7. When reviewing the potential for biological warfare, the nurse realizes that one of the greatest bioterrorism threats in the world today is:

1.	chickenpox.
----	-------------

2.	smallpox.
----	-----------

TESTBANKSELLER.COM

3.	rabies.
----	---------

4.	influenza.
----	------------

ANS: 2

Smallpox was considered eradicated worldwide, and much of the vaccine was destroyed. Much of the current population has not been vaccinated or received booster shots. As a result, the number of casualties would be great. The other communicable diseases listed would not cause a great number of casualties.

PTS: 1 DIF: Analyze REF: Smallpox

8. The nurse is a member of the emergency preparedness committee, and she learns that anthrax is a bioterrorism threat that could infect and kill large numbers of people. Because of this, the organization should have which of the following on stock to treat anthrax?

1.	No treatment available
----	------------------------

2.	Adrenaline injections
----	-----------------------

3. Intravenous or oral ciprofloxacin

4. Oral Benadryl

ANS: 3

If anthrax is suspected, ciprofloxacin IV is recommended. Oral ciprofloxacin is recommended for postexposure treatment. Adrenaline and oral Benadryl are not treatments for anthrax. There is a treatment for anthrax and the health care organization should be prepared for clients admitted with this disease.

PTS: 1 DIF: Apply REF: Anthrax: Pharmacology

9.A client is diagnosed with West Nile virus. The nurse should instruct the client that the most common carriers of this virus are:

1. rats.

2. birds.

3. mosquitoes. TESTBANKSELLER.COM

4. cows.

ANS: 3

The nurse can help provide education to clients about decreasing their risk of exposure to infected mosquitoes. Rats and cows do not carry the West Nile virus. Birds are the main reservoir for this virus.

PTS: 1 DIF: Apply REF: West Nile Virus: Etiology

10.Victims of a chemical spill are brought to the hospital for treatment. The nurse learns that 50 victims will be arriving within the hour. When preparing for these victims, the nurse should ensure that which of the following is available?

1. A small designated area to decontaminate the victims

2. A medium-sized area to decontaminate the victims

3. A large area to decontaminate the victims

-
4. The entire emergency department is available to decontaminate the victims

ANS: 3

The nurse is preparing to provide care to 50 victims. This is a large number to decontaminate, so the nurse should ensure that a large area is available to decontaminate the victims. A small area would be sufficient for a few victims. A medium-sized area would be sufficient for a slightly larger number of victims. It would not be reasonable to expect that the entire emergency department would be available to decontaminate the victims.

PTS:1 DIF:Apply REF: decontamination

11. The nurse is told to wear a Level B Hazmat suit when decontaminating victims of a mass casualty incident. The nurse realizes that this suit will provide:

-
1. the highest level of respiratory and skin protection.
-
2. resistance to chemicals, and it is impermeable to gases and vapors.
-
3. respiratory protection, but it will allow chemical vapors to permeate the suit.
-
4. splash protection and chemical resistance.
-

TESTBANKSELLER.COM

ANS: 3

There are three levels of personal protective equipment. With a Level A suit, the highest level of respiratory and chemical protection is provided. This suit provides resistance to chemicals and is impermeable to gases and vapors. In a Level B suit, there is respiratory protection but chemical vapors are able to permeate the suit. The Level C suit provides splash protection and chemical resistance.

PTS: 1 DIF: Analyze REF: Box 66-4 OSHA PPE Levels

MULTIPLE RESPONSE

1. The nurse is preparing to assess a client who was a victim of a blast injury. Prior to assessing the client, which are the mechanisms of a blast injury that the nurse will review? (Select all that apply.)

-
1. Primary
-
2. Secondary
-

3.	Acute
4.	Tertiary
5.	Quaternary
6.	Chronic

ANS: 1, 2, 4, 5

The four mechanisms of a blast injury are primary, secondary, tertiary, and quaternary. Each of these mechanisms of injury have associated injuries. The nurse should review the mechanisms of injury prior to assessing the client. Acute and chronic are not mechanisms of a blast injury.

PTS:1DIF:ApplyREF:Blast Injuries

2.The nurse believes a client is experiencing a reaction to a traumatic event when which of the following is assessed? (Select all that apply.)

1.	Client is not responding verbally to assessment questions.
2.	Client complains of dizziness.
3.	Clients blood pressure is 120/80 mmHg.
4.	Client complains of nausea.
5.	Client asks for medication for a headache.
6.	Client asks for something to drink.

ANS: 1, 2, 4, 5

Common responses to a traumatic event include silence or not responding to verbal stimuli, dizziness, nausea, and headache. A blood pressure of 120/80 mmHg is considered normal. The client requesting something to drink is not a reaction to a traumatic event.

PTS:1DIF:Analyze

REF: Table 66-5 Common Responses to a Traumatic Event

3.The nurse is concerned about developing post-traumatic stress disorder after working for several years in the emergency department. Which of the following should the nurse do to ensure this disorder does not manifest? (Select all that apply.)

-
- 1. Eat well-balanced meals.
 - 2. Drink water.
 - 3. Limit caffeine.
 - 4. Limit sugar intake.
 - 5. Exercise at least 4 times a week for 30 minutes.
 - 6. Ingest at least one alcoholic drink every evening.
-

ANS: 1, 2, 3, 4, 5

The nurse must learn to handle stress to reduce the onset of post-traumatic stress disorder by eating well-balanced meals, drinking water, limiting caffeine, limiting sugar intake, and exercising at least 4 times a week for 30 minutes. A daily intake of alcohol could be a sign that post-traumatic stress disorder is developing.

PTS:1DIF:Apply

REFost-traumatic Stress Disorder: Planning and Implementation

4.The nurse is participating in a debriefing session after participating in the care of victims of a mass casualty incident. Which of the following will occur during this debriefing session? (Select all that apply.)

-
- 1. The process for the debriefing will be explained.
 - 2. Individual reactions to the event will be discussed.
 - 3. Symptoms that the nurse may experience will be reviewed.
 - 4. Stress reduction techniques will be provided.
-

5. Prescriptions for antianxiety medications will be provided.

6. Physician follow-up appointments will be made.

ANS: 1, 2, 3, 4

There are eight (8) phases to a debriefing session. During the introduction phase, the process for the debriefing will be explained. During the reaction phase, individual reactions to the event will be discussed. During the symptom phase, symptoms that the nurse may experience will be reviewed. During the teaching phase, stress reduction techniques will be provided. Prescriptions for medication and physician appointments are not a part of the debriefing session.

PTS: 1 DIF: Analyze REF: Box 66-9 The Eight Phases of a Debriefing

5. The nurse is a member of a committee designing the hospital's emergency incident command system. Which of the following are the four components of the committee's design? (Select all that apply.)

1. Relief support

2. Staffing

3. Planning

4. Operations

5. Logistics

6. Finance

ANS: 3, 4, 5, 6

The hospital emergency incident command system has four components: 1) finance, 2) logistics, 3) operations, and 4) planning. Relief support and staffing are not parts of the hospital emergency incident command system.